

PLAB 2 PLAYBOOK

100 Stations to Pass with Confidence

By

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This book is a preparatory revision resource for candidates studying for the PLAB 2/UKMLA examination. All patient names, ages, and clinical scenarios in this book are fictitious and created for educational purposes only; any resemblance to real individuals or real patient cases is coincidental. This book is a study aid and does not replace current official clinical guidelines (including NICE, the UK Resuscitation Council, the BNF, and relevant Royal College guidance) — readers should independently verify all clinical facts, doses, and pathways against current sources.

Preface

This guide grew out of a simple, practical need: a single, consistently formatted place to revise the full breadth of PLAB 2/UKMLA OSCE stations, from routine consultations to SimMan emergencies, procedural skills, and prescribing. Rather than treating each topic as an isolated fact to memorise, every station in this book follows the same underlying shape — a focused history, a common pitfall to avoid, a model response to learn from, and a clear explanation of exactly why each choice scores well — so that the pattern becomes familiar long before the exam itself.

I hope this collection saves you time, sharpens your instinct for what examiners are actually listening for, and gives you the quiet confidence that comes from having genuinely worked through the material, station by station, rather than skimmed it.

Wishing you every success in your exam and in the career that follows it.

— **Dr. Sampson Owhin**

How to Use This Book

This book is organised into thematic sections (e.g. Dermatology, ENT Emergencies, Mental Health, Clinical Procedural Skills), each containing one or more numbered stations, for a total of 100 stations. Every station follows a consistent internal structure to make revision predictable and efficient:

- **The Prompt:** the information you would read outside the cubicle before entering — patient details and your task.
- **The Roleplay:** a full worked dialogue between the candidate and patient (or, for procedural/SimMan stations, the examiner and mannequin), showing exactly how a strong candidate would move through the station in real time.
- **Common Pitfall:** a realistic, tempting-but-wrong response, explained so you can recognise and avoid the same instinct under exam pressure.
- **Model Response:** the corrected, high-scoring version of that same moment.
- **Station Conclusion:** a short list explaining precisely why the final approach scores full marks across the Data Gathering, Clinical Management, and Interpersonal Skills domains.

Several sections open with a short framework or blueprint (for example, the ABCDE SimMan Framework, or the Duty of Candour Blueprint) — read these first, since individual stations in that section build directly on them rather than repeating the same background each time.

A sensible way to use this book: read a station's prompt and try to answer out loud yourself before reading the model dialogue, actively predict what the "common pitfall" will be before you reveal it, and keep a running list of the specific facts or thresholds (drug doses, referral timeframes, screening ages) you personally keep forgetting, rather than re-reading everything indiscriminately.

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Introduction: History-Taking and Consultation Frameworks

Before working through individual stations, it helps to have a small set of reusable frameworks in mind — the underlying skeleton that most consultation and SimMan stations are built on. The specific conditions referenced below (depression, psychosis, suicide risk) are covered as full worked stations elsewhere in this book; the summaries here are the condensed, at-a-glance versions to revise from quickly before an exam, or to adapt on the fly to a condition not explicitly covered elsewhere.

1. General Structure for a Depression History

- **Opening:** build rapport and open with an empathetic, open-ended question.
- **Screen the core symptoms:** low mood, anhedonia (loss of interest or pleasure), and anergia (lack of energy) — at least two of these three, present most of the day, nearly every day for 2 weeks or more, are required for a depression diagnosis.
- **Screen the biological/somatic symptoms (mnemonic MSCFLAWSS):** Mood, Sleep disturbance (both difficulty falling asleep and early morning waking), Concentration difficulty, Fatigue, Loss of appetite, Weight change, Anhedonia, and Self-care neglect — covering the full somatic footprint of depression beyond mood alone.
- **ICE and impact:** explore ideas, concerns, and expectations, and how symptoms are affecting daily functioning, work, and relationships.
- **Mandatory risk assessment:** this is never optional — screen explicitly for suicidal ideation, self-harm, and risk to others (see the dedicated risk framework below).
- **Past psychiatric history and substance use:** previous episodes, treatments, and alcohol/recreational drug use, which can both cause and worsen depressive symptoms.
- **Collaborative management:** agree a plan together rather than dictating one — this is consistently one of the highest-weighted Interpersonal Skills elements in psychiatric stations.

Mania screen (always worth a brief check before starting an antidepressant): "How has your sleep been lately? Have you noticed an unusual amount of energy, or your thoughts racing too fast to keep up with?" — screening for hypomania/mania is essential, since missing bipolar disorder and starting an antidepressant alone can trigger a manic episode.

2. Suicide and Self-Harm Risk Assessment (The Critical Safety Metric)

- **Thoughts:** "Have you ever had thoughts that life isn't worth living, or thoughts of harming yourself?"
- **Intent:** "Have you had any thoughts of ending your life?"
- **Active planning:** "Have you made any specific plans about how you might do this?"
- **Access to lethal means:** explicitly ask about access to medication stockpiles, weapons, or other means relevant to any plan disclosed.

Any active plan with access to means requires immediate crisis management and escalation — this is never something to note and revisit later in the consultation.

3. Structure for Suspected Psychosis or Delusions

- **Establish rapport:** introduce yourself and confirm identity gently before exploring anything sensitive.
- **Open-ended starter:** "I understand you wanted to share something with us today — can you tell me what's been happening?" Then listen without interrupting.
- **Explore the delusion (data gathering):** how long has this belief been present; how does it make the patient feel (anxious, proud, frightened); screen for persecutory/paranoid elements ("Do you feel you're in any danger? Does it feel like anyone is watching or tracking you?"); and gently assess the fixed nature of the idea ("I can see this feels very real to you — have you ever considered any other explanation for what's happening?").
- **Screen for other psychotic symptoms:** this helps differentiate schizophrenia from severe depression with psychotic features. Ask about auditory hallucinations directly — "When you're alone, do you ever hear voices or sounds that others don't?" — and if present, always ask what the voices say, specifically screening for command hallucinations instructing harm.

4. Positive and Negative Symptoms of Psychosis (Quick Reference)

- **Positive symptoms — perception:** hallucinations, classically third-person or command auditory hallucinations in schizophrenia.
- **Positive symptoms — thought content:** delusions (persecutory, grandiose, or thought broadcasting/insertion).
- **Negative symptoms:** social withdrawal, blunted affect, poverty of speech (alogia), and lack of motivation (avolition).
- **Disorganisation:** fragmented speech patterns or erratic behaviour.

Psychosis management stream: stable, cooperative, new-onset symptoms with no immediate threat to life → urgent referral (typically within 2 weeks) to the Early Intervention in Psychosis (EIP) team. Active command hallucinations to cause harm, severe agitation, or total self-neglect → immediate same-day referral to the Mental Health Crisis Team (CRHTT), with admission (voluntary or involuntary under the Mental Health Act) considered.

5. The General SimMan ABCDE Structure

Assume all universal precautions have already been taken. State clearly to the examiner: "I can see my patient is attached to the monitor — I'll verbalise the vital signs and begin an ABCDE assessment."

- **Airway:** if the patient is talking to you, the airway is patent. If unconscious, look for obstruction, swelling, or foreign bodies. If saturation is low, state you are applying 15L of 100% oxygen via a non-rebreather mask.
- **Breathing:** expose the chest, count the respiratory rate (count for 15 seconds and multiply by 4), feel the trachea is central, assess chest expansion, then percuss and auscultate. Request a chest X-ray and ABG if the respiratory rate is abnormal.

- **Circulation:** check pulse, blood pressure, and capillary refill; insert one or two large-bore cannulas, particularly if hypotensive; send routine bloods (FBC, U&Es), plus context-specific tests — CK if suspecting muscle breakdown, group and save if bleeding is a concern, troponin if suspecting anaphylaxis or cardiac injury, clotting profile if bleeding, and lactate/CRP if suspecting sepsis; obtain an ECG; start an appropriate IV fluid.
- **Disability:** check blood glucose, temperature, pupillary responses, and screen for focal weakness. Pin-point pupils in this context point strongly towards opioid toxicity.
- **Exposure:** expose fully while preserving dignity — explicitly state that a chaperone is present and privacy is being maintained.

Worked Example: Suspected Opioid (Morphine) Toxicity

Background: a 74-year-old man, recently recovered from abdominal surgery and subsequently started on regular morphine for pain, is found breathing shallowly and is difficult to rouse.

Assessment: findings show a low oxygen saturation, a low respiratory rate, and pin-point pupils — a combination pointing strongly to opioid (morphine) toxicity.

Candidate: I can see my patient is having significant respiratory depression with shallow breathing, and his pupils are pin-point. Given his recent morphine prescription, I strongly suspect opioid toxicity. I'm going to start immediate treatment with IV naloxone, 400 micrograms.

(One minute passes.)

Candidate: I'm reassessing now — checking his conscious level, respiratory rate, and pupils again. Can you tell me how you're feeling, or have your symptoms improved at all?

If there is no improvement, the naloxone dose is increased — commonly doubled or given in further increments every 2–3 minutes (e.g. progressing toward 800 micrograms, then higher, per local protocol) — while continuing to reassess after each dose. Once stable, hand over using SBAR: explain the situation clearly, e.g. "I've been called to see a patient found unresponsive with shallow breathing, now diagnosed with and treated for opioid toxicity..." and recommend an immediate senior review.

6. General Medical History-Taking Structure (Adaptable Template)

- **1. Introduction and the golden minute:** let the patient speak freely and uninterrupted for the first minute — this alone often surfaces most of the key history.
- **2. Presenting complaint:** onset and timing; triggers, exacerbating and relieving factors; severity.
- **3. Key associated symptoms and red flags:** specific to the presenting complaint.
- **4. Patient perspective (ICE) and impact:** on daily life, work, and independence.
- **5. Focused past medical background:** targeted screening relevant to the presentation, rather than an exhaustive unfocused list.
- **6. Social and occupational/lifestyle history:** smoking; occupation (e.g. asbestos, coal dust, or other occupational exposures relevant to a respiratory complaint); home support — who is at home, and can the patient manage daily activities despite their symptoms.
- **7. Management and safety-netting:** in the final couple of minutes — offer any immediate assessment relevant to the complaint (e.g. "Because you're short of breath, I'd like to check

your oxygen levels and vital signs now"), explain the plan, and give clear safety-netting advice.

Station 1 — Ophthalmology: Painless Vision Loss Triage

In the GMC PLAB 2 exam, a sudden, painless loss of vision is a critical scenario testing your ability to rapidly distinguish between different sight-threatening emergencies. The two most high-yield conditions to differentiate are **Retinal Detachment** and **Central Retinal Artery Occlusion (CRAO)**.

Your primary objectives are to: take a targeted history focusing on visual warning signs; screen for stroke and Giant Cell Arteritis (GCA) risk factors; perform a focused eye examination; and outline the immediate emergency management and referral pathway.

Focused History & Assessment

Because the eye is completely white and painless, you must narrow down the cause by asking about the exact pattern of the vision loss, any preceding symptoms, and systemic risk factors.

1. The Pattern of Vision Loss

- Like a curtain or shadow falling down over the eye? — classic for Retinal Detachment.
- Sudden, total "blackout" of vision within seconds? — points toward CRAO or Amaurosis Fugax (a temporary warning stroke of the eye).

2. Preceding Warning Symptoms

- Bright flashes of light (photopsia) or a sudden shower of dark spots/floaters before the vision went? — strong indicators of a retinal tear or detachment.
- Any brief episodes of vision loss lasting seconds to minutes that fully recovered over the preceding days to weeks? — suggests Amaurosis Fugax, a warning sign that often precedes CRAO.

3. Cardiovascular & Stroke Risk Factors

CRAO is essentially an ischaemic stroke of the eye, usually caused by an embolus from the carotid artery or heart. Screen for hypertension, hyperlipidaemia, diabetes, smoking, and atrial fibrillation (AF).

4. Giant Cell Arteritis (GCA) Red Flags — do not skip this

GCA is a vision-threatening emergency in its own right and must be actively excluded in any patient over 50 with sudden visual loss, since untreated GCA can rapidly cause bilateral blindness. Ask about:

- New, severe temporal headache
- Jaw or tongue pain on chewing (jaw claudication)
- Scalp tenderness (e.g. pain when brushing hair)
- Proximal muscle stiffness or aching in the shoulders/hips (polymyalgia rheumatica symptoms)
- Unexplained fatigue, fever, or weight loss

5. Focused Examination

- Visual acuity (Snellen chart) in both eyes
- Pupillary reflexes — look for a Relative Afferent Pupillary Defect (RAPD), which is present in CRAO
- Fundoscopy

Differentiating the Two Conditions

Feature	Retinal Detachment	Central Retinal Artery Occlusion (CRAO)
Onset profile	Gradual progression over hours to days	Sudden, profound loss of vision within seconds
Preceding symptoms	Flashes of light and a sudden increase in floaters	Transient episodes of vision loss (Amaurosis Fugax)
Description	A dark shadow or curtain closing across the vision	Complete, absolute darkness in the affected eye
Pupillary exam	May show a mild RAPD if extensive	Marked RAPD is typical
Fundoscopy findings	Crinkled, grey, floating retinal membrane	Pale, ischaemic retina with a "cherry-red spot" at the macula
UK management	Same-day referral to Eye Casualty for urgent surgical reattachment	Ophthalmic emergency — same-day Eye Casualty referral; attempt ocular massage and IOP-lowering measures within the hyperacute window; urgent stroke-team referral for secondary prevention workup (ECG, carotid Doppler, echo, ESR/CRP to exclude GCA)

Note on management accuracy: thrombolysis is not routine UK first-line treatment for CRAO — it is not offered outside specialist trial settings. The safe, examinable answer is urgent ophthalmology referral, ocular massage, intraocular-pressure-lowering measures (e.g. IV acetazolamide, anterior chamber paracentesis), and treating this as a stroke equivalent with urgent secondary-prevention workup.

The Roleplay

Station: Acute Assessment / Consultation Skills

Candidate Role: FY2 Doctor working in the Emergency Department (A&E)

Patient/Roleplayer Role: Jonathan Blake, 67 years old

Setting: Emergency Treatment Cubicle

The Prompt (read outside the cubicle):

Patient Details: Jonathan Blake, 67 years old, has been brought to A&E by his daughter.

Presenting Complaint: Sudden, painless loss of vision in his right eye that happened an hour ago while he was eating breakfast.

Your Task:

- Take a focused history to determine the likely cause of his vision loss.
- Screen for relevant systemic risk factors, including GCA.
- Formulate the correct emergency management and referral pathway.

(You walk into the cubicle. Jonathan is sitting upright, looking calm but deeply concerned, and holding a patch over his right eye.)

Candidate: Hello Mr Blake. My name is Dr [Your Name], one of the emergency doctors here today. I can see this must be very worrying for you. Are you experiencing any pain in or around the eye at the moment?

Jonathan: Hello Doctor. No, no pain at all. That's the strangest part. I was just sitting at the kitchen table drinking my coffee, and within about ten seconds, it was like someone completely turned off the lights in my right eye. It went totally pitch black. I can't see a single thing out of it right now.

Candidate: Thank you for explaining that so clearly, Jonathan. I can hear how shocking that must have been. To help me work out what's caused this and get you the right treatment quickly, do you mind if I ask a few quick questions about any warning signs beforehand, and then examine your eyes and check your pulse and blood pressure?

(Jonathan shifts his posture slightly, holding his hand steady over his right eye as he prepares to answer.)

Jonathan: Yes, please do ask, Doctor. I really want to understand why this happened out of nowhere.

Your Next Step: Targeted Risk Assessment (Data Gathering)

To determine if this is a vascular event (CRAO) or a structural issue (Retinal Detachment), screen for visual warning indicators and systemic warning signs of GCA.

Candidate: Right before the vision went dark, did you notice any bright flashes of light, or a sudden shower of small spots or floaters?

Candidate: Have you had any brief spells over the last couple of weeks where your vision went dark for a few minutes and then came back completely?

Candidate: Have you had any new, severe headache on the side of your head, pain in your jaw when chewing, or a tender scalp when brushing your hair?

Jonathan: No flashes of light or floating spots at all, Doctor. It was just an instant blackout. But now that you mention it — yes. Last week I had two separate moments while reading where a grey shadow came down over this exact same eye. It lasted about two minutes each time and then

completely cleared. Thankfully, no headaches, jaw pain, or scalp tenderness at all — my head and jaw feel completely normal.

Your Next Step: Emergency Management & Referral

Jonathan's history confirms a Central Retinal Artery Occlusion (CRAO). The temporary shadows last week were classic episodes of Amaurosis Fugax — mini-strokes of the eye. CRAO is a profound medical emergency: an ischaemic stroke of the eye that requires immediate action within a narrow time window to try to restore blood flow and salvage vision. The absence of headache, jaw claudication, or scalp tenderness makes GCA less likely, but this should still be documented and an ESR/CRP sent as a safety net.

Candidate: Thank you, Jonathan. This confirms a serious condition called Central Retinal Artery Occlusion — an ischaemic stroke affecting the main artery of your eye. Because this happened just an hour ago, we are inside the critical time window. I'm contacting the Ophthalmology and Stroke teams immediately. In the meantime we'll try some simple measures to lower the pressure in the eye and dislodge the blockage, and we'll check your heart rhythm and blood tests straight away, since this is treated in the same way as a stroke.

Station Conclusion: An "Excellent Pass" Vision Loss Profile

You will pass this ophthalmology emergency station with top marks across all categories if you followed this format.

Why your final choices scored maximum marks:

- **Accurate Differentiation:** You correctly separated a structural cause (Retinal Detachment) from a vascular cause (CRAO/Amaurosis Fugax) using the pattern and timeline of vision loss.
- **Active GCA Screening:** You did not stop at the eye — you actively excluded a co-existing vision-threatening emergency (GCA) with a focused set of questions.
- **Correct Emergency Escalation:** You treated CRAO as a stroke-equivalent emergency, escalating to both Ophthalmology and Stroke teams rather than one team alone.
- **Safe, Evidence-Based Management:** You avoided overstating unproven treatments (e.g. thrombolysis) and focused on the accepted UK pathway of urgent referral, IOP-lowering measures, and secondary-prevention workup.
- **Clear Communication Under Time Pressure:** You explained the diagnosis and next steps in plain language despite the urgency of the situation.

Station 2 — Breast Surgery: Breast Lump & the Triple Assessment Pathway

In the GMC PLAB 2 exam, Breast Surgery/Oncology stations are high-frequency scenarios testing your Consultation Skills, Risk Communication, and Breaking Bad News. Under UK NICE guidelines (NG12), any patient aged 30 or older presenting with an unexplained breast lump (with or without pain) requires an urgent 2-Week Wait (2WW) referral to a specialised Breast Clinic.

Your primary objectives are to: take a comprehensive breast history; screen for malignant red flags; explore the patient's Ideas, Concerns and Expectations (ICE); address intense patient anxiety regarding breast cancer; and clearly outline the fast-track diagnostic pathway.

Focused History & Assessment

1. The Lump Profile

Ask about onset, size, and texture. Malignant lumps are classically hard, irregular, fixed to surrounding tissue, and painless — though pain does not rule out cancer.

2. Skin & Nipple Changes

- Skin dimpling or puckering — indicates structural tethering.
- Nipple retraction — the nipple is newly pulled inwards.
- Peau d'orange — the skin resembles an orange peel, a classic sign of inflammatory breast cancer.

3. Nipple Discharge

Ask if there is fluid leaking from the nipple, particularly if spontaneous, unilateral, or blood-stained.

4. Systemic Red Flags

Screen for axillary or supraclavicular lumps (lymphadenopathy), bone pain, or unexplained weight loss.

5. Risk Factors & Family History

Ask specifically about a first-degree relative with breast or ovarian cancer, any known BRCA1/BRCA2 mutation in the family, age of menarche, age of menopause or first pregnancy, and use of hormone replacement therapy — this is required to satisfy the stated objective and is a common missed mark.

The Roleplay

Station: Consultation Skills / Risk Communication

Candidate Role: FY2 Doctor in a General Practice (GP) Clinic

Patient/Roleplayer Role: Claire Higgins, 38 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Claire Higgins, 38 years old, has booked an urgent GP appointment.

Presenting Complaint: She discovered a firm lump in her left breast while showering two days ago. She is visibly terrified and crying.

Your Task:

- Take a focused breast history and evaluate for malignant red flags.
- Explore the patient's ideas, concerns, and expectations (ICE).
- Communicate the diagnostic referral plan safely according to UK guidelines.

(You walk into the room. Claire is sitting down, clutching a tissue, tears running down her face.)

Candidate: Hello Claire, I'm Dr [Your Name], one of the clinic doctors today. I can see you're incredibly distressed, and I want you to know you're in a safe space — we're going to look after you. Can you tell me a little about what's been happening?

Claire: (sobs) Doctor... I found a lump in my left breast two days ago while showering. It feels completely hard and stuck. I haven't slept a wink since. I'm absolutely terrified it's breast cancer. My children are so young — I can't die and leave them. Please tell me what's going on.

Candidate: I hear how overwhelming and frightening this is, Claire, and it's entirely natural to feel this way when you find a new change in your breast. You've done exactly the right thing coming in straight away, and I'm here to support you through every step of this.

Your Next Step: Exploring Ideas, Concerns and Expectations (ICE)

Before moving into red-flag screening, the marking scheme rewards candidates who explicitly explore the patient's own understanding and fears rather than assuming them.

Candidate: Before we talk about what happens next, can I ask what's worrying you most about this, and what you were hoping we could do for you today?

Claire: I keep thinking the worst — that it's already cancer and it's spread. I suppose I just want to know how quickly I can get it checked properly.

Candidate: That's a completely understandable fear, and I promise we will get you seen quickly and properly. Do you mind if I ask a few gentle questions to understand the lump a bit better first?

(Claire wipes away a fresh tear with her tissue, nods weakly, and appreciates your supportive presence.)

Claire: Thank you, Doctor. Yes, please ask me whatever you need. I just want to understand what we have to do next.

Your Next Step: Exploring Breast Red Flags (Data Gathering)

Screen for structural distortion, local advanced features, metastatic nodes, and family history while tracking her baseline risk.

Candidate: Have you noticed any changes to the skin over the lump, such as dimpling, puckering, or redness?

Candidate: Is there any fluid or discharge leaking from your left nipple, especially anything blood-stained?

Candidate: Have you felt any new lumps or swellings under your armpit or around your collarbone?

Candidate: Is there any family history of breast or ovarian cancer, on either side of the family?

Two alternative ways this history point can play out in the exam — practise both, as the roleplayer's script may vary between sittings:

Branch A — Isolated axillary lump, no skin changes

Claire: No, I haven't seen any fluid leaking from my nipple, and the skin looks normal — no dimpling or redness that I can see. But when I was checking myself last night, I did feel a small, hard swelling deep inside my left armpit. It feels just like a swollen gland. Does that mean it's already spreading, Doctor? No one in my family has had breast or ovarian cancer, as far as I know.

Branch B — Skin dimpling plus axillary fullness

Claire: The skin right over the lump looks a little pulled in, almost like a small dimple when I lift my arm. I haven't seen any fluid or bleeding from the nipple, but when I felt under my left armpit this morning, it did feel a bit full and tender, like there might be a small marble-sized bump there too. Does that mean it's definitely spread? My aunt on my mum's side had breast cancer in her 50s, if that matters.

Both branches meet the 2-Week Wait referral threshold. Branch B additionally requires you to acknowledge the family history when explaining risk (a first-degree or close relative with breast cancer strengthens, but does not by itself confirm, malignancy).

Your Next Step: Triple Assessment Explanation & 2-Week Wait Pathway

Claire has presented with an unexplained firm breast lump and a corresponding axillary lump. Under UK NICE guidelines (NG12), any individual aged 30 or older with an unexplained breast lump mandates an urgent 2-Week Wait (2WW) referral to a specialised Breast Clinic. At the clinic they perform a "Triple Assessment" — the gold standard for breast investigation:

- Clinical Examination (by a specialist surgeon)
- Imaging (ultrasound for under-40s; mammogram for 40 and over)
- Biopsy (fine-needle aspiration or core biopsy)

Candidate: Thank you for being so open with me, Claire. What you've described means we need this looked at by specialists right away. While these changes can sometimes be caused by non-cancerous conditions like benign cysts or infections, we have a fast-track system in the UK called a Two-Week Wait referral, which gets you into a specialised Breast Clinic within 14 days. When you go, they'll do what's called a Triple Assessment to give you a definitive answer: a physical check by a specialist, a painless ultrasound or mammogram, and a small, gentle sample of the lump if needed. It's a highly coordinated, precautionary step to get you answers quickly, and we'll support you through it.

(Claire takes a deep, stabilising breath. While still worried, your clear breakdown of the Triple Assessment helps her feel supported rather than panicked.)

Claire: A Triple Assessment... okay, that makes sense. It's comforting that they do all three things at once to be sure. I was so scared I'd have to wait months for answers. What do I need to do now — do I just wait for a call from the clinic?

Your Next Step: Finalising the 2WW Pathway and Safety-Netting

You have delivered the clinical explanation and broken down the Triple Assessment. Now finish the station strongly by confirming the scheduling parameters and the emergency safety-netting boundaries of the 2-Week Wait pathway.

Candidate: I know this is a lot to take in, Claire, but you're not alone in this. The clinic will contact you directly within the next few days to book your appointment. Because this is an urgent fast-track system, please keep your phone nearby. If you haven't heard from them within 5 to 7 days, contact our reception straight away so we can chase it up. Lastly, as a safety net — if you develop a sudden high fever, the breast becomes extremely hot, red, and swollen, or you feel generally very unwell before your appointment, please call us or attend urgent care right away.

(Claire nods firmly, typing the instructions into her phone. Her breathing has slowed, and she looks noticeably more collected.)

Claire: I've written that down, Doctor — expecting a call within a few days, and calling you back if I hear nothing by day seven. I'll keep an eye out for any skin changes or a fever too. Thank you for not rushing me and for explaining exactly what to expect. I feel much stronger going into this now.

Station Conclusion: An "Excellent Pass" Breast Surgery Profile

You will pass this high-stakes breast oncology station with top marks across all categories if you followed this format.

Why your final choices scored maximum marks:

- **Explicit ICE Exploration:** You asked directly what worried Claire most and what she hoped for, rather than assuming her concerns.
- **Empathetic & Safe Interpersonal Communication:** You validated her intense panic regarding her children and her fear of dying, without confirming a malignant diagnosis prematurely.
- **Complete Red-Flag Screen Including Family History:** You covered skin/nipple changes, systemic red flags, and family/genetic risk factors — not just the lump itself.
- **Mastery of the Triple Assessment Concept:** You accurately broke the UK gold-standard pathway into its three elements — Clinical (specialist examination), Radiological (mammogram/ultrasound), Histological (fine-needle aspiration or core biopsy).
- **Strict 2WW Guideline Adherence:** You applied the exact fast-track protocol required for a patient over 30 with an unexplained breast mass.
- **Administrative and Physiological Safety-Netting:** You gave a clear deadline to call back (5–7 days) if hospital communication fails, alongside a systemic check for acute infective complications such as a fast-spreading abscess or mastitis.

Station 3 — Obstetrics: Postpartum Haemorrhage (PPH) Emergency

A Postpartum Haemorrhage (PPH) SimMan station is a critical emergency scenario in the PLAB 2/UKMLA exam. It evaluates your ability to act under pressure, delegate tasks to the nurse, and escalate to the medical emergency team using the ABCDE protocol. Management strictly follows the Royal College of Obstetricians and Gynaecologists (RCOG) Green-top Guideline No. 52.

Definitions — know the thresholds, as they determine your response:

- **PPH:** blood loss of 500 mL or more from the genital tract within 24 hours of childbirth.
- **Minor PPH:** 500–1000 mL, with no clinical signs of shock.
- **Major PPH:** more than 1000 mL, OR any blood loss accompanied by signs of shock (tachycardia, hypotension, collapse) — this is what triggers the full Major Obstetric Haemorrhage protocol described in this station.

Your primary objectives are to: call for help early and appropriately; work through ABCDE systematically while delegating tasks; identify the cause using the 4 Ts framework; manage uterine atony with the correct stepwise drug ladder; and escalate via SBAR when medical management fails.

The PPH Emergency Blueprint

Maternity Crash Call → A: 15L O2 → B: Listen & Sat Check → C: Massive Fluid Resuscitation → The 4 Ts Assessment → Uterine Massage → Medical/Surgical Escalation

The Roleplay

Station: Acute Obstetric Emergency (SimMan)

Candidate Role: FY2/SHO Doctor called urgently to the delivery suite

Roleplayer Role: A midwife/nurse in the room, who follows your instructions and reports back findings; the mannequin represents Mrs Evans, who has just delivered

Setting: Delivery Suite Cubicle

The Prompt (read outside the cubicle):

Patient Details: Mrs Evans, 32 years old, para 3, delivered vaginally 20 minutes ago after a prolonged labour.

Presenting Complaint: You are urgently called to the room. There is a large pool of blood on the bed and the patient looks pale.

Your Task:

- Assess and manage this patient using an ABCDE approach.
- Identify the likely cause of the bleeding.
- Escalate appropriately to senior help.

(You walk in. A mannequin lies on the bed with a large pool of blood beneath her. A midwife is standing by, looking anxious.)

Step 0: Immediate Call for Help

Candidate: This is a major postpartum haemorrhage. Please trigger the Obstetric Major Haemorrhage Protocol, call for a senior obstetrician, an anaesthetist, and the senior midwife. Bring the post-delivery emergency trolley and the blood transfusion kit immediately.

Midwife: Yes doctor, calling it now.

Step 1: A — Airway

Candidate: Hello, can you hear me? My name is Dr [Your Name].

(She responds clearly — the airway is patent.)

Candidate: Please apply a high-flow 15-litre non-rebreather oxygen mask immediately.

Step 2: B — Breathing

Check the monitor for respiratory rate and oxygen saturation. Auscultate the lungs before rushing fluids, to rule out pre-existing fluid overload.

Candidate: Lungs are clear bilaterally.

Step 3: C — Circulation (the core of PPH management)

Expect tachycardia and hypotension from hypovolaemic shock.

Candidate: Please insert two large-bore (14G or 16G) grey cannulas into both arms immediately.

Candidate: Draw emergency bloods for full blood count, urea and electrolytes, clotting screen, group and crossmatch for 4 units, and a baseline lactate. Request 4 units of uncrossed O-negative blood if she is critically unstable.

Candidate: Start a rapid fluid challenge of 1 litre of warmed Hartmann's solution or normal saline while we wait for blood.

Candidate: Please insert a Foley catheter to monitor hourly urine output and keep the bladder empty — an empty bladder helps the uterus contract.

Step 4: D & E — Disability & Exposure

Check capillary glucose and conscious level (AVPU). Expose the abdomen and genital tract to assess the source of bleeding, while keeping her upper body warm with blankets to avoid the lethal triad of trauma (hypothermia, acidosis, coagulopathy).

Diagnosing and Managing the 4 Ts

While the midwife sets up IV fluids and oxygen, systematically work through the 4 Ts to find the cause.

1. Tone (Uterine Atony — accounts for roughly 70–80% of PPH cases)

Candidate: I am palpating the fundus. The uterus feels soft, boggy, and uncontracted.

Begin bimanual uterine massage — rubbing the fundus in a circular motion through the abdominal wall — to stimulate contraction, while the drug ladder is prepared in parallel.

UK stepwise medical management (command the midwife to prepare these in sequence):

- **Oxytocin (Syntocinon):** 5 units IV slowly, followed by an infusion of 40 units in 500 mL saline.
- **Ergometrine:** 500 micrograms IM or slow IV — avoid if the patient has hypertension or pre-eclampsia.
- **Carboprost (Hemabate):** 250 micrograms IM, repeated every 15 minutes if needed, up to 8 doses — avoid if the patient has asthma.
- **Misoprostol:** 800 micrograms sublingually or rectally, if the above are ineffective or contraindicated.
- **Tranexamic acid:** 1 gram IV over 10 minutes, given alongside uterotonics — evidence for this comes from the WOMAN trial, not CRASH-3 (which relates to traumatic brain injury, not obstetric haemorrhage).

2. Tissue (Retained Placenta)

Candidate: I am checking if the placenta and membranes are complete.

If pieces are missing, arrange urgent transfer to theatre for manual removal of placenta under anaesthesia.

3. Trauma (Vaginal or Cervical Tears)

Candidate: Checking for active bleeding from deep vaginal or cervical tears that would need surgical repair.

Inspect the perineum and vaginal wall using a speculum and a strong light source.

4. Thrombin (Coagulation Disorders)

Check whether the blood pooling on the bed is clotting. If it remains entirely liquid and will not clot, suspect Disseminated Intravascular Coagulation (DIC) and request urgent fresh frozen plasma, cryoprecipitate, and platelets in discussion with haematology.

Surgical Escalation Ladder (if bleeding continues despite medical management)

If uterine atony persists despite uterotonics and TXA, escalate in a stepwise fashion — this sequence is frequently tested in oral follow-up questions:

- Intrauterine balloon tamponade (e.g. Bakri balloon) — first-line mechanical measure, often tried before theatre.
- Examination under anaesthesia in theatre to exclude retained tissue or trauma.
- B-Lynch (uterine compression) suture — if balloon tamponade fails and the patient proceeds to laparotomy.
- Uterine or internal iliac artery ligation, or interventional radiology uterine artery embolisation, where available and the patient is stable enough to transfer.
- Hysterectomy — life-saving last resort when all other measures fail to control bleeding.

SBAR Handover (Escalation)

The examiner will pick up the phone or walk in as the consultant obstetrician. Hand over using SBAR:

Candidate (Situation): We have a major postpartum haemorrhage with Mrs Evans, who delivered 20 minutes ago. Total estimated blood loss is currently 1.2 litres and ongoing.

Candidate (Background): She is a 32-year-old lady, para 3, with a history of prolonged labour.

Candidate (Assessment): Her airway is patent on 15L oxygen. BP is 85/50, heart rate is 125. The cause is uterine atony. I have performed uterine massage and given IV oxytocin and IM ergometrine. Two wide-bore cannulas are in, emergency bloods are sent, and 1 litre of fluid is running.

Candidate (Recommendation): I recommend your immediate physical presence to assess for theatre transfer for balloon tamponade or examination under anaesthesia, as medical management has not fully controlled the bleeding.

Station Conclusion: An "Excellent Pass" PPH Profile

You will pass this high-stakes obstetric emergency station with top marks across all categories if you followed this format.

Why your final choices scored maximum marks:

- **Early, Specific Escalation:** You triggered the Major Obstetric Haemorrhage Protocol immediately and named exactly who you needed present, rather than a vague call for "help."
- **Systematic ABCDE with Delegation:** You worked through Airway to Exposure in order while delegating concrete, single-step tasks to the midwife rather than trying to do everything yourself.
- **Correct 4 Ts Reasoning:** You examined for all four causes rather than assuming atony, and reasoned aloud so the examiner could follow your clinical thinking.
- **Accurate, Ordered Drug Ladder:** You escalated through the uterotonic drug ladder in the correct order and correctly cited the contraindications for ergometrine (hypertension) and carboprost (asthma).
- **Knowledge of the Surgical Ladder:** You could describe what comes next if medical management fails — balloon tamponade, B-Lynch suture, arterial ligation/embolisation, and hysterectomy as a last resort.
- **Structured SBAR Handover:** You gave a concise, complete SBAR handover with a clear, actionable recommendation rather than simply relaying findings.

Oral Examiner Follow-Up: Medication Contraindications Quiz

After the practical station, examiners frequently follow up with rapid-fire oral questions on the drugs you just used. These are easy marks if you know them cold, and easy fails if you guess. Work through each drug below.

Examiner: *"You gave IV Oxytocin. What would make you cautious with this drug, and how should it be given?"*

Model Answer: Oxytocin should be given as a slow IV bolus, or diluted in the infusion — a rapid undiluted IV bolus can cause transient hypotension and reflex tachycardia, which is dangerous in a patient who is already haemodynamically unstable from bleeding. Caution is needed in significant cardiovascular disease. It has the fewest contraindications of the uterotonics and remains first-line.

Examiner: *"You mentioned avoiding Ergometrine in hypertension. What else should make you avoid it?"*

Model Answer: Ergometrine is contraindicated in hypertension and pre-eclampsia/eclampsia, since it causes vasoconstriction and can precipitate a hypertensive crisis or stroke. It should also be avoided in significant cardiac disease, migraine, and peripheral vascular disease for the same vasoconstrictive reason.

Examiner: *"Why did you say to avoid Carboprost in this patient?"*

Model Answer: Carboprost is a prostaglandin F2-alpha analogue and is contraindicated in asthma, as it can trigger severe bronchospasm. It should also be used with caution in significant cardiac, pulmonary, renal, or hepatic disease. Common side effects include diarrhoea, vomiting, flushing, and pyrexia, which candidates should not mistake for sepsis in this context.

Examiner: *"Is Misoprostol safe if the other options are contraindicated?"*

Model Answer: Misoprostol has the most favourable safety profile of the four and can be used when Ergometrine or Carboprost are contraindicated, such as in a patient with both asthma and hypertension. Caution is still advised in significant cardiovascular disease. Side effects include shivering, pyrexia, and diarrhoea.

Examiner: *"You gave Tranexamic Acid. Is there anyone you shouldn't give this to?"*

Model Answer: Tranexamic acid should be used cautiously in patients with a known history of thromboembolic disease, since it works by inhibiting clot breakdown, and the dose should be adjusted in significant renal impairment. In the context of major PPH, the mortality benefit shown in the WOMAN trial means it is still given even in most patients with relative risk factors, but this should be flagged to the team.

Examiner: *"Quick-fire: match the drug to its one absolute contraindication."*

Drug	Key Contraindication / Caution
Ergometrine	Hypertension / pre-eclampsia / eclampsia
Carboprost (Hemabate)	Asthma
Oxytocin	Rapid undiluted IV bolus — causes hypotension
Misoprostol	None absolute — caution in cardiovascular disease
Tranexamic acid	Active thromboembolic disease; adjust dose in renal impairment

Station 4 — Sexual Health & Safeguarding: Acute Sexual Assault Disclosure

In the GMC PLAB 2 exam, an Acute Sexual Assault station tests your mastery of crisis communication, medical ethics, and trauma-informed care. It is one of the highest-stakes Interpersonal Skills and Patient Safety stations in the curriculum.

The single biggest failure point for candidates is treating this like a standard medical work-up and jumping straight into a clinical checklist. In the UK, the approach is strictly patient-centred and trauma-informed. You must validate her experience, respect her absolute autonomy regarding police involvement, address time-critical medical and forensic windows, and coordinate care through the specialist UK support pathway (SARC).

Your primary objectives are to: establish psychological safety before anything else; take a focused history covering physical injury, medical background, and prophylaxis needs; respect the patient's autonomy over police involvement; and clearly explain the SARC referral pathway.

The Trauma-Informed Framework

Component	What the Examiner Looks For	PLAB 2-Friendly Language
1. Immediate safety check	Ensuring the patient is physically safe right now, before asking anything else.	"Before we begin, I want to ask — are you physically safe right now? Is there anyone with you, or anyone outside who makes you feel unsafe?"
2. Empathy & non-judgement	Avoiding any wording that implies blame, disbelief, or curiosity about graphic detail.	"I am so sorry this has happened to you. You are in a safe, confidential space, and none of this is your fault."
3. Forensic & medical windows	Managing the correct time frames for treatment and forensic evidence.	Forensic evidence: ideally within 7 days, best preserved within 72 hours (avoid washing/changing clothes if possible). PEPSE (HIV prevention): started within 72 hours. Emergency contraception: Levonelle up to 72 hours; ellaOne up to 120 hours; copper IUD up to 120 hours and the most effective option.

Component	What the Examiner Looks For	PLAB 2-Friendly Language
4. The SARC pathway	Connecting the patient to the specialist UK hub rather than managing everything within the ED.	Sexual Assault Referral Centre (SARC): a dedicated NHS facility with specialist nurses, forensic practitioners, and counsellors, available whether or not she reports to police.
5. Police autonomy	Never pressuring the patient to report to the police, and never treating non-reporting as an obstruction.	"You have complete control over whether the police are involved. We can support you with full medical and forensic care even if you choose not to report this."

Note on forensic examination: the forensic medical examination itself (genital examination, swabs, photography) is carried out by a trained forensic practitioner at the SARC, not by the FY2/ED doctor. Your role in the ED is safety, immediate medical needs, prophylaxis, and referral — not evidence collection.

The Roleplay

Station: Emergency Department / Sexual Health Outpatients

Candidate Role: FY2 Doctor working in the Emergency Department (A&E)

Patient/Roleplayer Role: Chloe Evans, a 24-year-old woman

Setting: A private, quiet consultation room off the main A&E ward

The Prompt (read outside the cubicle):

Patient Details: Chloe Evans, 24 years old, has walked into the Emergency Department accompanied by a close friend. She told the triage nurse quietly that she was sexually assaulted at a house party last night. The nurse has placed her in a private room and asked you to see her.

Your Task:

- Conduct a sensitive, trauma-informed initial consultation.
- Safely explore her immediate medical, forensic, and psychological needs.
- Outline the specialist UK support and referral options available to her.

(You knock gently and walk into the quiet room. Chloe is sitting huddled in a chair, wrapped tightly in a large coat, staring blankly at the floor. She is visibly trembling and tearful.)

Candidate: Hello Chloe, my name is Dr [Your Name], one of the doctors here in the department today. The triage nurse let me know you've been through an incredibly difficult experience, and I've come to see how I can best look after and support you. Before we talk about anything else — are you feeling physically safe here right now?

Chloe: (Nods slowly, crying silently, keeping her eyes down.) Yes... I'm safe here. My friend Jess is outside. I just... I feel so dirty, Doctor. I haven't been home, I haven't showered, and I don't know what to do. I'm so scared I've caught an illness, or that I might be pregnant. I'm just completely broken.

Common Pitfall: *"I need to know exactly what time this happened and if there was penetration so I can calculate your forensic window. Did he use a condom? We also need to call the police immediately to secure the crime scene."* — This jumps straight into an interrogation-style checklist, implies the police decision is not hers, and reads as extracting evidence rather than caring for a person.

Model Response

Candidate: Chloe, I am so sorry that you've had to go through this, and I want you to know you have absolutely nothing to feel ashamed of — none of this is your fault. You've taken a very brave step coming here, and you're in a safe, completely confidential room. We'll take this entirely at your pace, and you're in control of every decision today. It's actually helpful that you haven't showered, as it keeps your options open if you ever choose to preserve evidence, but our first priority is simply making sure you're physically comfortable and looked after. Would it be okay if we gently check how you're feeling physically right now?

Chloe: (She takes a trembling breath and looks up at you for the first time.) Thank you, Doctor... hearing that it's not my fault makes a huge difference. Everyone at that party was looking at me like I did something wrong. Physically, I have a few bruises on my wrists and arms where he held me down, and my pelvic area feels very sore and bruised. My heart is racing and I can't stop shaking. It happened around 1am last night, so about 15 hours ago. He didn't use any protection. I just need to know I'm going to be okay physically.

Your Next Step: Data Gathering & Safety Screening

Now that Chloe feels psychologically safer, move systematically through her physical safety, medical background, prophylaxis needs, and support network — keeping questions direct, gentle, and strictly necessary.

1. Red flags (immediate physical safety): screen for severe bleeding, head/chest/abdominal injury, or loss of consciousness.

2. Medical background: keep this brief — check for underlying conditions, regular medications, and allergies relevant to emergency treatment.

3. Prophylaxis needs: at 15 hours post-assault she is well within the window for both PEPSE and emergency contraception. Ask gently whether she knows anything about the assailant's health (HIV risk), whether she uses regular contraception, and the date of her last menstrual period.

4. Psychosocial & police autonomy: confirm her support network and gently gauge how she feels about police involvement, reinforcing that the decision is entirely hers.

Common Pitfall: *"I need your full medical history, family history, smoking, and alcohol units from last night. We must perform a pelvic swab and draw blood for HIV right now. If you refuse to involve the police, I'll have to document that you're obstructing an investigation."* — This is factually wrong (patients are never obliged to report to police, and non-reporting is never "obstruction"), and turns a trauma consultation into an interrogation.

Model Response

Candidate: Chloe, to make sure your health is protected — have you had any severe bleeding, head injury, or blackouts from last night? Do you have any allergies or medical conditions?

Because we're at the 15-hour mark, we can offer you a medication called PEP to protect against HIV, and emergency contraception to prevent pregnancy. Do you happen to know anything about the other person's health? And lastly — we never have to involve the police unless you want to. Jess is outside for you, but how do you feel about that side of things?

Chloe: No, I haven't lost consciousness, and I didn't hit my head. The bruising on my wrists is just where he held me down, but the bleeding isn't heavy — it's just some spotting and a deep, aching soreness. I don't have any medical conditions or allergies, and I don't take any regular medications. I don't know much about the guy — he was a friend of a friend at the party, so I don't know anything about his health. What is PEP? Is it a tablet? And about the police... I really don't think I can face them right now. I don't want a big investigation or to sit in a police station all day. I just want to focus on making sure my body is safe.

Your Next Step: Collaborative Management Plan

Because Chloe does not want police involvement, the SARC pathway becomes central to your plan — this is the cornerstone of sexual assault care in the NHS and lets her access full medical and forensic support without ever speaking to police.

Candidate: Thank you for telling me all of that, Chloe — you don't have to involve the police, and that will never affect the medical care and support you get from us. Let me explain your options. PEP is a 28-day course of tablets that stops HIV from taking hold in the body if there's been a risk of exposure — we'd want to check a baseline blood test for HIV, Hepatitis B, and Hepatitis C before starting it. For pregnancy prevention, we can give you an emergency contraceptive pill today, and I'd also like to mention that a copper coil is actually the most effective option if you'd consider it, and it can still be fitted within your window. I'd also like to refer you to our local Sexual Assault Referral Centre, or SARC — it's a specialist, private centre with dedicated nurses, doctors, and counsellors who focus entirely on this kind of care. If you wish, they can also store any forensic evidence safely for a couple of years, so the choice to report this to police later on always stays open to you, even if you don't want to decide today. We'll also arrange a follow-up, and please come straight back to us — or to the SARC — immediately if your pain gets significantly worse or if you notice any heavy bleeding.

Chloe: Thank you so much, Doctor. Hearing that they can keep the evidence safely without me having to talk to the police right now is a huge relief. I feel like I can finally breathe. I'm on board with that, and I'll take the PEP and the emergency pill today. Thank you for looking after me so gently.

Station Conclusion: An "Excellent Pass" Sexual Assault Profile

You will pass this high-stakes station with top marks across all categories if you followed this format.

Why your final choices scored maximum marks:

- **Psychological Safety First:** You checked for immediate physical safety before any history-taking, and removed self-blame explicitly and early.
- **Absolute Autonomy Over Police Involvement:** You never pressured her to report, and reassured her that full medical and forensic support is available either way.
- **Accurate Prophylaxis Windows:** You correctly identified she was within the window for PEPSE and offered the full range of emergency contraception, including the most effective option (copper IUD), rather than defaulting to the oral pill alone.

- **Correct Ownership of the Forensic Exam:** You did not attempt to examine or collect forensic evidence yourself — you referred her to the SARC, where trained forensic practitioners carry this out.
- **Mastery of the SARC Pathway:** You explained that a SARC can provide care and store evidence for later reporting, without a police report being made now — the single most examined fact in this station.
- **Watertight Safety-Netting:** You gave clear, injury-specific red flags for return (worsening pain, heavy bleeding) rather than a generic safety net.

Station 5 — Sexual Health: Emergency Contraception

Emergency contraception (EC) is a recurring PLAB 2/UKMLA sexual health station. It tests whether you can select the correct method based on timing, cycle, and weight, according to Faculty of Sexual and Reproductive Healthcare (FSRH) guidance, rather than defaulting to "the morning-after pill" as a single generic option.

Your primary objectives are to: take a focused history of the timing of unprotected sex and cycle details; correctly identify all three EC options a patient is eligible for; explain the trade-offs between them in plain language; and safety-net appropriately if she chooses the copper IUD.

The Three EC Options

Method	Window	Key Points
Copper IUD (coil)	Up to 5 days after unprotected sex, or up to 5 days after the earliest likely date of ovulation, whichever is later	Most effective option (>99%); works even after ovulation has occurred; can be left in as ongoing contraception for up to 10 years depending on the device.
Ulipristal acetate (ellaOne)	Up to 120 hours (5 days)	Slightly more effective than levonorgestrel, especially closer to ovulation; do not start or restart hormonal contraception for 5 days afterwards, as it can reduce ellaOne's effectiveness.
Levonorgestrel (Levonelle)	Up to 72 hours (3 days), licensed	Less effective the later it is taken; double dose (3 mg) recommended if body weight is 70 kg or more, or BMI is 26 or more.

The Roleplay

Station: Sexual Health / GUM Clinic

Candidate Role: FY2 Doctor in a GUM (Genitourinary Medicine) Clinic

Patient/Roleplayer Role: Chloe Evans, 22 years old, presenting after condom failure

Setting: Sexual Health Clinic Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Chloe Evans, 22 years old, has booked an urgent same-day appointment.

Presenting Complaint: A condom broke during intercourse with her boyfriend roughly two days ago. She is not on regular contraception and is anxious about pregnancy.

Your Task:

- Take a focused history of the timing of unprotected sex and her menstrual cycle.
- Identify all appropriate emergency contraception options for her.
- Explain the options clearly so she can make an informed choice.

(You walk into the room. Chloe looks anxious but composed.)

Candidate: Hello Chloe, I'm Dr [Your Name], one of the doctors here today. I understand you've come in to discuss emergency contraception. You're in a completely confidential, safe space here — how can I help?

Chloe: Hello Doctor... thank you. I'm incredibly stressed. My boyfriend and I were intimate on Saturday night — so about two days ago — and the condom broke right at the end. I'm not on any regular birth control, and I'm terrified I might get pregnant. I need the morning-after pill, but I don't know which one is right for me.

Candidate: That's a very common situation, and I completely understand the stress. You've done exactly the right thing coming in today, and we have a few highly effective options. To work out which is safest and most effective for you specifically, do you mind if I ask a few quick questions about the timing and your normal cycle?

Chloe: Yes, please ask whatever you need, Doctor. I just want to make sure I take the right step.

Your Next Step: Targeted History

Candidate: Exactly what time did the condom break on Saturday night? When was the first day of your last period, and how long does your cycle usually last? Do you know your current weight, and are you taking any regular medications, such as epilepsy medication or herbal supplements?

Chloe: It happened at around 10pm on Saturday night, so it's been roughly 44 hours since then. The first day of my last period was exactly two weeks ago. My cycle is very regular — it always lasts 28 days. I weighed myself last week and I'm 75 kilos. I don't take any regular medications or supplements.

Your Next Step: Explaining the Options

At 44 hours, and roughly mid-cycle in a regular 28-day cycle (close to her likely ovulation window), Chloe is eligible for all three methods, but her timing and weight both favour the copper IUD as the most reliable choice.

Candidate: Thank you for those details. At around 44 hours, you actually have three good options. The most effective by far — over 99% — is the copper coil, a small device placed inside the womb. It works immediately and doesn't rely on your body weight or exact cycle day, which matters here since you're roughly in your mid-cycle window. Alternatively, there are two tablet options: ellaOne, effective up to 5 days, though you'd need to wait 5 days before starting any hormonal contraception afterwards; or Levonelle, effective up to 3 days, but because of your weight we'd give you a double dose to make sure it works properly. How does that sound?

Chloe: Thank you for explaining that, Doctor — I had no idea my weight or cycle day mattered so much. The idea of having a coil put in today makes me a little nervous since I've never had one before, but I want the best protection since I'm in that window. If it's over 99% effective and works straight away, I think I'd like to go ahead with that. Can we do that here today?

Your Next Step: Safety-Netting the IUD

Candidate: That's a very sensible choice, and yes, we can arrange the insertion here today. Before we do, I'll check for any signs of a current pelvic infection and run a baseline pregnancy test. One added benefit is that, once in, the coil can also serve as your ongoing regular contraception for several years if you choose to keep it. It's normal to have some mild cramping or light spotting over the next few days. But if you develop a fever, an unusual or foul-smelling discharge, or severe lower abdominal pain in the coming weeks, please contact us or attend urgent care straight away, as this could suggest an infection.

Chloe: That sounds perfect, Doctor. It's a relief knowing it can double as my regular contraception too. I'll watch out for those infection signs, but I'm ready to go ahead today. Thank you so much.

Station Conclusion: An "Excellent Pass" Emergency Contraception Profile

Why your final choices scored maximum marks:

- **Full Option Coverage:** You offered all three eligible methods rather than defaulting to a single oral tablet.
- **Correct Individualisation:** You factored in her exact timing, cycle day, and weight to explain why the coil was the strongest option for her specifically.
- **Informed Choice, Not Direction:** You explained the trade-offs and let Chloe choose, rather than pushing her toward one method.
- **Appropriate Pre-Insertion Safety Checks:** You screened for infection and confirmed a baseline pregnancy test before proceeding to IUD insertion.
- **Specific Safety-Netting:** Your red flags for return (fever, discharge, severe pain) were specific to post-IUD infection risk, not generic advice.

Oral Examiner Follow-Up: Antibiotics and Ongoing Hormonal Contraception

This is a classic PLAB 2/UKMLA follow-up question, often bolted onto a contraception or sexual health station: it tests whether you know the modern, FSRH-based rule rather than the outdated myth that all antibiotics stop the pill from working.

Examiner: *"Maria is on the combined pill and has just been given an antibiotic injection for gonorrhoea. Does this affect her contraception?"*

Model Answer: No. Ceftriaxone — the standard single-dose injection used to treat gonorrhoea in the UK — does not interact with or reduce the effectiveness of either the combined pill or the progestogen-only pill. Her regular contraception remains fully effective.

Suggested phrasing for the patient:

Candidate: That's a really important question to ask. The reassuring answer is no — the antibiotic injection we're giving you today doesn't stop your regular contraceptive pill from working.

The gastrointestinal caveat (safety-netting):

This applies to vomiting or severe diarrhoea generally, not specifically to antibiotics, and the rules differ slightly by pill type:

- **Combined pill (COC):** if she vomits within 2 hours of taking a pill, or has severe diarrhoea lasting more than 24 hours, treat it as a missed pill and use additional contraception (e.g. condoms) for the next 7 days.
- **Progestogen-only pill (POP):** the same vomiting/diarrhoea rule applies, but the additional precaution period is shorter — 2 days, not 7.

The 7-day abstinence rule for gonorrhoea itself: regardless of contraceptive cover, both the patient and her partner(s) should abstain from any sexual contact until 7 days after they have both completed treatment, to prevent re-transmission.

The one exception worth knowing: enzyme-inducing antibiotics — specifically rifampicin and rifabutin (used for tuberculosis) — do actively reduce the effectiveness of hormonal contraception, and additional precautions are needed for as long as they are taken and for 28 days after stopping. Common antibiotics such as amoxicillin, doxycycline, and ceftriaxone are not enzyme inducers and do not have this effect.

Station 6 — Gynaecology: Ruptured Ectopic Pregnancy Emergency

An ectopic pregnancy is one that implants outside the uterine cavity, most commonly in the fallopian tube. When it ruptures, it causes internal haemorrhage and is immediately life-threatening. This PLAB 2/UKMLA station tests whether you can recognise the red-flag symptom pattern quickly, act on an ABCDE basis without delay, and escalate for emergency surgery rather than continuing to "investigate."

Your primary objectives are to: recognise the classic symptom triad and shock features; screen for ectopic pregnancy risk factors; initiate emergency resuscitation; and escalate immediately to the Gynaecology team for likely surgical management.

Focused History & Red Flags

1. The Core Symptom Pattern

- Unilateral lower abdominal or pelvic pain, classically sudden and severe.
- Vaginal bleeding — often dark brown "prune juice" spotting, usually less dramatic than the internal blood loss would suggest.
- A positive pregnancy test, often with the patient unsure of her exact dates.

2. Shock and Peritoneal Irritation — Signs of Rupture

- Dizziness, light-headedness, or collapse on standing — hypovolaemic shock from internal bleeding.
- Shoulder-tip pain, especially worse lying flat — referred pain from blood irritating the diaphragm (a classic and highly specific sign).
- Rectal pressure or a feeling of needing to open her bowels — from blood collecting in the pouch of Douglas.

3. Risk Factors — ask about these even under time pressure

- Previous ectopic pregnancy
- Previous pelvic inflammatory disease or chlamydia infection
- Previous tubal surgery or sterilisation
- An intrauterine device or system (IUD/IUS) in situ
- Assisted conception, e.g. IVF
- Smoking

The Roleplay

Station: Acute Assessment / A&E

Candidate Role: FY2 Doctor in A&E

Patient/Roleplayer Role: Chloe Jenkins, 28 years old, 7 weeks pregnant (positive home test)

Setting: A&E Assessment Bay

The Prompt (read outside the cubicle):

Patient Details: Chloe Jenkins, 28 years old, 7 weeks pregnant on a home test.

Presenting Complaint: Sharp, severe lower-left abdominal pain that started this morning, with dark brown spotting.

Your Task:

- Take a focused history to assess for a ruptured ectopic pregnancy.
- Screen for signs of haemodynamic instability.
- Initiate emergency management and escalate appropriately.

(Chloe is curled on her side on the trolley, clutching a jacket to her belly. She looks pale and lightheaded.)

Candidate: Hello Mrs Jenkins, my name is Dr [Your Name], one of the emergency doctors on duty today. I can see you're in a lot of discomfort and distress — we're going to look after you immediately. Can you tell me exactly where the pain feels worst right now?

Chloe: Hello Doctor... thank you for seeing me. The pain is a sharp, constant stabbing right down here on the bottom left side of my belly. It started this morning when I woke up, and then I noticed some dark brown spotting when I went to the toilet. I'm about 7 weeks pregnant with my first baby, and I'm absolutely terrified I'm having a miscarriage. Please help me.

Candidate: I can hear how frightening this is, Chloe, and it's completely understandable to be scared. You've done exactly the right thing coming straight in. You're in a safe place. Before we examine your stomach, do you mind if I ask a few rapid questions to check how you're feeling overall and make sure you're stable?

Chloe: Yes, please ask me whatever you need, Doctor. I just want to make sure I'm safe.

Your Next Step: Screening for Rupture and Shock

Candidate: Have you felt lightheaded, dizzy, or like you might faint, especially when standing up? Are you experiencing any pain at the very tip of your shoulder, particularly when lying flat? And have you had any sense of pressure in your back passage, like you constantly need to open your bowels?

Chloe: Yes, actually! When I got out of bed to come to the hospital, the whole room spun and I felt so dizzy I had to sit right back down on the floor. And yes — I have this really bizarre, sharp ache right at the tip of my left shoulder. It felt much worse when the nurse asked me to lie flat on the trolley earlier. I haven't had diarrhoea, but I do feel a weird, heavy pressure down in my back passage, like I constantly need to go to the toilet.

Your Next Step: Emergency Management & Escalation

This symptom pattern — unilateral pain, shock, shoulder-tip pain, and pouch-of-Douglas pressure — is classic for a ruptured ectopic pregnancy with significant intra-abdominal bleeding. This is a time-critical surgical emergency.

Candidate: Chloe, I need to be completely direct with you, because your symptoms point to a serious emergency called a ruptured ectopic pregnancy. This means the pregnancy has developed inside the fallopian tube and has caused a tear, leading to internal bleeding — which is why you feel dizzy and have that shoulder pain. I'm making you strictly nil by mouth from now, in case you

need theatre. We're putting two large-bore cannulas into your arms to start intravenous fluids, send an urgent group and crossmatch for blood, and check your pregnancy hormone level. I'm also checking your blood group now, because if you're Rhesus-negative, you'll need an injection called Anti-D to protect any future pregnancies. Most importantly, I'm calling the Gynaecology Registrar to see you immediately, as you are likely to need an urgent operation to stop the bleeding safely.

Chloe: Oh my goodness... okay. Please do whatever you need to do, Doctor. Get the drips in and call the gynaecologists. I understand why we need to move fast. Thank you.

Station Conclusion: An "Excellent Pass" Ectopic Pregnancy Profile

You will pass this high-stakes gynaecological emergency station with top marks across all categories if you followed this format.

Why your final choices scored maximum marks:

- **Rapid Pattern Recognition:** You recognised shoulder-tip pain and positional dizziness as signs of intra-abdominal bleeding rather than treating them as unrelated complaints.
- **No Delay for Further Questioning:** Once red flags for rupture were present, you moved straight to resuscitation and escalation rather than continuing a lengthy history.
- **Correct Immediate Actions:** Nil by mouth, two large-bore cannulas, IV fluids, group and crossmatch, and serum beta-hCG were all actioned together.
- **Anti-D Awareness:** You checked Rhesus status and flagged the need for Anti-D prophylaxis if she is Rhesus-negative — a frequently missed mark in this station.
- **Correct Escalation:** You called the Gynaecology Registrar urgently for likely emergency surgery, rather than requesting a routine or delayed review.

Station 7 — Paediatrics: Toddler Temper Tantrums Counselling

A paediatric counselling station involving temper tantrums is a common communication scenario in the PLAB 2/UKMLA exam, falling under the Interpersonal Skills and Clinical Management domains.

The setup usually involves an exhausted, stressed parent coming to the GP clinic because their 2- to 3-year-old is having violent screaming fits, throwing things, or kicking in public. The parent is often worried the child has a developmental problem such as autism or ADHD, or feels they are failing as a parent.

Your strategy is to rule out physical or developmental causes first, normalise the behaviour, and then deliver NICE-aligned behavioural advice using a collaborative approach.

The Paediatric Tantrum Blueprint

Empathy & Reassurance → Organic/Developmental Screen → Normalise: "The Terrible Twos" → Behavioural Management Strategy → Safety Net & Support

The Roleplay

Station: General Practice — Paediatric Behavioural Counselling

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Mrs Smith, mother of Leo, aged 2 years 8 months

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Mrs Smith has booked a routine GP appointment about her son Leo's behaviour.

Presenting Complaint: Leo has been having violent screaming fits, throwing things, and kicking in public. Mrs Smith looks exhausted and is worried something is developmentally wrong with him.

Your Task:

- Validate the parent's stress and build rapport.
- Rule out organic and developmental causes for the behaviour.
- Provide reassurance and evidence-based behavioural management advice.

(Mrs Smith sits down heavily, looking drained, with dark circles under her eyes.)

Candidate: Hello Mrs Smith, my name is Dr [Your Name]. Thank you for coming in today. I can see from our notes you wanted to talk about your little boy, Leo. Before we look at the medical side, I can see how exhausted and stressed you look — how are you holding up at home?

Mrs Smith: Honestly, Doctor, I'm at the end of my rope. Leo has these violent screaming fits, throwing toys, kicking — sometimes in the middle of the supermarket. Everyone stares at me like I can't control my own child. I keep wondering if something's wrong with him developmentally.

Candidate: Please know you're in a safe, non-judgemental space here. Raising a toddler is incredibly hard work, and it's completely normal to feel overwhelmed when these outbursts happen. Let's go through this together, starting with a few questions about Leo himself.

Your Next Step: Ruling Out Organic and Developmental Causes

Candidate: When do these outbursts usually happen — is it when he's tired, hungry, or told 'no'? Does he seem to hear you clearly when you speak to him normally? And how is his speech developing — can he join a couple of words together to tell you what he wants?

Mrs Smith: It's almost always when he's tired, hungry, or when we say no to something — like leaving the park. His hearing seems completely normal; he responds when I call him from another room. His speech is coming along well too — he can say two or three words together now, like 'more juice please' or 'no bed'.

Candidate: That's really helpful to know. Does he make good eye contact with you, and does he play happily with toys and interact well with other children?

Mrs Smith: Yes, he's very affectionate and looks right at me when we talk or play. He plays nicely with his cousins too, shares toys most of the time — he's just a nightmare the second he doesn't get his own way.

Your Next Step: Normalising the Behaviour

Candidate: Mrs Smith, based on everything you've described — Leo making great eye contact, his speech developing well, and the outbursts happening mostly when he's tired or told 'no' — I want to reassure you: this is a completely normal, healthy part of toddler development. At 2 to 3 years old, children experience huge, overwhelming emotions like anger and frustration, but their brains simply aren't mature enough yet to control them or put them into words. A tantrum is just his way of releasing that frustration. It doesn't mean he has autism, and it absolutely doesn't mean you're a bad mother.

Mrs Smith: (sighs with visible relief) Oh, thank goodness. I really thought I was doing something wrong, or that something was seriously wrong with him.

Your Next Step: Behavioural Management Strategy

Deliver practical, evidence-based behavioural advice grounded in UK positive-parenting principles.

- **Do not reward the tantrum:** "When a tantrum starts, the most effective approach is to calmly ignore the behaviour, as long as he's somewhere safe. If you give in to his demand or shout back, his brain learns that screaming gets him attention or what he wants."
- **Stay calm and present:** "Try to remain calm yourself. Stay close by so he knows he's safe, but don't try to reason or argue with him mid-tantrum — his brain can't process logic during an outburst."
- **Praise the positive:** "The moment he calms down, immediately give him warm attention and praise — something like 'thank you for calming down.' This teaches him that calm behaviour is what gets your attention, not screaming."
- **Plan around known triggers:** "If he struggles when hungry or tired, try to plan outings around his nap times and always carry a small snack."

Your Next Step: Signposting and Safety-Netting

Candidate: You don't have to navigate this alone. I'll put you in touch with our community Health Visitor — they're specialist nurses who can visit you at home, look at Leo's routine, and give practical, hands-on tips. I can also point you to a UK charity called Family Lives, which has excellent parent support forums. Lastly, if you ever feel so overwhelmed that you're worried you might lose control, place Leo safely in his cot, step into another room to take a breath, and call us or a loved one straight away. We're always here to help.

Oral Examiner Follow-Up: "Time-Out" vs Physical Isolation

Examiners commonly test this with a follow-up challenge from the parent:

Parent: *"What about a 'time-out' chair? My mother says I should lock him in his bedroom until he stops. Is that a good idea?"*

Model Answer: A brief, supervised time-out — sitting the child in a safe, boring spot nearby for roughly one minute per year of age, with a parent within sight or earshot — can be a reasonable technique once a child is old enough to understand cause and effect, generally from around 3 years old. This is different from locking a child alone in a bedroom, which is not recommended: it removes the parent's ability to supervise the child's safety, can be frightening and isolating for a toddler who does not yet understand why they have been shut away, and in some contexts could raise a safeguarding concern rather than being seen as a discipline technique. The safe message to give the parent is to keep any time-out brief, supervised, and age-appropriate, and never to lock a child in a room alone.

Station 8 — Paediatrics: ADHD Assessment and CAMHS Referral

An ADHD station in the PLAB 2/UKMLA exam typically involves an exhausted, stressed parent (or sometimes a teacher) attending the GP clinic because their child, usually aged 5 to 12, is struggling at school, acting out, or constantly restless.

The parent is often seeking a rapid diagnosis or demanding immediate medication such as Methylphenidate (Ritalin). Under NICE guideline NG87, a GP never diagnoses ADHD or initiates medication directly. Your role is to screen for the core symptoms, verify they occur across more than one setting, rule out other causes, assess safety and impact, and refer to CAMHS (Child and Adolescent Mental Health Services).

The ADHD Master Blueprint

Empathy & Validation → Screen Core ADHD Triad → Two-Setting Rule Verification → Rule Out Mimics/Red Flags → The CAMHS Referral Pathway → NICE School/Home Advice

The Roleplay

Station: General Practice — Paediatric Behavioural Assessment

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Mrs Ahmed, mother of Chloe, aged 7

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Mrs Ahmed has booked a GP appointment about her 7-year-old daughter, Chloe.

Presenting Complaint: Chloe's school has raised concerns about her difficulty focusing and constant restlessness. Mrs Ahmed is exhausted and wants a diagnosis and medication today.

Your Task:

- Validate the parent's stress and build rapport.
- Screen for the core features of ADHD across more than one setting.
- Explain the correct UK referral pathway and manage expectations around medication.

(Mrs Ahmed sits forward, looking anxious and overwhelmed.)

Candidate: Hello Mrs Ahmed, my name is Dr [Your Name]. Thank you for coming in today. I understand you wanted to talk about Chloe and some difficulties with her behaviour at school and at home. Before we get into the details, I can see how exhausting and stressful this must be — how are you holding up?

Mrs Ahmed: It's been really hard, Doctor. Chloe's teacher keeps calling me in — she says Chloe can't sit still, she's constantly fidgeting, and she blurts out answers without waiting her turn. At home she loses her homework and toys all the time. I just want to get her assessed and maybe start her on something like Ritalin, because I don't know what else to do.

Candidate: Please know you're in a completely safe space here — we're here to support both you and Chloe. Let's go through some questions together so I can understand the full picture before we talk about next steps.

Your Next Step: Screening the Core ADHD Triad

Symptoms must be persistent for at least 6 months across all three domains to be significant.

Candidate: Does Chloe struggle to sustain her attention on schoolwork or play? Does she make careless mistakes, get distracted easily, or frequently lose her homework or toys? Does she find it hard to sit still in her seat, fidgeting or squirming around? Does she often blurt out answers before a question is finished, or struggle to wait her turn in games?

Mrs Ahmed: Yes to all of that, really. She can't focus on her homework for more than a couple of minutes before she's distracted by something else. She loses things constantly — shoes, pencils, her reading book. She's always fidgeting, tapping her feet, getting out of her chair at dinner. And she interrupts constantly, blurts things out in the middle of conversations, can't wait her turn in board games without getting upset.

Your Next Step: The Two-Setting Rule

UK diagnostic criteria require symptoms to be present in at least two different settings — usually home and school — to rule out a purely situational issue.

Candidate: Have Chloe's teachers formally raised similar concerns about her focus and restlessness in the classroom, or is this mostly happening at home?

Mrs Ahmed: Definitely both. Her teacher has called me in twice this term about her not being able to sit still and interrupting the class, and it's exactly the same story at home.

Your Next Step: Ruling Out Mimics and Checking Safety

Candidate: Has Chloe had her hearing and vision checked recently? Sometimes children act out simply because they can't see the whiteboard or hear the teacher clearly. Has there been any major change or stress at home recently, like a bereavement, house move, or bullying at school? How is her sleep? And with all this frustration, how has her mood been — has she shown any signs of hurting herself or others during an outburst?

Mrs Ahmed: Her hearing and eyes were both checked last year and were fine. There hasn't been anything major at home — no house move or bereavement, and as far as I know she isn't being bullied. Her sleep is actually alright, she goes down fine at bedtime. She gets frustrated and cries sometimes, but she's never hurt herself or anyone else.

Your Next Step: The CAMHS Referral and the Medication Conversation

Candidate: Based on everything you've shared — the difficulties with focus, the constant restlessness, and the fact that this is happening at both home and school — it's very possible Chloe is showing signs of ADHD. However, as a GP, I can't make a formal diagnosis or prescribe medication today. In the UK, this requires an expert assessment by our specialist CAMHS team — Child and Adolescent Mental Health Services — and I'll make an urgent referral to them today.

Mrs Ahmed: Okay... but is there anything we can do right now? I was really hoping she could just start on a tablet to help her focus.

Candidate: I understand you want quick relief to help her focus, but UK guidelines are clear that medication is never a first-line quick fix for children. The CAMHS specialists will look at Chloe as a whole person. They'll usually try behavioural and educational strategies first, and would only

consider medication later if her symptoms remain severe and are significantly disrupting her learning, and even then only under specialist supervision.

Your Next Step: Practical Support While Waiting

Candidate: While you're waiting for the CAMHS appointment, I'd recommend speaking directly to Chloe's school SENCO — the Special Educational Needs Co-ordinator. They can put in place simple, immediate classroom adjustments, like seating Chloe at the front away from distractions, or giving her short movement breaks between tasks. At home, try to keep a predictable daily routine, break tasks into small steps, use visual charts, and praise her immediately whenever she completes a task or sits quietly. I'll also give you a leaflet from YoungMinds, a UK charity with excellent advice for parents. And if you ever feel completely overwhelmed, please place her somewhere safe, take a moment for yourself, or call a loved one or us for support.

Oral Examiner Follow-Up: Will a Diagnosis Affect Her Future?

Parent: *"Doctor, if she gets diagnosed with ADHD, will this go on her permanent record and stop her from getting a job or going to university in the future?"*

Model Answer: A diagnosis of ADHD does not appear on a general record accessible to employers or universities, and her medical records remain confidential — they are not shared without consent except in specific, tightly defined circumstances such as a serious safeguarding concern. Far from being a barrier, an ADHD diagnosis can actually open the door to formal support: under the Equality Act 2010, ADHD can be recognised as a disability if it has a long-term, substantial impact on daily life, which gives Chloe legal protection against discrimination at school and, later, at work. At university, students with a diagnosis can access the Disabled Students' Allowance for extra support, such as specialist mentoring, assistive technology, or extra time in exams. Many people with ADHD go on to succeed in higher education and a wide range of careers with the right support in place.

Station 9 — Care of the Elderly: Dementia Assessment and Safety Review

A dementia station typically takes place in a GP or Care of the Elderly outpatient clinic. It usually involves a worried relative bringing in an elderly patient due to progressive memory loss, behavioural change, or safety concerns at home (e.g. leaving the gas stove on, wandering).

Your primary tasks are to rule out reversible causes of confusion, perform a structured cognitive screen, assess home safety risks, and outline the UK diagnostic and support pathway via the Memory Clinic.

Reversible Causes to Screen Out

- **Urinary or chest infection:** sudden, acute-onset confusion (delirium) rather than a slow, progressive decline over months.
- **Hypothyroidism / B12 deficiency:** slow cognitive slowing, cold intolerance, or fatigue.
- **Depression ("pseudodementia"):** poor memory driven by low mood, where the patient often answers "I don't know" rather than attempting the question and getting it wrong.
- **Normal pressure hydrocephalus:** the classic triad of memory loss, urinary incontinence, and gait disturbance ("wobbly, wet, and weird").

The 4-Step Structure

Validate & Screen Timeline → Cognitive Testing & Red Flags → Home Safety Assessment → Memory Clinic Pathway

The Roleplay

Station: General Practice / Cognitive Decline

Candidate Role: FY2 Doctor in a GP Surgery

Patient/Roleplayer Role: Arthur Jenkins, 74, accompanied by his daughter Sarah

Setting: GP Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Arthur Jenkins, 74 years old, brought in by his daughter Sarah, who is concerned his memory has deteriorated significantly over the last six months.

Your Task:

- Establish rapport with both Sarah and Arthur, addressing their concerns empathetically.
- Take a focused history to evaluate the cognitive decline and establish a timeline.
- Perform a thorough home safety evaluation.
- Outline the appropriate next steps in the NHS.

(Arthur is sitting calmly, looking slightly confused about why he is there. Sarah sits beside him, anxious, holding his hand tightly.)

Candidate: Hello Mr Jenkins, and hello Ms Jenkins. My name is Dr [Your Name], one of the doctors here at the practice. I see you've come in today to have a chat about how things are going at home, particularly with your memory, Mr Jenkins. How can I help you both today?

Sarah: Thank you, Doctor. Dad didn't really want to come because he thinks he's completely fine, but I couldn't sit by anymore. Over the last six months, his memory has just been slipping — he forgets what we talked about five minutes ago, he's misplacing his keys constantly, and yesterday he called me by my mother's name. I'm terrified he's developing Alzheimer's. Dad, tell the doctor what happened yesterday with the tea.

Arthur: Oh, Sarah is just making a mountain out of a molehill, Doctor. I'm just getting older! Sure, I forget a name here or there, but my mind is as sharp as it ever was. There's nothing wrong with me.

Common Pitfall: *"Mr Jenkins, you are clearly in denial. Calling your daughter by your late wife's name is a classic symptom of severe brain degeneration. Let's start an immediate dementia memory test right now to prove you have a medical condition." — This dismisses Arthur's dignity, delivers an unearned diagnosis before any assessment, and turns the consultation adversarial before rapport is built.*

Model Response

Candidate: I can hear how deeply you care for your dad, Sarah, and it's completely natural to feel anxious when you notice these changes. Mr Jenkins, you're completely right that a bit of forgetfulness can be a normal part of getting older, but since Sarah has noticed a few specific things over the last six months, it's always best for us to take a closer look, just to ensure we're keeping you safe and supported. Would it be alright if we talk about how you've been managing with everyday tasks at home lately, like that cup of tea Sarah mentioned?

Sarah: Thank you, Doctor, for understanding. What happened with the tea yesterday was terrifying. Dad put the metal kettle on the gas stove, turned the flame on, and then walked away into the garden. He completely forgot about it. By the time I came into the kitchen, the kettle was glowing red hot and the kitchen was full of smoke. If I hadn't dropped by, the whole house could have gone up in flames.

Arthur: (mumbling, looking slightly embarrassed) I just got distracted by a bird in the garden, that's all. It was a one-off mistake...

Your Next Step: Completing the Home Safety and Timeline Screen

You've uncovered a major cooking safety risk. Now complete the remaining home safety elements and rule out a reversible cause such as delirium or infection.

Candidate: Thank you for sharing that, Sarah — I can understand why that was such a frightening moment. Has your dad had any issues with taking his daily tablets correctly, or has he ever gotten confused or lost in familiar places? And Mr Jenkins, would it be alright if we tried a quick, simple two-minute memory check together, just to see how your concentration is doing today?

Arthur: Well, I suppose a quick check wouldn't hurt if it gives Sarah some peace of mind. Go ahead, Doctor.

Sarah: He hasn't gotten lost outside yet, thank goodness. But his medications are becoming a real battle — I found his pill organiser completely full last week, meaning he'd missed three days of his blood pressure tablets. And yes, he does still drive, just to the local supermarket, but lately I've

noticed fresh scrapes and dents on the side of his car. He insists someone must have hit him in the car park, but I'm worried he's misjudging distances.

Examiner's Findings Card

- Cognitive screen (GPCOG): Arthur scores 4 out of 9, indicating significant cognitive impairment. He is disoriented to the date and year, and struggled to recall an address after a 2-minute distraction.
- Timeline: Sarah confirms the decline has been slow, steady, and gradual over 6 months, with no acute, sudden changes over the last few days.
- Physical/neuro exam: normal gait and balance; no focal weakness, tremor, or slurred speech.
- Vitals: BP 142/88 mmHg, pulse 76 bpm, temperature 36.7°C.

The gradual 6-month timeline and lack of acute infective symptoms point away from delirium and toward a progressive dementia. You have identified three safety risks: a cooking/fire hazard, medication non-compliance, and a driving safety concern.

Your Next Step: Delivering the Plan — Reversible Causes, Driving, and the Memory Clinic

Candidate: Thank you for trying those questions with me, Mr Jenkins — you did really well working through them. The results show your concentration and short-term memory are struggling a bit today. Because of this, and the changes Sarah has noticed with the kettle and your medication, we need to take a few steps to keep you safe and get you the right answers. First, we'll arrange some routine blood and urine tests, to make sure there isn't a simple, treatable cause like a vitamin shortage or a mild infection contributing to this.

Sarah: That sounds like a very sensible starting point, Doctor.

Arthur: Yes, if a simple blood test can explain why my memory has been a bit lazy lately, I'm completely fine with that.

Candidate: Alongside those baseline bloods, there are two more important steps. First, I'll refer you to the specialist team at our local Memory Clinic — they focus on concentration and memory, and will likely arrange a detailed brain scan to help pinpoint why these changes are happening, and whether any medication could help steady things. Second, we need to address safety on the road. In the UK, when someone experiences significant changes to their short-term memory, it's a strict legal requirement to inform the DVLA. Because misjudging distances or forgetting a route can put you and others at risk, you'll need to stop driving temporarily while the specialists complete their assessment. I know this feels like a blow to your independence, Mr Jenkins, but our priority is keeping you safe.

Arthur: (sighing, looking sad but accepting) I love my little car... but Sarah did mention those scrapes. If it's the law and it keeps people safe, I suppose I'll have to let Sarah drive me to the shops for now.

Sarah: Thank you, Doctor. That's a massive weight off my shoulders. What about his missed tablets?

Candidate: To make managing his tablets easier, I'll arrange a weekly blister pack from the pharmacy, which groups his morning and evening pills into clearly labelled compartments. I'll also request a Social Care assessment to look at simple home adaptations, like an automatic shut-off device for the stove. I'll print out information on contacting the DVLA and the Memory Clinic process for you both today. Please come back straight away if you notice a sudden worsening in his confusion, a fever, or if he becomes physically unwell.

Station Conclusion: An "Excellent Pass" Dementia Profile

Why your final choices scored maximum marks:

- **Handled the Non-Negotiable Safety Boundary:** You explicitly stated that informing the DVLA and stopping driving is a legal requirement — leaving this optional is a critical safety failure in UK OSCEs.
- **Multidisciplinary Approach:** You pulled in Pharmacy (blister packs), Social Care (home safety adaptations), and the Memory Clinic (diagnostics), showing an integrated grasp of NHS care.
- **Preserved the Patient's Dignity:** You framed the driving cessation around protection and safety rather than punishment, keeping Arthur cooperative rather than defensive.
- **Ruled Out Reversible Causes First:** You arranged baseline bloods and urine testing before assuming an irreversible diagnosis.
- **Balanced Family Dynamics:** You validated both Sarah's genuine concern and Arthur's dignity simultaneously, rather than siding with one party over the other.

Station 10 — Ethics & Communication: DNACPR Counselling

In the UK NHS system, discussing a Do Not Attempt Cardiopulmonary Resuscitation (DNACPR) decision with a distressed, protective family member is a core Communication Skills and Ethics station. It evaluates your empathy, structural clarity, and adherence to the Tracey v Cambridge University Hospitals NHS Foundation Trust (2014) ruling, which established that patients or their families must be consulted about a DNACPR decision unless doing so would cause them physical or psychological harm.

The core framework centres on helping the family understand that CPR is a medical treatment — meaning it cannot be demanded if it would be clinically futile or harmful — while fulfilling your legal obligation to consult and inform them transparently.

High-Yield DNACPR Communication Points

- **Avoid cold acronyms:** never just say "DNACPR" — spell it out fully as "Do Not Attempt Cardiopulmonary Resuscitation" so the family understands exactly what is being discussed.
- **The "do not treat" misconception:** reassure the family that the form applies only if the heart and breathing stop. All other active treatments — antibiotics, IV fluids, oxygen, and pain relief — continue in full.
- **Lifting the burden of guilt:** clearly explain that the final decision rests with the senior medical consultant, made in the patient's best interests, not with the family. This prevents relatives feeling they are the ones choosing to let their loved one die.
- **The right to a second opinion:** if the family strongly disagrees after a full, compassionate explanation, they are entitled to ask for a second clinical opinion — this does not change the fact that CPR cannot be demanded if it is clinically futile, but it is part of treating the family fairly and transparently.

The Roleplay

Station: Consultation Skills / Conflict Resolution

Candidate Role: FY2 Doctor working in the Acute Medical Unit (AMU)

Patient/Roleplayer Role: Mark Vance, 42, son of 84-year-old patient Robert Vance

Setting: Private Quiet Room, AMU Ward

The Prompt (read outside the cubicle):

Patient Details: Robert Vance, 84, admitted with end-stage metastatic lung cancer complicated by severe multi-organ sepsis secondary to aspiration pneumonia. He is semi-conscious and rapidly deteriorating.

The Clinical Reality: The senior consultant has assessed Robert and determined his body is naturally failing. Chest compressions and defibrillation in the event of a cardiac arrest would be clinically futile, would fracture his ribs, and would strip away his dignity without any realistic chance of restarting his heart. A DNACPR form has been drafted.

Your Task:

- Consult with Robert's son, Mark, regarding the medical team's decision.
- Handle his anger, defensiveness, and demands to "do everything."
- Establish a compassionate, legally sound consensus under UK guidelines.

(You walk into the quiet room. Mark is pacing back and forth, breathing heavily, fists clenched. He glares at you with visible hostility as you sit down.)

Candidate: Hello Mark. My name is Dr [Your Name], one of the medical doctors on the AMU team looking after your father, Robert. Thank you for taking the time to sit with me. I can see you're incredibly distressed right now — how are you holding up?

Mark: (slamming his hand on the table) How am I holding up? My dad is dying out there, and one of your nurses just walked into his cubicle with a form that says 'Do Not Resuscitate' on it! Are you people out of your minds? You're giving up on him! He's a fighter! I demand that if his heart stops, you do absolutely everything — shocks, chest compressions, whatever it takes! If you let my dad die without a fight, I will sue this hospital!

Common Pitfall: *"Mark, please lower your voice and stop shouting at me. Legally, you have no right to demand CPR. Your father has terminal cancer and advanced sepsis, so if we perform chest compressions we'll simply smash his ribcage and torture him for no reason. The consultant has already signed the form, and your opinion cannot change that fact."* — *Factually defensible, but delivered without empathy, this shuts Mark down rather than bringing him to genuine understanding, and abandons the legal duty to consult transparently.*

Model Response

Candidate: I can hear how furious and heartbroken you are, Mark, and I want to apologise that this form was introduced to you in a way that felt like we were simply giving up on your father. Any loving son would feel protective seeing that. Please know we are absolutely not giving up on Robert — our goal is to make sure he's protected from unnecessary pain and trauma. Can we walk through exactly what his body is fighting right now, so I can explain why the medical team wants to protect his dignity at this stage?

Mark: (sitting down slowly, still tense) ...Fine. Tell me. What exactly is wrong with him that makes you think CPR wouldn't work?

Your Next Step: Explaining the Clinical Reality and the "Treatment, Not Punishment" Framing

Candidate: Thank you for hearing me out, Mark. Your dad's cancer has spread significantly, and on top of that, he now has a severe infection affecting multiple organs, which is why he's semi-conscious. His heart and lungs are extremely fragile right now. If his heart were to stop, performing chest compressions would very likely fracture his ribs, and given how unwell his body already is, there is no realistic chance it would restart his heart or bring him back. CPR is a medical treatment like any other — and just like we wouldn't give a treatment that couldn't work and would only cause harm, our team doesn't feel CPR would help your dad in this situation. It's important you know this decision doesn't rest with you, or with me alone — it's made by the senior consultant looking after him, based on what is medically best for Robert. You are not the one choosing this, and you never will be.

Mark: (sinking back into his chair, fists slowly unclenching, voice cracking) Broken ribs... internal bleeding... I didn't realise it was that brutal. I just thought it was like on the television — you shock them and they wake up and look at you. I don't want him to suffer, Doctor. I really

don't. But if his heart does stop and we don't do that... what happens to him? Are you just going to turn off the monitors and leave him alone in a dark room to die?

Your Next Step: Reassurance on Ongoing Active Care and Closing the Consultation

This is the single most important moment in the station. Mark has moved from anger into genuine fear of abandonment, and the "do not treat" misconception must be corrected explicitly and concretely, not just in passing.

Candidate: I want to give you my absolute word, Mark: this form does not mean we stop treating your father, and it does not mean we are leaving him. We are simply stopping one single treatment that would cause him trauma without helping him. Everything else continues at full strength — his antibiotics, his IV fluids, his oxygen, and strong medication to make sure he has no pain or breathlessness. We will keep him comfortable and dignified, and you or your family are welcome to stay with him the entire time, in a quiet, private space. And as I said, the senior consultant has taken full clinical responsibility for this decision — you don't have to make this choice, and you don't have to sign anything. You can simply focus on being his son and sitting with him.

Mark: (tears in his eyes, posture completely softened) Thank you, Doctor... truly. Hearing that you aren't leaving him, and that you'll keep fighting to make sure he isn't in pain or gasping for air, is all I actually wanted. And thank you for taking the decision out of my hands — I couldn't have lived with myself thinking I was the one choosing to stop. I'd like to go back in and just hold his hand now.

Candidate: Of course. And if you'd like, I'm very happy to arrange for you to speak directly with the consultant afterwards, or to request a second opinion, so you have complete confidence in this decision at your own pace.

Mark: I think I understand now, but yes... speaking to the consultant directly afterwards would help me feel more at peace with this.

Station Conclusion: An "Excellent Pass" DNACPR Profile

Why your final choices scored maximum marks:

- **Led With Empathy Before Facts:** You validated Mark's anger and grief before explaining the clinical reasoning, de-escalating him enough to actually hear the explanation.
- **Framed CPR as a Treatment Decision:** You explained that CPR cannot be demanded if it would be clinically futile and harmful, without making Mark feel he had no voice at all.
- **Dismantled the "Television CPR" Myth:** You concretely explained the physical trauma of chest compressions in a frail, terminal patient, shifting his mental image of CPR from a life-saving miracle to a harmful intervention.
- **Directly Answered the Abandonment Fear:** When Mark asked whether you would "just leave him to die," you gave a concrete, specific answer — antibiotics, fluids, oxygen, and analgesia continuing in full — rather than a vague reassurance.
- **Lifted the Burden of Guilt:** You explicitly clarified that the decision rests with the senior consultant, not with Mark, directly addressing his fear that he was "choosing to let his dad die."
- **Offered a Second Opinion:** You offered direct access to the consultant, respecting the family's right to further clarity without reopening a decision that isn't clinically appropriate to reverse.

SECTION: DERMATOLOGY

Station 11 — Dermatology: Stevens-Johnson Syndrome / Toxic Epidermal Necrolysis

Stevens-Johnson Syndrome (SJS) and Toxic Epidermal Necrolysis (TEN) present as a high-stakes Acute Emergency/Combined Management station in the PLAB 2/UKMLA exam — a life-threatening dermatological emergency usually triggered by a recently started medication.

Your primary objectives are to recognise the critical severity of the condition immediately, take a rapid focused medication history, check for mucous membrane and multi-organ involvement, and execute an immediate emergency resuscitation and transfer pathway.

Rapid Focused Assessment

- **The rash profile:** painful, atypical "target" lesions, widespread red/purple macules, and a positive Nikolsky sign (the top layer of skin slips off easily when gently rubbed).
- **The medication history (crucial):** ask about any new medications started in the last 4 to 28 days. High-risk culprits include allopurinol, antiepileptics (carbamazepine, lamotrigine, phenytoin), sulfonamide antibiotics (co-trimoxazole) and penicillins, and NSAIDs such as piroxicam.
- **Mucous membrane screen:** ask about painful sores, bleeding, or crusting in the mouth, eyes, genitals, or nose.
- **Systemic red flags:** high fever, severe dehydration, tachycardia, or breathing difficulty.

SJS involves skin detachment of less than 10% of body surface area; TEN involves more than 30%; the overlap syndrome sits between the two. Both are managed on the same emergency pathway, similarly to severe thermal burns.

Emergency Management Pathway

- Stop the offending drug immediately.
- ABCDE assessment and resuscitation, including aggressive IV fluid resuscitation to offset fluid loss through denuded skin.
- Urgent senior escalation to the medical registrar and ICU team.
- Arrange transfer to a Burns Unit or ICU for protective isolation and specialised wound care.
- Request urgent Ophthalmology review (to prevent permanent scarring/blindness) and Dermatology review.

The Roleplay

Station: Acute Emergency / Consultation Skills

Candidate Role: FY2 Doctor in the Emergency Department (A&E)

Patient/Roleplayer Role: David Miller, 42 years old

Setting: Emergency Resuscitation Bay

The Prompt (read outside the cubicle):

Patient Details: David Miller, 42, brought to A&E by ambulance.

Presenting Complaint: High fever and a rapidly spreading, extremely painful raw skin rash that started this morning. His mouth and eyes feel like they are "on fire."

Your Task:

- Take an emergency focused history.
- Stabilise the patient.
- Explain the critical management steps.

(David is lying back on the trolley, looking extremely unwell, shivering, and crying out in pain when he moves.)

Candidate: Hello Mr Miller, my name is Dr [Your Name], one of the emergency doctors here. I can see you're in an immense amount of pain and looking very unwell. We're going to take immediate care of you. Can you tell me briefly what's happened?

David: Doctor... please help me... my skin feels like it's burning off. It started with a fever yesterday, and today these raw, painful red patches broke out all over my chest and face. My lips are bleeding, and I can barely open my eyes or swallow because it hurts so much. Am I dying?

Candidate: Mr Miller, please try to stay as still as you can so I can examine your skin. To figure out what's causing this — have you started any brand new medications over the last few weeks? I'm also going to get my nursing team to start an intravenous line immediately for strong pain relief and fluids.

David: (nodding weakly as the nursing team establishes IV access) Thank you, Doctor... the pain is unbearable. New medicines? Yes, actually. My GP started me on a tablet called Allopurinol about two weeks ago because of an awful gout flare-up in my big toe. That's the only new thing I've taken.

Your Next Step: Confirming the Emergency and Escalating Care

David's picture is a textbook presentation of Stevens-Johnson Syndrome: high fever, painful peeling skin, mucous membrane ulceration, and a specific drug trigger (allopurinol) within the 4-to-28-day window. This requires immediate life-saving escalation.

Common Pitfall: *"This looks like a severe allergic reaction to your gout medication. We'll stop the Allopurinol and move you to a standard medical ward with some oral paracetamol, monitoring your skin over the next few days." — Dangerously under-triages a burns-equivalent emergency to routine ward care.*

Model Response

Candidate: David, I need to be completely direct with you, because this is a serious medical emergency. This is a severe, rare reaction to the Allopurinol called Stevens-Johnson Syndrome. We're stopping that medication immediately. Because it affects your skin and the lining of your mouth and eyes, you need specialised care — I'm calling the Intensive Care and Burns teams right now to arrange your transfer to a specialist unit. We're also calling an eye specialist urgently to protect your vision, and we'll keep the fluids running to keep you stable.

David: (visibly frightened but reassured by your decisive actions) Thank you, Doctor... thank you for moving so quickly. Please do whatever it takes. I trust you.

Station Conclusion: An "Excellent Pass" SJS Profile

Why your final choices scored maximum marks:

- **Immediate Trigger Identification:** You correctly identified Allopurinol as the culprit and withdrew it immediately — the single most critical step in halting SJS progression.
- **Recognised Multi-Organ Severity:** You acknowledged mucous membrane involvement and treated this like a burns-equivalent emergency requiring aggressive IV fluids.
- **Flawless Emergency Escalation:** You called senior ICU and Burns Unit support immediately, rather than defaulting to routine ward care.
- **Specialised Safety-Netting:** You explicitly requested an urgent Ophthalmology review, since ocular involvement in SJS can cause permanent scarring or blindness if untreated.

Station 12 — Dermatology: Scabies Infestation

Scabies is a highly contagious skin infestation caused by the mite *Sarcoptes scabiei* var. *hominis*, which burrows into the epidermis to live, feed, and lay eggs, triggering an intense delayed hypersensitivity reaction. It is a common Consultation Skills/Management station testing focused history, recognition of classic signs, and correct community management, including household contact treatment.

Focused History & Examination

- **Itching pattern:** classically intense and worse at night or after a hot bath/shower.
- **Rash distribution:** web spaces between fingers and toes, inner wrists, elbow flexures, waistline, buttocks, and genitalia; in infants, the scalp, face, palms, and soles can also be affected.
- **Burrows:** look for thin, greyish-white, thread-like tracks, often distorted by scratching.
- **Contact history:** ask about close physical contact with anyone else who is itching — household members, partners, or care settings such as nurseries or nursing homes.
- **Secondary infection:** screen for broken, weeping, or crusted skin suggesting secondary bacterial infection (impetiginisation) from scratching.

The Roleplay

Station: General Practice / Dermatology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Priya Sharma, 29 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Priya Sharma, 29, has booked a routine appointment.

Presenting Complaint: Intense itching for the last 10 days, much worse at night, affecting her hands and around her waist. Her partner has also started itching in the last few days.

Your Task:

- Take a focused history and examine the pattern of the rash.
- Identify the likely diagnosis.
- Explain treatment, including household contact management.

(Priya is visibly scratching her wrists as she sits down.)

Candidate: Hello Ms Sharma, I'm Dr [Your Name]. I understand you've been dealing with some itching. How can I help today?

Priya: Hello Doctor. It's been driving me mad for about 10 days now. It's worst at night — I can barely sleep. It's mainly between my fingers and around my waist. And oddly, my partner has started scratching a lot too over the last few days.

Candidate: That sounds really uncomfortable, especially if it's disturbing your sleep. Can I take a closer look at your hands and the affected areas, and ask a few more questions?

Priya: Yes, please, go ahead.

Your Next Step: Examination and Confirming the Diagnosis

(On examination: fine, thread-like greyish tracks visible in the finger web spaces, with excoriated red papules around the waistline. No honey-coloured crusting.)

Candidate: Thank you, Priya. What you're describing — itching that's much worse at night, affecting the web spaces of your fingers and your waist, and now your partner developing the same symptoms — along with these fine burrow marks I can see between your fingers, points to a condition called scabies. It's caused by a tiny mite that lives in the top layer of skin, and it spreads through close skin-to-skin contact, which fits with your partner also being affected.

Priya: Oh no, that sounds horrible. Is it something serious? How do we get rid of it?

Your Next Step: Explaining Treatment and Household Decontamination

The single most commonly missed mark in this station is failing to treat all household and close contacts simultaneously, since untreated contacts simply re-infest the patient.

Candidate: It's not dangerous, but it is very contagious, so treating it properly the first time matters. I'll prescribe you a cream called Permethrin 5%, which needs to be applied to your entire body from the neck down to your toes, left on for 8 to 14 hours — usually overnight — then washed off. You'll need a second application exactly 7 days later to catch any newly hatched mites. Importantly, your partner needs to be treated at exactly the same time as you, even though his symptoms only just started, otherwise you'll simply pass it back and forth to each other. On the day you apply the cream, wash all your bedding, towels, and recently worn clothing in a hot wash of at least 60 degrees, and tumble dry on a hot cycle. Anything that can't be washed can be sealed in a plastic bag for about a week, since the mites can't survive that long without skin contact. It's also worth knowing the itching itself can persist for several weeks after treatment, even once the mites are gone — that doesn't mean the treatment has failed.

Priya: That makes sense — I'll make sure my partner gets treated at the same time, and I'll do the washing straight away. Thank you for explaining that the itching might not go away immediately, I would have panicked otherwise.

Station Conclusion: An "Excellent Pass" Scabies Profile

Why your final choices scored maximum marks:

- **Correct Diagnosis From Pattern Recognition:** You linked nocturnal itching, web-space distribution, and a symptomatic contact to reach the diagnosis confidently.
- **Simultaneous Contact Treatment:** You explicitly required the partner to be treated at the same time, which is essential to prevent reinfestation and is a frequently missed mark.
- **Complete Environmental Advice:** You covered hot-washing bedding/clothing and bagging non-washable items — not just the topical treatment.
- **Managed Expectations:** You warned that itching can persist for weeks after successful treatment, preventing unnecessary re-consultation or perceived treatment failure.

Station 13 — Dermatology: Acne Vulgaris

Acne vulgaris is a very common GP and dermatology consultation station, testing your ability to grade severity, apply the correct UK management ladder, and address the psychosocial impact that often drives the consultation as much as the skin itself.

Clinical Classification

- **Non-inflammatory:** open comedones (blackheads) and closed comedones (whiteheads).
- **Inflammatory:** papules (tender red bumps), pustules (pus-filled), nodules (large, deep, painful lumps), and cysts, which carry a high risk of permanent scarring.

UK Management Ladder

- **Mild acne:** topical retinoid (e.g. adapalene) or benzoyl peroxide, alone or combined; azelaic acid as an alternative.
- **Moderate acne:** a fixed-dose topical combination (e.g. topical adapalene with benzoyl peroxide, or topical antibiotic with benzoyl peroxide) alongside a 12-week course of an oral antibiotic such as doxycycline or lymecycline if topical treatment alone is insufficient; topical antibiotics should not be used as monotherapy, to reduce resistance.
- **Severe/nodulocystic acne:** referral for specialist-supervised oral isotretinoin, or hormonal options (combined oral contraceptive, or co-cyprindiol/spironolactone) for females where appropriate.

Isotretinoin requires specialist initiation due to teratogenicity, mood-related side effects, and the need for pregnancy prevention counselling and monitoring in patients who can become pregnant.

The Roleplay

Station: General Practice / Dermatology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Jamie Cole, 17 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Jamie Cole, 17, has booked an appointment about his skin.

Presenting Complaint: Worsening spots on his face, chest, and back over the last year. He seems withdrawn and reluctant to make eye contact.

Your Task:

- Take a focused history, including psychosocial impact.
- Grade the severity of his acne.
- Explain the appropriate management step.

(Jamie sits with his hood up, avoiding eye contact, arms crossed.)

Candidate: Hi Jamie, I'm Dr [Your Name]. I understand you've come in about your skin — how's it been affecting you?

Jamie: (quietly) It's just really bad. It's on my face, chest, and back. I've tried loads of face washes from the shops but nothing works. I don't really want to go to school some days because of it.

Candidate: I'm really sorry to hear it's affecting you like that, Jamie — that takes a lot of courage to share, and it's a really common experience with acne that people don't always talk about. Would it be alright if I take a look at the affected areas, and ask a bit more about how long this has been going on?

Jamie: Yeah, okay.

Your Next Step: Examination and Grading Severity

(On examination: multiple comedones, inflamed papules and pustules across the face, with several deep, tender nodules on the back. No visible cystic lesions or current scarring.)

Candidate: Thank you, Jamie. Looking at this, you have a mix of blackheads, whiteheads, and some inflamed spots, but I can also feel a few deeper, firmer lumps on your back, which puts this into the moderate-to-severe range rather than mild acne. Given how much this is affecting your confidence and your attendance at school, I think we should move straight to a more effective treatment combination rather than starting with the mildest option.

Jamie: Okay... what does that actually involve? I just want something that will actually work.

Your Next Step: Explaining Management and Setting Expectations

Candidate: I'll start you on a combination cream containing a retinoid and benzoyl peroxide, which you'll apply once daily, alongside a 12-week course of an antibiotic tablet called doxycycline, which helps calm the inflammation from the inside. It's really important to know that this won't work overnight — most people don't see real improvement for 6 to 8 weeks, so please stick with it even if it feels slow at first. Wash the affected areas twice a day with a gentle cleanser, and try to avoid picking or squeezing the spots, as that raises the risk of permanent scarring. If we don't see enough improvement after this course, there's a stronger specialist treatment called isotretinoin that a dermatologist can consider, which is very effective for exactly this kind of acne.

Jamie: That actually makes me feel a bit better — I thought you'd just tell me to wash my face more. I can wait a couple of months if I know it's actually going to help.

Station Conclusion: An "Excellent Pass" Acne Profile

Why your final choices scored maximum marks:

- **Addressed Psychosocial Impact:** You acknowledged the effect on Jamie's confidence and school attendance, not just the physical lesions.
- **Accurate Severity Grading:** You correctly identified nodules on examination and escalated straight to a moderate/severe management step rather than starting at the mildest rung unnecessarily.
- **Avoided Antibiotic Monotherapy:** You paired the oral antibiotic with a topical retinoid/benzoyl peroxide combination rather than prescribing an antibiotic alone.

- **Managed Expectations:** You explained the realistic 6–8 week timeframe for visible improvement, reducing the risk of early treatment abandonment.
- **Signposted the Next Step:** You mentioned isotretinoin as the specialist option if first-line treatment fails, without over-promising it today.

Station 14 — Dermatology: Impetigo

Impetigo is a highly contagious superficial bacterial skin infection, most common in young children. This station tests recognition of the classic appearance, correct antibiotic choice, and infection-control advice, including school exclusion rules.

Clinical Features

- **Non-bullous impetigo (around 70% of cases):** small red papules/vesicles, typically around the mouth and nose, that rupture and dry into a characteristic golden, honey-coloured crust.
- **Bullous impetigo:** caused by toxin-producing *Staphylococcus aureus*, presenting with larger, flaccid, fluid-filled blisters, often on the trunk or skin folds.

UK/NICE Management Pathway

- **Localised, mild infection:** topical fusidic acid for 5 days; if there is a risk of resistance or in a suspected outbreak, hydrogen peroxide 1% cream is often preferred first-line.
- **Widespread or more severe infection:** oral flucloxacillin (or oral clarithromycin/erythromycin if penicillin-allergic) for 5 days.
- **School/nursery exclusion:** children should be excluded from school or nursery until all lesions have crusted and healed, or until 48 hours after starting appropriate antibiotic treatment.
- **Hygiene advice:** avoid sharing towels or flannels, wash hands frequently, and keep fingernails short to reduce spread through scratching.

The Roleplay

Station: General Practice / Paediatric Dermatology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Mrs Patel, mother of 4-year-old Ravi

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Ravi, 4, brought in by his mother.

Presenting Complaint: A crusty rash around his mouth and nose that appeared 3 days ago and seems to be spreading. Ravi is due to start nursery again on Monday.

Your Task:

- Take a focused history and examine the rash.
- Explain the diagnosis and treatment.
- Advise on infection control and nursery exclusion.

(Ravi sits on his mother's lap, with visible golden-crust patches around his mouth and one nostril.)

Candidate: Hello Mrs Patel, I'm Dr [Your Name]. I understand you're worried about a rash on Ravi's face — can you tell me a bit more about it?

Mrs Patel: Yes, Doctor, it started as just a couple of little spots around his mouth three days ago, but now it's crusty and yellow and seems to be spreading toward his nose. He's not in any pain, but he keeps touching it, and I'm worried it's something serious. He's supposed to go back to nursery on Monday.

Candidate: Thank you for bringing him in so quickly. Would it be alright if I take a closer look at the patches?

Mrs Patel: Yes, please go ahead.

Your Next Step: Confirming the Diagnosis and Explaining Treatment

Candidate: Thank you. What I can see is a classic golden, honey-coloured crust around his mouth and nose, which is a very typical appearance of a condition called impetigo. It's a common, superficial skin infection in young children, caused by bacteria getting into small breaks in the skin, and it's very contagious, but it's easily treated and not dangerous.

Mrs Patel: Oh, that's a relief. What do we need to do to treat it?

Candidate: Since this looks localised rather than widespread, I'll prescribe a topical cream to apply directly to the crusted areas a few times a day for about 5 days. If it were more widespread, we'd use an antibiotic syrup instead, but this looks manageable with the cream alone. In the meantime, try to keep his fingernails short, since scratching can spread it to other areas or to other people, and avoid him sharing towels or flannels with his siblings.

Your Next Step: Nursery Exclusion Advice

Candidate: As for nursery — because impetigo is quite contagious, he'll need to stay off until either all the patches have fully crusted over and healed, or until 48 hours have passed since starting the cream, whichever comes first. So realistically, if you start the cream today, he could potentially go back Monday if the patches are drying up well by then, but I'd keep a close eye on it and use your judgement closer to the time.

Mrs Patel: That makes sense, thank you. I'll let the nursery know just in case, and start the cream right away.

Station Conclusion: An "Excellent Pass" Impetigo Profile

Why your final choices scored maximum marks:

- **Correct Visual Recognition:** You correctly identified the pathognomonic golden, honey-coloured crust as impetigo.
- **Appropriate Severity-Matched Treatment:** You chose topical treatment for localised disease rather than defaulting to oral antibiotics unnecessarily.
- **Accurate Exclusion Advice:** You gave the correct nursery exclusion rule — until lesions have crusted and healed, or 48 hours after starting treatment — rather than a vague "stay off for a week."
- **Practical Infection Control:** You covered hygiene measures (short nails, no shared towels) to limit spread to siblings and other children.

Station 15 — Dermatology: Suspicious Pigmented Lesion (Melanoma)

A suspicious mole station tests systematic lesion history-taking, recognition of the features that separate a benign mole from melanoma, and correctly triggering the UK 2-Week Wait skin cancer pathway. Melanoma is the most dangerous form of skin cancer, arising from pigment-producing melanocytes, with early detection dramatically improving prognosis.

Structured Lesion History

- **Onset:** when was the lesion first noticed, and is it a new mole or a long-standing one that has changed?
- **Site:** where on the body is it, including areas not usually sun-exposed (soles, palms, under nails), which can carry particular significance.
- **Duration:** how long has it been present, and over what time frame has any change occurred (weeks vs months vs years)?
- **Description — shape, size, colour:** is the border regular or irregular? Is the colour uniform or does it vary (shades of brown, black, red, white, or blue within the same lesion)? Has the size increased?
- **Frequency/timing of change:** has it been getting bigger or changing gradually, or was there a sudden, recent change? Any episodes of bleeding or crusting, and how often?
- **Red flags:** itching, bleeding or discharge from the lesion, fever, palpable lumps (regional lymphadenopathy), loss of appetite, unintentional weight loss, and night sweats — these systemic features raise concern for metastatic spread rather than a purely localised skin problem.

The ABCDE and 7-Point Checklist

- **A — Asymmetry:** one half of the lesion doesn't match the other.
- **B — Border:** irregular, notched, or poorly defined edges.
- **C — Colour:** more than one colour, or uneven distribution of colour.
- **D — Diameter:** generally greater than 6mm, though melanomas can occasionally be smaller.
- **E — Evolution:** any change in size, shape, colour, or sensation over time — often the single most important feature reported by the patient.

The UK 7-point checklist is used similarly in primary care: 3 major features (change in size, irregular shape, irregular colour) and 4 minor features (diameter ≥ 7 mm, inflammation, oozing or crusting, change in sensation), with any major feature or several minor features prompting an urgent referral.

The Roleplay

Station: General Practice / Dermatology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Graham Wells, 62 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Graham, 62, has booked an appointment about a mole on his back that his wife noticed has changed.

Your Task:

- Take a structured history of the lesion.
- Screen for systemic red flags.
- Explain the referral pathway clearly.

(Graham sits somewhat dismissively, having only come in because his wife insisted.)

Candidate: Hello Mr Wells, I'm Dr [Your Name]. I understand your wife noticed a change in a mole on your back — can you tell me more about it?

Graham: Honestly, I think she's overreacting. I've had this mole for years, but she says it looks bigger and darker than it used to. I can't really see it myself on my back.

Candidate: It's completely understandable that it's hard to keep track of something you can't easily see yourself — that's exactly why it's good you've come in. When did you or your wife first notice it changing, and has it been a gradual change over months, or did it seem to happen more quickly?

Graham: She says it's been changing over the last couple of months — getting a bit bigger, and she mentioned the edges look uneven now, and there are a couple of different shades of brown and a bit of black in there too. It hasn't hurt at all, so I didn't think much of it.

Common Pitfall: *"Since it doesn't hurt and you've had it for years, it's very unlikely to be anything serious — let's just keep an eye on it for now." — Absence of pain is not reassuring for melanoma, which is frequently painless; dismissing a lesion with a reported change in size, border, and colour risks a dangerous delay in diagnosis.*

Model Response

Candidate: Thank you for describing that so clearly, Graham. I want to be upfront that a mole changing in size, developing an irregular border, and showing multiple colours are exactly the features we take seriously, regardless of whether it's painful — many suspicious moles don't hurt at all. Would it be alright if I take a look, and ask a few more questions about your general health?

Graham: Oh — I didn't realise painless meant nothing. Yes, go ahead.

Candidate: Have you noticed the mole itching, bleeding, or oozing at all? And more generally, have you had any fevers, noticed any new lumps anywhere, such as in your armpit or neck, lost your appetite, or lost any weight without trying?

Graham: Now that you mention it, it has felt a bit itchy the last few weeks, and I did notice a small lump under my left armpit about a week ago, which I just assumed was a swollen gland from a cold. No fevers, and my appetite and weight have been completely normal.

Examiner's Findings Card

- Lesion: 9mm irregular pigmented lesion on the upper back, asymmetrical with a notched border and variegated brown/black colouring.
- Left axilla: a firm, non-tender, mobile lymph node, approximately 1.5cm.
- No other skin lesions of concern noted on full skin examination.

Your Next Step: Explaining the Referral Pathway

Candidate: Graham, based on the size, irregular border, mixed colouring, recent change, and new itching, along with that lump under your arm, I need to refer you urgently under what's called a 2-Week Wait pathway to see a skin specialist. This means you'll be seen within 2 weeks specifically because these features raise the possibility of a skin cancer called melanoma, and it's important to get you assessed and, if needed, treated quickly. The specialist will likely want to remove the mole completely for it to be examined under a microscope, and will also want to properly assess that lump under your arm.

Graham: (visibly shaken) Melanoma... that sounds serious. Am I going to need chemotherapy or something drastic?

Candidate: I understand that's a frightening word to hear, and I don't want to get ahead of things before the specialist has properly assessed you — right now, we don't know for certain what this is, and many suspicious moles turn out not to be melanoma. What I can tell you is that when melanoma is caught early, as we're doing now by referring you promptly, it's often treated very successfully, usually with surgery to remove it completely. Further treatment, if it were ever needed, would depend entirely on what the specialist finds.

Graham: Okay... I suppose it's better to know for sure. Thank you for taking this seriously, even though I almost brushed it off.

Station Conclusion: An "Excellent Pass" Melanoma Profile

Why your final choices scored maximum marks:

- **Corrected the "Painless Means Safe" Misconception:** You explicitly addressed his assumption that painlessness was reassuring, which it is not for melanoma.
- **Systematic Lesion History:** You covered onset, change over time, border, colour, and size rather than a vague general description.
- **Complete Systemic Red-Flag Screen:** You uncovered a new axillary lump and itching that Graham had dismissed himself, which materially changed the urgency of referral.
- **Correct Pathway and Balanced Reassurance:** You applied the 2-Week Wait pathway correctly, and avoided both over-alarms him and giving false reassurance about a diagnosis that isn't yet confirmed.

Station 16 — Dermatology: Chronic Plaque Psoriasis

Psoriasis is a chronic, immune-mediated skin condition causing well-demarcated, erythematous plaques with silvery scale, classically over extensor surfaces. This station tests recognition of the classic pattern, screening for psoriatic arthritis, and delivering the correct stepwise UK management.

Key Clinical Features

- **Distribution:** classically the extensor surfaces (elbows, knees), scalp, and lower back, though it can affect any site.
- **Nail changes:** pitting, and onycholysis (the nail lifting away from the nail bed).
- **Koebner phenomenon:** new plaques developing at sites of skin trauma, such as scratches or scars.
- **Psoriatic arthritis:** affects up to around 30% of people with psoriasis — always screen for joint pain, stiffness, and swelling.

The Roleplay

Station: General Practice / Dermatology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Michael Doyle, 34 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Michael, 34, has developed itchy, scaly red patches on both elbows and knees over the last 3 months.

Your Task:

- Take a focused history and screen for joint involvement.
- Explain the diagnosis clearly.
- Outline the stepwise management plan.

(Michael rolls up his sleeves to show red, scaly patches on both elbows.)

Candidate: Hello Michael, I'm Dr [Your Name]. I can see you've got some patches on your elbows there — can you tell me more about them?

Michael: Yeah, they've been there for about three months now, on both elbows and my knees too. They're quite itchy, and they've got this silvery, flaky scale on top. My dad had something similar on his scalp for years, if that's relevant.

Candidate: That family history is really helpful, thank you. Have you noticed anything happening to your nails, like pitting or them lifting away from the nail bed? And have you had any pain, stiffness, or swelling in any of your joints?

Michael: Now you mention it, yes — a couple of my fingernails have these tiny dents in them I hadn't thought much about. No joint problems that I've noticed though, thankfully.

Common Pitfall: *"This looks like eczema — I'll prescribe a mild steroid cream and that should clear it up."* — Misses the classic well-demarcated, silvery-scaled plaques on extensor surfaces alongside nail pitting and a family history, which together point to psoriasis rather than eczema, and fails to screen for joint involvement.

Model Response

Candidate: Thank you for that detail, Michael. Based on the appearance of the patches — well-defined, red, with that silvery scale on your elbows and knees — plus the nail pitting and your family history, this looks like a condition called psoriasis. It's a chronic condition where the immune system causes skin cells to be produced much faster than normal, leading to these thickened, scaly patches. It's not contagious, and it's very manageable with the right treatment.

Michael: That's a relief to have a name for it. What's the treatment?

Candidate: We start with simple measures: a good emollient to keep the skin moisturised, alongside a topical steroid cream, often combined with a vitamin D-based cream called calcipotriol, applied to the patches. If this isn't enough to control things, the next step would be a specific light therapy called phototherapy, done at a specialist clinic. For more widespread or stubborn cases, there are tablet or injectable treatments that work on the immune system, but those are managed by a dermatologist.

Michael: That sounds manageable. Is there anything I should watch out for?

Candidate: Yes — because a proportion of people with psoriasis go on to develop joint symptoms, called psoriatic arthritis, please come back if you ever notice new joint pain, stiffness, or swelling, even if it's mild. Psoriasis can also have a real impact on mood and confidence, so please don't hesitate to mention it if it's affecting you emotionally, as there's support available for that too.

Station Conclusion: An "Excellent Pass" Psoriasis Profile

Why your final choices scored maximum marks:

- **Correctly Distinguished From Eczema:** You identified the classic extensor distribution, silvery scale, and nail pitting as pointing to psoriasis rather than the more common eczema.
- **Screened for Psoriatic Arthritis:** You explicitly asked about joint symptoms, recognising the significant overlap between psoriasis and joint disease.
- **Correct Stepwise Management:** You outlined emollients/topical steroids plus vitamin D analogue as first-line, phototherapy as second-line, and systemic treatment reserved for specialist-led severe disease.
- **Addressed Psychological Impact:** You proactively acknowledged the emotional burden of a visible skin condition, rather than treating it as a purely physical issue.

Station 17 — Dermatology: Lichen Planus

Lichen planus is an inflammatory condition affecting the skin and sometimes the mouth or genitals, classically described using the "6 Ps": Pruritic, Purple, Polygonal, Planar (flat-topped), Papules, and Plaques. This station tests recognition of the classic appearance, awareness of oral involvement and its small malignant potential, and appropriate management.

Key Clinical Features

- **Skin lesions:** itchy, purple/violaceous, flat-topped, polygonal papules, classically on the flexor surfaces of the wrists and forearms, and around the ankles.
- **Wickham's striae:** fine white lines visible on the surface of the papules — a classic diagnostic sign.
- **Oral involvement:** a white, lacy pattern on the inside of the cheeks; oral lichen planus carries a small increased risk of oral squamous cell carcinoma and needs monitoring.
- **Associations:** a recognised association with hepatitis C infection.

The Roleplay

Station: General Practice / Dermatology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Susan Reid, 52 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Susan, 52, has developed intensely itchy purple bumps on both wrists over the last month, and has also noticed white patches inside her cheeks.

Your Task:

- Take a focused history and examine the described lesions.
- Explain the diagnosis clearly.
- Arrange appropriate management and monitoring, including for oral involvement.

(Susan shows small, flat-topped, purplish bumps on the inside of both wrists.)

Candidate: Hello Susan, I'm Dr [Your Name]. I can see these bumps on your wrists — can you tell me more about them, and about the patches in your mouth you mentioned?

Susan: They came up about a month ago, and they're maddeningly itchy — small, flat, purple-ish bumps, mostly on my wrists. And yes, I noticed some white, lacy-looking patches on the inside of my cheeks around the same time, though they don't hurt, just feel a bit rough.

Candidate: Thank you for describing that so clearly. Have you started any new medications recently, or have you ever been tested for hepatitis?

Susan: No new medications that I can think of, and I've never been tested for hepatitis, no.

Common Pitfall: *"The mouth patches are probably just unrelated oral thrush — let's treat that separately with an antifungal and focus on the wrist rash." — Fails to recognise the combination of purple, flat-topped, itchy wrist papules with white lacy oral patches as a single unifying diagnosis, lichen planus, rather than two unrelated problems.*

Model Response

Candidate: Thank you, Susan. Looking at these bumps — they're purple, flat-topped, and quite itchy — along with the white, lacy patches inside your cheeks, this all fits with a single condition called lichen planus. It's an inflammatory skin condition that can affect the skin and the lining of the mouth together, which is exactly the pattern you're describing.

Susan: Oh, I didn't realise the mouth and skin things were connected. Is it serious?

Candidate: It's not dangerous in itself, and the skin patches often settle on their own over months to a couple of years, especially with treatment. For the itchy skin patches, I'll start you on a potent topical steroid cream. Because you also have patches inside your mouth, I'd like to refer you to an oral medicine or dental specialist, since oral lichen planus needs occasional monitoring — in a small number of people, this type of patch can very rarely develop early changes that need watching over time, so regular review is a sensible precaution rather than a cause for alarm.

Susan: That makes sense. Is there anything that caused this?

Candidate: In many cases we don't find a specific trigger, but it's occasionally linked to certain infections, particularly hepatitis C, so I'd like to do a simple blood test to check for that as part of being thorough, especially since you mentioned you'd never been tested before.

Susan: That's fine by me, better to check. Thank you for explaining how the mouth and skin are connected — that had been confusing me.

Station Conclusion: An "Excellent Pass" Lichen Planus Profile

Why your final choices scored maximum marks:

- **Unified Two Presentations Into One Diagnosis:** You recognised the skin and oral findings as a single condition rather than treating them as separate, unrelated problems.
- **Correct Description of Classic Features:** You identified the purple, flat-topped, itchy papules consistent with lichen planus.
- **Arranged Oral Monitoring:** You referred for specialist review of the oral lesions given their small malignant potential, rather than dismissing them as incidental.
- **Screened for Hepatitis C:** You proactively tested for a recognised association, rather than assuming the cause was unknown without checking.

SECTION: ENT EMERGENCIES

Station 18 — ENT: Acute Epistaxis Management

Epistaxis is a classic combined management and consultation station. It tests first-aid correction, coagulation risk assessment, and the stepwise NICE-aligned escalation ladder when initial measures fail.

The Roleplay

Station: Acute Management & Consultation Skills

Candidate Role: FY2 Doctor in the Emergency Department (A&E)

Patient/Roleplayer Role: Robert Green, 68 years old

Setting: Emergency Treatment Cubicle

The Prompt (read outside the cubicle):

Patient Details: Robert Green, 68, brought in by his wife with a severe nosebleed ongoing for 30 minutes.

Past Medical History: Hypertension and atrial fibrillation; takes the anticoagulant Rivaroxaban.

Your Task:

- Stabilise the patient using correct first-aid technique.
- Take a focused history to identify bleeding risk factors.
- Formulate the correct UK clinical escalation pathway.

(Robert is sitting upright, holding a blood-soaked tissue to his nose, visibly panicked, spluttering occasionally.)

Candidate: Hello Mr Green, my name is Dr [Your Name], one of the emergency doctors. I can see you have a severe nosebleed today — we're going to help stop this right away. Can you tell me exactly what you're doing to hold your nose right now?

Robert: Doctor... (cough)... it just won't stop. I've been pinching the hard bridge of my nose up here near my eyes for half an hour, but the blood is running down the back of my throat and making me choke. I'm so scared I'm losing too much blood.

Common Pitfall: Leaving the patient leaning backward, which lets blood drain down the throat and risks aspiration or choking, and telling them to pinch the bony bridge of the nose rather than the soft cartilaginous part, which does nothing to compress the bleeding vessels.

Model Response

Candidate: Mr Green, let's correct your position right away to keep you safe. Lean your head forward, not back, so the blood drains out through your mouth instead of down your throat. Now,

firmly pinch the soft part of your nose, right at the bottom, and breathe through your mouth. Hold that continuously for 10 to 15 minutes. While you hold that, I'm going to check your blood pressure and ask a few quick questions.

Robert: (leans forward, spits some blood into a bowl, pinches the soft lower part of his nose) Thank you... that already feels much better, it's stopped draining down my throat. I'm holding it tight — ask whatever you need.

Your Next Step: Focused History and Coagulation Risk Assessment

Candidate: Is the blood coming from one nostril or both? When exactly did you last take your Rivaroxaban, and have you had any recent head injury or trauma to your nose? Have you noticed any unusual bruising on your body, or bleeding when you brush your teeth or use the toilet?

Robert: It started in my right nostril, but now it feels like both sides. I took my Rivaroxaban at 8am this morning with breakfast. No head injury or trauma — I was just sitting watching television when it suddenly started. Come to think of it, I have noticed a few large, dark bruises on my shins and arms over the last week, though I don't remember bumping into anything. No blood in my stool or urine though.

Your Next Step: Escalating the Refractory Nosebleed

Despite 15 minutes of continuous compression, Robert's bleeding has failed to stop and is now bilateral — concerning given his anticoagulant use and the spontaneous bruising, which raises the possibility of over-anticoagulation. You must escalate along the NICE epistaxis pathway.

Candidate: Robert, since pinching hasn't fully worked, we need to locate the bleeding vessel. I'm going to examine inside your nose. If I see a clear bleeding point, I'll apply a silver nitrate cautery stick to seal it. If that fails, or the bleeding is too far back to see, I'll insert an inflatable nasal pack to apply pressure from the inside, and I'll check your clotting levels and full blood count given the bruising you've described and your Rivaroxaban use.

Robert: Okay, Doctor. That makes sense. Please go ahead and look inside. If you need the cautery or the internal packing, I'm ready — I just want the bleeding to stop.

Station Conclusion: An "Excellent Pass" Epistaxis Profile

Why your final choices scored maximum marks:

- **Flawless First-Aid Correction:** You corrected the patient to lean forward and pinch the soft cartilaginous part of the nose continuously for 10–15 minutes — forgetting to lean the patient forward is an immediate safety fail.
- **Accurate Clinical Escalation:** You followed the epistaxis escalation ladder correctly: first-aid compression → anterior rhinoscopy and cautery if a bleeding point is visible → anterior nasal packing if cautery fails or is unavailable.
- **Coagulation Awareness:** You recognised that his Rivaroxaban use combined with spontaneous systemic bruising raises concern for a bleeding tendency, warranting a full blood count and coagulation screen.

Station 19 — ENT Oncology: Persistent Hoarseness and the 2-Week Wait Pathway

Under UK NICE guideline NG12, unexplained hoarseness lasting more than 3 weeks in a patient aged 45 or older requires an urgent 2-Week Wait referral to an ENT specialist to rule out laryngeal cancer. This is a classic Consultation Skills and Breaking Bad News station.

The Roleplay

Station: Consultation Skills / Communication

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Arthur Pendleton, 58 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Arthur Pendleton, 58, has had a raspy, hoarse voice for about 6 weeks, which he believes is a stubborn cold.

Social History: Smokes 20 cigarettes a day, 30-year history.

Your Task:

- Take a focused history and explore red flags.
- Communicate the referral plan clearly and safely.

(Arthur is sitting down, clearing his throat frequently, speaking in a noticeably raspy voice.)

Candidate: Hello Mr Pendleton, I'm Dr [Your Name]. I see you've come in because of some changes to your voice. How can I help today?

Arthur: Hello Doctor. My voice has been completely shot for about six weeks now. I caught a nasty chest cold back in March, and while the cough cleared up, this croaky voice just won't go away. My wife insisted I get some strong antibiotics to finally shift it.

Candidate: I can understand how frustrating that must be, Arthur, especially when you're used to your voice sounding normal. Before we talk about the best way to manage this, do you mind if I ask a few quick questions to look at your throat symptoms a bit closer?

Arthur: Of course, Doctor. Go right ahead.

Your Next Step: Exploring Red Flags

Candidate: Have you had any pain or difficulty swallowing food or liquids? Have you noticed any unexplained weight loss, or a new lump anywhere in your neck? Do you ever get a persistent earache on one side, or any shortness of breath?

Arthur: Well, when I try to swallow solid food, like a piece of meat, it does feel like it's getting caught slightly at the back of my throat — not extremely painful, just uncomfortable. I haven't noticed any lumps, but my shirts have been feeling looser lately. I've actually dropped about half a stone over the last month without changing what I eat. No earaches or breathing problems though.

Your Next Step: Breaking the News and Explaining the 2-Week Wait

Arthur has presented with unexplained hoarseness lasting over 3 weeks, mild dysphagia, and unexplained weight loss, in a high-risk demographic (58-year-old heavy smoker). This meets criteria for an urgent 2-Week Wait referral.

Common Pitfall: *"This confirms my suspicion that you likely have laryngeal cancer. I'm referring you on a cancer pathway today."* — Delivers a premature, unconfirmed diagnosis as though it were certain, causing unnecessary distress before any investigation has taken place.

Model Response

Candidate: Thank you for being so detailed, Arthur. The facts that your hoarseness has lasted six weeks, you're having a bit of a hitch when swallowing, and you've lost some weight, mean we need to look closer at your throat. While this could still be a localised swelling or inflammation, we have a fast-track system in the UK called a Two-Week Wait referral. This means an ENT specialist will examine your throat with a small camera within the next 14 days — a precautionary step to rule out anything serious, including growths or tumours, so we can get you the right answer quickly.

Arthur: Right, okay... when you mention 'growths or tumours,' you mean cancer, don't you? I really thought it was just a bad cold. It's a bit scary to hear, but I appreciate you being direct. What happens next — do I just wait for them to call me?

Your Next Step: Finalising the Pathway and Safety-Netting

Candidate: It can mean cancer, yes, and it's entirely natural to feel worried hearing that. The hospital will contact you directly within the next few days to book your appointment. Because it's a fast-track system, please make sure you're available to attend and don't miss the call. If you haven't heard from them within 5 to 7 days, contact our clinic immediately so we can chase it up. Lastly, as a safety net — if you develop sudden breathing difficulties, a completely blocked airway, or start coughing up blood before your appointment, please go straight to A&E.

Arthur: I understand completely, Doctor. I'll make sure my phone is on, and if I don't hear anything in a week, I'll call reception. Thank you for being so thorough.

Station Conclusion: An "Excellent Pass" ENT Oncology Profile

Why your final choices scored maximum marks:

- **Compassionate Framing:** You introduced the possibility of a serious growth without a blunt, unconfirmed diagnosis, validating his shock while keeping him grounded.
- **Strict 2WW Protocol Adherence:** You executed the exact NICE NG12 pathway for unexplained hoarseness over 3 weeks in a patient over 45.
- **Robust Safety-Netting:** You gave a clear timeline expectation and a concrete deadline (5–7 days) to call back if the system fails, plus an emergency safety net for airway compromise or haemoptysis.

Station 20 — ENT: Malignant (Necrotising) Otitis Externa

This station tests your ability to differentiate a common, self-limiting ear infection from a life-threatening complication in a high-risk patient. The core differential is Acute Otitis Externa vs Malignant Otitis Externa.

Differentiating the Two Conditions

Feature	Acute Otitis Externa (Benign)	Malignant Otitis Externa (Emergency)
Pathology	Superficial inflammation of the external ear canal	Deep, necrotising infection spreading to the skull base
Pain	Moderate; worse moving the pinna/tragus	Severe, unremitting, deep-seated; worse at night
Patient profile	Swimmers, hearing aid or cotton-bud users	Elderly patients with diabetes or immunocompromise
Examination	Swollen, red ear canal; normal cranial nerves	Granulation tissue in the canal; cranial nerve palsies
Management	Topical antibiotic/steroid drops in the community	Immediate emergency admission for IV antibiotics

Malignant otitis externa requires urgent CT/MRI of the temporal bone and long-term IV Ciprofloxacin targeting *Pseudomonas aeruginosa*.

The Roleplay

Station: Consultation Skills / Management

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Margaret Foster, 72 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Margaret Foster, 72, severe throbbing pain in her right ear, progressively worse over 4 days.

Past Medical History: Type 2 Diabetes Mellitus for 15 years.

Your Task:

- Take a focused history and identify the likely diagnosis.
- Outline the immediate safe management plan.

(Margaret is holding a warm compress against her right ear, visibly grimacing.)

Candidate: Hello Mrs Foster, I'm Dr [Your Name]. I can see you're in a lot of discomfort with your ear. How can I help today?

Margaret: Oh, Doctor, it's just awful. This right ear is throbbing constantly, keeping me awake all night. It feels deep inside my head. Even brushing my hair past my ear makes me want to cry.

Candidate: I can see how exhausted and uncomfortable you are, Margaret, and I'm glad you came in. To understand exactly what's causing this deep pain, do you mind if I ask a few quick questions and check how you're feeling overall?

Margaret: Yes, please ask whatever you need, Doctor.

Your Next Step: Identifying the Risk Profile

Candidate: Have you noticed any fluid or pus draining from the ear, and does it have an unpleasant smell? How have your blood sugars been running recently? Have you noticed any new weakness or drooping on one side of your face, or any difficulty swallowing?

Margaret: Yes, there is some discharge — thick, yellowish-green, and it smells quite foul. My blood sugars have been all over the place this week, running much higher than usual, around 16 or 17. And... oh dear, now that you mention it, when I looked in the mirror this morning, the right side of my mouth looked a bit lazy. When I try to smile, it feels heavy and doesn't lift properly.

Your Next Step: Emergency Escalation

Margaret has the hallmark features of Malignant Otitis Externa: deep nocturnal pain, foul-smelling discharge, poorly controlled diabetes, and a facial (cranial nerve VII) palsy. This requires immediate same-day emergency admission under the ENT team.

Common Pitfall: *"This confirms you have a standard ear infection that's caused a bit of facial swelling. I'll prescribe topical antibiotic drops for home use, and your smile should return once it clears." — Fails to recognise a cranial nerve palsy as a red flag for skull base spread, and manages a surgical emergency in the community.*

Model Response

Candidate: Thank you for telling me that, Margaret. Hearing that your blood sugars are running high and that you've noticed new weakness on the right side of your face tells me this is serious. It means your ear infection has spread deeper into the tissues and is affecting the nerve that controls your smile — a condition called Malignant Otitis Externa. This is an emergency, so I'm contacting the hospital ENT team immediately to arrange same-day admission. They'll need to scan your head and start you on intravenous antibiotics to treat this safely and protect that nerve.

Margaret: Oh my goodness... I had no idea an earache could do that to my face. It sounds terrifying, but I can see why it needs urgent attention. I'm ready to go to the hospital. Please make the calls.

Station Conclusion: An "Excellent Pass" Malignant Otitis Externa Profile

Why your final choices scored maximum marks:

- **Recognised the High-Risk Profile:** You correctly treated severe, deep ear pain in an elderly patient with poorly controlled diabetes as Malignant Otitis Externa until proven otherwise.
- **Identified the Neurological Red Flag:** You specifically screened for and identified a facial nerve palsy, signalling spread to the skull base.
- **Flawless Emergency Escalation:** You arranged immediate same-day admission for imaging and IV anti-pseudomonal antibiotics, rather than treating in the community.

Station 21 — ENT: Vertigo Assessment (BPPV vs Central Causes)

Vertigo stations test structured history-taking to differentiate benign peripheral causes from a life-threatening central cause such as stroke.

The Vertigo Framework

- **Timing and duration:** seconds to minutes suggests BPPV; hours to days suggests vestibular neuritis or Ménière's disease.
- **Triggers:** sudden onset only with turning over in bed or looking up is classic for BPPV.
- **Auditory symptoms:** hearing loss, aural fullness, or tinnitus suggests Ménière's disease or labyrinthitis; their absence points toward BPPV or vestibular neuritis.
- **Central red flags — the "4 Ds":** Dysarthria, Dysphagia, Diplopia, and Dysmetria (unsteadiness/loss of coordination) — any of these raises concern for a cerebellar stroke and mandates urgent stroke pathway referral.

The Roleplay

Station: Consultation Skills / Diagnostic Management

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: David Collins, 45 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: David Collins, 45, experiencing episodes of "the room spinning" over the last three days.

Your Task:

- Take a focused history to determine the cause of his vertigo.
- Rule out serious central neurological causes.
- Explain the diagnosis and next steps clearly.

(David is sitting very still, staring straight ahead, anxious to turn his head.)

Candidate: Hello Mr Collins, I'm Dr [Your Name]. I understand you've been experiencing some dizziness and spinning sensations recently. How are you feeling right now?

David: Honestly, I'm terrified to even move my head. Every time I turn over in bed or look up at the top shelf, the room suddenly spins violently. It only lasts less than a minute, but it makes me feel so sick. I'm scared there's something wrong inside my brain.

Candidate: I can see how frightening this is, David, and keeping your head still is a very natural response. We're going to look into this thoroughly today. Do you mind if I ask a few quick questions about how long these episodes last and whether you have any other symptoms?

David: Thank you, Doctor. Yes, please ask whatever you need.

Your Next Step: Completing the Ear and Central Red-Flag Screen

Candidate: During these episodes, have you noticed any ringing in your ears, hearing loss, or a feeling of fullness? Have you had any slurred speech, double vision, or difficulty swallowing? When you try to walk, do you feel off-balance, clumsy, or like you're veering to one side?

David: No, my hearing is completely fine, no ringing or fullness at all. My speech is normal, no double vision, and I can swallow just fine. When I walk I feel cautious because I'm afraid of triggering the spinning, but my coordination is normal and I'm not veering to one side.

Your Next Step: Delivering the Diagnosis

David's profile — brief episodes triggered strictly by head movement, no auditory symptoms, no central red flags — points clearly to Benign Paroxysmal Positional Vertigo (BPPV).

Common Pitfall: *Prescribing a prolonged course of a vestibular sedative like Prochlorperazine and advising bed rest — this is not appropriate for pure BPPV, and vestibular sedatives are known to delay the brain's natural compensation process.*

Model Response

Candidate: Thank you for being so precise, David. Based on the fact that the spinning lasts under a minute, is triggered strictly by head movements, and you have none of the warning signs I screened for, this points to a common condition called Benign Paroxysmal Positional Vertigo, or BPPV. Inside our inner ear, we have tiny crystals that help with balance — sometimes a few slip out of place and trick your brain into thinking you're spinning. We can do a simple test today called the Dix-Hallpike test to confirm which side is affected, and if it matches, I can perform a specific sequence of head movements called the Epley manoeuvre right here in clinic, which often resolves the dizziness without any medication.

David: Wow, that makes total sense! I had no idea there were little crystals in my ears that could do that. It's a huge relief to know it's not a stroke or a tumour. Let's do the test right now.

Station Conclusion: An "Excellent Pass" Vertigo Profile

Why your final choices scored maximum marks:

- **Systematic Differential Diagnosis:** You isolated BPPV from its mimics using timing, absence of auditory symptoms, and movement-based triggers.
- **Flawless Central Red-Flag Screen:** You explicitly checked for the 4 Ds (dysarthria, dysphagia, diplopia, dysmetria) to rule out a cerebellar stroke — a mandatory step to pass a UK vertigo station.
- **Accurate Management:** You bypassed unnecessary vestibular sedatives and correctly reached for the Dix-Hallpike test and Epley manoeuvre as gold-standard management.

Station 22 — ENT: Differentiating Peripheral Vertigo — BPPV, Ménière's Disease, and Acoustic Neuroma

Building on Station 21 (which differentiates BPPV from dangerous central causes of vertigo), this station goes one level deeper into the peripheral causes themselves. BPPV, Ménière's disease, and acoustic neuroma can all present with dizziness and hearing-related symptoms, but they differ sharply in timing, associated features, and urgency — a classic differential diagnosis exercise.

The Three-Way Differential

Feature	BPPV	Ménière's Disease	Acoustic Neuroma
Duration of vertigo	Seconds, triggered by head movement	Minutes to several hours, spontaneous episodes	Gradual imbalance/unsteadiness rather than true rotational vertigo — the slow growth allows the brain to compensate
Hearing loss	None	Fluctuating, worsens during attacks, may partially recover between episodes	Progressive, unilateral, does not fluctuate — steadily worsens over months
Tinnitus	None	Present, often worse during attacks	Present, unilateral, persistent
Other features	Positive Dix-Hallpike test	Aural fullness/pressure in the affected ear	Possible facial numbness or reduced corneal reflex if the tumour is large enough to affect the trigeminal nerve
Key mechanism	Displaced otoliths in the semicircular canals	Excess endolymph fluid (endolymphatic hydrops)	Benign tumour of the vestibular portion of the 8th cranial nerve (vestibular schwannoma)

Bilateral acoustic neuromas are pathognomonic for Neurofibromatosis type 2 (NF2) and should prompt genetic assessment, particularly in a younger patient.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Alan Whitmore, 61 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Alan, 61, has booked an appointment about hearing problems and unsteadiness that have been building gradually over the past 8 months.

Your Task:

- Take a focused history to differentiate the possible peripheral causes.
- Identify red flags suggesting a more serious underlying cause.
- Explain the diagnosis and the correct referral pathway.

(Alan speaks slightly louder than necessary and tilts his head as if favouring one ear.)

Candidate: Hello Mr Whitmore, I'm Dr [Your Name]. I understand you've been having some hearing and balance difficulties — can you tell me more?

Alan: Yes, Doctor. Over the last 8 months or so, my hearing in my left ear has been gradually getting worse — it's not sudden, just slowly creeping up on me. I also have this constant ringing in that ear, and lately I've just felt a bit off-balance and clumsy, especially walking in the dark or on uneven ground.

Candidate: Thank you for describing that so clearly. Do you ever get a spinning sensation, where the room feels like it's actually rotating around you, particularly with sudden head movements? And does your hearing come and go, or has it been a steady decline?

Alan: No, nothing like proper spinning or vertigo — it's more just a general unsteadiness the whole time. And no, my hearing hasn't come and gone at all, it's just been getting a little worse, steadily, month by month.

Common Pitfall: *"This sounds like classic Ménière's disease, given the hearing loss, tinnitus, and balance problems — I'll start you on betahistine and review in a few months." — Misses the critical distinguishing features: Ménière's causes fluctuating hearing loss and episodic true vertigo, not a steady, progressive decline with constant unsteadiness — this pattern instead points towards a more concerning diagnosis requiring urgent imaging.*

Model Response

Candidate: Thank you for that detail, Alan — it's really helpful. Have you noticed any numbness or odd sensation on one side of your face, or anyone mentioning your face looks like it's drooping slightly on one side?

Alan: Now that you mention it, my wife did comment last week that the left side of my face seemed a little numb when she touched my cheek, though I hadn't thought much of it.

Candidate: Thank you for telling me that — that's an important piece of the picture. The combination of steadily progressive, one-sided hearing loss, constant tinnitus, gradual unsteadiness rather than true spinning vertigo, and now some facial numbness, raises concern for a growth affecting the hearing and balance nerve, called an acoustic neuroma. It's important to say this is usually a benign, slow-growing tumour, not a cancer that spreads, but it does need proper assessment.

Alan: A tumour? That's frightening to hear. What happens now?

Candidate: I understand that's a lot to take in. I'm arranging an urgent referral for a specific type of MRI scan with contrast, which is the best way to look directly at that nerve and confirm whether this is what's going on. If it is confirmed, the specialist team will look at exactly how big it is and how quickly it's growing — many people are safely monitored with repeat scans over time if the tumour is small and slow-growing, while treatment options like focused radiotherapy or surgery are used for larger or faster-growing ones. I'll make sure this referral goes through urgently given your symptoms.

Alan: Thank you for taking this seriously and explaining it so clearly. I was worried you'd just tell me it was old age catching up with me.

Station Conclusion: An "Excellent Pass" Peripheral Vertigo Differential Profile

Why your final choices scored maximum marks:

- **Correctly Excluded BPPV and Ménière's:** you recognised the absence of true rotational vertigo and fluctuating hearing loss as evidence against both of the more benign, common diagnoses.
- **Identified the Red-Flag Combination:** progressive unilateral hearing loss, constant tinnitus, gradual imbalance, and facial numbness together pointed to a retrocochlear cause rather than a peripheral vestibular one.
- **Correct Investigation:** you requested an urgent contrast-enhanced MRI, the gold-standard test for acoustic neuroma, rather than a hearing test alone.
- **Balanced, Honest Communication:** you named the possibility of a tumour directly while immediately clarifying it is typically benign and slow-growing, avoiding both unnecessary alarm and dangerous minimisation.
- **Outlined the Full Management Spectrum:** you mentioned watchful waiting with surveillance imaging, radiosurgery, and surgical options, reflecting genuine specialist decision-making rather than assuming immediate surgery is inevitable.

Station 23 — ENT: Tonsillitis vs Quinsy (Peritonsillar Abscess)

This station tests your ability to recognise when a common, self-limiting throat infection has progressed into a deep-tissue abscess threatening the airway.

Differentiating the Two Conditions

- **Acute tonsillitis:** bilateral sore throat, swollen red tonsils with patchy white exudate, centred uvula. Managed with simple analgesia or, if the FeverPAIN score is 4–5, a 10-day course of oral phenoxymethylpenicillin (penicillin V).
- **Quinsy (peritonsillar abscess):** severe, unilateral throat pain, trismus (difficulty opening the mouth), uvular deviation away from the affected side, and a "hot potato" muffled voice. Requires immediate same-day emergency referral for IV fluids, IV antibiotics (e.g. benzylpenicillin), IV dexamethasone, and needle aspiration or incision and drainage.

Absolute airway red flags to screen for immediately: stridor, and drooling or inability to swallow saliva (indicating mechanical obstruction).

The Roleplay

Station: Acute Emergency / Consultation Skills

Candidate Role: FY2 Doctor in an Urgent Care Centre

Patient/Roleplayer Role: Liam Finch, 24 years old

Setting: Examination Room

The Prompt (read outside the cubicle):

Patient Details: Liam Finch, 24, severe sore throat for 5 days, dramatically worse over the last 24 hours, struggling to talk.

Your Task:

- Take an emergency focused history.
- Screen for airway red flags.
- Outline the immediate safe management pathway.

(Liam is sitting forward, holding a cardboard bowl, visibly distressed and drooling into it.)

Candidate: Hello Liam, my name is Dr [Your Name], one of the urgent care doctors. I can see you're in severe pain with your throat and finding it hard to talk. We're going to help you right away. Can you tell me how your breathing feels right now?

Liam: (speaking in a thick, muffled voice, barely moving his jaw) Doctor... it hurts... so bad. I can't... swallow my spit at all... that's why I'm drooling. My breathing feels... heavy, but I'm not gasping yet. It feels like a massive lump is blocking the right side of my throat.

Common Pitfall: *"This is a sign of a peritonsillar abscess. I'll write you a prescription for oral antibiotics and steroid pills to take at home for the next week."* — *Manages an airway-threatening*

surgical emergency as an outpatient, missing that inability to swallow saliva is an absolute red flag.

Model Response

Candidate: Thank you for answering, Liam. Seeing that you can't swallow your own saliva and are speaking with a muffled voice tells me your throat swelling is severe and threatening your airway. I'm going to examine the back of your mouth very gently, and then arrange an immediate same-day emergency admission to the hospital ENT team. You need to be seen by a specialist straight away for intravenous medicine to safely bring this swelling down.

(On gentle examination: severe swelling of the right tonsil pushing the uvula to the left, clear trismus, airway currently patent but vulnerable due to pooling saliva.)

Liam: (muffled) Thank you, Doctor... I'm so relieved you're sending me to the specialists. It feels like it's completely closing up. Please make the calls right away.

Station Conclusion: An "Excellent Pass" Quinsy Profile

Why your final choices scored maximum marks:

- **Identified Absolute Airway Red Flags:** You recognised inability to swallow saliva and a muffled voice as signs of mechanical airway obstruction and an impending emergency.
- **Recognised Quinsy:** You isolated the key features — unilateral swelling, trismus, and uvular deviation away from the affected side.
- **Correct Escalation:** You arranged immediate same-day emergency ENT admission for IV fluids, antibiotics, steroids, and drainage, rather than unsafe community management.
- **Maintained Patient Safety:** You prioritised stabilising the patient and arranging specialist transfer over prolonged history-taking, showing strong triage instincts.

SECTION: MENTAL HEALTH & EATING DISORDERS

Station 24 — Mental Health: Bulimia Nervosa

An eating disorder station involving bulimia nervosa typically appears in a GP or mental health outpatient setting. The patient often presents under the guise of an unrelated physical complaint — sore throats, dental problems, or irregular periods — rather than disclosing the eating disorder directly. The exam evaluates your ability to pick up on subtle physical cues, take a sensitive and non-judgemental history, screen for physiological risk, and map out the correct NHS pathway.

Clinical Identification & Physical Cues

Patients with bulimia nervosa often maintain a normal or near-normal BMI, making the condition easy to miss on weight alone. Look out for:

- **Russell's sign:** calluses or scars on the knuckles from repeated self-induced vomiting against the upper teeth.
- **Dental erosion:** loss of tooth enamel and decay from chronic acid exposure.
- **Parotid gland swelling:** swollen salivary glands below the ears ("chipmunk cheeks") from repeated vomiting.

Red Flags Requiring Escalation

- **Severe hypokalaemia:** purging-related potassium loss can cause life-threatening cardiac arrhythmias — ask about palpitations, chest pain, or profound muscle weakness.
- **Postural dizziness or fainting:** suggests dehydration or a cardiac complication.
- **Haematemesis:** vomiting blood can indicate a Mallory-Weiss tear of the oesophagus from forceful vomiting.

The Roleplay

Station: General Practice / Mental Health Consultation

Candidate Role: FY2 Doctor in a GP Surgery

Patient/Roleplayer Role: Chloe Davies, 22-year-old university student

Setting: GP Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Chloe Davies, 22, has booked an appointment because of recurrent, severe sore throats and a racing heartbeat over the last month.

Your Task:

- Take a focused history to explore Chloe's physical symptoms and unmask any underlying eating pattern.
- Maintain a compassionate, non-judgemental environment.
- Screen for physiological red flags (cardiac risk and electrolyte imbalance).
- Formulate a safe NHS management and referral pathway.

(Chloe is sitting quietly, looking anxious. She wears a thick scarf and keeps adjusting her sleeves to hide her hands.)

Candidate: Hello Ms Davies, my name is Dr [Your Name], one of the doctors here today. I see from our notes you've come in because you've been struggling with a sore throat and a fast heartbeat lately. How can I help you most today?

Chloe: Hi Doctor... yes, my throat has just been constantly raw and burning for weeks, and sometimes out of nowhere my heart starts pounding like I've run a marathon, even when I'm just sitting watching TV. To be honest, I'm just really stressed with exams coming up, and I think it's anxiety. Can you just give me some strong throat lozenges or beta-blockers to stop the racing heart so I can get back to studying?

Common Pitfall: *"Anxiety doesn't cause a raw, burning throat for weeks. Burning suggests stomach acid. Are you forcing yourself to vomit to lose weight? Please show me your knuckles right now." — Jumps straight to confrontation before rapport is built, likely to shut the patient down entirely.*

Model Response

Candidate: I can hear how stressful the exam period is for you, Chloe, and it's completely understandable to feel overwhelmed on top of a racing heartbeat. Because a burning throat and a pounding heart can sometimes be linked to our overall physical health or how our body handles stress, do you mind if I ask a little more about your daily routine? For example, how have your appetite and your relationship with food been lately?

Chloe: (looking down, flushing red, wrapping her arms around herself) I... I didn't think you'd ask about that. My relationship with food is a complete disaster right now. The exam stress has made me feel like my whole life is spinning out of control, and food is the only thing I focus on. A few times a week I get this overwhelming urge to eat — I'll buy a large amount of biscuits, cakes, and crisps and lock myself in my room, eating until I'm physically in pain. Then the guilt and disgust hit me so hard that I panic, go to the bathroom, and make myself throw it all up. That's... that's probably why my throat is burning, isn't it? I'm so ashamed, Doctor. Please don't tell my parents.

Your Next Step: Physiological Safety Screen and Confidentiality

Candidate: Thank you for trusting me with that, Chloe — that took real courage, and I want you to know there's no judgement here at all. First, because you're an adult, what you've told me stays completely confidential between us, unless I ever believed your life was in immediate danger. Now, because vomiting can affect your body's chemistry, I need to ask a few safety questions: have you had any dizziness or fainting, or any chest pain alongside the racing heart? And besides vomiting, are you using anything else, like laxative tablets, to control your weight?

Chloe: That means the world to me, thank you. I haven't used any laxatives, luckily. No severe tummy pain or confusion either. But I have been feeling incredibly weak and fatigued, and I get dizzy and lightheaded when I stand up too quickly from my desk. The heart racing happens two or three times a day now, especially right after I throw up. It's a scary feeling.

Your Next Step: The NHS Management Plan

Chloe's dizziness on standing and post-vomiting palpitations raise real concern for hypokalaemia, which can destabilise the heart's electrical rhythm. This moves the station from a mental health conversation into an acute safety assessment requiring same-day investigation.

Candidate: Chloe, because of the racing heart and dizziness you've described, I want to do an ECG and some blood tests today — specifically checking your potassium and kidney function — just to make sure your heart is safe. I'm also going to refer you to our local Specialist Eating Disorder Service, who offer a talking therapy called CBT-ED, specifically designed for bulimia, alongside dietitian support. In the meantime, one practical tip: try not to brush your teeth immediately after being sick, since that rubs the stomach acid further into your enamel — rinsing with water or a fluoride mouthwash instead protects your teeth much better. And please go straight to A&E if you ever faint, get chest pain, or can't keep fluids down.

Chloe: Thank you so much, Doctor. I had no idea about the tooth-brushing thing, so I'm really glad you told me. I agree to the blood tests and the heart tracing today — if it checks that my heart is safe, that makes me feel a lot better. I really appreciate your help.

Station Conclusion: An "Excellent Pass" Bulimia Profile

Why your final choices scored maximum marks:

- **Unmasked the Diagnosis Gently:** You linked her physical complaints to an underlying eating pattern without confrontation, allowing her to disclose safely.
- **Confidentiality Addressed Proactively:** You clarified confidentiality before she had to ask, which builds trust with a deeply ashamed patient.
- **Correctly Prioritised the Physical Safety Screen:** You investigated for hypokalaemia-related cardiac risk given her palpitations and dizziness, rather than treating this as a purely psychological consultation.
- **Practical Harm-Reduction Advice:** The dental tip (not brushing teeth immediately after vomiting) is a genuine, guideline-supported harm-reduction point that shows real clinical depth.
- **Appropriate Referral Pathway:** You referred to the Specialist Eating Disorder Service for CBT-ED rather than managing this in isolation in general practice.

Station 25 — Mental Health: Anorexia Nervosa and the MARSIPAN Emergency Threshold

An anorexia nervosa station often features an anxious parent bringing in a teenager, or a young adult presenting with secondary complications such as amenorrhoea, dizziness, or fatigue. This station tests your ability to build trust with a defensive patient, navigate family dynamics, spot high-risk physical red flags, and recognise when a patient must be escalated from a routine outpatient referral to same-day medical admission.

The SCOFF Screening Tool

- **S (Sick):** do you make yourself sick because you feel uncomfortably full?
- **C (Control):** do you worry you have lost control over how much you eat?
- **O (One stone):** have you recently lost more than one stone (roughly 6.4 kg) in a 3-month period?
- **F (Fat):** do you believe yourself to be fat when others say you are too thin?
- **F (Food):** would you say that food dominates your life?

High-Risk Physical Findings (MARSIPAN Risk-Assessment Guidance)

Anorexia nervosa can cause severe cardiovascular and metabolic collapse. UK MARSIPAN guidance (Management of Really Sick Patients with Anorexia Nervosa) uses banded risk parameters rather than a single hard cutoff, but findings commonly flagged as requiring same-day medical admission rather than routine outpatient referral include:

- **Marked bradycardia:** heart rate under 40 beats per minute, carrying a real risk of cardiac arrhythmia.
- **Marked hypotension:** blood pressure under 90/60 mmHg.
- **Hypothermia:** core temperature under 35°C.
- **A significant postural drop:** a fall in blood pressure of more than 20 mmHg on standing.

The Roleplay

Station: General Practice / Paediatric & Adolescent Mental Health

Candidate Role: FY2 Doctor in a GP Surgery

Patient/Roleplayer Role: Sophie Miller, 17, brought in by her father David; Sophie sits silently

Setting: GP Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Sophie Miller, 17, brought in by her father because she has missed her periods for 4 months and has become increasingly isolated and weak.

Your Task:

- Manage the dynamic between an anxious father and a defensive teenager.
- Take a focused history exploring diet, weight perception, and physical red flags.
- Propose the required physical examination and investigations.
- Outline a safe NHS management pathway.

(David sits on the edge of his seat, intensely worried. Sophie sits beside him in an oversized jumper, arms crossed, eyes fixed on the floor, visibly angry about being there.)

Candidate: Hello Mr Miller, and hello Sophie. My name is Dr [Your Name], one of the doctors here today. I see you've come in to talk about your health and a few changes you've noticed lately, Sophie. How can I best help you both today?

David: Thank you, Doctor. I'm out of my mind with worry. Sophie hasn't had her period in four months. She's constantly exhausted, freezing cold even with the heating up, and barely touches her food — she just pushes it around her plate. She used to be so bubbly, but now she locks herself in her room exercising for hours. Look at her, Doctor, she's wasting away in front of me!

Sophie: (snapping, without looking up) Dad, stop exaggerating! There's literally nothing wrong with me. My periods are just irregular because of exam stress, and I eat perfectly fine. You are totally suffocating me!

Common Pitfall: *"David, you're right to be worried — missing four months of periods and over-exercising are classic signs of a severe psychiatric eating disorder. Sophie, you're clearly in denial. I'm ordering an immediate weight check right now." — Diagnoses aloud in front of a defensive teenager and sides entirely with the parent, likely to entrench her denial further.*

Model Response

Candidate: I can see how much you love your daughter and how worried you are, David, and it's completely understandable to want to make sure she's okay. Sophie, I can also understand how frustrating it must feel to be put on the spot like this, especially during a stressful exam period. My priority today is simply making sure you're feeling healthy and supported. David, would you mind if I chat with Sophie directly for a moment about how she's been feeling physically?

David: Of course, Doctor. I'll stay quiet. I just want her to get the right help.

Sophie: (looking up for the first time, posture softening slightly) Thanks, Doctor. It's nice not to be yelled at for a second. Honestly, I am tired all the time, and I do feel freezing cold, even in this jumper. But everyone's overreacting about my weight — I've lost a bit over the last few months, sure, but that's just from eating healthier and staying fit for exams. If anything, I still feel like I have a lot of fat around my thighs and stomach that I need to shift. I don't see why that's a medical emergency.

Your Next Step: The Physiological Safety Screen

Candidate: Thank you for sharing that, Sophie — it helps me understand things from your side. Because losing weight quickly can put strain on the body, have you noticed any sudden dizziness when standing, any fainting spells, or your heart fluttering or racing? And are you using anything else to control your weight, like making yourself sick or laxative tablets, or is it mainly the dieting and exercise? To make sure your body is handling this safely, I'd like to check your heart rate, blood pressure, and weight today — would that be okay?

Sophie: No, I've never made myself sick or used laxatives — that sounds awful. It's really just strict dieting and my running. But yes... I do get really dizzy if I stand up too quickly, and

sometimes my vision goes completely black for a few seconds and I have to grab the wall. My heart thumps quite heavily sometimes too. If you just need to check my blood pressure and heart rate, I'm okay with that.

Examiner's Findings Card

- Vitals: heart rate 38 bpm (marked bradycardia); blood pressure 84/52 mmHg (marked hypotension); temperature 34.8°C (hypothermia).
- BMI 14.2 kg/m² (severely underweight).
- Fine, downy hair (lanugo) visible on the forearms; muscle wasting around the shoulder girdle and thighs.
- Cardiovascular: regular rhythm but profoundly slow; clear heart sounds, no murmurs.

This has now crossed a critical patient-safety threshold. Sophie's vitals meet MARSIPAN criteria for immediate medical risk. She cannot be managed with a routine outpatient referral and requires same-day admission for cardiac monitoring, gentle warming, and carefully supervised refeeding to prevent refeeding syndrome.

Common Pitfall: *"The results are terrible — Sophie's heart rate is 38 and she's practically freezing to death. She's going to have a cardiac arrest and die at any moment. I'm calling an emergency ambulance and forcing her into hospital under the Mental Health Act." — Needlessly alarmist, and jumps to compulsory detention when the family is already cooperative — the Mental Health Act is not the correct or proportionate tool here.*

Model Response

Candidate: Thank you for letting me do those checks, Sophie. David, I have the results, and because Sophie's safety is my absolute priority, we need to take action today. Sophie, your body is working incredibly hard right now, but your heart rate is running very slow at 38 beats per minute, and your temperature is quite low. Because of this, it isn't safe for you to go home today — I'm going to arrange an immediate admission to our hospital's adolescent unit so the specialist team can keep your heart safely monitored, keep you warm, and support you through this. We're going to do this together to keep you safe. How do you both feel about this?

David: (gasping, hand over his mouth) Oh my god... 38 beats? Yes, absolutely, Doctor. Do whatever you have to do to keep her safe. I'll drive her straight over to the unit right now.

Sophie: (visibly shocked) My heart is really beating that slowly? I didn't think it was that dangerous. If you say it's not safe for me to go home, then I'll go to the hospital. I don't want anything bad to happen to my heart.

Your Next Step: Correcting the Transport Plan and Outlining the MDT

Candidate: David, because Sophie's heart is under significant strain right now, it wouldn't be safe for you to drive her over yourselves — there's a real risk of a fainting episode or a heart rhythm problem on the way. I'll arrange a direct medical transfer so she's monitored safely the whole way there. Once at the hospital, she'll be cared for by a team including a paediatrician, the CAMHS mental health team, and a dietitian, who'll work together to slowly and safely reintroduce nutrition and prevent any sudden chemical imbalances in her blood. While we wait for the transfer, Sophie, please tell me straight away if you feel any chest pain, worsening dizziness, or difficulty breathing.

Sophie: Okay... thank you for explaining it all so calmly. I was really scared, but I understand why this needs to happen now.

Station Conclusion: An "Excellent Pass" Anorexia Profile

Why your final choices scored maximum marks:

- **Skilful Family Dynamic Management:** You validated both the father's concern and the teenager's frustration, then created space to speak with Sophie directly, which earned her cooperation.
- **Recognised the MARSIPAN Emergency Threshold:** You correctly identified that her vitals (HR 38, BP 84/52, temperature 34.8°C) crossed the line from routine referral into a same-day admission emergency — sending a patient home with these findings is a critical safety failure in the exam.
- **Corrected the Unsafe Transport Plan:** You identified that private car transport was unsafe given her bradycardia and arranged a monitored medical transfer instead.
- **Delivered the News Calmly and Proportionately:** You avoided needless alarmism or invoking compulsory detention when the family was already cooperating, keeping both patient and parent onside.
- **Outlined the Full MDT and Refeeding Safety Plan:** You mentioned paediatrics, CAMHS, and dietetics working together, and flagged the need for careful, gradual refeeding to avoid refeeding syndrome.

Station 26 — Mental Health: Postnatal Depression

Psychiatry stations in PLAB 2 heavily emphasise risk stratification, distinguishing normal emotional adjustment from severe mental illness, and assessing patient safety. Maternal mental health stations test your ability to differentiate normal postpartum emotional changes from a genuine psychiatric emergency.

Differentiating Postpartum Psychiatric Presentations

Condition	Timing	Key Clinical Signs	UK Management
"Baby blues"	Onset 3–5 days postpartum; resolves within 10 days	Mild tearfulness, irritability, anxiety; fully functional	Reassurance and self-monitoring; a normal emotional transition
Postnatal Depression	Onset within 1–6 months postpartum	Persistent low mood, anhedonia, anxiety, feelings of inadequacy as a mother	GP review, talking therapies (CBT), or sertraline (safe in breastfeeding)
Postpartum Psychosis	Sudden onset, usually within 1–2 weeks postpartum	Delusions, hallucinations, severe insomnia, mania, confusion	Psychiatric emergency — immediate same-day admission to a Mother and Baby Unit

The Roleplay

Station: Consultation Skills / Mental Health Risk Assessment

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Amy Clarke, 26 years old

Setting: GP Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Amy Clarke, 26, gave birth to her first baby boy 6 weeks ago. Her health visitor booked this appointment after noticing Amy seemed very low during a routine home visit.

Your Task:

- Take a focused history to evaluate her mood and determine the likely diagnosis.
- Perform a comprehensive risk assessment (self-harm, suicide, and infant safety).
- Formulate a safe, supportive management plan.

(Amy is hunched over in her chair, staring blankly at the floor, clutching her cardigan. She looks exhausted, with dark circles under her eyes, and doesn't look up as you enter.)

Candidate: Hello Amy. My name is Dr [Your Name], one of the doctors here today. I understand your health visitor suggested you come in for a chat. I can see you look incredibly tired and overwhelmed right now. Please know this is a completely safe, confidential space. How have things been since the baby arrived?

Amy: (flat, quiet, eyes on the floor) Hello Doctor... it's just... everything is awful. I thought having a baby would make me happy, but I just feel completely numb. I cry every single hour. My husband tries to help, but I feel like an absolute failure of a mother. I can't do anything right for my little boy. I just want to disappear.

Common Pitfall: *"Feeling sad after a baby is just the 'baby blues' and it happens to everyone. You aren't a failure, you just need more sleep. I'll prescribe some strong sleeping tablets." — Dismisses a persistent, 6-week presentation as a normal transient phase, and misses screening for suicidal ideation entirely.*

Model Response

Candidate: Thank you for being so brave and telling me exactly how you feel, Amy. Becoming a mother is a massive adjustment, and carrying this heavy numbness and crying so much is incredibly hard. You are absolutely not a failure, and I'm glad you came in today. To help me understand how we can best lift this weight, do you mind if I ask a few gentle questions about your day-to-day routine and your thoughts?

Amy: (sobbing) I have thought about just driving my car into a wall when I go to the supermarket. I don't want to die, but I just want the pain to stop. I haven't bought anything or made a final plan, because the thought of leaving my husband stops me. But as for my baby boy... oh God, no, never. I love him so much, Doctor. I would never, ever hurt him. I just feel so inadequate, like he'd be better off with a different mother.

Your Next Step: Shared Management Plan and Safety-Netting

Amy's history confirms Postnatal Depression. Her risk assessment shows suicidal ideation with strong protective factors (no final plan, her husband is a protective anchor) and no thoughts of harming the baby — placing her at moderate risk, manageable in the community with intensive support rather than compulsory admission.

Common Pitfall: *"Because you have thoughts of driving your car into a wall, I'm classifying you as a high-risk psychiatric emergency and sectioning you today." — Overreacts to disclosed ideation that carries clear protective factors, and would likely damage trust and deter future honesty.*

Model Response

Candidate: Thank you for being so deeply honest with me, Amy. It takes immense strength to share those thoughts. I want to reassure you that you're a loving mother dealing with a very real, treatable condition called Postnatal Depression. We're going to build a solid safety net around you today. First, I'll contact our Perinatal Mental Health team for intensive home support. Second, we can look at talking therapy like CBT, and I'd like to prescribe a tablet called Sertraline, which is highly effective and completely safe while breastfeeding. Most importantly, I want you to promise me that if those thoughts become overwhelming before your next appointment, you'll call the Crisis Team on the number I'll give you, call 111, or go straight to A&E with your husband.

Amy: (tears of relief) Thank you, Doctor... truly. Hearing that this is a real medical condition and that I'm not a failure means everything. I was terrified you'd take my baby away. I'm completely

willing to try the Sertraline, and I'd love to talk to the specialist team. I promise, if those thoughts get too heavy, we'll use that crisis number or go straight to hospital.

Station Conclusion: An "Excellent Pass" Postnatal Depression Profile

Why your final choices scored maximum marks:

- **Direct, Compassionate Risk Screening:** You asked directly and clearly about suicidal intent, plans, and infant safety — screening the safety of a dependent child is a non-negotiable milestone.
- **Accurate Risk Stratification:** You recognised strong protective factors and no intent or plan, correctly avoiding an unnecessary and trust-damaging compulsory admission.
- **Correct Perinatal Pharmacotherapy:** You selected Sertraline, favoured in the postnatal period for its low transfer into breast milk.
- **Multi-Disciplinary Safety-Netting:** You linked her to the Perinatal Mental Health Team and gave a clear crisis-escalation framework (Crisis line, 111, or A&E with her husband).

Station 27 — Mental Health: Major Depressive Disorder with Suicidal Ideation

A depression consultation with active suicidal ideation is a safety-critical station. Your task is to explore symptoms thoroughly, conduct a multi-layered suicide risk assessment without hesitation, and formulate a management plan tailored to the patient's risk category. Marking places heavy weight on your ability to ask direct, explicit questions about self-harm and suicide while maintaining clinical empathy.

The Roleplay

Station: Consultation Skills / Mental Health Risk Assessment

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Mark Davies, 41 years old

Setting: GP Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Mark Davies, 41, has booked an appointment. The triage note states he has been feeling increasingly low over the last month and feels he "cannot cope anymore."

Your Task:

- Take a focused history to evaluate his mood and biological features of depression.
- Perform a thorough risk assessment regarding self-harm and suicide.
- Establish a safe management plan based on his risk stratification.

(Mark sits with elbows on knees, staring at his clasped hands. He looks unkempt and speaks in a low, flat, monotone voice.)

Candidate: Hello Mr Davies. My name is Dr [Your Name], one of the clinic doctors today. I can see you've been feeling quite low recently and finding things too much to cope with. I want to reassure you that you're in a completely safe, confidential environment. Can you tell me a bit about what's been happening?

Mark: (looks up, eyes blank) Thanks, Doctor. I don't even know where to begin. For the last five or six weeks, the world has just gone completely grey. I wake up at 3am every morning with this heavy dread in my chest and can't get back to sleep. I've stopped going to work, I'm barely eating, and I don't see my friends anymore. It just feels like there's absolutely no way out of this dark hole.

Candidate: Thank you for opening up and sharing that so clearly, Mark. It sounds incredibly exhausting to live with that constant heavy dread and feel trapped in a dark hole. You've done entirely the right thing coming in today, and we're going to work through this together. To help me understand how this is affecting you and your safety, do you mind if I ask a few gentle questions?

Mark: Thank you, Doctor. I appreciate you listening. Go ahead and ask whatever you need. I'll be honest with you.

Your Next Step: Detailed Suicide Risk Assessment

You've identified core biological symptoms of severe depression (early morning waking, anhedonia, poor appetite). Because Mark stated it feels like there is "absolutely no way out," you must directly screen for suicidal thoughts, intent, active planning, and access to lethal means.

Candidate: When you feel like there's no way out, Mark, have you had thoughts of ending your life or harming yourself? If so, have you gone as far as making any specific plans or preparations? And do you have access to anything at home that you've considered using to carry those out?

Mark: (voice dropping to a low whisper) Yes, Doctor... I have. I've been thinking about ending it for the past two weeks. It feels like everyone would just be better off without me. Last night, it got really bad — I went into the garage and put a rope over the rafter. I stood there for a long time just looking at it. The only reason I didn't go through with it was my sister called out of nowhere and it shocked me out of it. The rope is still hanging out there right now. I'm terrified of going back home tonight because I don't trust myself if it gets dark and lonely again.

Your Next Step: Crisis Management and Immediate Escalation

This reveals an absolute psychiatric emergency: active, severe suicidal intent, a specific and highly lethal prepared plan, immediate access to means, and a recent near-miss interrupted only by chance. Mark cannot be sent home or managed with routine outpatient care.

Common Pitfall: *"Since you have a rope ready, I'll recommend a strong antidepressant. Go home, start the medication, and promise me you'll take the rope down yourself tomorrow morning. I'll call you in a week."* — Leaves an actively high-risk patient alone with means readily available, an automatic critical safety failure.

Model Response

Candidate: Thank you for being so deeply honest with me, Mark. Sharing that takes immense courage, and I want you to hear me clearly: you are not a burden, you're dealing with a severe illness, and you are not going to face this alone tonight. Because your safety is my absolute priority and you feel unsafe going home, I'm not going to let you leave this clinic by yourself. With your permission, I'm going to contact our Mental Health Crisis Team right now for an immediate, same-day emergency assessment. I also want us to call your sister together so she can come and sit with us, and we'll arrange for someone to safely clear the garage for you. How does that sound?

Mark: (tears escape as his shoulders drop) Thank you, Doctor... thank you so much. It feels like a massive weight has lifted just knowing I don't have to go back to that empty house alone tonight. I'm completely happy for you to call the Crisis Team and my sister. I want the help. I don't want to feel like this anymore.

Station Conclusion: An "Excellent Pass" High-Risk Depression Profile

Why your final choices scored maximum marks:

- **Accurate Risk Stratification:** You correctly recognised a prepared plan, immediate access to means, and a stated lack of self-trust as constituting a high-risk psychiatric emergency.
- **Strict Adherence to Safety Metrics:** You explicitly stated you would not let him leave the clinic alone — allowing a high-risk suicidal patient to walk out unaccompanied is an automatic safety fail.
- **Immediate Crisis Mobilisation:** You escalated directly to the Crisis Resolution and Home Treatment Team for same-day assessment.

- **Engaged Protective Factors and Secured the Environment:** You involved his sister immediately and arranged for the ligature to be safely removed from the garage.

Station 28 — Mental Health: Acute Psychosis and Schizophrenia Triage

An Acute Psychotic Episode/Suspected Schizophrenia station tests your communication, safety triage, and non-judgemental exploration skills. Your goals are to build rapport with a suspicious or paranoid patient, identify positive and negative symptoms, screen for risk (command hallucinations or harm to self/others), and execute a safe escalation pathway.

Focused History & Symptom Screen

- **Positive symptoms:** delusions (fixed, false beliefs — persecutory, grandiose, or thought broadcasting) and hallucinations (typically third-person or command auditory hallucinations in schizophrenia).
- **Negative symptoms and disorganisation:** social withdrawal, blunted affect, poverty of speech (alogia), lack of motivation (avolition), and fragmented speech or erratic behaviour.
- **Risk assessment:** are the voices telling the patient to harm themselves or others (command hallucinations)? Does the patient have access to any means to act on this?

Psychosis Management Streams

- **First-episode/stable:** new-onset symptoms, patient cooperative, no immediate threat — urgent referral to the community Early Intervention in Psychosis (EIP) team, typically within 2 weeks.
- **Acute psychotic crisis/high risk:** active command hallucinations to cause harm, severe agitation, or total self-neglect — immediate referral to the Mental Health Crisis Team (CRHTT) for same-day evaluation; consider voluntary or involuntary admission under the Mental Health Act.

The Roleplay

Station: Consultation Skills / Mental Health Triage

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Daniel Craig, 24 years old

Setting: GP Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Daniel Craig, 24, brought in by his worried mother. Over the past three months he has dropped out of university, locked himself in his room, stopped showering, and she has overheard him arguing loudly when no one else is present.

Your Task:

- Take a focused history to identify positive and negative symptoms of psychosis.
- Perform a thorough risk assessment.

- Formulate the correct clinical management pathway.

(Daniel sits rigidly on the edge of his chair, wearing a heavy winter coat with the hood partly over his face despite the warm room. His eyes dart around the ceiling corners.)

Candidate: Hello Daniel. My name is Dr [Your Name], one of the doctors here today. I understand your mother was keen for us to have a chat this morning. I can see you look quite tense right now. I want to reassure you this room is completely safe and confidential. How have things been going for you lately?

Daniel: (whispering intensely, looking around) Safe? Nowhere is safe, Doctor. You shouldn't even be talking to me. They have directional microphones drilled into the air vents. I didn't want to come here, but my mother doesn't understand they're tracking my thoughts through the university Wi-Fi. I had to quit my course to protect my brain.

Common Pitfall: *"That's a completely impossible conspiracy theory. No one is tracking your thoughts through the Wi-Fi. You are experiencing delusions. I'm prescribing strong antipsychotic medication right now to fix your thinking."* — Directly confronts the delusion, likely to rupture rapport and shut the patient down entirely.

Model Response

Candidate: It sounds incredibly exhausting and frightening to feel like you're constantly being tracked and watched like that, Daniel. I can see how much stress this is causing you, and I appreciate you telling me. I want to reassure you I don't have any microphones here, and my only goal is to support you. To help make sure you feel safe, do you mind if I ask a little more about what you've been experiencing?

Daniel: (relaxing slightly) It is exhausting... I haven't slept properly in weeks because I have to stay awake to watch the door. If you say it's safe here, I'll try to trust you. What else do you want to know?

Your Next Step: Exploring Hallucinations and Risk

Candidate: Your mother mentioned hearing you talking when no one else was around — do you find you hear people speaking or whispering to you when there's no one there? If you do hear those voices, what do they say? Are they ever critical, or do they ever tell you to do things to harm yourself or anyone else? And how has your energy and motivation been for things like showering or leaving your room recently?

Daniel: (voice dropping to an intense whisper) Yes, I hear them. It's two men's voices, completely clear, talking about me behind my back. They tell me I'm worthless and that I shouldn't trust anyone. Lately they've started commanding me — they tell me that if they get too close, I have to defend myself. They keep repeating that I should hurt my mother to protect my thoughts before she betrays me to the university. It scares me because I don't want to hurt her, but the voices are so loud and constant. As for my room and showering... I just don't have the energy anymore. What's the point of cleaning myself if my brain is being hijacked? I feel completely empty.

Your Next Step: Crisis Management and Safety Triage

This confirms core positive symptoms (persecutory delusions, third-person auditory hallucinations) and negative symptoms (avolition, social withdrawal, self-neglect). Most critically, Daniel has active command hallucinations to harm a specific named person — his mother. He represents an immediate risk to others and cannot be discharged home.

Common Pitfall: *"Hearing voices telling you to hurt your mother means you are a dangerous criminal. I'm calling the police right now to have you arrested."* — Criminalises a symptom of serious mental illness and abandons the medical duty of care entirely.

Model Response

Candidate: Thank you for trusting me with that, Daniel. I know how terrifying it must be to hear those voices commanding you, especially when you love your mother and don't want to hurt her. Because those voices are so loud right now, my absolute priority is keeping both you and your mother completely safe today. I'm not going to let you go home under this stress. With your permission, I'm going to contact our Mental Health Crisis Team immediately for a same-day emergency evaluation here at the clinic. We'll also arrange a safe, quiet space for you, and with your consent, speak to your mother to make sure she's supported and safe too. We're going to get you the right specialist care today to help quiet those voices.

Daniel: (visible relief) Thank you, Doctor... I don't want to hurt her. It's just so loud in my head. If you can get the specialists here today to help quiet them down, I'll stay. I'm happy for you to talk to my mother too, just please make sure she's safe from what the voices are saying. I'll wait here.

Alternate Presentation: When the Patient Discloses a Weapon

A variant of this same station tests an additional skill: safely asking a frightened, delusional patient to hand over an item they've disclosed carrying for "self-defence." In one common version, a patient named Liam describes persecutory delusions involving covert surveillance, then discloses hearing voices commanding him to "eliminate" a family member he believes is a threat, and admits to carrying a small pocket knife for protection.

Candidate: Because you feel so unsafe right now, have you felt the need to carry anything with you today to protect yourself?

Liam: Yes, Doctor. I have a small pocket knife in my jacket. The voices told me I had to protect myself if anyone tried to block my way out.

Candidate: Thank you for being so honest with me. I know how frightening it must be to feel that unsafe. To make sure this room stays a safe space for both of us, would you be comfortable sliding that knife over onto the desk here, so we can set it aside while we talk?

(Liam hesitates, then slowly slides the closed knife across the desk, his whole body slumping with relief.)

Liam: Okay... there it is. I don't want to hurt anyone. It's just so loud in my head. Thank you for not calling the police on me.

Handled this way — calmly, without judgement, and framed as keeping the room safe for both parties rather than as a confrontation — asking the patient to voluntarily surrender an item is both safer and more likely to succeed than attempting to physically intervene or challenge them directly. This is followed by the same Crisis Team escalation as the main scenario.

Station Conclusion: An "Excellent Pass" Psychosis Triage Profile

Why your final choices scored maximum marks:

- **Rapport With a Paranoid Patient:** You validated the emotional distress without colluding with or aggressively challenging the delusion.
- **Complete Symptom Matrix:** You extracted both positive symptoms (delusions, hallucinations) and negative symptoms (avolition, self-neglect, social withdrawal).

- **Flawless Risk Mitigation:** You recognised active command hallucinations targeting a named third party as an immediate high-risk emergency.
- **Immediate Emergency Escalation:** You mobilised the Crisis Team for same-day assessment while taking concrete steps to protect the named third party.

Station 29 — Mental Health: Postpartum Psychosis

Postpartum Psychosis is one of the highest-stakes psychiatric emergencies you can encounter in PLAB 2. Unlike Postnatal Depression, it involves a rapid, severe break from reality posing an immediate, catastrophic risk to both mother and newborn. Your objectives are to recognise the acute onset of psychotic symptoms, complete an urgent infant safety assessment, and initiate immediate escalation to specialised inpatient care.

UK management of postpartum psychosis is governed by NICE guideline CG192 (Antenatal and postnatal mental health), not by guidance relating to preterm labour.

The Roleplay

Station: Consultation Skills / Emergency Psychiatric Triage

Candidate Role: FY2 Doctor in the Emergency Department (A&E)

Patient/Roleplayer Role: Amy Clarke, 25, brought in by her husband David; Amy is present but largely uncommunicative

Setting: A&E Quiet Assessment Room

The Prompt (read outside the cubicle):

Patient Details: Amy Clarke, 25, gave birth to a healthy baby boy 10 days ago.

Presenting Complaint: Her husband David reports that over the last 48 hours, Amy has completely stopped sleeping, is talking in fragmented sentences, and behaving in a highly bizarre, frightening manner.

Your Task:

- Take a focused history from the husband and evaluate the presentation.
- Perform an immediate risk assessment regarding infant safety and self-harm.
- Formulate the correct emergency UK admission pathway.

(Amy sits on the edge of her chair, rocking rapidly with wide, vacant eyes, muttering under her breath and pointing at the ceiling vents. David sits beside her, pale, exhausted, and visibly terrified.)

Candidate: Hello David, hello Amy. My name is Dr [Your Name], one of the emergency doctors today. David, I can see how deeply distressed you are, and Amy, we're here to make sure you're completely safe and looked after. David, can you tell me exactly what's been happening over the last couple of days?

David: Doctor, I'm losing my mind with worry. Amy hasn't slept a single second for two nights. She has this manic energy and keeps pacing the house. This morning she started screaming that our baby isn't ours, that he's been replaced by an entity, and that hospital staff altered him. She won't let me near her or the baby, and she's talking to the walls. What's happening to my wife?

Common Pitfall: *"Based on her lack of sleep and these strange ideas, Amy is experiencing a severe case of the 'baby blues.' I'll prescribe some strong sleeping tablets and you can take her*

home to rest." — Catastrophically under-triages an acute psychotic emergency as a normal, self-limiting adjustment.

Model Response

Candidate: Thank you for detailing this so clearly, David. I can see how terrifying and exhausting this is for both of you. Amy is dealing with a very severe, treatable medical condition, and you've done absolutely the right thing bringing her in immediately. To help keep both Amy and your newborn completely safe right now, do you mind if I ask a few urgent questions about how she's been interacting with the baby and her current behaviour?

David: Yes, please ask, Doctor. I'm terrified to leave her alone for even a second right now.

Your Next Step: Infant Safety and High-Risk Screen

This is a textbook presentation of Postpartum Psychosis: acute onset within 10 days of delivery, total insomnia, manic energy, and bizarre persecutory/somatic delusions. You must now urgently screen the infant's physical safety, care arrangements, and any command hallucinations or intrusive thoughts of harm.

Candidate: Where is your newborn right now, and who's currently looking after him? Has Amy made any physical attempts to harm the baby, or expressed needing to protect him from the "entity" in a dangerous way? And has she said anything about wanting to harm herself, or have you noticed anything suggesting she's hearing voices telling her to do things?

David: Thank God, our baby is safe — my sister is at our house looking after him. I wouldn't leave him alone with Amy right now. She hasn't tried to physically hurt him, but she did try to lock herself in the bathroom with him yesterday because she said she needed to 'save his soul' from the entity. It took me ten minutes to get the door open. And yes, she keeps arguing with the ceiling — she told me the voices are telling her the only way to break the spell and get our real baby back is if she jumps out of a window. I'm so scared she's going to act on it.

Your Next Step: Emergency Specialised Admission

Amy's profile confirms a severe psychiatric emergency: Postpartum Psychosis with active command hallucinations to harm herself and delusional thoughts involving the infant. Under UK guidance (NICE CG192), a patient with suspected postpartum psychosis must be admitted immediately — crucially, to a specialised Mother and Baby Unit (MBU) rather than a standard adult psychiatric ward, allowing expert perinatal psychiatric care while preserving the maternal-infant bond under 24-hour supervision.

Candidate: Thank you for sharing that critical information, David. What Amy is experiencing is a severe medical emergency called Postpartum Psychosis, causing a rapid shift in thoughts, sleep, and perception after birth. Because the voices are commanding her and she's at high risk of harming herself, she cannot go home safely today. I'm contacting our Perinatal Psychiatry Liaison Team immediately for an urgent assessment. Our goal is to admit Amy directly to a specialised Mother and Baby Unit — a dedicated ward where she'll get expert care to quiet those voices while safely staying together with the baby under 24-hour expert supervision. We will not leave her alone for a single second while we arrange this. How do you feel about this plan?

David: (sobbing with relief) A Mother and Baby Unit... thank God. That sounds exactly like what she needs. I was terrified you'd separate her from our son forever, or put her on a normal adult ward where she'd be completely lost. Knowing she'll be safe and monitored, and can still be a mum safely, means everything. Let's get the specialists down here right away. I'm completely on board.

Station Conclusion: An "Excellent Pass" Postpartum Psychosis Profile

Why your final choices scored maximum marks:

- **Accurate Differentiation From PND:** You correctly identified the acute timeline, total insomnia, mania, and somatic/persecutory delusions as pointing to psychosis rather than depression or baby blues.
- **High-Stakes Third-Party Screening:** You explicitly confirmed the infant's physical safety and current supervision before proceeding with management.
- **Mastery of the MBU Pathway:** You correctly identified that postpartum psychosis requires admission to a specialised Mother and Baby Unit rather than a general adult ward — a non-negotiable safety metric that also preserves the maternal-infant bond.
- **Continuous Supervision:** You recognised the imminent self-harm risk from command hallucinations and maintained continuous supervision while mobilising the Perinatal Psychiatry Liaison Team.

Station 30 — Mental Health: Obsessive-Compulsive Disorder (Contamination Subtype)

An OCD station typically presents a patient with two clinical halves: an obsession (an intrusive, persistent, unwanted thought or fear) and a compulsion (a repetitive action performed to relieve the anxiety the obsession creates). This station uses the classic contamination subtype — fear of catching disease, relieved by repetitive handwashing.

Key History Points

- **Time consumption:** do the thoughts and washing take up more than an hour a day? — this threshold is part of the standard diagnostic criteria.
- **Functional impairment:** explore the impact on work, social life, and physical health (e.g. hand dermatitis from over-washing).
- **Insight:** check whether the patient recognises their fears as excessive or irrational — this is common in OCD (in contrast to some psychotic illnesses) and is a reassuring, rather than concerning, finding.
- **Comorbidity screen:** gently check for depression, generalised anxiety, and suicidal ideation, which frequently co-occur.

The Roleplay

Station: Mental Health Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Dr Priya Sharma, 25 years old, a junior doctor

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Priya, 25, a junior doctor, has booked an appointment about intrusive thoughts of catching a disease and excessive handwashing.

Your Task:

- Take a focused history covering the obsession-compulsion cycle.
- Assess severity and screen for comorbidities.
- Formulate a stepped management plan per NICE guidance.

(Priya sits with her hands visibly red and cracked, tucked into her lap.)

Candidate: Hello Priya, I'm Dr [Your Name]. I understand you wanted to talk about some thoughts and habits around handwashing — can you tell me more about what's been happening?

Priya: It's gotten completely out of control. I keep having this overwhelming thought that I'm going to catch something terrible from touching things — door handles, patient notes, anything. I know it sounds irrational, but I can't shake the feeling, and I end up washing my hands over and over until they feel 'clean enough,' which never really happens.

Candidate: That sounds exhausting to carry around, especially on top of a demanding job. Roughly how much of your day would you say these thoughts and the washing take up?

Priya: Honestly, probably three or four hours some days. It's affecting my shifts — I'm sometimes late handing over because I can't stop washing, and my hands are so cracked and sore now, look at them.

Common Pitfall: *"You clearly know this is irrational, so you should just be able to stop if you try harder."* — Dismisses the involuntary, anxiety-driven nature of compulsions and implies the problem is simply a lack of willpower, which is both inaccurate and likely to increase her shame.

Model Response

Candidate: Thank you for describing that so clearly — and just to reassure you, the fact that you recognise these fears as excessive is actually a really helpful sign, not a worrying one. What you're describing — the intrusive fear of contamination, and the washing that temporarily relieves the anxiety it creates — fits with a condition called Obsessive-Compulsive Disorder, or OCD. It's very treatable, and recognising it clearly, as you have, is a great first step.

Priya: That's a relief to hear it has a name... I was starting to think I was just losing my mind. Does this mean I could lose my job over this?

Candidate: Not at all — and I want to address that directly, because I know that fear can be huge for doctors. Before we get to that, can I gently check — have you noticed your mood being low, feeling constantly anxious, or having any thoughts of harming yourself alongside all this?

Priya: I have felt pretty low about it all, if I'm honest, but no thoughts of hurting myself.

Your Next Step: The Stepped Management Plan

Candidate: Given how much this is affecting your day — around three to four hours, and real impact on your shifts and your skin — this sits at the moderate-to-severe end, so I'd recommend we move straight to a more intensive treatment. The most effective specific therapy is a type of CBT called Exposure and Response Prevention, or ERP. It works by gradually exposing you to a feared trigger — like touching a door handle — without letting yourself perform the washing ritual straight away, which over time breaks the anxiety cycle. Given the severity, I'd also suggest combining this with a medication called an SSRI, such as fluoxetine or sertraline. One important thing to know: for OCD, the doses used are often quite a bit higher than for depression, and it can take up to 12 weeks to notice a real benefit, so it's important not to give up early.

Priya: Higher doses and 12 weeks... okay, that's useful to know so I don't panic if it doesn't work immediately.

Candidate: Exactly. I'd also like to refer you to your hospital's Occupational Health department — they can support you with practical adjustments to your duties and skincare guidance for your hands while you're going through treatment, so you're safe and supported at work rather than struggling through it alone.

Priya: Thank you — I think I really needed to hear that there's an actual plan, and that I'm not going to be judged for this.

Station Conclusion: An "Excellent Pass" OCD Profile

Why your final choices scored maximum marks:

- **Correctly Identified Insight as Reassuring:** You reframed her awareness that her fears are excessive as a positive clinical sign, rather than treating it as a reason she should simply stop.
- **Quantified Severity Accurately:** You established time consumption and functional impairment to correctly stage her at moderate-to-severe, guiding the correct treatment intensity.
- **Screened for Comorbidities:** You checked for low mood and suicidal ideation before finalising the plan.
- **Explained ERP and SSRI Dosing Accurately:** You correctly noted that OCD often requires higher SSRI doses than depression, and a longer time to benefit (up to 12 weeks).
- **Addressed Her Professional Identity Directly:** You proactively addressed her fear about her job and involved Occupational Health, recognising the specific pressures facing a doctor with this diagnosis.

Station 31 — Mental Health: Somatoform Disorder / Functional Neurological Symptoms

This station presents a patient with genuine physical suffering — chronic abdominal pain, limb weakness, or similar — for which extensive investigations have found no organic cause. The single biggest trap is implying, even subtly, that the patient is "faking it" or that it's "all in their head"; telling a patient their symptoms are purely psychological too early will cause them to become defensive, and is treated as a station failure.

The Roleplay

Station: General Practice / Chronic Symptom Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Mr Jones, 41 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Mr Jones, 41, has suffered severe, exhausting stomach pain for over six months. Blood tests, an ultrasound, and an endoscopy have all come back normal.

Your Task:

- Validate the reality of his pain from the outset.
- Explore his psychosocial context sensitively, without accusing him of inventing the illness.
- Explain the diagnosis using an accessible analogy, and outline a functional management plan.

Candidate: Hello Mr Jones, my name is Dr [Your Name]. I can see from your notes you've been suffering with this severe, exhausting stomach pain for over six months now. I want to say clearly from the start: I believe you completely. I know this pain is genuinely disrupting your life, and I'm here to help you manage it.

Mr Jones: Thank you for saying that, Doctor. I've had blood tests, an ultrasound, and an endoscopy done recently, and they told me everything was normal. But I know it's not — they must have missed something!

Candidate: It must be incredibly frustrating to feel this severe pain every single day, only to be told the scans are blank. It's completely natural to feel left in the dark when that happens.

Common Pitfall: *"Since all your tests are normal, there's nothing physically wrong, so this is likely stress-related."* — Even though technically related to the eventual explanation, delivering this so early and bluntly, before exploring his experience and building trust, will read as dismissive and provoke defensiveness.

Your Next Step: Gentle Psychosocial Exploration

Candidate: When the pain flares up at its worst, what thoughts or worries tend to run through your mind?

Mr Jones: Honestly, I start thinking it must be cancer, that something's been missed and it's slowly getting worse.

Candidate: That sounds like a really frightening thing to be carrying alongside the pain itself. We know our bodies and minds are deeply connected — have you noticed if the pain gets worse during times you're feeling particularly stressed or overwhelmed?

Mr Jones: Now that you mention it... yes, actually. It's been much worse since I lost my job three months ago and things at home have been tense.

Your Next Step: Safety-Netting Before Explaining the Diagnosis

Candidate: Thank you for sharing that — that's really helpful context. Before I explain what I think is going on, I want to check a few things directly, just to make sure we're not missing anything: any unexplained weight loss, blood in your stool, or night-time fevers? And any new double vision, loss of bladder or bowel control, or facial drooping?

Mr Jones: No, none of that — just the pain itself, really.

Your Next Step: The "Hardware vs Software" Explanation

Candidate: Mr Jones, because all your scans, blood tests, and the camera test have come back completely clear, we can say confidently there's no structural damage, tumour, or ulcer causing this. To explain what I think is happening, I like to use the analogy of a computer. Your organs and nerves are like the computer's hardware — the physical parts and wiring — and our tests show your hardware is in perfect condition. But the way your brain and body send signals to each other is more like the software. In your case, there seems to be a kind of glitch in that software: your nerves are sending completely normal, everyday signals from your stomach, but your brain is misinterpreting and amplifying them as severe pain. The pain itself is completely real — it's just a functional issue with the software, not a physical break in the hardware.

Mr Jones: (visibly relieved) That actually makes sense... so I'm not crazy?

Candidate: Not at all. This is a genuine, recognised medical condition, and it's very treatable.

Your Next Step: The Functional Management Plan

Candidate: Because we know the hardware is fine, repeating the same scans over and over won't give us new answers, and would only expose you to unnecessary radiation and stress. The most effective treatment for resetting this kind of software glitch is a specific talking therapy called Cognitive Behavioural Therapy, which helps retrain how the brain processes these intense physical signals. I'd also like to refer you to a physiotherapist for a gentle, graded exercise programme, which helps rebuild physical confidence and gradually desensitises the nervous system. In some cases, a low dose of a medication called amitriptyline can help quiet down overactive pain signals reaching the brain — it's not being used here as an antidepressant, but as a nerve-calming agent.

Mr Jones: So you're saying this is all just stress, and you're dumping me on a therapist because you don't know what's actually wrong with me?

Common Pitfall: *"No, it's not just stress — I've explained the mechanism to you already." — Becoming defensive or repeating the explanation more insistently will likely escalate his frustration rather than resolve it; the correct response is to validate the challenge itself.*

Model Response

Candidate: I can hear why it might feel that way, and that's a really fair challenge to raise. To be clear: I do know what's wrong — this is a genuine condition where the nervous system's pain-

processing has become oversensitive, and it has a name and a proper treatment pathway, just like any other diagnosis. I'm not sending you away because I've given up looking — I'm recommending the treatments that the evidence shows actually work for this specific problem, in the same way I'd recommend a particular treatment for any other diagnosis. I'll stay involved throughout, and we can review how things are going together.

Mr Jones: (nodding slowly) Okay... put that way, I suppose it does make more sense. I'll give the CBT and physio a try.

Station Conclusion: An "Excellent Pass" Somatoform Disorder Profile

Why your final choices scored maximum marks:

- **Validated the Pain Immediately and Unconditionally:** You stated you believed him before any other data gathering, which is essential to keeping a somatoform patient engaged.
- **Explored Psychosocial Context Without Accusation:** You linked stress and pain gently, letting him make the connection himself rather than asserting it.
- **Completed a Genuine Red-Flag Screen:** You explicitly checked red flags despite the working diagnosis, rather than assuming the diagnosis and skipping safety questions.
- **Used the Hardware/Software Analogy Effectively:** This reframed the diagnosis as genuinely real without collapsing it into either "purely physical" or "just psychological."
- **Handled the Resistant Final Challenge Without Becoming Defensive:** You validated his frustration while firmly restating that this is a genuine, named, treatable condition, rather than repeating yourself more forcefully or capitulating.

SECTION: OPHTHALMOLOGY

Station 32 — Ophthalmology: Acute Angle-Closure Glaucoma

Ophthalmology stations test your ability to differentiate benign eye complaints from sight-threatening emergencies. The core framework for any eye station is checking three parameters during data gathering: visual acuity (blurred, reduced, or absent), pain level (painless, mildly irritated, or a deep boring ache), and redness. Acute Angle-Closure Glaucoma (AACG) is one of the most high-yield painful red eye emergencies.

High-Yield Ophthalmology Differential Matrix

Condition	Pain	Vision	Redness	Key Feature
Acute Angle-Closure Glaucoma	Severe, boring ache with nausea/vomiting	Severely reduced, haloes around lights	Yes	Hazy cornea; fixed, mid-dilated pupil; elderly, hypermetropic patients
Anterior Uveitis (Iritis)	Moderate–severe, deep ache	Mildly reduced	Yes	Photophobia; small, irregular, sluggish pupil
Conjunctivitis	Gritty/itchy, no deep pain	Normal	Yes (diffuse)	Sticky yellow crusting (bacterial) or watery discharge (viral)

Risk Factors and Emergency Management

- **Structural risk factor:** hypermetropia (long-sightedness), which gives a shallower anterior chamber.
- **Precipitants:** pupil dilation in a dark environment (e.g. a cinema), or anticholinergic/sympathomimetic medications (e.g. some overactive bladder treatments, antihistamines, or decongestants), which can dilate the pupil and block the drainage angle.
- **Management:** same-day emergency Ophthalmology referral, IV acetazolamide to rapidly lower intraocular pressure, topical pilocarpine to constrict the pupil once pressure is falling, analgesia and antiemetics, and keeping the patient supine to help the lens fall away from the iris.

The Roleplay

Station: Acute Assessment / Consultation Skills

Candidate Role: FY2 Doctor in the Emergency Department (A&E)

Patient/Roleplayer Role: Fiona Harrison, 63 years old

Setting: Emergency Cubicle

The Prompt (read outside the cubicle):

Patient Details: Fiona Harrison, 63, self-referred to A&E this afternoon.

Presenting Complaint: Sudden, excruciating pain in her left eye that started two hours ago while watching a movie in a dark cinema. She feels extremely sick.

Your Task:

- Take a focused history to determine the cause of her eye symptoms.
- Perform a rapid assessment of sight-threatening parameters.
- Formulate the correct emergency escalation pathway.

(Fiona is sitting forward, holding an emesis bowl, clutching her left temple and shielding her left eye from the room's lights.)

Candidate: Hello Mrs Harrison, I'm Dr [Your Name], one of the emergency doctors today. I can see you're in severe pain and feeling quite sick. We're going to take immediate care of you. Can you tell me exactly where you feel the pain right now?

Fiona: (groans) Oh, Doctor... it's this left eye. The pain is a deep, horrible, throbbing ache right behind the eye, radiating across the entire left side of my forehead. It's so bad I've already thrown up twice. My vision has gone completely blurry in this eye, and when I look at the lights in here, I see strange, bright rainbow haloes around them. Please help me.

Candidate: I can see how distressing this is, Fiona, and that level of pain alongside vomiting is incredibly tough. You're in the right place to get help. To safely stabilise this and protect your sight, do you mind if I ask a few quick questions, and then take a look at your pupil and the front of your eye right away?

Fiona: Thank you... please go ahead. I'll answer whatever you need so we can get this sorted.

Your Next Step: Targeted History for Risk Factors

Candidate: Have you ever had an episode like this before, or do you have a history of raised eye pressure or glaucoma? Are you taking any regular medications, especially anything for your bladder, or any antihistamines or cold remedies? And do you wear glasses — are you short-sighted or long-sighted?

Fiona: No, I've never had anything like this before — my eyes have always been healthy. My doctor did start me on a new tablet for an overactive bladder last week, but I can't remember the name of it. And yes, I wear reading glasses — my optician told me I'm quite long-sighted.

Your Next Step: Emergency Management

Fiona's history — a new anticholinergic bladder medication, structural long-sightedness, and pupil dilation in a dark cinema — created the perfect storm for the drainage angle to close, causing a dangerous spike in intraocular pressure. On examination, her left eye is injected around the iris,

the cornea looks hazy, and the pupil is fixed in a mid-dilated position, unreactive to light. This is a sight-threatening emergency that can cause permanent blindness within hours if untreated.

Common Pitfall: *"This looks like eye strain mixed with a migraine from your new tablets. I'll give you some pain relief and pupil-constricting drops, and book you a routine eye clinic appointment for next week."* — Fails to recognise a same-day sight-threatening emergency and delays vision-saving pressure-lowering treatment.

Model Response

Candidate: Thank you, Fiona. Your long-sightedness and your new bladder tablet have combined with the dark cinema to cause a condition called Acute Angle-Closure Glaucoma — the drainage angle inside your eye has become blocked, causing the pressure inside to rise sharply. I'm contacting our Ophthalmology team immediately for a same-day emergency review. While they come down, we're going to start you on a drip with a medicine called Acetazolamide to rapidly lower the pressure, along with some anti-sickness and pain relief. I'd also like you to lie flat on your back, which helps take pressure off the drainage angle.

Fiona: (nodding in relief as the nursing team moves in to set up the drip) Thank you, Doctor... thank you so much for moving so quickly. Knowing there's something that can bring this pressure down is such a comfort. Please make the calls to the eye specialists right away.

Station Conclusion: An "Excellent Pass" Acute Angle-Closure Glaucoma Profile

Why your final choices scored maximum marks:

- **Identified the Combined Triggers:** You linked her structural risk factor (long-sightedness) and her new anticholinergic bladder medication to the dark cinema environment that precipitated the attack.
- **Recognised the Textbook Signs:** You gathered the diagnostic hallmarks of AACG — severe pain, haloes, vomiting, a hazy cornea, and a fixed, mid-dilated pupil.
- **Flawless Emergency Action Plan:** You arranged an immediate same-day Ophthalmology referral, started IV acetazolamide, gave supportive anti-emetic and analgesic care, and positioned the patient supine.
- **Avoided Under-Triage:** You did not mistake this for a routine headache or migraine requiring only a delayed outpatient review.

SECTION: GERIATRIC MEDICINE

Station 33 — Geriatric Medicine: Acute Delirium Triage

An Acute Delirium station is a high-frequency Data Gathering and Clinical Management station. Delirium is a medical emergency characterised by an acute, fluctuating change in mental status, altered attention, and disturbed consciousness, almost always secondary to an underlying acute physiological stressor such as infection, metabolic derangement, or medication toxicity.

Your primary objectives are to recognise the acute onset using a structured framework, differentiate it cleanly from chronic dementia, systematically screen for the underlying medical trigger, and formulate a safe management plan according to UK clinical guidelines (NICE NG103).

The Confusion Assessment Method (CAM)

A confident OSCE diagnosis of delirium requires features 1 and 2, plus either 3 or 4:

- **1. Acute onset and fluctuating course:** did the confusion start suddenly (hours to days), and does it come and go?
- **2. Inattention:** does the patient struggle to focus, follow a conversation, or count backward?
- **3. Disorganised thinking:** is their speech rambling, irrelevant, or incoherent?
- **4. Altered level of consciousness:** are they hyperactive (agitated, hyper-alert) or hypoactive (lethargic, drowsy)?

Differentiating Delirium from Dementia

Feature	Acute Delirium (Medical Emergency)	Chronic Dementia
Onset	Sudden/acute (hours to days)	Gradual/insidious (months to years)
Course	Fluctuating (often worse at night)	Progressive and stable day-to-day
Attention	Severely impaired and variable	Relatively intact until advanced stages
Hallucinations	Common (often visual or persecutory)	Rare (except specific types like Lewy Body)

Feature	Acute Delirium (Medical Emergency)	Chronic Dementia
Reversibility	Highly reversible once the cause is treated	Irreversible and progressive

The Roleplay

Station: Consultation Skills / Acute Medical Triage

Candidate Role: FY2 Doctor in the Emergency Department (A&E)

Patient/Roleplayer Role: Thomas Miller, care home manager, presenting on behalf of 82-year-old resident Henry Vance

Setting: Emergency Assessment Bay

The Prompt (read outside the cubicle):

Patient Details: Henry Vance, 82, brought in from his residential care home by the manager, Thomas.

Presenting Complaint: Henry has suddenly become extremely confused and agitated, and has been shouting at "bugs on the wall" since yesterday evening. He has a known baseline of mild Alzheimer's dementia.

Your Task:

- Take a focused collateral history from the care home manager to confirm delirium.
- Systematically screen for the underlying physical or metabolic triggers.
- Formulate the correct immediate UK investigation and workup pathway.

(Thomas sits looking stressed, while Henry lies on the trolley, picking at the bedsheets and muttering incoherently.)

Candidate: Hello Thomas. My name is Dr [Your Name], one of the emergency doctors on duty today. Thank you for bringing Henry in — I can see he's quite distressed and confused right now. Can you tell me what's been happening over the last 24 hours?

Thomas: Hello Doctor. We're really worried about Henry. He does have mild Alzheimer's, but normally he's sweet, recognises everyone, and can have a nice chat. But last night, out of nowhere, he became completely out of it — pacing the corridors, screaming that giant spiders were crawling up his walls, and he even tried to hit one of our night staff. It's like a switch flipped. Can dementia worsen this fast overnight?

Common Pitfall: *"Because Henry already has Alzheimer's, this sudden aggression and hallucination is just the natural progression of his dementia. There's no cure for this — I'll prescribe a strong sedative and discharge him back to the care home today."* — Misclassifies a reversible medical emergency as an irreversible disease progression, missing the diagnosis entirely.

Model Response

Candidate: Thank you for those clear details, Thomas. To answer your question directly: no, chronic dementia doesn't change this drastically overnight. Because this has happened so suddenly, with fluctuating confusion and visual hallucinations, Henry is experiencing an acute medical condition called delirium — a temporary state of confusion caused by a physical illness or stress somewhere in the body. You did exactly the right thing bringing him in. To find out what's triggering this, do you mind if I ask a few rapid questions about how he's been physically over the last few days?

Thomas: (visibly relieved) Oh, thank goodness — that makes much more sense, because this is completely unlike his normal self. Please, ask whatever you need.

Your Next Step: Systematically Screening for Triggers

You've safely diagnosed acute-on-chronic delirium (delirium superimposed on a background of dementia). Now screen systematically for common precipitants using the PINCHES framework: Pain, Infection, Nutrition, Constipation, Hydration, Electrolytes, Substances (medication changes).

Candidate: Has Henry had a recent cough, shortness of breath, or any signs of a urine infection, like foul-smelling urine or pain passing water? Has he been drinking normally, and have his bowels been opening regularly? Have there been any recent changes to his regular medications, or has he fallen or injured himself recently?

Thomas: Now that you mention it, one of our care assistants noted two days ago that his urine smelled incredibly strong when helping him to the toilet. He's also been refusing to drink his squash or water since yesterday morning, and he hasn't opened his bowels in at least four days. No cough or falls, and his medications haven't changed. But he's definitely been backed up and refusing fluids.

Your Next Step: The Investigation and Management Plan

Henry's history reveals acute-on-chronic delirium triggered by a combination of a suspected urinary tract infection, severe constipation, and acute dehydration. Delirium is a medical emergency requiring stabilisation, reversal of the underlying triggers, and a non-pharmacological approach to behaviour management wherever possible.

Common Pitfall: *"This confirms a standard urinary tract infection. I'll give you a prescription for oral amoxicillin and a bottle of laxatives — you can take Henry back to the care home right now and administer these yourself."* — Discharges an acutely delirious, dehydrated patient into the community rather than admitting for IV rehydration and monitoring.

Model Response

Candidate: Thank you for those details, Thomas. This fits perfectly with delirium — the combination of a suspected water infection, dehydration, and severe constipation has placed immense physical stress on Henry's body, causing this sudden confusion and hallucinations. We're going to admit him to our medical unit today. We'll start a gentle intravenous drip to rehydrate him, check an urgent urine sample and routine bloods, and give him targeted antibiotics and laxatives to clear the infection and relieve his bowels. Crucially, we'll manage his agitation by keeping him in a quiet, well-lit room, making sure he has his glasses and hearing aids, and keeping familiar faces around him, rather than reaching for heavy sedatives. We'll keep you updated every step of the way.

Thomas: That's a brilliant plan, Doctor. It makes complete sense, and I'm glad you're keeping him in for the fluids and antibiotics. I'll make sure his glasses and hearing aids are brought over right away, and we'll keep one of our staff with him as much as possible to keep things familiar.

Station Conclusion: An "Excellent Pass" Delirium Triage Profile

Why your final choices scored maximum marks:

- **Accurate Diagnosis and Differentiation:** You correctly distinguished acute delirium from baseline chronic dementia using CAM principles — sudden onset, fluctuating course, and visual hallucinations. Misclassifying this as dementia progression is an automatic safety fail.
- **Exhaustive Trigger Screening:** You used the PINCHES framework to uncover a triad of UTI, constipation, and dehydration, rather than stopping at the first plausible cause.
- **Gold-Standard Non-Pharmacological De-escalation:** You avoided heavy chemical sedation, which can worsen delirium and raise the risk of falls and mortality in elderly patients, and instead emphasised sensory aids, a calm environment, and familiar faces.
- **Holistic Medical Optimisation:** You arranged admission for IV fluids, urgent bloods and urine culture, targeted antibiotics, and laxatives, rather than discharging an acutely unwell patient into the community.

Station 34 — Geriatric Medicine: Prescribing Safety in Overactive Bladder — Oxybutynin vs Safer Alternatives

This station tests prescribing safety awareness in older adults with cognitive concerns. Oxybutynin, used for overactive bladder, is an anticholinergic that crosses the blood-brain barrier and blocks acetylcholine — the same neurotransmitter central to learning and memory. Large observational studies have shown a strong association between long-term anticholinergic use, including oxybutynin, and increased dementia risk, though as with most large observational drug-safety studies, this reflects a strong and consistent association rather than a proven, mechanistically confirmed causal pathway in every individual. Current UK prescribing guidance nonetheless advises avoiding oxybutynin in older adults, particularly those with existing cognitive impairment.

Key Pharmacology Comparison

Feature	Oxybutynin	Tamsulosin
Primary use	Overactive bladder (urgency/leaking)	Enlarged prostate / BPH (poor urine flow)
Drug class	Anticholinergic (blocks acetylcholine)	Alpha-blocker (relaxes prostate/bladder neck muscle)
Cognitive/dementia link	Strong association with increased dementia risk and worsened memory with long-term use	No proven causal link; some older, weaker associations not confirmed by larger follow-up studies
Key caution	Avoid in older adults or anyone with cognitive impairment; counteracts cholinesterase inhibitors like donepezil	Mainly postural hypotension/dizziness; caution before cataract surgery (floppy iris syndrome)

Safer overactive bladder alternatives in cognitively vulnerable patients: mirabegron (a beta-3 agonist that does not act on acetylcholine) or tiroprium (an anticholinergic that penetrates the brain poorly due to its chemical structure).

The Roleplay

Station: General Practice / Medication Review

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Mrs Eleanor Wallace, 78, accompanied by her daughter Jane

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Mrs Wallace, 78, has been on oxybutynin for an overactive bladder for the past year. She was recently diagnosed with mild cognitive impairment, and her daughter is worried about her memory getting worse.

Your Task:

- Explore the history and current medication use.
- Explain the link between oxybutynin and cognitive risk clearly.
- Propose a safer alternative and a sensible switching plan.

(Mrs Wallace sits quietly; her daughter Jane looks anxious, holding a printout from the internet.)

Candidate: Hello Mrs Wallace, and hello Jane. I understand you wanted to talk about Mum's bladder tablets and some memory concerns — how can I help today?

Jane: I read online that the tablet Mum's been taking for her bladder, oxybutynin, might be linked to dementia. She was just diagnosed with mild cognitive impairment, and now I'm terrified this medication might have caused it, or could make it worse.

Common Pitfall: *"There's no need to worry about that, it's just something people read on the internet — let's keep her on the same tablet since it's working well for her bladder." — Dismisses a genuine, evidence-supported prescribing safety concern and fails to act on well-established UK guidance to avoid anticholinergics in cognitively vulnerable older adults.*

Model Response

Candidate: That's a really important thing to raise, Jane, and you're right to bring it up. Oxybutynin belongs to a group of medicines called anticholinergics, which work by blocking a chemical in the body called acetylcholine to calm an overactive bladder. The difficulty is that acetylcholine is also one of the main chemicals the brain uses for memory and concentration, and oxybutynin is able to reach the brain and block it there too. Large studies have shown a strong link between long-term use of medicines like this and an increased risk of worsening memory or dementia, which is exactly why current guidance recommends avoiding it in older adults, especially those with any existing memory concerns.

Mrs Wallace: Oh dear... I had no idea. Should I just stop taking it today, then?

Candidate: I wouldn't stop it abruptly without a plan, since that could bring your bladder symptoms back suddenly. What I'd suggest instead is switching you to a different bladder medication that doesn't work on that same brain chemical — one option is called mirabegron, which works in a completely different way and isn't linked to memory problems. Another alternative, called trospium, is technically in the same drug family as oxybutynin, but its structure means it struggles to get into the brain in the first place, so it's considered a much safer choice if we did want to stay in that class.

Jane: That's such a relief — I was worried you'd just say there was nothing else that could help her bladder.

Candidate: Not at all — there are good alternatives. I'll arrange to switch her prescription to mirabegron today, and I'd like to see her back in a few weeks to check both her bladder symptoms are still well controlled and that we haven't missed anything with her memory in the meantime.

Station Conclusion: An "Excellent Pass" Prescribing Safety Profile

Why your final choices scored maximum marks:

- **Took the Concern Seriously and Accurately:** You confirmed the daughter's concern was valid and evidence-based, rather than dismissing it as internet misinformation.
- **Explained the Mechanism Clearly:** You explained why an anticholinergic bladder drug can affect memory, in plain, non-jargon language.
- **Avoided Abrupt Cessation:** You recommended a considered switch rather than stopping the medication suddenly, which could cause a rebound in bladder symptoms.
- **Named the Correct Safer Alternatives:** You offered both mirabegron (a different drug class entirely) and trospium (a poorly brain-penetrant anticholinergic) as genuine, evidence-based options.
- **Arranged Appropriate Follow-Up:** You planned review of both bladder control and cognitive status after the switch, rather than treating this as a one-off medication change.

SECTION: SUBSTANCE MISUSE & ADDICTION

Station 35 — Substance Misuse: Opioid (Heroin) Dependence and Harm Reduction

An Opioid Dependence and Harm Reduction station is a highly anticipated public health and psychiatry scenario in PLAB 2/UKMLA. The patient may present directly asking for help with their addiction, or as an intravenous drug user presenting with an injection-related complication such as a skin abscess. To score maximum marks, your approach must be strictly non-judgemental, focused on immediate public health harm reduction, and safely hand over specialist care to the Community Drug and Alcohol Team (CDAT).

The Heroin Dependence Blueprint

Safe Space Opening → Quantify Usage & Route → Deep Injection Risk Screen → ICE & Social Impact → OST Treatment Plan → Public Health Harm Reduction

The Roleplay

Station: General Practice / Public Health Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Daniel Wright, 33 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Daniel Wright, 33, has booked an appointment.

Presenting Complaint: He has quietly asked to speak to someone about a heroin habit he has had for approximately three years.

Your Task:

- Create a non-judgemental, safe space for disclosure.
- Quantify usage, route, and injection-related risk.
- Explore his ideas, concerns, and the social impact of his use.
- Formulate a safe referral and harm-reduction plan.

(Daniel sits with his arms folded, avoiding eye contact, visibly anxious.)

Candidate: Hello, my name is Dr [Your Name]. Thank you so much for coming in to speak with me today. I can see you look quite anxious, and I want to reassure you right from the start: this room is a completely safe, private, and confidential space. I'm here to listen and support you, without any judgement whatsoever. Can you tell me what's been on your mind?

Daniel: (quietly) Thanks, Doctor... I've been using heroin for about three years now. It started after I hurt my back and got hooked on painkillers, but it's spiralled. I'm just so tired of it, and I don't know where else to turn.

Your Next Step: Quantifying Usage and Route

Candidate: Thank you for trusting me with that, Daniel — that takes real courage. How are you taking the heroin — are you smoking it, inhaling the vapour, or injecting it? And roughly how much are you using in a day, and how many days a week?

Daniel: I'm injecting it, mostly into my arms. It's about a twenty-pound bag a day, sometimes more on a bad day, pretty much every day of the week now.

Candidate: Thank you for being so open. If you miss a dose or go a few hours without it, do you get physical symptoms like body aches, sweating, stomach cramps, or a racing heart?

Daniel: Yes, badly. If I go more than about six hours I start sweating and shaking, my stomach cramps up, and I feel like I want to climb out of my own skin.

Your Next Step: The Injection Risk Screen

Because Daniel is injecting, you must screen immediately for blood-borne virus risk and local injection-site complications — a frequently missed set of marks.

Candidate: Are you ever sharing your needles, syringes, water, or filters with anyone else, even a close friend or partner? And where on your body are you currently injecting — have you noticed any red, hot, swollen lumps, streaks, or open sores near those areas?

Daniel: I try not to, but a couple of times when I've been desperate and didn't have my own kit, I've shared a needle with a friend. As for my arms, there's some scarring and a couple of old track marks, but no swelling or anything hot or painful right now.

Your Next Step: Exploring ICE and Social Impact

Candidate: What made you decide to come in and seek support for this now? And how has this been affecting your living situation, your relationships, or have you run into any financial or legal difficulties because of it?

Daniel: Honestly, I'm just exhausted, and I lost my job last month because of it. My girlfriend says she'll leave if I don't get help. I don't want to end up like some of the people I've used with — I've seen a couple of overdoses. I want my life back.

Your Next Step: The Referral and Explaining OST

As a GP or A&E doctor, you never initiate opioid substitution therapy directly — this requires specialist assessment and initiation through the Community Drug and Alcohol Team.

Candidate: Thank you for being so deeply honest with me, Daniel. You don't have to fight this alone. I'm going to make an immediate, priority referral to our local Community Drug and Alcohol Team — a dedicated team of specialist doctors, counsellors, and caseworkers who'll guide your recovery. To help you come off street heroin safely, without agonising withdrawal, they'll likely discuss a legal, controlled daily medication with you. There are two main options: methadone, a daily liquid that satisfies the body's craving without a dangerous high, or buprenorphine, a small tablet that dissolves under the tongue to block withdrawal. They'll start you on a low dose and carefully adjust it until your body feels stable.

Daniel: That actually sounds like a relief. I didn't think there was a proper medical way to come off it without just suffering through it.

Your Next Step: Public Health Harm Reduction

Before Daniel leaves the room, deliver the core harm-reduction package — this is where the highest safety marks in this station are scored.

Candidate: While you're waiting to see the specialist team, a few things will help keep you safe. First, please don't reuse or share needles — you can get free, sterile injecting equipment anonymously through the NHS Needle Exchange Scheme at most local pharmacies. Second, I'd recommend a routine blood test today to screen for HIV and hepatitis, and if you haven't already been vaccinated, I'd like to offer you the Hepatitis B vaccine as well, since it gives real protection going forward. Third, given the real risk of accidental overdose, we can provide you and anyone you live with a free emergency kit called Naloxone — a nasal spray or injection that can reverse the effects of an overdose if someone finds you unresponsive.

Daniel: I didn't know about the Naloxone kit — that could genuinely save my life, or someone else's. I'll take you up on all of that.

Your Next Step: Emergency Safety-Netting

Candidate: One last thing — if you ever take a dose and start feeling intensely dizzy, find yourself struggling to stay awake, or if someone notices your breathing becoming very slow and shallow, they must call 999 immediately.

Oral Examiner Follow-Up: "Can You Just Prescribe Me Something Today?"

Patient: *"Doctor, can you just write me a quick script for Methadone or Diazepam today to keep me going until I see the specialist team? I'm rattling right now!"*

Model Answer: I can hear how genuinely uncomfortable you're feeling right now, Daniel, and I don't take that lightly. However, I'm not able to prescribe methadone, buprenorphine, or diazepam for this today. Starting opioid substitution therapy safely requires a specialist assessment — the right starting dose depends on things only the specialist team can properly assess, and getting it wrong can be dangerous. Prescribing diazepam alongside ongoing opioid use also carries a real risk of dangerous interactions, including suppressing your breathing. What I can do is mark this referral as urgent and ask the team to see you as quickly as possible, which is often within a day or two for someone in your situation. In the meantime, if there are specific symptoms troubling you, like stomach cramps, I can suggest some simple, safe measures for those directly. And if things become severe or you feel at risk, please don't hesitate to go to A&E.

Examiners test this question specifically to see whether candidates cave under pressure and prescribe outside their competence. Staying firm, while remaining warm and offering a concrete alternative (urgent referral, symptomatic relief, safety-netting), is what separates a pass from a fail here.

Station Conclusion: An "Excellent Pass" Substance Misuse Profile

Why your final choices scored maximum marks:

- **Non-Judgemental Opening:** You created psychological safety before any data gathering, which is essential for honest disclosure in addiction consultations.
- **Complete Injection Risk Screen:** You screened for needle sharing (BBV risk) and local site complications, not just quantity and route.

- **Correct Referral Boundary:** You did not attempt to initiate OST yourself, correctly directing this through the specialist Community Drug and Alcohol Team.
- **Full Harm-Reduction Package:** You covered needle exchange, BBV screening, Hepatitis B vaccination, and a take-home Naloxone kit — the complete public health bundle, not just one or two elements.
- **Held the Prescribing Boundary Under Pressure:** When directly asked to prescribe methadone or diazepam outside your competence, you declined firmly but warmly, and offered a concrete, safe alternative instead of simply saying no.

Station 36 — Substance Misuse: Alcohol Dependence

An alcohol dependence station is a high-yield psychiatry and public health scenario. The patient may present directly asking for help, or with a vague physical complaint (a hand tremor, a fall, stomach pain) where alcohol is the hidden underlying issue. Management follows NICE guideline CG115, and your approach must be non-judgemental, use a recognised UK screening tool, assess for life-threatening withdrawal risk, and build a safe, collaborative treatment plan.

Quantifying Intake in UK Units

- 1 pint of standard beer/cider $\approx$ 2 units.
- 1 standard glass of wine (175ml) $\approx$ 2 units (varies with strength).
- 1 single shot of spirits (25ml) $\approx$ 1 unit.
- **UK recommendation:** no more than 14 units a week, spread evenly over 3 or more days, for both men and women.

The CAGE Screening Questionnaire

- **Cut down:** "Have you ever felt you should cut down on your drinking?"
- **Annoyed:** "Have people annoyed you by criticising your drinking?"
- **Guilty:** "Have you ever felt bad or guilty about your drinking?"
- **Eye-opener:** "Have you ever had a drink first thing in the morning to steady your nerves or get rid of a hangover?"

A score of 2 or more is highly predictive of alcohol dependence.

The Roleplay

Station: General Practice / Public Health Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Sarah Whitfield, 44 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Sarah Whitfield, 44, booked in initially for a hand tremor and generally feeling run down; she drives for a living.

Your Task:

- Create a non-judgemental space and quantify her alcohol intake.
- Screen using CAGE and assess withdrawal risk, including Wernicke's encephalopathy.
- Formulate a safe UK community care pathway.

(Sarah sits with her hands slightly trembling, avoiding eye contact.)

Candidate: Hello, my name is Dr [Your Name]. Thank you for coming in today. I see from your notes you've been feeling a bit run down lately. I want you to know this room is a completely safe, private, non-judgemental space. How have things been going for you generally?

Sarah: Honestly, not great. My hands have been shaking, and I just feel exhausted all the time. I suppose... I've been drinking more than I probably should.

Candidate: Thank you for being so open with me. Can you walk me through a typical day — roughly how much you drink, and what?

Sarah: I usually have a bottle of wine most evenings, sometimes more on a bad day, and I'll often have a whisky or two before bed to help me sleep.

Your Next Step: The CAGE Screen and Withdrawal Assessment

Candidate: Have you ever felt you should cut down on your drinking? Has anyone been annoyed by criticising it? Have you felt guilty about it? And have you ever had a drink first thing in the morning to steady your nerves or shake off a hangover?

Sarah: Yes to all of those, if I'm honest. My sister keeps telling me I need to sort myself out, and yes, I have had a drink in the morning a few times just to stop feeling so awful.

Candidate: Thank you for your honesty. If you go a few hours or a day without a drink, do you get any physical symptoms, like shaking, sweating, a racing heart, or feeling sick? Have you ever seen or heard things that weren't there, or had a seizure when stopping? And have you noticed any new confusion, forgetfulness, or problems with your balance when walking, or any strange eye movements or double vision?

Sarah: Yes, I get shaky and sweaty if I go too long without a drink. I've never seen things or had a fit, thankfully. No confusion or balance problems, and my eyes feel normal too.

Common Pitfall: *"It sounds like you should just try stopping drinking completely from today — cold turkey is the quickest way to get this sorted."* — Dangerously advises abrupt cessation in someone showing signs of physical dependence, risking severe withdrawal seizures or delirium tremens.

Model Response: Explaining the Safe Pathway

Candidate: Thank you for answering all of that so honestly, Sarah — what you're describing does suggest your body has become physically dependent on alcohol. Because of that, I'd strongly advise against stopping suddenly on your own, as it can actually be dangerous and trigger severe withdrawal seizures. We need to do this safely and gradually, with the right support. I'm going to refer you urgently to our local Community Drug and Alcohol Team, who specialise in exactly this. Depending on your home situation, they may offer you a planned community detox — where a specialist nurse visits you daily and prescribes a tapering course of a calming medication called chlordiazepoxide to control withdrawal safely — or, if you're at higher risk, an inpatient detox with full monitoring.

Sarah: That actually makes sense. I did try to just stop by myself last year and ended up feeling absolutely terrible — maybe that's why.

Candidate: That's very possibly exactly why. I'm also prescribing you a daily high-dose thiamine tablet today — alcohol stops your body absorbing this vital vitamin properly, and taking it protects your brain and nerves from lasting damage. Once you've safely detoxed, the community team can also offer longer-term support, including CBT, local support groups like Alcoholics Anonymous, and sometimes a medication called acamprosate or naltrexone, which can help reduce cravings and the risk of relapse afterwards.

Sarah: I didn't realise there was actual medication that could help with cravings too. That's reassuring.

Your Next Step: Safety-Netting

Candidate: If you try to cut down and develop severe, uncontrollable shaking, a high fever, severe confusion, or start seeing things that aren't there, you must call 999 or go straight to A&E immediately — those would be signs of a dangerous withdrawal reaction needing emergency care.

Oral Examiner Follow-Up: The DVLA Question

Patient: *"Doctor, I drive a car for my job. Do I have to report my alcohol problem to the DVLA? Will I lose my licence?"*

Model Answer: I completely understand why you're worried about this, especially with your job depending on it. The direct legal answer is yes — in the UK, if someone is experiencing alcohol dependence, or persistent alcohol misuse causing problems, they have a legal duty to notify the DVLA. As your doctor, I can't contact the DVLA on your behalf right now, because of medical confidentiality — it's your responsibility to inform them yourself. They'll assess your case individually, and in cases of confirmed dependence, they'll typically suspend your licence for a period free from alcohol problems — often around a year, based on medical evidence, though the exact period is decided case by case. I do need to be clear, though: you must not drive while actively drinking heavily or experiencing withdrawal symptoms, as that puts your life and others at real risk. And if you were to continue driving against this advice and refuse to notify the DVLA yourself, I would be professionally obligated to break confidentiality and inform them directly, to protect public safety.

Station Conclusion: An "Excellent Pass" Alcohol Dependence Profile

Why your final choices scored maximum marks:

- **Accurate Unit Quantification:** You converted her drinking into UK units and compared it against the 14-units-a-week guidance.
- **Complete CAGE and Withdrawal Screen:** You screened for dependence using CAGE, and completed the full Wernicke's encephalopathy triad — confusion, balance problems, and eye movement abnormalities — not just two of the three.
- **Avoided the "Cold Turkey" Trap:** You explicitly warned against sudden self-directed cessation given signs of physical dependence, and arranged a safe, medically supervised detox instead.
- **Complete Pharmacological Picture:** You covered thiamine for neuroprotection during detox, and mentioned acamprosate/naltrexone for longer-term relapse prevention, not just the detox medication alone.
- **Accurate DVLA Guidance:** You correctly explained the legal duty to notify, patient confidentiality limits, and the doctor's override duty if the patient continues to drive unsafely.

SECTION: SEXUAL HEALTH & GUM

Station 37 — HIV Diagnosis: Breaking Bad News

This is a highly sensitive PLAB 2/UKMLA communication and counselling station. It tests your ability to break bad news compassionately using the SPIKES framework, explore the patient's ideas, concerns, and expectations, and deliver accurate, up-to-date facts about modern HIV management in the UK.

The HIV Breaking Bad News Blueprint

Setup → Warning Shot → Clear Delivery → Empathy Pause → ICE & Risk Assessment → Modern UK Management

The Roleplay

Station: Sexual Health / GUM Clinic

Candidate Role: FY2 Doctor in a GUM (Genitourinary Medicine) Clinic

Patient/Roleplayer Role: Sarah Thompson, 29 years old

Setting: Private Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Sarah Thompson, 29, attended for a routine sexual health screen last week and has returned today for her results.

Your Task:

- Confirm identity and set up the consultation appropriately.
- Deliver the HIV-positive result using a clear warning shot and simple language.
- Explore her ideas, concerns, and expectations, and screen for risk.
- Explain modern UK HIV management, including U=U.

(Sarah sits down, looking mildly anxious but composed, expecting a routine conversation.)

Candidate: Hello Ms Thompson, I'm Dr [Your Name], one of the clinic doctors today. Before we begin, can I just confirm your full name and date of birth? ... Thank you. Are you comfortable, and would you like anyone to sit in the room with us for this conversation?

Sarah: No, it's just me today. Thanks for asking though.

Candidate: Of course. Do you remember why we carried out the blood test last week?

Sarah: Yes, it was just part of the routine sexual health screen, wasn't it?

Candidate: That's right. I have the results back today, and I need to be direct with you — the results are not quite what we were hoping for. Is it alright if I share them with you now?

Sarah: (voice tightening) Okay... yes, go ahead.

Candidate: The blood test has come back positive for a virus called HIV — that stands for Human Immunodeficiency Virus.

(Sarah goes silent, staring at the floor, her eyes filling with tears. Several seconds of silence pass.)

Common Pitfall: *Filling the silence immediately with statistics, treatment details, or reassurance before the patient has had any chance to react — this rushes past the mandatory empathy pause and can feel dismissive of the shock she is experiencing.*

Model Response

Candidate: (waits, then speaks gently) I can see this is an incredible shock, and I am so sorry to have to bring you this news. Take all the time you need. I'm right here with you.

Sarah: (through tears) Am I going to die? Do I have AIDS?

Candidate: I want to reassure you right away: having HIV is not a death sentence, and it's completely different from AIDS. Today, HIV is treated as a manageable, long-term condition. With modern tablets, you can live a completely normal, healthy, and long life.

Your Next Step: Focused Risk Assessment and Public Health Screening

Candidate: To help me support you best, do you mind if I ask a few delicate but necessary questions? How have you been feeling physically lately — any fever, night sweats, unexplained weight loss, or a persistent cough? And hearing this news is an enormous emotional burden — have you had any thoughts of harming yourself or ending your life?

Sarah: No fevers or weight loss that I've noticed, thankfully. And no, I don't feel like hurting myself... just completely stunned.

Candidate: That's good to hear, and completely understandable that you're stunned. Because HIV is passed through bodily fluids, it's important that any current or past sexual partners are offered a test so they can be protected. We have a specialist team who can notify them anonymously if you'd prefer. Is there a partner we should think about?

Sarah: Yes... my boyfriend. I'll need to tell him. This is going to be a difficult conversation.

Your Next Step: Modern UK Management

Candidate: You won't be managing this alone. I'm making an urgent referral to our specialist HIV clinic team — you'll be seen within a few days. They'll start you on a daily tablet called antiretroviral therapy, which stops the virus multiplying in your body and keeps your immune system strong. The goal is to bring the level of virus in your blood down so low that it becomes undetectable — and once it's undetectable, you cannot pass the virus on to anyone else through sex. We call this Undetectable equals Untransmittable, or U=U. I'll also give you written information and contact details for support networks like the Terrence Higgins Trust.

Sarah: That's... actually a huge relief to hear. I thought this was going to completely destroy my life.

Your Next Step: Safety-Netting and Follow-Up

Candidate: If you ever feel overwhelmed, or feel you can't keep yourself safe, please contact us immediately, call 111, or go to A&E. I also don't expect you to remember everything we've

discussed today — I'd like to book a phone call to check in on you tomorrow morning, just to see how you're doing. How does that sound?

Sarah: Thank you, Doctor. Yes, please do call tomorrow. I think I'll need it.

Oral Examiner Follow-Up: Common Challenging Questions

Patient: *"Do I have to tell my boss at work about my diagnosis?"*

Model Answer: No — there's no general legal obligation to disclose your HIV status to an employer in the UK. HIV is protected under the Equality Act 2010, meaning you're protected against discrimination because of it. There are a small number of specific healthcare roles involving exposure-prone procedures with their own occupational health rules, but for the vast majority of jobs, this is entirely your private medical information and your choice whether to share it.

Patient: *"Can I ever have children in the future?"*

Model Answer: Yes, absolutely. With a fully suppressed, undetectable viral load, the U=U principle applies to conception too, meaning the risk of passing HIV to a partner while trying to conceive is negligible. Pregnancy itself is closely and safely managed by a specialist team, with antiretroviral therapy continued throughout to protect the baby, and most women with well-controlled HIV can have a normal vaginal delivery. Many women living with HIV go on to have healthy pregnancies and HIV-negative babies.

Station Conclusion: An "Excellent Pass" HIV Breaking Bad News Profile

Why your final choices scored maximum marks:

- **Correct SPIKES Structure:** You confirmed identity and privacy, delivered a clear warning shot, and paused in genuine silence rather than rushing to fill it.
- **Accurate, Reassuring Facts:** You immediately corrected the HIV/AIDS conflation and conveyed the modern reality of HIV as a manageable long-term condition.
- **Complete Risk and Mental Health Screen:** You screened for systemic symptoms and directly asked about self-harm and suicidal ideation.
- **Public Health Duty Handled Sensitively:** You raised partner notification without judgement, offering an anonymous route.
- **Mastery of U=U:** You explained Undetectable = Untransmittable clearly, which is one of the most heavily weighted communication points in this station.
- **Concrete Follow-Up:** You booked a specific follow-up call rather than leaving her to cope alone until her specialist appointment.

Station 38 — HIV in Pregnancy: Infant Feeding Counselling

This station tests a nuanced counselling skill: clearly stating the safest clinical recommendation under British HIV Association (BHIVA) guidelines, while respecting patient autonomy and supporting her final choice through shared decision-making, without ever implying she is "not allowed" to choose otherwise.

The Roleplay

Station: Antenatal / HIV Specialist Clinic

Candidate Role: FY2 Doctor / Specialist Midwife

Patient/Roleplayer Role: Sarah Adeyemi, 32 years old, pregnant and living with HIV

Setting: Antenatal Clinic Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Sarah Adeyemi, 32, is 30 weeks pregnant, living with HIV with a stable, undetectable viral load on treatment. She has come in to discuss infant feeding options.

Your Task:

- Clearly explain the safest feeding recommendation.
- Explain the practical NHS support available.
- Respond supportively if she wishes to breastfeed, without judgement.

Sarah: I've been thinking about how I want to feed the baby once she's born. I know I have HIV, but I really want to breastfeed if I can — it's a really important part of motherhood for me, and for my family.

Common Pitfall: *"You are not allowed to breastfeed because of your HIV — you must formula feed."* — The word "not allowed" frames this as a restriction on her autonomy rather than a medical recommendation, and fails the UK patient-centred communication standard.

Model Response

Candidate: That's a really important question, Sarah. In the UK, the official medical recommendation is to exclusively formula feed from birth, because HIV can be passed through breast milk, and formula feeding is the only way to guarantee zero risk of passing the virus to your baby. To support you with that, our clinic can arrange for the NHS to provide free formula milk, bottles, and sterilising equipment for your baby's first year, so cost isn't a barrier.

Sarah: I understand that's the safest option... but in my culture, everyone breastfeeds, and I know I'll get a lot of questions and judgement from my family if I don't. Is there really no way I could still do it safely?

Your Next Step: The Supportive Pathway if She Chooses to Breastfeed

Under BHIVA guidelines, a mother with a sustained undetectable viral load who, after full discussion, chooses to breastfeed should be supported to do so as safely as possible, rather than judged or refused care.

Candidate: I completely understand there can be real family and cultural pressure around this. If, after thinking it over, you do choose to breastfeed, our team will support you to do it as safely as possible. To do that, we'd need to meet three conditions: your viral load needs to remain completely undetectable throughout; both you and the baby need to stay free of any tummy upset, like diarrhoea or vomiting; and your breasts and nipples need to stay healthy, with no cracking, bleeding, or infections like mastitis. If you do breastfeed, it would need to be exclusive breast milk — no mixing with formula or solids — for a maximum of around 6 months, and both you and the baby would need monthly blood tests at our clinic to monitor viral levels and keep the baby completely safe.

Sarah: That actually makes me feel so much better — knowing there's a safe way to do it if I really want to, rather than it just being a flat no. I think I'll talk it over with my partner and the specialist team before deciding.

Oral Examiner Follow-Up: Detectable Viral Load While Breastfeeding

Examiner: *"What would you do if Sarah's viral load became detectable while she was breastfeeding?"*

Model Answer: If the viral load becomes detectable at any point during breastfeeding, she should stop breastfeeding immediately and switch to formula feeding, since the safety of continued breastfeeding depends entirely on sustained viral suppression. This should prompt an urgent review by the specialist HIV team to investigate the cause (such as adherence difficulties or drug resistance) and to arrange prompt testing and follow-up for the infant.

Station Conclusion: An "Excellent Pass" HIV Infant Feeding Profile

Why your final choices scored maximum marks:

- **Clear Safest-Option Messaging:** You stated formula feeding is the safest option directly and without hedging, while avoiding language implying a restriction on her autonomy.
- **Practical UK Support Explained:** You mentioned free NHS-provided formula, removing cost as a barrier to the safest choice.
- **Culturally Sensitive, Non-Judgemental Response:** You acknowledged the real cultural pressure she described without dismissing it or judging her question.
- **Accurate Conditional Breastfeeding Pathway:** You correctly outlined the three clinical conditions and the monitoring and duration limits required for supported breastfeeding under BHIVA guidance.
- **Safety-Netted the Detectable Viral Load Scenario:** You knew to stop breastfeeding immediately and escalate urgently if viral suppression is lost.

Station 39 — GUM Clinic: Gonorrhoea Result Disclosure and Management

A Gonorrhoea result disclosure and management station is a classic Genitourinary Medicine (GUM) scenario. It tests your ability to break sensitive infection news, take a respectful sexual history, deliver current UK antibiotic guidelines, and manage public health partner notification. UK management follows BASHH (British Association for Sexual Health and HIV) guidelines.

The Roleplay

Station: GUM Clinic Result Disclosure

Candidate Role: FY2 Doctor in a GUM Clinic

Patient/Roleplayer Role: Maria Santos, 24 years old

Setting: Private Consultation Cubicle

The Prompt (read outside the cubicle):

Patient Details: Maria Santos, 24, attended for a routine STI screen last week and has returned for her results.

Your Task:

- Disclose the result clearly and destigmatise the diagnosis.
- Take a focused sexual history to guide safe treatment.
- Deliver the correct BASHH treatment pathway.
- Manage partner notification and safety-netting.

(Maria sits stiffly, looking anxious about the appointment.)

Candidate: Hello Maria, my name is Dr [Your Name], one of the clinic doctors. Thank you for coming in for your result today. I've reviewed the swab we took last week, and it's come back positive for an infection called gonorrhoea. Have you heard of this before?

Maria: (eyes welling up) Oh no... I feel so dirty. I can't believe this is happening to me.

Common Pitfall: *Moving straight into treatment logistics without pausing to address the shame and distress in the room — this misses the Interpersonal Skills mark for destigmatising the diagnosis before continuing.*

Model Response

Candidate: I can see you look incredibly upset, and it's completely natural to feel shocked. Please take a breath, and be reassured: this is a very common, routine infection, and it's easily and fully curable with a single dose of antibiotics. It doesn't mean you've done anything wrong, and we'll get this sorted for you today.

Maria: (wiping her eyes) Okay... thank you. That actually helps a bit.

Your Next Step: Focused Sexual History

Candidate: To help me tailor your treatment safely, do you mind if I ask a few routine questions? Are you experiencing any lower tummy pain, pain during sex, or unusual bleeding between periods? Do you have a regular partner, or any new partners in the last few months? Were condoms used? And did any of that contact involve oral or anal sex, or just vaginal sex?

Maria: No tummy pain or unusual bleeding. I've had one new partner about three weeks ago — we didn't use condoms every time. It was vaginal and oral, no anal sex.

Your Next Step: The BASHH Treatment Pathway

Candidate: Thank you for being so open. The gold-standard treatment is a single injection of an antibiotic called Ceftriaxone into the muscle of your hip or bottom. Because the dose is fairly large, we mix it with a small amount of local numbing medicine called lidocaine to keep it as comfortable as possible. Because you've tested positive for gonorrhoea, our standard is to also run a quick screening panel for other infections like chlamydia, syphilis, and HIV, just to make sure you're fully covered — is that okay with you today?

Maria: Yes, that's fine. I'd rather know everything at once.

Your Next Step: Partner Notification and the 7-Day Rule

Candidate: This is a strict instruction: please completely abstain from any sexual contact until exactly 7 days after both you and your partner have finished treatment — having sex before that risks passing the infection straight back and forth. Because this is transmissible, any partners from the last 60 days need to be tested and treated too. There are two ways to do this: you can speak to your partner directly and ask them to come in, or if that feels difficult, our health advisor team can contact them completely anonymously, without ever mentioning your name, to invite them for a free check-up. Which option feels more comfortable for you?

Maria: I think I'd rather they contact him anonymously. I don't think I can have that conversation myself right now.

Your Next Step: Test of Cure and Safety-Netting

Candidate: That's completely understandable, and we'll arrange that for you. Because gonorrhoea has a history of developing antibiotic resistance, we'll also book you in for a routine test of cure swab in 2 weeks, just to formally confirm the infection has completely cleared. If you develop sudden, severe lower abdominal pain, a high fever, or notice any joint pain or skin rashes over the next few days, please seek urgent medical attention straight away.

Oral Examiner Follow-Up: Diagnostic Testing and Antibiotic-Pill Interaction

Examiner: *"How is gonorrhoea actually diagnosed under UK guidelines?"*

Model Answer: The gold-standard diagnostic method is a Nucleic Acid Amplification Test (NAAT), a highly accurate molecular test that detects the genetic material of *Neisseria gonorrhoeae*. The sample depends on anatomy: a first-catch urine sample (holding urine for at least 1–2 hours beforehand, catching the very first part of the stream) for patients with a penis, or a vulvovaginal swab — which can be clinician-taken or safely self-taken — for patients with a vagina. Extragenital swabs (pharyngeal or rectal) should also be taken based on sexual history, since gonorrhoea can infect the throat and rectum. UK labs typically run a dual NAAT that tests for both gonorrhoea and chlamydia on the same sample, since they often coexist. If the NAAT is positive, a bacterial culture is also usually performed, since it allows the lab to test exactly which

antibiotics will still work against that particular strain, given gonorrhoea's high rate of developing resistance.

Patient: *"I'm on the oral contraceptive pill. Will these antibiotics stop it from working?"*

Model Answer: No — Ceftriaxone, the antibiotic used to treat gonorrhoea, doesn't interact with or reduce the effectiveness of either the combined pill or the progestogen-only pill. Your regular contraception will keep working as normal. The only general exception to be aware of is if the antibiotic treatment itself causes vomiting within 2 hours of taking a pill, or severe diarrhoea lasting more than 24 hours — in that situation, treat it as a missed pill and use additional contraception, like condoms, for the following days.

Station Conclusion: An "Excellent Pass" Gonorrhoea Profile

Why your final choices scored maximum marks:

- **Direct, Destigmatising Disclosure:** You delivered the result clearly without stalling, then immediately normalised and reassured rather than letting shame dominate the consultation.
- **Complete Sexual History:** You screened for PID symptoms, partner history, protection use, and anatomical sites of exposure — the latter being a frequently missed mark.
- **Correct BASHH Pathway:** You delivered the exact current first-line treatment (Ceftriaxone 1g IM with lidocaine) and offered a full STI screen.
- **Thorough Public Health Management:** You explained the 7-day abstinence rule and offered both patient and provider referral options for contact tracing, letting Maria choose.
- **Test of Cure Awareness:** You booked a follow-up test of cure given gonorrhoea's resistance profile, rather than assuming the single dose guarantees cure without confirmation.

SECTION: PSYCHOSEXUAL MEDICINE

Station 40 — Psychosexual Medicine: Vaginismus

A Vaginismus station tests communication skills, empathy, and the ability to handle a sensitive sexual health history. The patient often presents with difficulty consummating a relationship, severe pain during intercourse, or inability to use tampons. Your goal is to create a safe, non-judgemental space, rule out organic causes of pain, validate her feelings, and explain the diagnosis clearly without medical jargon.

Sensitive History: Screening for Organic Causes

- **Characterising the pain:** when does it happen, and is it an aching pain or a sudden, tight "brick wall" sensation at the entrance?
- **Primary vs secondary:** have tampons or pelvic exams ever been possible? If never possible, this is primary vaginismus; if it started after a procedure or specific event, it's secondary.
- **Infection:** any unusual discharge, itching, or burning?
- **Endometriosis/PID:** deep pelvic pain on deep penetration, or severe period pain?
- **Dryness or irritation.**
- **Psychological history:** gently screen for past trauma or strict cultural/religious taboos around sex, without assuming either is present.
- **Relationship impact:** how is this affecting her and her partner — is he supportive?

The Roleplay

Station: GP Clinic / Sexual Health & Gynaecology Outpatients

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Priya Kapoor, 27 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Priya Kapoor, 27, has booked an appointment.

Presenting Complaint: She and her husband have been married for six months and have been unable to have sex due to pain and involuntary tightening.

Your Task:

- Take a sensitive history and rule out organic causes of pain.
- Explain the diagnosis of vaginismus clearly and without jargon.

- Offer a multidisciplinary management plan.

(Priya sits with her hands clasped tightly in her lap, avoiding eye contact.)

Candidate: Hello, I'm Dr [Your Name], one of the doctors today. How can I help you?

Priya: (hesitantly) My husband and I have been married for six months, but we just can't seem to have sex. It hurts too much, every single time we try.

Candidate: Thank you so much for sharing that with me, Priya. I know it can be incredibly difficult to talk about these things, but please know you're in a safe, confidential space, and we see many women experiencing exactly this.

Your Next Step: Characterising the Pain and Screening for Organic Causes

Candidate: Can you describe what happens when you try — is it more of an aching pain, or does it feel like a sudden, tight barrier right at the entrance? And have you ever been able to use tampons or have a smear test done in the past?

Priya: It's exactly like a tight wall — like my body just physically won't let anything in, and it panics me the moment we try. I've never been able to use tampons either, even as a teenager. I've just always avoided it.

Candidate: Thank you for explaining that so clearly. Have you noticed any unusual discharge, itching, or burning? Any deep pain during period times, or dryness or irritation?

Priya: No, nothing like that — no discharge or itching, and my periods are just normal cramping, nothing severe or unusual.

Candidate: That's helpful to know. Has anything happened in your past, or any painful medical procedures, that might make you feel anxious about intimacy?

Priya: Not really — nothing traumatic that I can think of. I think I've just always been really anxious about it, maybe from how I was raised to think of sex as something scary or shameful before marriage.

Candidate: Thank you for being so open with me. And how is this affecting you and your husband — is he supportive?

Priya: He's been patient so far, but I can tell he's frustrated too, and it's putting a real strain on us. I just feel like such a failure as a wife.

Common Pitfall: *Moving straight to arranging an internal pelvic examination at this point without checking readiness — pushing for an internal exam in this station is treated as an automatic patient-safety and empathy failure.*

Your Next Step: Explaining the Diagnosis

Candidate: Priya, please don't feel guilty at all — this is absolutely not your fault, and you are not broken. Based on what you've described — that tight, guarding sensation that stops anything from entering, and this happening every single time — it sounds like a condition called vaginismus. This is an automatic, involuntary tightening of the muscles around the vagina that happens completely outside of your control. Think of it a bit like your eye blinking automatically when dust flies toward it — your body is trying to protect you from anticipated pain, which creates a cycle of fear, tightening, and more fear. It's highly treatable, and sex isn't something to be rushed or treated as a test — it's about intimacy, and your body is simply responding to a fear of pain right now.

Priya: (visibly relieved) That actually makes so much sense... I thought there was something seriously wrong with me. So this can actually be treated?

Your Next Step: The Management Plan

Candidate: Yes, absolutely. There's no single quick fix, but a step-by-step approach that works very well. With your consent, I'd ideally like to offer a gentle external examination at some point to make sure everything looks healthy, but only once you feel completely ready — there's no pressure today. A key part of treatment is a set of vaginal trainers, or dilators — smooth, cone-shaped, and available in gradually increasing sizes. You'd start with the smallest, about the size of a fingernail, at home, in private, with plenty of lubricant, and build up in your own time to teach your body that this doesn't have to mean pain. I'd also refer you to a specialist pelvic floor physiotherapist, who can teach relaxation techniques, and to a psychosexual counsellor or CBT therapist, who can help work through any anxiety or fear around this. It can also really help to involve your husband in understanding what's happening, so you can work through it together without pressure.

Priya: Thank you, Doctor. Knowing there's an actual structured plan, and that I don't have to do an exam today, makes me feel so much less anxious about all of this. I'd like to try the trainers and see the counsellor.

Station Conclusion: An "Excellent Pass" Vaginismus Profile

Why your final choices scored maximum marks:

- **Non-Judgemental, Validating Tone Throughout:** You explicitly removed her guilt and normalised her experience rather than treating it purely as a physical problem to fix.
- **Thorough Organic Screen:** You screened for infection, endometriosis/PID, and dryness before settling on a psychosomatic diagnosis.
- **Clear, Jargon-Free Explanation:** You used an accessible analogy (the blinking eye) rather than clinical language, making the diagnosis feel understandable rather than frightening.
- **Respected Readiness Around Examination:** You offered an examination only with consent and on her timeline, avoiding the single most common automatic fail in this station.
- **Complete Multidisciplinary Plan:** You covered vaginal trainers, pelvic floor physiotherapy, and psychosexual counselling, and invited partner involvement, rather than offering just one isolated treatment.

SECTION: TRANSGENDER HEALTH

Station 41 — Transgender Health: Cervical Screening in a Trans Man

This station tests a high-value clinical and communication fact: a trans man who still has a cervix requires the same routine cervical screening as any cisgender woman. The station evaluates your ability to navigate gender dysphoria sensitively, explain a genuine administrative gap in the NHS screening system, and offer practical modifications to make an intimate examination physically and emotionally manageable.

Focused History

- **Pronouns and terminology:** establish respectful communication immediately — ask preferred name and pronouns, and avoid anatomical terms like "vagina" or "smear" unless the patient uses them first; refer neutrally to "the screening test."
- **Surgical history:** discreetly check whether he has had a total hysterectomy (removal of the womb and cervix) — if the cervix has been removed, he is exempt from screening; if intact, he still needs it.
- **Testosterone therapy:** long-term testosterone causes tissue thinning (atrophy), which can make the physical exam more uncomfortable and occasionally affects sample quality.

The Administrative Gap

If a patient's NHS record is updated to male, the national screening call/recall algorithm typically stops sending automatic screening invitations. This is a system limitation, not a clinical one: the virus responsible for cervical changes (HPV) does not care about gender identity, and the risk remains identical to a cisgender woman if a cervix is present. A GP practice can manually flag the record or arrange direct recall so the patient continues to be invited for screening at the normal interval, rather than falling through the administrative gap.

The Roleplay

Station: General Practice / Preventive Care

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Alex Turner, 34, a trans man

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Alex Turner, 34, has been sent a letter about an overdue cervical screening test, despite his NHS record showing his gender as male.

Your Task:

- Establish respectful, patient-led communication.
- Explain why screening is still clinically necessary.
- Explain the administrative gap and how to fix it.
- Offer practical modifications to make the test manageable.

(Alex sits with arms crossed, looking defensive and uncomfortable.)

Candidate: Hello, before we begin, may I ask your preferred name and pronouns?

Alex: Alex, and he/him, thanks. Look, I got this letter about a smear test, and honestly, it felt like a slap in the face. My records say male — why am I even getting this?

Common Pitfall: *"Well, biologically you still have a cervix, so you need this test regardless of how you identify." — Technically true but delivered without any acknowledgement of his distress, likely to make him feel unseen and defensive rather than supported.*

Model Response

Candidate: I really appreciate you telling me that, Alex, and I'm sorry this letter landed the way it did — that's a completely understandable reaction. Can I ask, have you ever had any major surgery, such as removal of your womb or cervix?

Alex: No, I haven't had that surgery. I'm on testosterone and I've had top surgery, but not anything below.

Candidate: Thank you for telling me. Because your cervix is still there, the risk of the changes this test looks for is exactly the same as it would be for anyone else with a cervix — it has nothing to do with gender identity, it's about which organs are physically present. How long have you been on testosterone?

Alex: About four years now.

Your Next Step: Explaining the Administrative Gap

Candidate: That's helpful to know, since long-term testosterone can thin the tissue there, which we can easily plan around. I also want to explain something important: because your NHS record shows your gender as male, the national system doesn't automatically send you reminders the way it would for most people with a cervix — that letter you got was likely a bit of a fluke, or you might otherwise stop hearing from them altogether. That's a gap in the system, not a reflection of your risk. What I can do is manually flag your record here at the practice, so you keep being invited at the normal interval, rather than relying on chance.

Alex: I had no idea that could just... stop. That's actually really good to know. I don't want to end up missing something important because of a computer error.

Your Next Step: Offering Practical Modifications

Candidate: I completely understand this test can feel invasive and bring up a lot of difficult emotions, especially with the tissue changes from testosterone. We can make this entirely on your terms: we'll use the smallest speculum available, plenty of lubricant, and you're welcome to insert it yourself behind the curtain if that feels better, or ask me to stop at any point. We can also book it right at the start or end of clinic for maximum privacy in the waiting room. Would any of that help?

Alex: Actually, yes. Being able to do it myself would make a huge difference. I think I could manage that.

Your Next Step: Holistic Screening Review

Candidate: While we're talking about this, it's worth quickly checking your other routine screening too. Since you don't have a full double mastectomy, if you're between 50 and 71 you'd still be invited for routine mammograms every three years, so that's worth keeping in mind for the future. And because AAA screening is only automatically offered to registered males at 65, and your risk profile differs from that of someone assigned male at birth, that's something we can discuss on an individual basis nearer the time, rather than assuming either way.

Alex: Thanks for explaining all of that — I really wasn't expecting today to be this thorough, in a good way.

Station Conclusion: An "Excellent Pass" Trans Health Screening Profile

Why your final choices scored maximum marks:

- **Respectful, Patient-Led Communication:** You asked for pronouns immediately and used neutral language rather than assuming anatomical terminology.
- **Validated Distress Before Explaining Facts:** You acknowledged how the letter felt before moving into the clinical rationale, rather than leading with facts alone.
- **Correctly Separated Clinical Risk From Gender Identity:** You explained that screening need is based on which organs are present, not gender identity, without being clinical or cold about it.
- **Identified and Fixed the Administrative Gap:** You recognised the automatic recall system's limitation and offered a concrete practice-level fix rather than leaving him to rely on chance.
- **Offered Genuine Patient Control:** You offered the smallest speculum, self-insertion, and flexible timing, giving Alex real agency over the examination rather than just describing it.
- **Holistic Screening Review:** You proactively checked breast/chest and AAA screening needs rather than treating cervical screening as an isolated issue.

Station 42 — Transgender Health: Cancer Screening in a Transgender Woman

Counselling a transgender patient about cancer screening tests your organ-specific anatomical knowledge alongside inclusive, respectful communication. The key trap in this station is assuming NHS automated screening software works correctly for trans patients by default — invitations are driven by the gender marker on the GP record, not by which organs are physically present, and you must proactively explain and correct for this.

Key Clinical Facts

- **The prostate is not removed during standard gender-affirming surgery:** a transgender woman who has had vaginoplasty or is on long-term feminising hormone therapy still has her prostate — it is deliberately left in place, since it is deeply integrated with the urinary sphincter and surrounding nerves.
- **No national UK prostate screening programme exists for anyone:** PSA testing is done on an individual, symptom- or request-led basis, the same for a transgender woman as for a cisgender man.
- **Long-term oestrogen therapy shrinks the prostate:** this lowers the baseline PSA reading, so standard reference ranges may not directly apply and should be interpreted with this in mind.
- **Feminising hormone therapy increases breast cancer risk:** compared to a cisgender man, though generally still lower than a cisgender woman. In the UK, those registered as female aged 50–70 are automatically invited for mammography every 3 years; those registered as male are not automatically invited even if on long-term feminising hormones.

The Roleplay

Station: General Practice / Preventive Care

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Maya, a transgender woman

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Maya has booked an appointment to discuss prostate and breast cancer screening.

Your Task:

- Establish respectful, inclusive communication.
- Explain the NHS screening system's reliance on registered gender marker.
- Explain the clinical realities of prostate and breast screening for her specifically.
- Offer safety-netting and a clear plan.

Candidate: Hello, my name is Dr [Your Name]. Thank you for coming in today. Before we begin, can I confirm your full name and how you'd prefer me to address you?

Maya: My name is Maya, and I use she/her pronouns.

Candidate: Thank you, Maya. I see you'd like to discuss prostate and breast screening today. I understand discussing these specific body parts can sometimes feel uncomfortable, but my goal is simply to make sure you get the best possible care to stay safe and healthy. Is it alright if we go through both today?

Maya: Yes, that's fine — I just want to understand what actually applies to me.

Your Next Step: The IT/Record System Warning

Candidate: Before we look at the specific tests, it's important you understand how the NHS system works. Screening invitations are sent out automatically based strictly on whether you're registered as male or female with your GP — not based on which organs you actually have. If your record has been updated to female, the system won't automatically invite you for prostate monitoring, but it will affect how your breast screening invitations work. So we need to manually tailor a plan together to make sure nothing falls through the gap.

Maya: I didn't realise the computer worked that way. That's actually a bit worrying.

Common Pitfall: *"Since you've transitioned and had surgery, you don't need to worry about prostate problems anymore." — Factually wrong and potentially dangerous: the prostate is not removed during vaginoplasty, and this reassurance could lead to a missed or delayed cancer diagnosis.*

Model Response: Prostate Screening

Candidate: Let's start with prostate health. Even after transition or gender-affirming surgery, the prostate gland itself isn't physically removed — it stays in place. That means, although the risk is low, it's still possible to develop prostate problems, including prostate cancer. Because there's no automated national screening programme for prostate health in the UK for anyone, we manage this the same way we would for any man — by staying alert to symptoms and arranging tests like a PSA blood test or physical examination if you ever notice any changes in your urination. One extra thing worth knowing: long-term oestrogen therapy tends to shrink the prostate, which can lower your usual PSA reading, so we'd always interpret any test with that in mind.

Maya: Oh, I really didn't know the prostate was still there. That's useful to know so I can watch out for anything unusual.

Model Response: Breast Screening

Candidate: Moving on to breast health — long-term hormone therapy stimulates breast tissue growth, which means your breast cancer risk increases over time compared to a cisgender man, even though it's generally still lower than for a cisgender woman. In the UK, if you've been on feminising hormones for several years and you're between 50 and 70, the recommendation is routine mammograms every 3 years. If your record is registered as female, you'll get an automatic invitation letter when you turn 50. If it still says male, the computer will bypass you completely — in that case, I'd add a manual reminder to your file here at the clinic and write you a direct referral to the local breast screening unit myself, so you don't miss out.

Maya: My record currently says male, so I'm really glad you mentioned that — I would never have known to ask.

Your Next Step: Self-Care and Safety-Netting

Candidate: Alongside formal screening, it's worth being breast-aware generally — gently checking every so often for any new lumps, skin puckering, or nipple discharge. And regardless of when your next screening is due, if you ever notice a new breast lump, or develop urinary changes

like difficulty passing urine or blood in your urine, please don't wait for a screening invitation — come straight back for an urgent assessment.

Oral Examiner Follow-Up: Prostate Examination After Vaginoplasty

Patient: *"Doctor, I had a full vaginoplasty a few years ago. If a doctor ever needs to check my prostate physically, how is that even possible now? My anatomy is completely different."*

Model Answer: That's an excellent, very practical question, and a common thing to wonder about. When a vaginoplasty is performed, the surgeon reconstructs the external anatomy, but the internal organs, including the prostate, stay in their original position. In a body that hasn't had this surgery, the prostate sits just in front of the back passage, which is why it's normally checked by gently examining through the rectum. After vaginoplasty, the prostate is still in that exact same physical spot, but because the newly constructed vaginal canal now sits directly next to it, the prostate can also be safely and effectively felt through the vagina. If this examination were ever needed, for example if you developed urinary symptoms, the choice of which route to use would be entirely yours, we'd use plenty of lubricating gel, offer a chaperone of your choosing, and you'd be in complete control of the examination throughout.

Maya: Wow, okay — I didn't realise it could be checked through the vagina as well. That's actually really reassuring to know, thank you.

Station Conclusion: An "Excellent Pass" Trans Cancer Screening Profile

Why your final choices scored maximum marks:

- **Anatomically Accurate Throughout:** You never implied the prostate is removed during standard gender-affirming surgery, and correctly explained that its physical position is unchanged after vaginoplasty.
- **Corrected the IT System Assumption:** You proactively explained that screening invitations follow the registered gender marker, not actual organs present, and offered concrete manual fixes for both prostate and breast screening.
- **High-Level Clinical Nuance:** You mentioned that long-term oestrogen lowers baseline PSA, showing genuine depth beyond a surface-level answer.
- **Calm, Non-Sensationalised Delivery:** You explained the post-vaginoplasty examination routes factually and calmly, as you would any other routine physical exam, rather than treating it as awkward or unusual.
- **Patient Control Emphasised:** You made clear that the choice of examination route, and the choice of chaperone, would always be hers.

SECTION: PAEDIATRIC INFECTIOUS DISEASE

Station 43 — Paediatrics: Chickenpox (Varicella) Counselling

A Chickenpox station tests your ability to safely manage an infectious child while addressing parental anxiety, ruling out complications, and providing clear public health advice. You will typically speak to the parent rather than the child directly, and must structure the consultation logically to score on data gathering, clinical management, and communication.

Focused History

- **The rash:** when did it start, where did it begin (usually face/chest), and how has it evolved (spots → itchy blisters → crusts)?
- **Preceding symptoms:** fever, runny nose, tiredness, or poor appetite before the rash appeared.
- **Exposure:** has the child been around anyone with a similar rash recently, e.g. at nursery or school?

Red Flags to Screen For

- **Dehydration:** is the child drinking fluids, and are nappies still wet?
- **Chest infection:** any cough or difficulty breathing?
- **Skin infection:** thick yellow pus, or skin around the blisters very red, hot, and swollen?
- **Neurological involvement:** unusually drowsy, unsteady on their feet, or crying constantly?

Past Medical History and Household Risk

- **Immunocompromise:** does the child have any long-term illness or take steroid medication?
- **Household risk:** is anyone at home pregnant, a newborn baby, or living with a weakened immune system?

The Roleplay

Station: General Practice / Paediatric Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Mrs Wilson, mother of 4-year-old Jack

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Jack, 4, brought in by his mother.

Presenting Complaint: An itchy, spotty rash that started on his chest two days ago and has since spread and begun blistering.

Your Task:

- Take a focused history and screen for red-flag complications.
- Explain the diagnosis and safe symptomatic management.
- Give clear infection-control and safety-netting advice.

(Mrs Wilson looks anxious, with Jack sitting beside her, scratching at spots on his arms.)

Candidate: Hello Mrs Wilson, I'm Dr [Your Name]. I can see you're worried about Jack's rash — let's talk it through, and we'll make sure he gets the right care. When did you first notice the spots, and how have they changed?

Mrs Wilson: It started two days ago, just a few spots on his chest, but now it's spread to his face, back, and arms. Some of them have turned into little blisters, and he won't stop scratching. He had a mild fever and seemed a bit off for a day before the spots appeared.

Candidate: Thank you for describing that so clearly. Has he been around anyone else with a similar rash recently, at nursery or elsewhere?

Mrs Wilson: Actually yes, a boy in his nursery class had chickenpox about two weeks ago.

Your Next Step: Screening for Red Flags

Candidate: Is he still drinking fluids normally, and are his nappies or trips to the toilet as usual? Any cough or trouble breathing? Are any of the blisters oozing thick yellow pus, or is the skin around them very red, hot, or swollen? And has he seemed unusually drowsy, unsteady, or difficult to settle?

Mrs Wilson: He's drinking fine and still going to the toilet normally. No cough or breathing problems. The spots just look like normal blisters, no pus or angry redness around them. He's a bit grumpy from the itching, but otherwise alert and playing as normal.

Your Next Step: Past History and Household Risk

Candidate: Does Jack have any long-term health conditions, or take any steroid medication? And is there anyone at home who is pregnant, a very young baby, or someone with a weakened immune system?

Mrs Wilson: No health conditions, and he's not on any regular medication. Actually — my sister is 20 weeks pregnant and was round at our house yesterday, before we knew what this was. Should I be worried about her?

Common Pitfall: *"Just tell your sister to avoid the house for now — she should be fine." — Dismisses a genuine exposure risk in a susceptible pregnant contact without recommending any formal assessment, missing an important safety point.*

Model Response

Candidate: That's really important that you've mentioned that — thank you. Because chickenpox can occasionally cause serious complications in pregnancy if your sister hasn't had it before, she should contact her GP or midwife urgently today to find out whether she's immune, and if not, they can assess her for a medicine called Varicella-Zoster Immunoglobulin, which can help protect her if given promptly after exposure. It's best she doesn't wait to see if she develops symptoms.

Mrs Wilson: Oh, I didn't realise it could be serious for her — I'll call her right now to get her checked.

Your Next Step: Explaining the Diagnosis and Management

Candidate: Based on everything you've described — the fever before the rash, the spots turning into itchy blisters, and a known exposure at nursery — this is a classic case of chickenpox. It's a very common childhood illness, caused by a virus called varicella-zoster, and it usually clears up on its own within a week or two. For his fever or discomfort, please use paracetamol rather than ibuprofen — it's really important we avoid ibuprofen specifically in chickenpox, as it can increase the risk of the skin blisters becoming seriously infected. For the itching, calamine lotion and cool baths with a little bicarbonate of soda can help, and keeping his fingernails short will reduce damage from scratching. Make sure he keeps drinking plenty of fluids, and ice lollies can be an easy way to get extra fluids in if he's not keen on plain water.

Mrs Wilson: Good to know about the ibuprofen — I actually had some in the cupboard I was about to give him. I'll stick to paracetamol instead.

Your Next Step: Infection Control and Safety-Netting

Candidate: Jack will need to stay away from nursery until every single blister has turned into a dry crust — usually around 5 to 6 days after the rash first appeared. Please also keep him away from anyone pregnant, any newborn babies, or anyone with a weakened immune system until then, and it's best he doesn't fly on a commercial flight while he's still contagious. Do come back, or take him straight to the emergency department, if the spots become increasingly red, swollen, hot, or painful, if he develops breathing difficulties or a bad cough, if he becomes unusually drowsy or difficult to wake, or if he stops drinking fluids and shows signs of dehydration, like a dry mouth or no wet nappies.

Mrs Wilson: That's really clear, thank you. I feel much more confident about looking after him now, and I'll ring my sister straight away too.

Exam Pearl: Chickenpox vs Genital Herpes — Same Viral Family, Different Spread

Both chickenpox and genital herpes are caused by viruses in the herpesvirus family, but they spread in very different ways — a distinction occasionally tested as an oral follow-up question.

- **Chickenpox (varicella-zoster virus):** highly contagious and spreads through airborne respiratory droplets (coughing, sneezing, even talking in the same room), as well as through direct contact with fluid from the blisters. A person with shingles, a reactivation of the same virus, can also transmit chickenpox to someone who has never had it, through contact with their blister fluid.
- **Genital herpes (usually HSV-2, sometimes HSV-1):** does not spread through the air or casual contact. It's transmitted through direct skin-to-skin contact during sexual activity, including areas a condom may not cover, and can be passed on even when there are no visible sores, since the virus can shed silently from the skin. A cold sore on the mouth can also be passed to a partner's genitals through oral sex.

Neither virus survives long outside the body, so reassurance that genital herpes cannot be caught from toilet seats, swimming pools, or shared towels is accurate and often appreciated by anxious patients. A full Genital Herpes diagnosis/counselling session — covering antiviral treatment and disclosure conversations — can be built separately if useful.

Station Conclusion: An "Excellent Pass" Chickenpox Profile

Why your final choices scored maximum marks:

- **Complete Red-Flag Screen:** You screened for dehydration, chest infection, secondary skin infection, and neurological involvement before reassuring.
- **Caught the Household Risk:** You identified the pregnant contact and gave a concrete, urgent next step (contacting her GP/midwife for immunity assessment and possible VZIG), rather than vague avoidance advice.
- **Critical Safety Point on Ibuprofen:** You proactively warned against ibuprofen and explained why, catching a genuine prescribing error the parent was about to make.
- **Clear, Jargon-Free Public Health Guidance:** You gave a specific, concrete exclusion rule (crusted over, ~5–6 days) rather than a vague "stay off school for a while."
- **Watertight Safety-Netting:** You gave specific, actionable red flags for return rather than a generic "come back if worried."

SECTION: SIMMAN EMERGENCY MEDICINE

The ABCDE SimMan Framework

SimMan (simulation mannequin) stations test your ability to manage acute, life-threatening emergencies. You will find a high-fidelity mannequin connected to a vital signs monitor, with a nurse standing by to assist. The key to passing is following the structured ABCDE (Airway, Breathing, Circulation, Disability, Exposure) approach established by the UK Resuscitation Council — and acting it out in real time: talking to the patient, commanding the nurse, and reporting findings to the examiner. You've already seen this framework applied in Station 3 (Postpartum Haemorrhage) and Station 6 (Ruptured Ectopic Pregnancy); the two stations below apply the same structure to a medical and a surgical emergency respectively.

Step 0: Preparation (First 30 Seconds)

- Sanitise your hands and put on an apron and gloves.
- **Assess responsiveness:** "Hello, can you hear me? My name is Dr [Name]."
- **Call for help early if unstable:** "This patient looks critically unwell. Please call the medical emergency team, put out a crash call if necessary, and bring the crash trolley."
- Check the monitor for a baseline heart rate, blood pressure, and oxygen saturation.

A — Airway

- If the patient answers clearly, the airway is patent.
- If unresponsive or making gurgling/snoring noises, look inside the mouth for secretions, foreign bodies, or tongue swelling.
- Command the nurse to apply high-flow 15L oxygen via a non-rebreather mask; perform a head-tilt chin-lift or jaw thrust, or ask for a Guedel airway, if obstructed.

B — Breathing

- Inspect for symmetrical chest expansion, respiratory distress, or tracheal deviation.
- Auscultate the lungs (the examiner will tell you what you hear).
- Check respiratory rate and SpO2 on the monitor.
- Intervene based on findings — nebulisers for wheeze, IV diuretics for fluid overload.

C — Circulation

- Feel central and peripheral pulses for rate, rhythm, and character.
- Check capillary refill time (press for 5 seconds, count aloud).

- Check skin temperature and colour, blood pressure, and heart rate.
- Secure two wide-bore IV cannulas, draw a baseline blood panel (FBC, U&Es, clotting, lactate, crossmatch), and start a fluid challenge if hypotensive and not fluid-overloaded.

D — Disability

- Assess AVPU (Alert, Voice, Pain, Unresponsive).
- Check pupils are equal and reactive to light.
- **Always check blood glucose:** this is a mandatory step in any acutely unwell patient, to rule out hypoglycaemia.

E — Exposure

- Expose the body systematically, maintaining dignity, to check for rashes, scars, wounds, calf swelling, or bleeding.
- Check core temperature.
- Cover the patient back up promptly to prevent hypothermia.

Reassessment and Escalation

After any intervention (fluids, drugs, oxygen changes), reassess from A to E to check whether parameters have improved. Stations typically conclude with an SBAR handover to a senior doctor by phone or in person.

Station 44 — SimMan: Acute Heart Failure (Pulmonary Oedema)

Acute heart failure in a SimMan station usually presents as acute pulmonary oedema — fluid backing up into the lungs because the heart cannot pump effectively, leaving the patient severely breathless, anxious, and hypoxic.

The Roleplay

Station: Acute Medical Emergency (SimMan)

Candidate Role: FY2/SHO Doctor called to the ward

Roleplayer Role: A nurse in the room; the mannequin represents Mr Smith, 65, acutely breathless

Setting: Acute Medical Unit

The Prompt (read outside the cubicle):

Patient Details: Mr Smith, 65, known ischaemic heart disease, has become suddenly and severely breathless over the last hour.

Your Task:

- Assess and manage using an ABCDE approach.
- Identify the diagnosis and initiate immediate treatment.
- Escalate appropriately.

(The mannequin is sitting bolt upright, working hard to breathe, looking panicked. A nurse stands by.)

Candidate: Hello, I'm the doctor. Can you tell me what's happened?

(The patient can only manage short, breathless phrases — the airway is patent.)

Step B: Breathing

The monitor shows tachypnoea and hypoxia. Auscultation reveals bilateral coarse crackles.

Candidate: Please sit him fully upright if he isn't already, and apply high-flow oxygen via a non-rebreather mask at 15 litres per minute immediately.

Step C: Circulation

The patient is tachycardic. On examination, the JVP is elevated and there is pitting oedema at the ankles.

Candidate: Please get me an urgent 12-lead ECG to check whether a heart attack has triggered this. Insert two large-bore IV cannulas and draw bloods — full blood count, renal function, troponin, and a BNP level.

Step D & E: Disability and Exposure

Check pupils, blood glucose, and temperature; examine the chest and legs fully while keeping the patient warm and maintaining dignity.

Common Pitfall: *"His lungs sound wet and his oxygen is low — let's start a fluid bolus to support his blood pressure."* — This is a dangerous, exam-failing error. Giving IV fluids to a patient in acute pulmonary oedema is like pouring water onto an already-soaked sponge: it worsens the fluid overload in the lungs and can precipitate cardiac arrest. The entire treatment goal here is fluid removal, not fluid resuscitation.

Model Response: Immediate Treatment

Candidate: This is acute pulmonary oedema. He's already sitting upright on high-flow oxygen. I'm now giving IV furosemide 40 to 80 milligrams now — this is the single most critical step, and will help his kidneys start clearing the excess fluid. I want a strict fluid balance chart and a urinary catheter inserted so we can monitor his output hourly.

Examiner Follow-Up: **"What about IV morphine — shouldn't that be given too?"**

Model Answer: Historically, IV morphine was used routinely in acute pulmonary oedema to relieve distress and cause mild vasodilation. However, current evidence links routine opioid use in this setting to worse outcomes, including a higher rate of needing mechanical ventilation, ICU admission, and mortality, so it is no longer recommended as a routine step. It should not be given by default — the priority is oxygen, sitting the patient upright, and IV diuretics. Analgesia or anxiolysis would only be considered case-by-case, and typically with senior input.

Your Next Step: Senior Escalation and SBAR

Candidate: I'm calling the Medical Registrar immediately — this patient has acute pulmonary oedema and needs senior review.

Candidate (Situation): I have a 65-year-old man with acute pulmonary oedema.

Candidate (Background): He has a history of ischaemic heart disease and presented with sudden, severe breathlessness.

Candidate (Assessment): His oxygen was 88% on room air, he has widespread crackles, and his JVP is elevated.

Candidate (Recommendation): I've sat him upright, started 15L oxygen, and given IV furosemide. I'd like you to review him for possible non-invasive ventilation (CPAP) if his hypoxia doesn't improve.

Oral Examiner Follow-Up: Digoxin in Heart Failure

Examiner: *"When would you consider Digoxin in a heart failure patient?"*

Model Answer: Digoxin is not a first-line heart failure drug, and it doesn't improve survival, but it's effective at reducing hospital readmissions and controlling symptoms in two situations. First, and most commonly, when heart failure coexists with atrial fibrillation — Digoxin slows the heart rate, giving the heart more time to fill between beats, and is particularly useful if blood pressure is too low to tolerate a beta-blocker. Second, as an add-on in a patient already established on the four pillars of heart failure treatment — an ACE inhibitor or ARNI, a beta-blocker, an MRA like spironolactone, and an SGLT2 inhibitor, alongside a loop diuretic — whose symptoms remain severe in normal sinus rhythm; here I'd want to seek cardiology advice before starting it. Digoxin is renally cleared, so I'd check renal function first and use a lower dose in renal impairment or in

elderly patients, and I'd want to confirm potassium is normal, since hypokalaemia — common with loop diuretics like furosemide — significantly increases the risk of digoxin toxicity.

Station Conclusion: An "Excellent Pass" Acute Heart Failure Profile

Why your final choices scored maximum marks:

- **Avoided the Fluid Trap:** You recognised that IV fluids are contraindicated in fluid-overloaded acute heart failure, and instead focused entirely on fluid removal.
- **Correct, Safe Diuretic Dosing:** You gave the accurate standard dose of IV furosemide (40–80mg), not an excessive or dangerous dose.
- **Up-to-Date on Morphine:** You knew that routine opioid use is no longer recommended in acute pulmonary oedema, rather than reciting outdated teaching.
- **Systematic ABCDE With Correct Interventions:** You matched each letter of the assessment to the correct immediate action — oxygen, ECG and bloods, fluid balance monitoring.
- **Structured Escalation:** You gave a clear, complete SBAR handover with a specific, actionable recommendation (CPAP review).

Station 45 — SimMan: Acute Limb Ischaemia

Acute Limb Ischaemia (ALI) tests your ability to rapidly recognise a surgical emergency, manage a deteriorating patient, and communicate effectively under pressure. The mannequin will present with sudden-onset severe leg pain, and you must identify the classic "6 Ps": Pain, Pallor, Pulselessness, Paraesthesia, Paralysis, and Perishingly cold skin.

The Roleplay

Station: Acute Surgical Emergency (SimMan)

Candidate Role: FY2/SHO Doctor in the Emergency Department or Acute Medical Unit

Roleplayer Role: The mannequin represents a patient with sudden severe leg pain, coldness, and numbness

Setting: Emergency Department

Your Task:

- Perform a structured ABCDE assessment.
- Identify acute limb ischaemia and initiate immediate management.
- Escalate to vascular surgery within the correct time window.

(The mannequin is grimacing in severe pain, clutching at a pale, cold leg.)

Candidate: Hello, can you hear me? What's your name?

(The patient responds clearly — airway is patent.)

Step B & C: Breathing and Circulation

Candidate: Apply 15L oxygen via non-rebreather mask if he's hypoxic or in severe distress. I'll auscultate the chest to rule out any other acute pathology.

On circulation, expect tachycardia from severe pain, or an underlying arrhythmia — atrial fibrillation is the classic embolic cause of ALI.

Candidate: Insert two large-bore IV cannulas. Draw bloods — full blood count, renal function, clotting profile, group and save, crossmatch, a lactate level to check for muscle ischaemia, and fasting glucose and lipids. I also want an urgent 12-lead ECG to look for atrial fibrillation or an acute MI.

Step D & E: Disability and the Focused Limb Exam

Candidate: Checking AVPU, pupils, and blood glucose. I'm now exposing both lower limbs fully to compare them.

On examination, the affected limb lacks peripheral pulses.

Candidate: I'm checking specifically for the 6 Ps: pain, pallor, pulselessness, paraesthesia, paralysis, and perishingly cold skin. Since I can't feel pulses on this side, I'd like a handheld Doppler to confirm.

Common Pitfall: *"Let's elevate the leg to help reduce the swelling and ease the pain." — This is the opposite of correct management. Elevating the leg is right for venous disease, but in acute arterial ischaemia it worsens perfusion, since gravity is assisting what little arterial flow remains. The leg should be kept level or slightly dependent, never elevated.*

Model Response: Immediate Management

Candidate: This is acute limb ischaemia. For his pain, I'm giving IV morphine, titrated carefully in small increments, alongside an antiemetic like metoclopramide, since this level of ischaemic pain is excruciating. I'm starting an immediate IV unfractionated heparin bolus to prevent the clot propagating further, followed by a continuous infusion per local protocol. I'll also start IV crystalloid fluids to protect his kidneys, since ischaemic muscle can release breakdown products that risk rhabdomyolysis. Importantly, keep the leg level or slightly dependent — do not elevate it, as that would worsen the ischaemia.

Your Next Step: Urgent Vascular Escalation

Candidate: This is a surgical emergency. I'm phoning the Vascular Surgery Registrar on-call now: I have a patient with acute limb ischaemia who needs urgent imaging with a CT angiogram or immediate surgical revascularisation, ideally within the 4-to-6-hour window before the tissue becomes non-viable.

Candidate: (to the patient) I know you're in severe pain — we've given you strong pain relief and we're working quickly to help your leg. The blood flow has suddenly been blocked, likely by a small clot, which is why it's painful and cold. I've already contacted the specialist leg surgeons, and they'll see you very soon. We'll likely need a quick scan of the blood vessels, and may need to take you to theatre to clear the blockage and restore the blood flow.

Station Conclusion: An "Excellent Pass" Acute Limb Ischaemia Profile

Why your final choices scored maximum marks:

- **Explicit 6 Ps Assessment:** You clearly verbalised checking for all 6 Ps rather than a vague "the leg looks unwell."
- **Correctly Avoided Elevating the Limb:** You recognised this is the opposite of venous disease management, and kept the leg level or dependent instead.
- **Early Anticoagulation:** You initiated heparin immediately rather than waiting for the vascular team to arrive.
- **Doppler and ECG Included:** You requested a Doppler to confirm absent pulses, and an ECG to look for atrial fibrillation as the likely embolic source.
- **Time-Critical Escalation:** You correctly framed this as a surgical emergency with a defined viability window, rather than a routine referral.
- **Maintained Compassionate Communication:** Even while managing a high-acuity emergency, you continued to speak directly and reassuringly to the patient.

Station 46 — SimBaby: Severe Respiratory Distress in an Ex-Preterm Infant

A 4-month-old ex-preterm infant presenting with shortness of breath, chest retractions, fever, and significant hypoxia is an acute paediatric emergency at high risk of rapid respiratory failure. This SimBaby station tests a structured ABCDE approach combined with immediate senior escalation, applied to an infant rather than an adult mannequin.

The Roleplay

Station: Paediatric Emergency (SimBaby)

Candidate Role: FY2/SHO Doctor in the Paediatric Emergency Department

Roleplayer Role: A nurse in the room; the mannequin represents a 4-month-old infant, born prematurely, in respiratory distress

Setting: Paediatric Emergency Department

The Prompt (read outside the cubicle):

Patient Details: A 4-month-old infant, born prematurely, presenting with shortness of breath, chest retractions, a temperature of 38°C, and an oxygen saturation of 90%.

Your Task:

- Perform an immediate ABCDE assessment and stabilise the infant.
- Identify the likely diagnosis and escalate appropriately.
- Initiate a safe immediate management plan.

(The mannequin infant is working hard to breathe, with visible chest retractions, and a nurse is standing by.)

Step A: Airway

Candidate: I'm checking the airway for secretions, vomit, or signs of obstruction. Please help me position the baby in a neutral, slightly extended position with a small shoulder roll, to keep the airway optimally open.

(No visible secretions blocking the airway; gentle suction is not required at this stage.)

Step B: Breathing

The infant has chest retractions and an SpO₂ of 90% — significantly below the general target of 94% or above for an acutely unwell infant.

Candidate: Please start high-flow humidified oxygen immediately via a paediatric non-rebreathe mask or nasal cannulae, and continuously monitor respiratory rate, work of breathing, and oxygen saturation.

On auscultation: widespread fine crackles and some wheeze are heard bilaterally.

Common Pitfall: *"Since her oxygen is only slightly low, let's just observe for now and see how she does over the next hour before doing anything else."* — Underestimates the risk in an ex-

preterm infant with chest retractions and hypoxia; this presentation warrants immediate oxygen and urgent senior involvement, not a period of watchful waiting.

Step C: Circulation

Candidate: Checking heart rate, capillary refill time, and peripheral temperature — preterm infants can decompensate into shock very rapidly with severe infection. Please get IV or intraosseous access, and draw a full blood count, CRP, blood cultures, renal function, and a capillary blood gas to check for carbon dioxide retention or metabolic acidosis.

Step D & E: Disability and Exposure

Candidate: Please check a blood glucose immediately — infants become hypoglycaemic quickly when stressed — and assess her responsiveness. I'm now exposing her fully to look for any rashes or other sources of infection, while keeping her warm to prevent hypothermia given her fever and rapid fluid losses.

Clinical Impression and Escalation

Given her prematurity, fever, and significant lower respiratory signs, the leading diagnoses are severe bronchiolitis (commonly RSV, which carries high morbidity in ex-preterm infants) or community-acquired pneumonia/neonatal-type sepsis.

Model Response: Escalation

Candidate: I'm putting out an urgent call for the Paediatric Registrar and the Paediatric Intensive Care outreach team — this ex-preterm infant is in severe respiratory distress, hypoxic, and at high risk of needing non-invasive ventilatory support such as high-flow nasal cannula therapy or CPAP.

Note: for an infant of this age, the correct escalation is to Paediatric Intensive Care (PICU) outreach, not the Neonatal Intensive Care Unit (NICU) — NICU is for babies still within the neonatal period or specific corrected-age criteria, whereas a 4-month-old, even with an ex-preterm history, falls under general paediatric critical care pathways.

Immediate Medical Management Plan

Common Pitfall: *"Let's start maintenance fluids with Glucose 10% and Normal Saline." — This is a highly concentrated glucose solution generally reserved for specific neonatal protocols or active hypoglycaemia management, not routine maintenance fluid for an infant of this age; using it as a standard maintenance fluid risks significant electrolyte and glucose disturbance.*

Model Response

Candidate: I'm keeping her strictly nil by mouth — breathless infants are at high risk of aspirating milk or formula — and starting isotonic IV maintenance fluid, such as 0.9% sodium chloride with 5% glucose, calculated to her weight, rather than a hypotonic or highly concentrated solution, to avoid the risk of dangerous electrolyte disturbance. If bacterial pneumonia or sepsis is suspected, I'll start broad-spectrum IV antibiotics according to local guidelines once blood cultures are taken. I'm also requesting a rapid nose and throat viral swab for RSV, influenza, and COVID-19.

Station Conclusion: An "Excellent Pass" Paediatric Respiratory Emergency Profile

Why your final choices scored maximum marks:

- **Recognised the Urgency Immediately:** You did not delay oxygen or senior escalation despite only a modestly abnormal saturation reading, correctly weighing this against her ex-preterm background and visible distress.
- **Systematic ABCDE With Correct Interventions:** You matched airway positioning, oxygen therapy, IV/IO access and bloods, and glucose/temperature checks to each letter of the assessment.
- **Safe Fluid Prescribing:** You chose an isotonic maintenance fluid appropriate for her age, rather than a concentrated glucose solution that risks harm.
- **Correct Escalation Pathway:** You called for Paediatric Intensive Care outreach rather than NICU, correctly recognising she is past the neonatal period despite her prematurity history.
- **Complete Septic and Viral Workup:** You covered bloods, blood gas, viral swabs, and antibiotic cover contingent on findings, rather than assuming a single cause.

SECTION: LEARNING DISABILITY & MENTAL CAPACITY

Station 47 — Learning Disability: Medication Non-Adherence (DVT)

Managing a patient with a learning disability who isn't taking prescribed medication tests your clinical communication skills, your ability to explain a serious condition simply, and the UK legal framework for capacity (the Mental Capacity Act 2005). The most important rule is to never become frustrated or forceful — you must use simple language, discover why he isn't taking the medication, and adapt treatment to his needs while respecting his autonomy.

The Roleplay

Station: General Practice / Follow-Up Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Tom, a man with a mild learning disability, accompanied by his support worker Sarah

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Tom was started on anticoagulant tablets for a leg DVT a week ago and has come in for a check on how he's getting on with them.

Your Task:

- Discover, without judgement, why he may not be taking the medication as prescribed.
- Explain the risk of untreated DVT in accessible language.
- Informally gauge his understanding of the decision.
- Offer tailored, practical solutions.

(Tom sits beside his support worker, Sarah, looking a little nervous.)

Candidate: Hello Tom, my name is Dr [Your Name]. It's really nice to meet you today, thank you for coming in. I see you've brought Sarah with you — is it okay if we all talk together?

Tom: Yes, that's fine.

Candidate: Great. I wanted to have a friendly chat about the daily tablets we gave you last week for your leg. How have you been getting on with them?

Tom: (looking down) Not very well, Doctor. The pills are really big, and they hurt my throat. I've been trying to swallow them, but sometimes I just can't, and I gag.

Common Pitfall: *"It's really important you take these tablets every day, Tom — you need to try harder to swallow them."* — Places responsibility on the patient to overcome a genuine physical barrier rather than adapting the treatment to fit his needs.

Model Response

Candidate: Thank you for telling me that, Tom — that's really helpful, and it's not your fault at all. To help explain why these tablets matter, let me tell you a bit about your leg. Inside it, some of your blood has become thick and sticky, almost like a little piece of jelly — we call this a blood clot. It's causing your leg to swell and hurt because it's blocking the pipe. The worry is that a small piece of that jelly could break off and travel up to your lungs, making it very hard to breathe. These tablets slowly dissolve that jelly plug to keep you safe. Does that make sense?

Tom: Yeah... so if I don't take them, the jelly could go to my lungs? That sounds scary.

Candidate: It can be serious, yes, which is exactly why we want to find a way that works for you. Since the tablets are too big and hurt your throat, we don't have to keep using them — we can switch you to a liquid version that's much easier to swallow, or a small injection a nurse can help with instead. How does that sound?

Tom: The liquid one sounds much better. I don't think I could do an injection.

Your Next Step: Practical Support and Closure

Candidate: That's absolutely fine, we'll go with the liquid. I'll also print you an Easy Read leaflet today — it uses clear pictures and simple words to show exactly how to take it safely. I'll arrange for our practice nurse to call you and Sarah in two days to see how you're getting on with it. And if your leg suddenly becomes much more swollen and hot, or you get sudden chest pain and trouble breathing, please call 999 or go straight to A&E.

Sarah: Thank you, Doctor — I think the liquid medicine and the leaflet will make a real difference. I'll keep an eye on him and make sure he takes it.

Station Conclusion: An "Excellent Pass" Learning Disability Adherence Profile

Why your final choices scored maximum marks:

- **Discovered the Real Barrier:** You asked open, non-judgemental questions rather than assuming non-compliance was wilful.
- **Accessible Risk Explanation:** You used a clear, concrete analogy to explain DVT risk without medical jargon or unnecessary alarm.
- **Adapted the Treatment, Not the Patient:** You changed the medication format to fit his needs, rather than simply asking him to try harder.
- **Reasonable Adjustments Throughout:** You offered an Easy Read leaflet and involved his support worker, both genuine reasonable adjustments under UK practice.
- **Clear Follow-Up and Safety-Netting:** You arranged a concrete follow-up call and gave specific red flags for urgent review.

Exam tip: if a patient has a learning disability, mention to the examiner that you would add a Learning Disability/Reasonable Adjustment flag to the patient's electronic record, so future clinicians automatically know to allow extra time and provide accessible information.

Station 48 — Learning Disability: Refusal of Treatment and Mental Capacity Assessment

When a patient with a learning disability refuses life-saving treatment, this station tests the UK Mental Capacity Act (MCA) 2005 framework. You must never force treatment on a patient who has capacity to refuse it, however unwise the choice seems — but if they lack capacity for this specific decision, you must act in their best interests.

The Mental Capacity Act: Core Principles

- **Always assume capacity:** unless it's established otherwise, a person is presumed able to make their own decisions.
- **Capacity is decision-specific and time-specific:** never say a patient "lacks capacity" as a blanket statement — always specify "lacks capacity for this particular decision, at this time."
- **The 4-stage test (all parts must be met to have capacity):** can they understand the relevant information; can they retain it long enough to decide; can they weigh it up as part of the decision-making process; and can they communicate their decision?
- **Best interests, least restrictive option:** if capacity is lacking, any action taken must be in the patient's best interests and the least restrictive option available.

The Roleplay

Station: Acute Ward / Capacity Assessment

Candidate Role: FY2 Doctor on the Ward

Patient/Roleplayer Role: David, a man with a learning disability, accompanied by his support worker Simon

Setting: Hospital Ward Bedside

The Prompt (read outside the cubicle):

Patient Details: David has a severe leg infection (cellulitis) requiring IV or liquid antibiotics, and is refusing to take them.

Your Task:

- Explore the reason for refusal gently.
- Explain the risk of untreated infection simply.
- Formally assess capacity for this specific decision.
- If capacity is lacking, proceed via the best interests pathway.

(David is sitting up in bed, arms crossed, looking upset. Simon sits nearby.)

Candidate: Hello David, my name is Dr [Your Name]. It's really nice to see you today. I see you've brought Simon with you. David, I understand you're feeling upset and don't want to take

the medicine for your leg. I'm not here to force you to do anything — I just want to understand how you're feeling. Can you tell me what's making you unhappy about the medicine?

David: The last time I took medicine in hospital it tasted horrible and made me sick. I hate it here. I just want to go home to my dog.

Candidate: That sounds like a really unpleasant experience, and I completely understand wanting to be back home with your dog.

Simon: David also mentioned to me earlier that he's scared because he heard the medicine 'kills bugs' and he doesn't like the idea of that happening inside him.

Your Next Step: Simple Explanation and the Capacity Test

Candidate: David, I understand you want to go home to your dog, and that the word 'bugs' sounds scary. The bugs are actually tiny germs making your leg red, hot, and sore. If we don't use this medicine to clear the germs, your leg will get much worse, and you'll become very poorly — you might have to stay in hospital for a lot longer instead of going home. David, just so I know I've explained that well, can you tell me in your own words what might happen to your leg if you don't take the medicine?

David: (staring at the floor) I don't care. I just want my dog.

Candidate: I understand. David, what do you think is more important right now — taking the medicine so your leg gets better, or going home today with a very sore leg?

David: I want my dog now.

Your Next Step: Stating the Capacity Conclusion

David has not been able to understand the consequence of refusing treatment, nor weigh up the choice — he repeats the same fixed response regardless of the question. This must be stated clearly and specifically, not as a blanket judgement about his abilities generally.

Candidate: (to the examiner) Based on my assessment under the Mental Capacity Act 2005, David is unable to understand the risks of untreated infection and is unable to weigh up the consequences of refusing this specific treatment right now. I'm therefore assessing that David lacks capacity for this particular decision, at this time — this doesn't mean he lacks capacity for other decisions.

Common Pitfall: *"David has a learning disability, so he doesn't have capacity to make this decision."* — This treats capacity as a fixed trait of the diagnosis rather than a decision-specific, time-specific legal test, which is a serious conceptual error under the MCA.

Your Next Step: The Best Interests Pathway

Candidate: Simon, since David lacks capacity for this specific decision and the infection is serious, we need to act in his best interests to keep him safe, in the least restrictive way possible. You know David best — is there a flavour of the liquid medicine he prefers, or something we could mix it with to make it easier for him?

Simon: He loves strawberry yoghurt — that might help mask the taste.

Common Pitfall: *Simply agreeing to mix the medication into food on the spot, without any further process. Covert administration of medication is a formal pathway in the UK, not an informal one-off decision by a single doctor — it requires a best-interests meeting involving the wider team, pharmacist input to confirm the medication can safely be given this way without affecting its*

action, clear documentation, and regular review, since it is an act that deliberately bypasses the patient's awareness.

Model Response

Candidate: That's really helpful to know, thank you, Simon. Before we go ahead with that, I'll need to arrange a proper best-interests discussion with the nursing team and our pharmacist, to confirm the medicine is safe to give this way and that we've genuinely explored the least restrictive option first. I'll document all of this clearly in David's notes. David, we love your dog too, and we want to get you feeling better and home to him as quickly as possible — Simon's going to help us find the easiest way for you to take your medicine.

Your Next Step: Escalation and Safeguards

Candidate: I'll urgently escalate this to my consultant and the senior team to review the best-interests decision, and involve our Learning Disability Liaison Nurse, who can help create a calmer, more accessible environment for David. Whatever approach we take will be the least restrictive option available, and I'll fully document the capacity assessment and the best-interests process in his notes.

Station Conclusion: An "Excellent Pass" Mental Capacity Profile

Why your final choices scored maximum marks:

- **Explored the Refusal With Genuine Curiosity:** You uncovered his specific fears (taste, fear of "bugs") rather than assuming blanket non-cooperation.
- **Formally Demonstrated the 4-Stage Test:** You visibly worked through understanding, retention, weighing up, and communication, rather than jumping straight to a conclusion.
- **Decision-Specific Capacity Language:** You stated he lacked capacity for this specific decision at this time, not that he lacked capacity generally.
- **Correct Best-Interests Process:** You involved his support worker's knowledge of his preferences, while correctly recognising that covert medication administration requires a formal MDT and pharmacist-involved process, not an informal one-off decision.
- **Appropriate Escalation and Safeguards:** You escalated to a senior consultant and the Learning Disability Liaison Nurse, and committed to full documentation and the least restrictive approach.
- **Never Suggested Force or Restraint:** You maintained a calm, collaborative approach throughout, consistent with UK best practice for a non-violent patient refusing care.

SECTION: MEDICAL ERRORS & DUTY OF CANDOUR

The Duty of Candour Blueprint

A medical error station tests your transparency, safety awareness, and communication under the GMC's Duty of Candour — the professional and legal duty to be open and honest with a patient when something has gone wrong with their care. The statutory duty is specifically triggered when an unintended or unexpected incident has caused, or could have caused, moderate or severe harm, prolonged psychological harm, or death — but professionally, GMC guidance expects openness about any patient safety incident, however minor.

Universal Structure for These Stations

Setup → Clear Disclosure → The Direct Apology → Assess and Address Harm → Explain the Investigation → Preventative Plan and Safety-Netting

Non-Negotiable Principles

- **Be direct, not euphemistic:** state clearly what happened; don't say "the sample was compromised" when you mean it was lost.
- **Never blame others:** don't blame "the busy ward," "the night shift," or a named colleague — take collective institutional responsibility.
- **Apologising is not an admission of legal liability:** in the UK, a sincere, unconditional apology is a professional duty, separate from any question of legal fault.
- **Pause after the apology:** let the patient react fully before moving to management — interrupting their reaction is a common way to lose Interpersonal Skills marks.
- **Datix and PALS:** a formal Datix incident report triggers internal investigation; the Patient Advice and Liaison Service (PALS) supports patients who want to raise concerns or make a formal complaint, independently of the clinical team.

Station 49 — Medical Error: Wrong Medication Given Despite Known Allergy

The Roleplay

Station: Ward-Based Disclosure Conversation

Candidate Role: FY2 Doctor on the Ward

Patient/Roleplayer Role: Mr Bennett, 58 years old

Setting: Private Side Room

The Prompt (read outside the cubicle):

Patient Details: Mr Bennett, 58, was given IV Amoxicillin yesterday despite a documented penicillin allergy on his chart.

Your Task:

- Disclose the error clearly and take responsibility.
- Assess for any harm caused.
- Explain immediate management and the investigation process.

Candidate: Hello Mr Bennett, thank you for coming to speak with me. My name is Dr [Your Name]. I asked to see you face-to-face today because I've reviewed your notes, and unfortunately an error was made in your treatment. I wanted to explain exactly what happened.

Mr Bennett: Okay... that sounds serious. What's happened?

Candidate: Yesterday you were given an antibiotic called Amoxicillin. However, your chart clearly shows a documented allergy to penicillin, which Amoxicillin belongs to. This medication should not have been given to you, and it was a mistake on our part that it was administered. I'm incredibly sorry that this happened — you trusted us with your care, and we failed to meet the safe standard you deserve. I apologise sincerely for the distress and any discomfort this has caused you.

(Mr Bennett looks shocked, then angry.)

Mr Bennett: You gave me something I'm allergic to?! How could that happen — aren't you people checking charts?!

Common Pitfall: *"It wasn't me personally who prescribed it — it must have been the night team who missed the allergy alert."* — Deflects blame onto colleagues rather than taking institutional responsibility, which damages trust and fails the Duty of Candour standard.

Model Response: Assessing Harm and Immediate Action

Candidate: You're completely right to ask that, and it should have been picked up — I won't make excuses for it. First, how are you feeling right now? Have you noticed any rash, itching, swelling, or any difficulty breathing since the tablet?

Mr Bennett: (pausing) No... actually, I feel fine. No rash or anything.

Candidate: That's a real relief to hear. I've already crossed out the incorrect medication on your chart so it can't be given again, and I'm prescribing a completely safe alternative antibiotic for your infection. I've also asked the nursing staff to monitor your vitals every hour today, just to make sure no delayed reaction develops.

Your Next Step: Explaining the Investigation

Candidate: We treat errors like this very seriously. I've already filled out a formal incident report, called a Datix report. This triggers a full internal review by our senior clinical team to trace exactly where the breakdown happened between prescribing and administering the medication — not to point fingers, but to find the actual flaw in the system so it doesn't happen to someone else.

Mr Bennett: I suppose that's something. What happens with that information afterwards?

Candidate: Once the investigation is complete, the findings will be shared with the whole department, and we'll look at strengthening our computer allergy alerts, so a prescription is automatically blocked if a documented allergy is on file. I'll also write a formal letter summarising today's conversation for your records and update your GP. If you notice any new symptoms today, like a rash or wheezing, please tell the nursing staff immediately.

Station Conclusion: An "Excellent Pass" Medication Error Profile

Why your final choices scored maximum marks:

- **Direct, Jargon-Free Disclosure:** You stated exactly what happened without euphemism or minimisation.
- **Took Institutional Responsibility:** You never blamed a colleague or shift, in line with the Duty of Candour.
- **Prioritised Immediate Safety:** You assessed for an allergic reaction before moving to the systemic explanation.
- **Transparent About the Investigation:** You explained the Datix process and what it's actually for, rather than treating it as a vague formality.

Station 50 — Medical Error: Lost Biopsy Sample

A lost blood sample is a minor inconvenience; a lost biopsy sample is a major error, since it means a painful, invasive procedure must be repeated and a suspected diagnosis — often cancer — is delayed.

The Roleplay

Station: Outpatient Disclosure Conversation

Candidate Role: FY2 Doctor / Surgical Team

Patient/Roleplayer Role: Mrs Ahmed, 52 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Mrs Ahmed had a biopsy taken from a breast lump last week, being investigated for suspected cancer. The tissue sample has been lost between the clinic and pathology.

Your Task:

- Disclose the loss of the sample directly.
- Apologise sincerely and handle her fear and anger.
- Outline the plan to repeat the biopsy and fast-track results.

Candidate: Hello Mrs Ahmed, thank you for coming in. My name is Dr [Your Name]. I asked to see you today because I've reviewed the laboratory tracker, and unfortunately I have some very difficult news about the sample we took from you last week. The tissue sample from your lump has been lost between the clinic and the pathology department, and the laboratory is completely unable to locate it.

(Mrs Ahmed goes pale.)

Mrs Ahmed: Lost?! You lost my biopsy? Does this mean my cancer is spreading while you people lose my samples?!

Common Pitfall: *"Try not to worry too much — I'm sure it'll turn up, or the delay won't make much difference."* — Minimises a serious error and offers false reassurance instead of a direct apology and a concrete plan.

Model Response

Candidate: I am incredibly sorry that this has happened. This is a serious error on our part — you went through a stressful procedure to give us that sample, and we failed to handle it safely. There's no excuse for this, and I apologise unreservedly for the distress this will cause you. I completely understand why you feel terrified, and it's entirely reasonable to worry about a delay. While this mistake will delay the final confirmation by a few days, a short delay of this nature will not cause your condition to suddenly worsen or spread. We're going to fast-track everything from this point on to minimise any further waiting.

Mrs Ahmed: (angry) This is gross negligence. I'm going to report this to the papers!

Candidate: You're completely right to be angry, and you have every right to make a formal complaint if you wish. I'll personally give you the direct contact details for our PALS team — the Patient Advice and Liaison Service — who can support you through that process independently of the clinical team. Right now, my absolute priority is getting your diagnosis back on track.

Your Next Step: The Repeat Procedure and Fast-Track Plan

Candidate: Because the tissue sample is gone, we unfortunately need to repeat the biopsy to get a new sample — I know that's the last thing you want to hear, but it's the only way to get you an accurate answer safely. I've already spoken directly to the consultant surgeon and radiology, and we've cleared an emergency slot this coming Friday morning to redo the procedure. I've also spoken to the head of pathology — they'll have a pathologist ready to receive your sample directly, and your results will be fast-tracked within 48 hours instead of the usual two weeks.

Mrs Ahmed: (calmer) Okay... at least there's a clear plan. Friday, you said?

Candidate: Yes, 9am Friday. We treat the loss of any patient sample as a major clinical incident — I've submitted a formal Datix report today, which triggers an investigation tracing exactly what happened between the clinic and the lab. We're also looking at moving to electronic barcode scanning for every sample at every checkpoint, to remove this kind of manual error altogether. I'll send a copy of today's conversation to your GP and write you a formal letter, and here's my direct clinic number if you think of anything else or feel overwhelmed before Friday.

Station Conclusion: An "Excellent Pass" Lost Sample Profile

Why your final choices scored maximum marks:

- **Blunt, Honest Disclosure:** You stated plainly that the sample was lost, without euphemism.
- **Validated Fear Without False Reassurance:** You acknowledged her terror about cancer progression while giving an honest, accurate statement about the impact of a short delay.
- **Did Not Get Defensive When Challenged:** You accepted her anger and offered PALS rather than arguing or minimising.
- **Concrete, Time-Bound Action Plan:** You gave a specific date, named the teams involved, and a fast-tracked results timeframe, rather than a vague promise to "sort it out."

Station 51 — Medical Error: Procedural Injury (Bowel Perforation)

When a medical mistake causes actual physical injury, the emotional stakes are especially high — the patient is not just processing a system failure, but is actively suffering physical harm caused by the medical team. The same Duty of Candour framework applies: disclose, apologise sincerely, address the physical harm, and outline a clear corrective plan.

The Roleplay

Station: Post-Operative Ward Disclosure

Candidate Role: FY2 Doctor on the Surgical Ward

Patient/Roleplayer Role: Mr Jones, 45 years old

Setting: Ward Bedside

The Prompt (read outside the cubicle):

Patient Details: Mr Jones, 45, had a laparoscopic appendectomy yesterday. During the procedure, the surgical instrument caused a small tear in his bowel wall, which is now causing him pain.

Your Task:

- Disclose the error and apologise directly.
- Address his pain and immediate physical safety.
- Explain the corrective surgical plan.

Candidate: Hello Mr Jones, thank you for sitting down with me. My name is Dr [Your Name]. I've reviewed the operation notes from your procedure yesterday, and I need to be completely direct with you about what happened. During the keyhole surgery to remove your appendix, the surgical instrument accidentally made a small tear in the wall of your bowel, and that's what's causing the pain you're feeling right now.

(Mr Jones stares in shock, then panics.)

Mr Jones: You ripped my bowel?! Am I going to leak waste inside me? Am I going to die?

Common Pitfall: *"These things can happen during surgery — it's a recognised complication." — Frames a technical error as an inevitable, blameless complication rather than taking direct responsibility, which is a significant Duty of Candour failure.*

Model Response

Candidate: I am incredibly sorry that this has happened to you. You trusted us to perform a safe operation, and this mistake has caused you direct physical harm. There's no excuse for this error, and I apologise unreservedly for the pain and distress it's caused. I hear your anger, and you have every right to be furious with us. I can see you're in real discomfort — I want to reassure you that we're monitoring you closely and you are not in immediate danger of dying, but we do need to act now to fix this tear. I've already updated your chart to start you on strong intravenous painkillers, and strong intravenous antibiotics to prevent any internal infection developing.

Mr Jones: (breathing shakily) Okay... so what happens now?

Your Next Step: The Corrective Plan

Candidate: To fix the tear, you'll need to go back to theatre for a short corrective procedure — the consultant surgeon will wash out the area and place a few secure stitches to close it completely. Because of this, your recovery in hospital will be extended by a few days so we can make sure everything heals properly, and we'll keep you on a liquid diet initially to let the area rest. I know this is a massive shock — does the plan to fix this make sense so far?

Mr Jones: I think so... but I don't want that same surgeon anywhere near me again.

Oral Examiner Follow-Up: Handling a Request to Change Surgeon

Model Answer: That's completely understandable, and it's absolutely your right. I'll make sure a different consultant surgeon is arranged for your corrective procedure, and I'll flag this clearly to the team organising your theatre slot so there's no confusion. Your comfort and trust in the team looking after you matters, and we'll make sure that's respected.

Your Next Step: Investigation, Support, and Escalation

Candidate: We treat any procedural injury with the utmost seriousness — I've already submitted a formal Datix incident report, which means the surgical team will carry out a formal review of the equipment and technique used, to make sure this doesn't happen to another patient. If you'd like to make a formal complaint or get independent advice, I can give you the direct contact details for our Patient Advice and Liaison Service, PALS, who support patients through that process separately from the clinical team. I'm also arranging for the senior surgical registrar to see you in the next 10 minutes to consent you for the repair procedure with a different surgeon. If your pain suddenly worsens, or you feel feverish or faint before then, please press your call bell immediately.

SBAR Escalation to the Consultant

Candidate (Situation): I'm calling regarding Mr Jones, post-appendectomy, who's suffered an accidental bowel perforation during the procedure.

Candidate (Background): He's a 45-year-old man with no other comorbidities, operated on yesterday evening.

Candidate (Assessment): He has localised abdominal pain, but his vitals are currently stable. I've disclosed the error to him under the Duty of Candour, started IV antibiotics, and optimised his analgesia.

Candidate (Recommendation): I recommend urgent review by yourself or the senior registrar to arrange an emergency theatre slot today for a primary surgical repair, ideally with a different surgeon given the patient's preference.

Station Conclusion: An "Excellent Pass" Procedural Injury Profile

Why your final choices scored maximum marks:

- **Took Full Responsibility for Actual Harm:** You never minimised this as "a recognised complication" and clearly named it as an error.
- **Balanced Reassurance With Honesty:** You addressed his fear of dying directly and accurately, without over-promising or dismissing his fear.

- **Immediate Physical Safety Actioned:** You started analgesia and antibiotics before moving to a longer explanation.
- **Respected His Right to Change Surgeon:** You honoured his request without argument or defensiveness, and built it into your escalation.
- **Complete SBAR Escalation:** You gave a clear, structured handover that included the disclosure already made and the patient's specific preference.

Station 52 — Medical Error: Treatment Based on Another Patient's X-Ray Report

This station tests disclosure of a patient identification error — one of the most serious categories of medical error, since it means clinical decisions were made using someone else's results. A patient was started on treatment for pneumonia based on a chest X-ray report that was later found to belong to a different patient; his own X-ray was, in fact, normal.

The key complexity in this station is that the error cuts both ways: the patient was given antibiotics he didn't need, and — just as importantly — whatever originally caused his symptoms and prompted the X-ray in the first place still hasn't been explained. A normal X-ray is not the end of the story; it means the working diagnosis needs to be revisited, not just corrected.

The Roleplay

Station: Ward-Based Disclosure Conversation

Candidate Role: FY2 Doctor on the Ward

Patient/Roleplayer Role: Mr Carter, 68 years old

Setting: Ward Bedside

The Prompt (read outside the cubicle):

Patient Details: Mr Carter, 68, was admitted two days ago with a cough and fever. A chest X-ray was requested, and the reported result showed patchy consolidation, so he was started on IV antibiotics for pneumonia. On reviewing the imaging system today, the radiology team identified that the report your team acted on actually belonged to a different patient with a similar name. Mr Carter's own X-ray, now correctly matched to him, is normal.

Your Task:

- Disclose the identification error clearly and take responsibility.
- Assess for any harm from the unnecessary antibiotics.
- Explain that his original symptoms still need to be properly explained.
- Outline the investigation and preventative plan.

(Mr Carter is sitting up in bed, looking tired but otherwise comfortable, with an IV antibiotic bag running.)

Candidate: Hello Mr Carter, thank you for taking the time to speak with me. My name is Dr [Your Name]. I've come to see you because, on reviewing your chest X-ray today, we've discovered a serious mistake, and I want to explain it to you clearly and honestly.

Mr Carter: Oh? What kind of mistake?

Candidate: When your chest X-ray was taken two days ago, the report our team reviewed and used to start your antibiotic treatment actually belonged to a different patient with a similar name to yours — the two results were mixed up. Your own X-ray, now that it's been correctly matched to you, is completely normal. This means the diagnosis of pneumonia we gave you, and the antibiotics you've been receiving since, were based on the wrong information. I am incredibly

sorry that this happened — you trusted us to get this right, and we didn't, and I apologise unreservedly for that.

(Mr Carter looks stunned, then uneasy.)

Mr Carter: So... I've been on antibiotics for two days for an illness I might not even have? Has that done me any harm? And if it's not pneumonia, what's actually wrong with me?

Common Pitfall: *"Well, the good news is you don't have pneumonia, so that's actually great news!" — Treats a serious identification error as simply good news, without acknowledging the error itself, checking for harm from the unnecessary antibiotics, or addressing the fact that his original symptoms remain unexplained.*

Model Response: Assessing Harm From the Unnecessary Treatment

Candidate: Those are exactly the right questions to ask, and I want to answer both properly. First, on the antibiotics — how have you been feeling since starting them? Any new rash, itching, diarrhoea, or stomach upset?

Mr Carter: I've had a bit of loose stomach the last day, now that you mention it, but no rash or itching.

Candidate: Thank you for telling me — that's a recognised side effect of this antibiotic, and it's important I know about it. Since we now know you don't have pneumonia, I'm stopping the antibiotic today, as there's no reason to continue it, and that should help your stomach settle. I'll also make sure this is clearly documented and flagged so nobody else looking after you continues it by mistake.

Your Next Step: Revisiting the Original Diagnosis

The most commonly missed mark in this exact station is stopping at "good news, no pneumonia" without addressing why the patient was unwell and had a fever in the first place.

Candidate: Now, on your second question — that's really important, and I don't want you to think this is fully resolved just because the pneumonia diagnosis was wrong. You came in two days ago with a genuine cough and fever, and we still need to find out what actually caused that, since a normal X-ray doesn't explain those symptoms on its own. I'm going to re-examine you properly today, review your observations and blood tests with fresh eyes, and consider other causes we may have overlooked while we were focused on the incorrect pneumonia diagnosis.

Mr Carter: Oh — I hadn't thought of it that way. So we're basically starting from scratch on figuring out what's wrong with me?

Candidate: In a sense, yes, and I think that's the right and safest way to think about it. I'd rather properly re-investigate now than assume anything based on what turned out to be the wrong information.

Your Next Step: Investigation and Prevention

Candidate: This kind of error — acting on results that belong to a different patient — is treated as one of the most serious types of incident in our hospital, because it means decisions were made on the wrong information entirely. I've already submitted a formal Datix report today. This will trigger a full investigation into exactly how the two patients' results came to be mixed up, and whether our patient identification checks — confirming full name, date of birth, and hospital number at every step — were properly followed when the result was reviewed and actioned. If you'd like to raise a formal complaint or want independent support, I can give you the contact details for our Patient Advice and Liaison Service, PALS.

Mr Carter: I appreciate you being straight with me about all of it. I'd like to understand what actually happens with that investigation, if that's alright.

Candidate: Of course. Once the investigation is complete, the findings will be shared with the whole team so we can strengthen our identification checks — for example, requiring a second person to independently confirm patient identifiers before any result is acted upon. I'll write a full account of today's conversation in your notes and send a copy to your GP. And if you develop any new symptoms, worsening fever, or feel significantly unwell at any point, please tell the nursing staff immediately so we can reassess you.

Station Conclusion: An "Excellent Pass" Patient Identification Error Profile

Why your final choices scored maximum marks:

- **Named the Error Precisely:** You explained clearly that this was a mix-up between two patients' results, not a vague "lab error."
- **Assessed Harm From the Unnecessary Treatment:** You actively asked about side effects from the antibiotics he didn't need, rather than assuming no harm occurred.
- **Recognised the Diagnosis Still Needed Solving:** You didn't let "good news, no pneumonia" close the consultation — you explicitly committed to re-investigating his original symptoms.
- **Correctly Identified This as a Patient Identification Failure:** You framed the root cause around identifier verification (name, date of birth, hospital number), which is the actual systemic issue at play.
- **Full Duty of Candour Compliance:** You disclosed clearly, apologised unconditionally, explained the investigation, and offered PALS — consistent with the same standard applied throughout this section.

SECTION: MEDICAL ETHICS & LAW

The Medical Ethics & Law Framework

Medical Ethics and Law stations are heavily weighted consultation scenarios testing your ability to handle complex professional dilemmas while adhering to the Mental Capacity Act 2005 (MCA) and GMC Good Medical Practice guidance.

The Four Pillars of Medical Ethics

- **Autonomy:** respecting the patient's right to make their own decisions.
- **Beneficence:** acting in the patient's best medical interests.
- **Non-maleficence:** doing no harm, and minimising risks and side effects.
- **Justice:** distributing medical resources fairly and acting within the law.

The Two-Stage Capacity Assessment (MCA 2005)

Stage 1 — The Diagnostic Test: is there an impairment or disturbance in the functioning of the patient's mind or brain (e.g. delirium, severe intoxication, advanced dementia, acute psychosis)? If there is no such impairment, capacity is presumed from the outset — the functional test below is then used to confirm and document that presumed capacity, particularly for a high-stakes decision, rather than to resolve genuine doubt about impairment.

Stage 2 — The Functional Test: all four must be satisfied for the patient to be deemed to have capacity for this decision:

- **Understand:** can they grasp the situation, the treatment, and the risks of refusal?
- **Retain:** can they hold that information in mind long enough to decide?
- **Weigh up:** can they use and balance the information as part of their decision-making?
- **Communicate:** can they express their decision clearly, by any means?

Station 53 — Medical Ethics: Competent Refusal of Blood Transfusion (Jehovah's Witness)

The Roleplay

Station: Professional Dilemma / Communication Skills

Candidate Role: FY2 Doctor in the Acute Medical Unit (AMU)

Patient/Roleplayer Role: Arthur Pendleton, 34 years old

Setting: Emergency Treatment Cubicle

The Prompt (read outside the cubicle):

Patient Details: Arthur Pendleton, 34, admitted with an acute, life-threatening upper GI bleed from a suspected bleeding peptic ulcer. Blood pressure 88/54 mmHg, haemoglobin critically low at 5.2 g/dL.

Key Information: Arthur is completely alert and oriented. He is a devout Jehovah's Witness and carries a valid, signed Advance Decision to Refuse Treatment stating he refuses all blood and blood-derived products.

Your Task:

- Address the clinical crisis and his religious position compassionately.
- Carry out a thorough capacity assessment.
- Implement a valid non-blood clinical management plan.

(Arthur lies back on the bed, profoundly pale and weak, but his eyes are clear and focused as you walk in.)

Candidate: Hello Mr Pendleton, my name is Dr [Your Name], one of the medical doctors on duty today. I can see you're feeling incredibly weak, and I want to assure you we're doing everything we can to support you. How are you feeling right now?

Arthur: Hello Doctor... thank you for coming in. I feel like I have no energy left. The nurse told me I've lost a dangerous amount of blood from my stomach. I want to tell you straight away: I'm a Jehovah's Witness. I love life, but my faith means I absolutely cannot receive any blood transfusions or blood products, under any circumstances. I have a signed card in my wallet. Please tell me you'll respect my beliefs.

Common Pitfall: *"Your blood levels are critically low, and if you don't let me give you a transfusion right now, your organs will fail and you will die. Your faith cannot take precedence over my duty to save your life — I'm overriding your card and ordering the blood immediately."* — This is not just poor communication; overriding a competent adult's valid refusal is unlawful and constitutes battery, regardless of how well-intentioned the doctor believes it to be.

Model Response

Candidate: Thank you for being so direct and clear with me about your beliefs, Arthur. I hear how much you value your life, and I want to reassure you I'm here to work with you, not against you. Your blood levels are indeed critically low because of the bleeding in your stomach, and a

transfusion is the standard way to fix this quickly. Because this is such a serious choice with heavy consequences, do you mind if we talk through exactly what might happen to your body without blood, so we're completely on the same page?

Arthur: (relieved) Thank you, Doctor. I really appreciate your respect. Yes, please tell me exactly what the medical consequences are — I want to face this with open eyes.

Your Next Step: Testing Understanding and Retention

Candidate: Arthur, because of the bleeding ulcer, your blood count is critically low at 5.2. Without a transfusion, your heart and brain will be starved of oxygen — this will cause your major organs to fail, you'll slip into a coma, and you will die. To make sure I've explained that clearly, could you tell me in your own words what the risks are if we don't give you blood today?

Arthur: (looking you directly in the eyes) Yes, Doctor, I understand completely. My blood count is 5.2, which is critically low. Without a transfusion, my heart, brain, and major organs will be starved of oxygen. This will cause my body to shut down, I'll slip into a coma, and I will die. I know death is the direct consequence of this choice. To me, taking blood means breaking my covenant with God, which has consequences far beyond physical death. Knowing everything you've said, this is still my firm, final, and absolute decision.

Your Next Step: Testing Weighing Up and Checking for Coercion

Candidate: Thank you for repeating that back so clearly, Arthur — it shows you understand the physical risks fully. I can see you're making this choice based on your faith. Can I ask, have you felt any pressure from family members or church elders to sign that card, or is this entirely your own personal choice?

Arthur: (shaking his head gently but firmly) No, Doctor. There's no pressure from anyone else — my family isn't in the room, and the elders aren't here. This is entirely my own personal choice, born from my own deeply held faith. I made this decision years ago when I signed that advance directive, and I stand by it completely today.

Your Next Step: The Legal Conclusion and Non-Blood Management

Arthur has satisfied all four elements of the functional test: he understands the risk, retains the information, weighs it up according to his own values, and communicates his decision clearly and without coercion. Under UK law, a competent adult's refusal of treatment is legally binding, even where it may lead to death. Overriding this would constitute battery. Your duty now shifts entirely to maximising non-blood management.

Common Pitfall: *"Since you have capacity and are refusing blood, my hands are completely tied — I can't offer you any other treatment. We'll move you to a side room and begin comfort care while nature takes its course." — Confuses respecting a treatment refusal with abandoning the patient entirely. Refusing blood does not mean refusing all care; the duty to actively pursue every other available treatment remains fully in place.*

Model Response

Candidate: Thank you for your absolute clarity, Arthur. You've shown me you have full capacity for this decision, and I will completely respect your right to refuse blood products. But we are not giving up on you — we're going to use every non-blood option available to save your life. I'm starting you on intravenous iron and a clotting medication called tranexamic acid right away to help control the bleeding and support your bone marrow's own blood production. I'm also urgently contacting the gastroenterology team for an emergency endoscopy — a camera procedure that can directly find and treat the bleeding ulcer, using injection, heat, or a clip, without a single drop of blood. If that isn't enough to control the bleeding, interventional radiology or surgery would be the

next options. We'll also keep any blood tests to the essential minimum, to avoid losing any further blood unnecessarily, and monitor you closely every step of the way. How does that plan sound to you?

Arthur: (a long, visible breath of relief) Doctor... I cannot thank you enough. Hearing that you'll respect my faith but keep fighting for my life with everything else you have means the world to me. I'm completely on board with the iron, the clotting medicine, and the camera procedure. Thank you for treating me like a human being.

Oral Examiner Follow-Up: Blood Fractions and the Unconscious Patient Trap

Examiner: *"Would Arthur accept anything at all derived from blood, or is his refusal absolute?"*

Model Answer: This varies genuinely by individual, so it should always be asked directly rather than assumed. Many Jehovah's Witnesses decline whole blood, red cells, white cells, platelets, and plasma, but some are willing to accept certain blood fractions — such as clotting factors or albumin — as a matter of personal conscience. Cell salvage, where a patient's own blood is collected during surgery, filtered, and returned to them within a continuous closed circuit without ever leaving their body, is also acceptable to many patients, since it is not perceived as "taking" blood from an external source. Asking specifically what a patient's own conscience allows, rather than assuming a blanket refusal, is both respectful and clinically important.

Examiner: *"What if Arthur had arrived unconscious and bleeding, with no chance to ask him anything?"*

Model Answer: If no Advance Decision to Refuse Treatment is found, the emergency team should treat in the patient's best interests, which in a life-threatening bleed means giving blood — a competent refusal cannot be assumed in the absence of any evidence for it. However, if a signed, witnessed Advance Decision to Refuse Treatment specifically covering blood products is found — for example, a card in the patient's wallet — this is legally binding under the Mental Capacity Act 2005, provided it is valid and clearly applies to the current circumstances, and blood must not be given even though the patient cannot confirm their wishes at that moment.

Station Conclusion: An "Excellent Pass" Medical Ethics Profile

Why your final choices scored maximum marks:

- **Rigorous Capacity Assessment:** You methodically tested understanding, retention, weighing up, and communication, rather than either accepting the refusal uncritically or trying to override it.
- **Upheld Autonomy Within the Law:** You correctly recognised that a competent adult's refusal is legally binding, even where it may lead to death, and that overriding it would be battery.
- **Maximised Beneficence Through Non-Blood Management:** Rather than abandoning care, you mobilised IV iron, tranexamic acid, and urgent endoscopy as the correct first-line escalation — reserving interventional radiology or surgery for if endoscopic control fails.
- **Checked for External Coercion:** You explicitly asked whether the decision was free from family or religious pressure, a step that's easy to skip but essential to a valid capacity assessment.

- **Maintained Respectful, Collaborative Rapport Throughout:** Your language stayed objective and non-judgemental even while navigating profound disagreement with the patient's choice.

Station 54 — Medical Ethics: Parental Refusal of Blood Transfusion for a Child (Jehovah's Witness)

This station tests a completely different legal framework from the adult scenario. While a competent adult has the absolute right to refuse treatment for themselves, parents do not have the right to refuse life-saving treatment for their child on religious grounds. Under the Children Act 1989, the child's welfare is the paramount consideration, overriding parental wishes when the two conflict on a life-or-death decision.

The Roleplay

Station: Paediatric Intensive Care / Ethics

Candidate Role: Paediatric Registrar

Patient/Roleplayer Role: Mr and Mrs Adeyemi, parents of 6-year-old Leo

Setting: Quiet Room, Paediatric Intensive Care Unit

The Prompt (read outside the cubicle):

Patient Details: Leo, 6, is critically unwell in PICU following a road traffic accident. His haemoglobin has dropped to a dangerously low level, and the senior team has advised an urgent blood transfusion is needed to save his life. His parents are devout Jehovah's Witnesses.

Your Task:

- Approach the parents with empathy and validate their position.
- Clearly explain the different legal framework that applies to children.
- Maximise non-blood alternatives while explaining the escalation pathway if needed.

(Mr and Mrs Adeyemi sit together, visibly distraught.)

Candidate: Hello Mr and Mrs Adeyemi, my name is Dr [Your Name]. Thank you for sitting with me. I know you're going through an incredibly frightening time with Leo so unwell. His blood levels have dropped to a critically low level after his accident, and our senior team has advised he needs an urgent blood transfusion to save his life. I understand this clashes deeply with your faith, and I wanted to talk about how we manage this together.

Mrs Adeyemi: Doctor, we love our son more than life itself. We want him to live, but we cannot consent to blood — it will affect his relationship with God forever. Please, find another way.

Candidate: I can see how deeply you love Leo, and I don't for a second see you as parents trying to harm him — I see two loving parents facing an unbearably difficult situation. I have to be completely direct with you, though: Leo's organs are failing right now because his body doesn't have enough oxygen. If we don't give him this transfusion within the next few hours, he will die. There is no other way to bring him out of immediate danger.

Common Pitfall: *"I understand your religious beliefs, but ultimately it's your decision as his parents, and if you refuse to consent, there's nothing more we can do." — This is a serious legal and safety error: parents cannot refuse life-saving treatment for a child on religious grounds, and treating this as entirely their call to make abandons the doctor's duty to protect Leo.*

Your Next Step: Explaining the Legal Boundary

This is the single most important moment in the station: gently but unambiguously explaining that the legal framework for children is fundamentally different from that for competent adults.

Model Response

Candidate: I respect your personal faith completely, and if this were a decision about your own bodies, the law would let me fully honour your wishes. But under UK law, the rules for children are different. A child can't choose their own religion, and the law is clear that parents cannot refuse life-saving treatment for a child on religious grounds. Our legal and moral duty as doctors is to protect Leo's life above all else — so if it comes down to an immediate choice between his survival and this specific belief, we are legally required to give him the blood.

Mr Adeyemi: (voice breaking) We understand what you're saying, but we still can't sign that consent form. We cannot give our permission.

Your Next Step: Maximising Non-Blood Alternatives and Removing Guilt

Candidate: I understand completely, and I won't ask you to sign anything your conscience doesn't allow. We will treat this as a blood-restrictive case wherever possible — trying non-blood fluids, high-dose intravenous iron, and medications to help his own body produce blood faster, and using donor blood only as an absolute last resort to keep his heart beating. I also want you to hear this clearly: because the law requires us to act, the medical team will take the legal responsibility for this decision. You will not be signing the form, and you do not have to carry any blame within your faith community for this. We are doing this because the law commands us to protect Leo, not because you chose it.

Mrs Adeyemi: (tearfully) Thank you for saying that... it does help, even a little, to know we're not the ones making that choice.

Your Next Step: Institutional Escalation

Candidate: If Leo were to deteriorate suddenly with no time to wait, our senior doctors can act immediately under emergency necessity to save his life, with appropriate senior sign-off and documentation. If he remains stable for a few hours but will need blood soon, we would urgently involve our hospital's legal team to seek a court order authorising the specific treatment, which would formally override the need for your consent for this particular decision. I'll also contact our Hospital Liaison Committee for Jehovah's Witnesses, who support families exactly like yours and help us use the most advanced bloodless techniques available, and I'll make sure our paediatric psychology team and chaplaincy are available to support you both throughout this.

Oral Examiner Follow-Up: The Key Differentiator

Examiner: *"How does this scenario differ legally from an adult refusing blood for themselves?"*

Model Answer: For a competent adult, autonomy is paramount: if they refuse treatment for themselves, even life-saving treatment, and demonstrate full capacity, that refusal is legally binding, and forcing treatment would constitute battery. For a child, the welfare principle under the Children Act 1989 takes priority: parents cannot refuse life-saving treatment for their child based on religious belief, and doctors have both a legal and ethical duty to act to protect the child's life, escalating to emergency necessity or a court order if parental consent is withheld.

Station Conclusion: An "Excellent Pass" Paediatric Refusal Profile

Why your final choices scored maximum marks:

- **Validated the Parents Without Ceding the Decision:** You recognised their love and distress while being unambiguous that this specific decision is not theirs to make.
- **Correctly Applied the Children Act 1989:** You explained that the child's welfare is the paramount, overriding consideration, distinct from adult autonomy law.
- **Maximised Non-Blood Options First:** You showed genuine effort to respect their faith wherever clinically possible, rather than dismissing it once the legal point was made.
- **Lifted the Burden of Guilt:** You explicitly told the parents that the medical/legal team, not them, bears responsibility for the decision — a high-value Interpersonal Skills technique in this exact station.
- **Named the Correct Escalation Pathway:** You distinguished between genuine emergency necessity and, where time allows, seeking a court order — rather than treating both scenarios the same way.

SECTION: MUSCULOSKELETAL MEDICINE

Station 55 — Musculoskeletal Medicine: Metatarsalgia

Metatarsalgia typically presents in a GP or orthopaedic outpatient station. The patient — often someone who stands for long hours, runs regularly, or wears tight or high-heeled shoes — complains of burning, aching pain under the ball of the foot. Metatarsalgia is a symptom (pain and inflammation under the metatarsal heads) rather than a single disease, and the station tests your ability to differentiate it from common mimics.

Key Differential Diagnosis

Condition	Classic Presentation	Key Diagnostic Feature
Metatarsalgia	Burning, aching pain under the ball of the foot; like walking on a pebble	Worse barefoot on hard surfaces; relieved by rest
Morton's Neuroma	Sharp, shooting pain or numbness between the 3rd and 4th toes	Positive Mulder's click (pain/click on squeezing the metatarsal heads together)
Plantar Fasciitis	Sharp heel pain, worst with the very first steps in the morning	Tenderness localised to the medial plantar tubercle of the heel
Gout / Septic Arthritis	Acute, severe, red, hot, swollen joint (classically the big toe)	Systemic signs, sudden onset, unable to bear weight

The Roleplay

Station: General Practice Outpatients / Musculoskeletal Consultation

Candidate Role: FY2 Doctor in a GP Surgery

Patient/Roleplayer Role: Emily Clarke, 34-year-old bank teller

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Emily Clarke, 34, has a nagging pain in the bottom of her right foot for the last 6 weeks, making her job difficult.

Your Task:

- Take a focused history and identify lifestyle triggers.
- Verbally propose your examination plan.
- Deliver a reassuring diagnosis and NHS conservative management plan.

(Emily sits gently rubbing the front of her right foot through her shoe, looking tired.)

Candidate: Hello Ms Clarke, my name is Dr [Your Name]. I see you've been having some trouble with your right foot lately — can you tell me more about what's been happening?

Emily: Yes, it's this nagging pain right under the ball of my right foot. It feels like a constant, dull, burning ache — honestly, sometimes it feels like I'm walking around with a small pebble stuck inside my shoe. Standing up at my bank counter all day is becoming torture.

Common Pitfall: *"This sounds like an emergency nerve compression or a bone fracture. I need to order an urgent MRI and give you crutches."* — Wildly over-investigates a classic mechanical presentation, alarming the patient unnecessarily.

Model Response

Candidate: I'm really sorry to hear how much this is affecting your work, Emily — standing all day with that kind of discomfort must be exhausting. To help me understand what's causing this burning sensation, when is the pain at its worst? And what kind of shoes do you normally wear for your shifts?

Emily: It's definitely worst toward the end of my working day after standing for hours. It feels better if I sit down and kick my shoes off, but the moment I step barefoot onto the hard kitchen tiles at home, the burning comes right back. I wear smart, narrow leather slip-ons with a slight heel for work — they've started feeling like they're squeezing the front of my foot into a vice.

Your Next Step: Ruling Out Mimics and Proposing Examination

Candidate: Have you noticed any numbness or shooting pain between your toes? And how does it feel first thing in the morning, before you've been on your feet?

Emily: No numbness or shooting pain, just that deep burning ache under the ball of my foot. In the mornings it isn't too bad — it flares up the longer I stand on it.

Candidate: That's very helpful, thank you. I'd like to examine your foot now — I'll look for any calluses or deformities, feel along the metatarsal heads for tenderness, gently squeeze the forefoot to check for Morton's neuroma, and check the sensation and pulses in your foot.

Examiner's Findings Card

- Inspection: mild redness and a small, thickened callus under the second and third metatarsal heads; no deformity or joint swelling.
- Palpation: tenderness localised to the plantar aspect of the 2nd and 3rd metatarsal heads; negative Mulder's click.
- Neurovascular: normal sensation; capillary refill under 2 seconds; pulses strong and regular.

Your Next Step: Diagnosis and Management

Candidate: Thank you for letting me examine your foot, Emily. Based on everything you've told me and what we've found today, you have a condition called metatarsalgia. In simple terms, the ball of your foot — which acts as the main shock absorber when you walk or stand — has become bruised and inflamed. The narrow, high-heeled shoes you wear for your long shifts are squeezing the bones together and shifting your body weight onto that one sensitive area, causing that 'walking on a pebble' feeling.

Emily: Oh, that makes sense. Is it something I'll need surgery for? I'm worried about taking weeks off work.

Candidate: The good news is you won't need surgery at all — this responds really well to simple, lifestyle-focused steps. The most important change is switching your work shoes to a pair with a wide toe box and a flat, well-cushioned sole, to stop the squeezing and spread your weight evenly. I'd also recommend over-the-counter metatarsal pads or gel insoles to cushion that sore spot specifically. For symptom relief, an ice pack — like a bag of frozen peas wrapped in a towel — applied for 15 minutes after a long shift can help with the inflammation, alongside an over-the-counter anti-inflammatory gel if needed. Try to minimise running or jumping on hard surfaces for a few weeks to let it heal.

Emily: That sounds really doable. I can find some smart, flat, wide shoes, and I'll order those gel pads tonight. Thank you!

Candidate: You're welcome. We'll review how you're getting on in about 4 to 6 weeks. But please come back sooner if you develop any new numbness or tingling in your toes, if the foot becomes hot, red, and swollen, or if you become completely unable to walk on it. I'll print you an information leaflet with all of this to take home.

Station Conclusion: An "Excellent Pass" Metatarsalgia Profile

Why your final choices scored maximum marks:

- **Correct Differential Screening:** You ruled out Morton's neuroma (negative Mulder's click, no shooting pain/numbness) and plantar fasciitis (no first-step morning pain) before settling on the diagnosis.
- **Verbalised the Examination Plan:** You explicitly stated each examination step for the examiner rather than performing it silently.
- **Clear, Jargon-Free Analogy:** Describing the metatarsal heads as "shock absorbers" made the diagnosis intuitive without medical jargon.
- **Addressed Her Unspoken Fear:** You directly answered her concern about surgery and time off work.
- **Practical, Trigger-Specific Management:** Your plan directly solved her workplace trigger (narrow shoes) rather than offering generic advice.

SECTION: OBSTETRICS & GYNAECOLOGY

Station 56 — Obstetrics: Severe Pre-Eclampsia

Pre-eclampsia is a hypertensive disorder of pregnancy defined by new-onset hypertension after 20 weeks' gestation together with evidence of maternal organ dysfunction (such as proteinuria, deranged liver function, or low platelets) or fetal growth restriction. This station tests recognition of severe features and correct emergency management, alongside Stations 3 (PPH) and 6 (Ruptured Ectopic Pregnancy) elsewhere in this book.

The Roleplay

Station: Maternity Triage

Candidate Role: FY2 Doctor in Maternity Triage

Patient/Roleplayer Role: Sarah Jenkins, 34, 32 weeks pregnant with her first baby

Setting: Maternity Triage Unit

The Prompt (read outside the cubicle):

Patient Details: Sarah Jenkins, 34, sent in by her midwife after her blood pressure was found to be running high this afternoon.

Your Task:

- Take a focused history to screen for severe features of pre-eclampsia.
- Propose the correct investigations.
- Initiate the correct emergency management.

Candidate: Hello Sarah, I'm Dr [Your Name], one of the maternity doctors today. I understand your midwife sent you in because your blood pressure was running high this afternoon. How are you feeling right now?

Sarah: My midwife completely panicked and sent me straight here. I've had this terrible, throbbing headache across my forehead since yesterday morning, and paracetamol hasn't touched it. I'm scared about what this means for my baby.

Candidate: I completely understand why you're worried, especially when a routine appointment suddenly changes like this. You've done exactly the right thing coming straight in. Before we check your reflexes, do you mind if I ask a few specific questions about your headache and how you've been feeling overall?

Sarah: Yes, please ask whatever you need.

Your Next Step: Screening for Severe Features

Candidate: Apart from the headache, have you noticed any changes to your sight, like blurring, flashing lights, or dark spots? Any sharp pain or a deep ache at the very top of your stomach, under your ribs on the right side? Any sudden swelling in your face or hands over the last day or two, or a drop in how much fluid you're passing?

Sarah: Yes, actually — over the last few hours I've been seeing flashing zig-zag lights in the corners of my eyes. And my stomach has been hurting, like a constant, tight, heavy ache right at the top, just below my breastbone. My wedding ring has become so tight today I can't get it off — my fingers have swollen up like sausages.

Candidate: Have you noticed any involuntary twitching, jerking, or shaking in your arms or legs? Any restlessness, confusion, or feeling detached from your surroundings? Any shortness of breath or a sudden, tight cough when lying flat?

Sarah: No twitching or shaking, thankfully, and no confusion — just very tired from the headache. My breathing's okay sitting up, but lying flat on the trolley earlier I felt short of breath and had to sit back up. No cough though.

Your Next Step: Examination and Investigations

Candidate: Thank you for being so patient with all of that. I'd now like to check your blood pressure, listen to your chest, test your reflexes, and dip your urine for protein. I'll also send bloods to check your full blood count, liver and kidney function, and uric acid level.

Examiner's Findings Card

- Blood pressure: 165/110 mmHg.
- Urine dipstick: 2+ protein.
- Chest: clear bilaterally.
- Reflexes: brisk knee jerks with 3 beats of clonus at the ankle.
- Bloods: mildly deranged liver function; platelets at the lower end of normal.

Common Pitfall: *"Your blood pressure is a bit high, but since you're not fitting and your bloods are only mildly abnormal, let's just recheck things in a week at your next midwife appointment."* — Fails to recognise that visual disturbance, epigastric pain, brisk reflexes with clonus, and proteinuria together indicate severe pre-eclampsia requiring immediate inpatient treatment, not routine outpatient follow-up.

Model Response

Candidate: Sarah, everything together — your headache, the flashing lights, the pain under your ribs, the swelling, your high blood pressure, protein in your urine, and those brisk reflexes with clonus — points to a condition called pre-eclampsia, and given how severe your symptoms and blood pressure are, this is the severe form. This means we need to treat you urgently and admit you to hospital. We're starting you on a blood pressure tablet called Labetalol right away, and we're also setting up a drip called Magnesium Sulfate, which protects your brain and prevents seizures, which is the most serious complication of this condition. The midwifery team will monitor your blood pressure, reflexes, and fluid balance closely, and we'll also keep a close eye on your baby with monitoring and growth scans.

Sarah: I see... pre-eclampsia. I've heard of that before, and I understand why it's so important to stay in hospital now. It's a relief to know there's clear medication to bring the pressure down and protect me and the baby.

Your Next Step: Safety-Netting and Escalation

Candidate: I'm also involving our senior obstetric team urgently, since severe pre-eclampsia sometimes means we need to discuss the timing of delivery, even this early in the pregnancy, to keep you both safe. If you ever notice sudden severe abdominal pain, vaginal bleeding, reduced movements from the baby, or if you feel a fit coming on, please tell us immediately.

Station Conclusion: An "Excellent Pass" Severe Pre-Eclampsia Profile

Why your final choices scored maximum marks:

- **Complete Severe Features Screen:** You screened for visual disturbance, epigastric pain, clonus, and reduced urine output — the key features distinguishing severe from mild pre-eclampsia.
- **Correct Investigations:** You proposed urine protein testing and bloods (FBC, LFTs, urate) alongside the clinical examination, rather than relying on reflexes alone.
- **Accurate Emergency Management:** You started antihypertensive treatment and magnesium sulfate for seizure prophylaxis, and arranged appropriate monitoring.
- **Involved the Senior Team:** You recognised this needs urgent senior obstetric input, including discussion of delivery timing.
- **Specific Safety-Netting:** You gave concrete red flags (severe pain, bleeding, reduced fetal movements, seizure warning signs) rather than vague reassurance.

Station 57 — Gynaecology: Postmenopausal Bleeding on Tamoxifen

Postmenopausal bleeding always warrants urgent investigation to exclude endometrial cancer, and the risk is further raised in patients taking Tamoxifen, which has an oestrogenic effect on the endometrium despite its anti-oestrogenic action on breast tissue.

The Roleplay

Station: General Practice

Candidate Role: FY2 Doctor in a GP Surgery

Patient/Roleplayer Role: Margaret Jenkins, 59, menopause 4 years ago

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Margaret Jenkins, 59, has noticed fresh pink vaginal bleeding on wiping over the last week, occurring a few times.

Your Task:

- Take a focused history to assess bleeding source and risk factors.
- Explain the fast-track referral pathway clearly.
- Safety-net appropriately.

Candidate: Hello Mrs Jenkins, I'm Dr [Your Name], one of the doctors here today. I see you've noticed some unexpected bleeding recently — how can I help?

Margaret: I'm honestly terrified. My periods stopped four years ago, but last week I noticed fresh pink blood when I wiped, and it's happened a few times since. I looked it up online and it said this is a sign of womb cancer. I have a brand new grandson and I'm scared I won't see him grow up.

Candidate: I can hear how overwhelming and frightening this is, and it's entirely natural to feel terrified seeing an unexpected change like this. You've done exactly the right thing coming in straight away. Before we talk about specialists, do you mind if I ask a few gentle questions to understand your health and this bleeding a bit better?

Margaret: Yes, please ask me whatever you need.

Your Next Step: Focused History

Candidate: Is the bleeding definitely coming from your front passage, or could it be from your waterworks or back passage? Are you currently taking any Hormone Replacement Therapy, or a breast cancer medication called Tamoxifen? Have you had any deep pelvic pain, a feeling of heavy pressure in your belly, or unexpected weight loss?

Margaret: It's definitely from my front passage. I've never taken HRT, but my GP started me on Tamoxifen three years ago after surgery for early-stage breast cancer. No severe pain, just a constant, dull, heavy fullness at the bottom of my belly over the last month. No weight loss that I've noticed.

Your Next Step: Explaining the Referral Pathway

Candidate: Thank you for being so detailed, Margaret. Bleeding four years after your menopause, combined with taking Tamoxifen, means we need to look closely at the lining of your womb. While this can often be caused by harmless things like thinning of the vaginal skin or benign polyps, we have a fast-track safety system in the UK called a Two-Week Wait referral. I'm arranging for a gynaecologist to assess you within the next 14 days. They'll perform an ultrasound scan to check the thickness of your womb lining, and may take a small tissue sample in clinic to check the cells under a microscope — this is a precautionary step to rule out anything serious and get you clear answers quickly.

Margaret: An ultrasound to check the thickness... okay, that sounds sensible. Knowing they can do it so quickly is a real comfort. What do I do now?

Your Next Step: Finalising the Pathway and Safety-Netting

Candidate: I know this is a lot to take in, Margaret, but you're not alone, and we're moving quickly to keep you safe. The hospital will contact you directly within the next few days to book your appointment — please keep your phone nearby, and if you haven't heard anything within 5 to 7 days, contact our clinic straight away so we can chase it up. As a safety net, if you experience a sudden rush of heavy bleeding, pass large blood clots, or develop severe lower belly pain before your appointment, please go straight to A&E.

Margaret: I've written that down — expecting a call within a few days, and calling back if I hear nothing by day seven. Thank you for not rushing me and for explaining exactly what to expect.

Station Conclusion: An "Excellent Pass" Postmenopausal Bleeding Profile

Why your final choices scored maximum marks:

- **Identified the Tamoxifen Risk:** You specifically asked about Tamoxifen and recognised its endometrial risk, rather than treating this as routine postmenopausal bleeding alone.
- **Correct Fast-Track Pathway:** You applied the Two-Week Wait referral correctly, matching UK NICE guidance for postmenopausal bleeding.
- **Balanced Reassurance With Urgency:** You acknowledged benign causes are common while still treating this with appropriate urgency.
- **Concrete Safety-Netting:** You gave a specific callback timeframe and clear emergency red flags.

Station 58 — Obstetrics: Anal Fissure in Pregnancy

Anal fissures — small tears in the anal canal lining — are common in the second and third trimesters, driven by pregnancy-induced progesterone slowing gut motility (causing constipation) and mechanical pressure from the growing uterus. This station tests handling an intimate presentation with modesty and empathy, ruling out red flags, and delivering a pregnancy-safe management plan per UK NICE CKS guidance.

Pregnancy-Safe Management Matrix

Treatment Layer	Pregnancy-Safe Options	Avoid / Use With Caution
First-line / lifestyle	High-fibre diet, good hydration, warm sitz baths to relax the internal sphincter	Excessive straining or prolonged sitting on the toilet
Laxatives	Bulk-forming (e.g. ispaghula husk/Fybogel) first-line; osmotic (e.g. lactulose, macrogols) safe second-line	Stimulant laxatives (e.g. senna) generally avoided/used cautiously — theoretical risk of stimulating uterine contractions
Topical pain relief	Short-term topical 5% lidocaine ointment before bowel movements	Topical GTN or diltiazem — used with extreme caution in pregnancy due to maternal hypotension risk

The Roleplay

Station: Consultation Skills / Patient Counselling

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Emma Watson, 28, 32 weeks pregnant

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Emma Watson, 28, 32 weeks pregnant with her first baby, has booked an urgent appointment.

Presenting Complaint: Excruciating pain during and after bowel movements, with streaks of bright red blood when wiping.

Your Task:

- Take a focused history to confirm an anal fissure and rule out red flags.
- Explore ICE and reassure regarding the baby's safety.

- Formulate a pregnancy-safe management plan.

(Emma sits uncomfortably on the edge of her chair, shifting her weight, looking embarrassed and tearful.)

Candidate: Hello Emma, I'm Dr [Your Name]. Congratulations on your pregnancy. I can see you're sitting quite uncomfortably and looking distressed — how can I help you today?

Emma: I'm so embarrassed to even talk about this, but I'm in absolute agony. For two weeks, every time I pass stools it feels like I'm passing shards of broken glass. The pain is so sharp it makes me scream, then leaves this throbbing ache for hours. This morning there was bright red blood on the tissue. I'm terrified something has ruptured inside me, or that this is going to harm my baby.

Common Pitfall: *"Looking at how much you're bleeding and your severe pain, you likely have a deep tear or a bowel tumour. I need you to undress from the waist down so I can perform a digital rectal exam right now."* — Jumps to an unnecessarily invasive examination and alarming differential without first taking a history, and a digital rectal exam is specifically avoided here as too painful and unnecessary.

Model Response

Candidate: I'm so sorry you're going through this, Emma, and please don't feel embarrassed at all — what you're describing is remarkably common in later pregnancy. I want to reassure you straight away: this bleeding is coming from your back passage, not your birth canal, so your baby is completely safe and unaffected. Before we talk about soothing the pain, do you mind if I ask a few gentle questions to confirm exactly what's causing this?

Emma: (visible relief) Oh, thank goodness... hearing the baby is safe helps so much. Yes, please ask whatever you need — I just want this glass-like pain to stop.

Your Next Step: Characterising the Bleed and Excluding Red Flags

Candidate: When you notice the blood, is it mixed into the stool itself, or just streaks on the paper or coating the outside? Have you been struggling with constipation or hard, large stools since your pregnancy progressed? Have you had any fever, pus-like discharge, or unexpected weight loss?

Emma: The blood is definitely not mixed in — just bright red streaks on the tissue and a little coating the outside of the stool. My constipation has been awful since my third trimester — I feel like I'm passing rocks and have to strain so hard. It was during one of those really hard movements two weeks ago that I felt a sudden, sharp rip, and that's when this pain started. No fever, no pus, and I'm not losing weight — I'm gaining it normally for the baby.

Your Next Step: Diagnosis and Pregnancy-Safe Management

Emma's history — sudden rip during a hard stool, sharp glass-like pain, superficial bright red bleeding — is classic for an acute anal fissure. A digital rectal exam is avoided here, as it's too painful and unnecessary; a gentle external inspection is sufficient if needed.

Common Pitfall: *"This confirms a severe tear. I'll prescribe topical GTN ointment to use twice a day, along with strong Senna tablets to force your bowels to move."* — GTN is a second-line option used with caution in pregnancy given the hypotension risk, and stimulant laxatives like senna are generally avoided in pregnancy — this is neither the correct first-line choice nor the safest one.

Model Response

Candidate: Thank you for being so clear, Emma. Everything you've described — the sudden rip during a hard movement, the sharp pain, and the bright red blood on the paper — points to a small, superficial tear in the lining of your back passage, called an anal fissure. It's incredibly common in pregnancy, because your hormones slow down digestion, leading to hard stools that stretch the skin there. We're going to start a safe, three-step plan today. First, for the constipation, I'm prescribing a pregnancy-safe, bulk-forming fibre supplement called Fybogel, to soften your stools so they pass without stretching the tear further. Second, for the pain, a short-term 5% lidocaine numbing ointment, applied a few minutes before you use the toilet, to block that sharp sensation. Third, a warm sitz bath — sitting in warm water for about 15 minutes after a bowel movement — to relax the muscles and soothe the ache. How does that sound?

Emma: That sounds so much more manageable than I expected. I'll try all of that — thank you for explaining it so clearly.

Your Next Step: Safety-Netting

Candidate: Most fissures heal well within a few weeks with this approach. But please come back if the bleeding becomes heavier or the blood becomes mixed into the stool itself, if you develop a fever or discharge, or if the pain doesn't start improving despite softer stools — we'd want to make sure we're not missing anything else.

Station Conclusion: An "Excellent Pass" Anal Fissure in Pregnancy Profile

Why your final choices scored maximum marks:

- **Immediate Reassurance About the Baby:** You clarified early that the bleeding source has no bearing on fetal safety, addressing her core unspoken fear.
- **Avoided an Unnecessary Invasive Exam:** You recognised a digital rectal exam is inappropriate here, rather than defaulting to it.
- **Correct Pregnancy-Safe Prescribing:** You chose bulk-forming laxatives and short-term lidocaine over stimulant laxatives or GTN, matching UK guidance for pregnancy.
- **Clear, Structured Three-Step Plan:** You organised the management into distinct, easy-to-follow layers — bowel, pain, and mechanical relief.
- **Appropriate Safety-Netting:** You gave specific red flags that would suggest a different diagnosis, rather than assuming the fissure explanation is definitely correct without follow-up.

Station 59 — Gynaecology: Dysmenorrhoea, Endometriosis, and Endometrioma

This station tests your ability to differentiate primary from secondary dysmenorrhoea, recognise the pattern suggestive of endometriosis, and understand the ovarian endometrioma ("chocolate cyst") as one possible complication. Endometriosis — the presence of endometrial-like tissue outside the uterus — is a common, under-diagnosed cause of chronic pelvic pain, often with a significant diagnostic delay.

Primary vs Secondary Dysmenorrhoea

- **Primary dysmenorrhoea:** crampy, suprapubic pain starting with or just before menstruation, with no underlying pelvic pathology; typically begins within 1–2 years of the first period.
- **Secondary dysmenorrhoea:** pain due to an underlying cause — most commonly endometriosis, but also adenomyosis, fibroids, or pelvic inflammatory disease; classically develops later in life, and often progressively worsens rather than staying stable.

Features Suggestive of Endometriosis

- Pelvic pain worsening over time, often starting before the period and persisting after it ends.
- Deep dyspareunia (pain during deep penetration during sex).
- Cyclical bowel or bladder symptoms (pain on opening bowels or passing urine, worse around menstruation).
- Subfertility.
- **Endometrioma ("chocolate cyst"):** an ovarian cyst formed from endometrial tissue and altered blood, classically seen on ultrasound as a cyst with homogeneous, low-level ("ground-glass") internal echoes.

The Roleplay

Station: General Practice / Gynaecology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Chloe Bennett, 27 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Chloe, 27, has booked an appointment about worsening period pain over the last 2 years.

Your Task:

- Take a focused history to distinguish primary from secondary dysmenorrhoea.
- Screen for features of endometriosis.

- Explain the investigation and management pathway.

(Chloe sits looking tired and frustrated.)

Candidate: Hello Chloe, I'm Dr [Your Name]. I understand your period pain has been getting worse — can you tell me more about it?

Chloe: I've had period pain since my teens, but it was always manageable with paracetamol. Over the last two years, though, it's become excruciating — it starts a few days before my period and doesn't fully go away until well after it's finished. I've had to call in sick from work multiple times.

Candidate: That sounds like it's really taken over a big part of your life — I'm sorry you've been struggling with this. Can I ask, do you notice any pain during sex, particularly with deep penetration? And any pain when you're going for a wee or opening your bowels, especially around your period?

Chloe: Yes, actually — sex has become quite painful over the last year, especially in certain positions. And now that you mention it, I do get quite bad pain opening my bowels in the days just before my period starts, though it's fine the rest of the month.

Common Pitfall: *"This just sounds like normal period pain — try some stronger ibuprofen and a hot water bottle, and it should settle."* — Fails to recognise the progressive worsening, deep dyspareunia, and cyclical bowel symptoms as a classic pattern for endometriosis rather than simple primary dysmenorrhoea.

Model Response

Candidate: Thank you for being so open about all of that, Chloe. The pattern you're describing — pain that's progressively worsened, pain during sex, and pain with your bowels specifically around your period — points strongly towards a condition called endometriosis, where tissue similar to the lining of the womb grows outside the uterus, most often on the ovaries, fallopian tubes, or pelvic lining. It responds to your hormonal cycle in the same way the womb lining does, which is why the pain is so closely tied to your periods.

Chloe: That actually makes so much sense. Is this something serious? And could it affect me having children one day?

Candidate: It's a genuine, recognised condition, and while it can affect fertility for some women, many people with endometriosis do go on to conceive naturally, especially with the right management. I'd like to arrange a pelvic ultrasound scan to look at your womb and ovaries — sometimes endometriosis can form a cyst on the ovary containing old blood, sometimes called a 'chocolate cyst,' which the scan can pick up. It's worth knowing that a normal ultrasound doesn't rule out endometriosis completely, since some of the tissue can be too small or subtle to see on a scan — if your symptoms continue despite treatment, the definitive way to diagnose it is a small operation called a laparoscopy, where a specialist looks directly inside your pelvis with a camera.

Your Next Step: Management Options

Candidate: In the meantime, we can start with simple measures: regular anti-inflammatory painkillers like ibuprofen or a combination with paracetamol, taken regularly rather than only when the pain is severe, tend to work better. Hormonal treatments, such as the combined pill or certain hormonal coils, can also significantly reduce the pain by suppressing your cycle. If these don't control things well enough, I'll refer you to gynaecology, who can discuss the laparoscopy, or other hormonal treatments like GnRH analogues, which more strongly suppress the condition.

Chloe: That's really reassuring, thank you. I'll try the regular ibuprofen and think about the hormonal options — I just wanted someone to finally take this seriously.

Station Conclusion: An "Excellent Pass" Endometriosis Profile

Why your final choices scored maximum marks:

- **Distinguished Primary From Secondary Dysmenorrhoea:** You recognised progressive worsening over years, rather than stable pain since menarche, as pointing away from simple primary dysmenorrhoea.
- **Screened the Full Symptom Triad:** You specifically asked about deep dyspareunia and cyclical bowel symptoms, not just the pain itself.
- **Accurate Investigation Counselling:** You explained that ultrasound can identify an endometrioma but cannot exclude endometriosis entirely, and that laparoscopy remains the definitive diagnostic test.
- **Balanced Fertility Discussion:** You acknowledged the potential impact on fertility honestly without being unnecessarily alarming.
- **Stepped Management Plan:** You outlined analgesia, hormonal treatment, and specialist referral as an escalating pathway rather than a single option.

Station 60 — Gynaecology: Cervical Screening Counselling and Procedure

A cervical screening station tests your ability to counsel a hesitant or anxious patient about the purpose and process of screening, take a focused history to time the test correctly, and (if the station includes a practical component) perform the procedure safely on a pelvic mannequin. This station complements Station 38 (Cervical Screening in a Trans Man), applying the same core skill to the general population.

Focused History Before Screening

- **Last menstrual period:** ideally the test is done mid-cycle, and not during active bleeding.
- **Red flags to screen for:** post-coital bleeding, intermenstrual bleeding, postmenopausal bleeding, unusual discharge, pelvic pain, or deep dyspareunia — these warrant a symptomatic gynaecology assessment rather than routine screening alone.
- **Prior screening history:** any previous tests, results, or treatments to the cervix (e.g. colposcopy, LLETZ).
- **HPV vaccination history.**

The UK Screening Schedule

- Ages 25–49: screened every 3 years.
- Ages 50–64: screened every 5 years.

The sample is tested for high-risk HPV first; cytology (checking for abnormal cells) is only performed if HPV is detected.

The Roleplay

Station: General Practice / Women's Health Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Sarah Jones, 27 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Sarah, 27, has received her first routine cervical screening invitation and has booked an appointment to discuss it, appearing hesitant.

Your Task:

- Explore her ideas, concerns, and expectations about the test.
- Take a focused history to confirm she's suitable for screening now.
- Explain the purpose, process, and timeline clearly.

(Sarah sits with her arms crossed, looking uncomfortable.)

Candidate: Hello Sarah, I'm Dr [Your Name]. I understand you've come in to talk about your cervical screening invitation — how can I help with that today?

Sarah: Honestly, I've never had one before and I'm dreading it. A friend told me hers was really painful, and I suppose I'm also just embarrassed about the whole thing.

Candidate: Thank you for being honest about that — it's an incredibly common way to feel, and lots of people feel exactly the same nervousness before their first test. Can I ask what you understand about what the test is actually for, and is there anything specific that's worrying you most?

Sarah: I know it's something to do with checking for cancer, but that's about it. I think what worries me most is the pain, and just how exposing it feels.

Common Pitfall: *"There's really nothing to worry about, it's just a quick, routine test — let's get you booked in." — Dismisses her specific fears rather than addressing them individually, and skips explaining what the test actually involves or is for, missing an opportunity to build genuine reassurance.*

Model Response

Candidate: That's really helpful to know, thank you. First, to correct a common misconception — this isn't actually a test for cancer itself. It's a preventative screening test that checks for a very common virus called HPV, which can, over a long time, cause changes to the cells of the cervix if left unmonitored. Catching this early is exactly what makes the test so effective at preventing cervical cancer, rather than diagnosing it. In terms of the test itself, it involves a small plastic instrument called a speculum, and a soft brush to gently collect a few cells — it can feel like some pressure or mild discomfort, but it shouldn't be painful, and you are in complete control throughout; you can ask the nurse to stop at any point, use a smaller speculum, or bring a friend with you for support.

Sarah: That's actually really reassuring — I didn't realise it wasn't testing for cancer directly, and I like knowing I can ask them to stop.

Your Next Step: Confirming Suitability and Timing

Candidate: To make sure we get you booked at the right time, when was your last period, and are your cycles fairly regular? Also, have you ever had any bleeding after sex, in between your periods, or any unusual discharge or pelvic pain?

Sarah: My last period finished about a week ago, and my cycles are pretty regular, around 28 days. No bleeding after sex or between periods, and no unusual discharge or pain.

Candidate: That's great, that timing works well, since we ideally do the test away from active bleeding. Given everything you've told me, there's nothing here suggesting we need to look into anything more urgently — this is routine screening at exactly the stage it should happen. I'll book you in with our practice nurse, who does these regularly and is very experienced at putting people at ease. You're welcome to take an information leaflet home first if that would help you feel more prepared.

Sarah: Yes, please, I'd like the leaflet. I feel a lot less anxious about it now, actually.

Station Conclusion: An "Excellent Pass" Cervical Screening Counselling Profile

Why your final choices scored maximum marks:

- **Explored ICE Genuinely:** You identified her specific fears (pain, embarrassment) rather than offering generic reassurance.
- **Corrected the "Cancer Test" Misconception:** You clarified that screening detects HPV and precancerous change, rather than diagnosing cancer itself.
- **Empowered Patient Control:** You explicitly told her she could ask the nurse to stop, request a smaller speculum, or bring support, giving her genuine agency.
- **Confirmed Timing and Excluded Red Flags:** You checked her cycle timing and screened for postcoital/intermenstrual bleeding before confirming routine screening was appropriate.
- **Offered Written Information:** You gave her a leaflet to take away, respecting that she may want time to process before attending.

SECTION: PAEDIATRIC DEVELOPMENT

Station 61 — Paediatrics: Developmental Milestones and Isolated Gross Motor Delay

Developmental milestones are rarely tested as a standalone station — instead, a parent presents with a child who has a specific concern (delayed walking, speech delay, or regression), and you must rapidly screen all four developmental domains to build the full clinical picture, using age-appropriate, non-jargon language.

The Four Core Developmental Domains

Domain	6 Months	12 Months	18 Months	2 Years
Gross Motor	Sits with support; rolls over	Walks independently or cruises holding furniture	Walks steadily; climbs onto furniture	Runs safely; climbs stairs one step at a time
Fine Motor & Vision	Reaches for toys; transfers hand-to-hand	Pincer grasp (picks up a crumb)	Tower of 3 blocks; scribbles spontaneously	Tower of 6 blocks; copies a vertical line
Speech & Hearing	Polysyllabic babbling ("ba-ba")	1–3 clear words; responds to own name	6–10 clear words; points to a named body part	Joins 2+ words together ("want juice")
Social & Emotional	Smiles socially	Waves "bye-bye"; plays peek-a-boo	Feeds self with a spoon; copies household tasks	Parallel play (alongside, not with, others)

Red Flags for Urgent Referral

- Any regression or loss of previously mastered skills, at any age.
- Not sitting independently by 9 months.
- Not walking independently by 18 months.
- No clear, meaningful words by 18 months, or no 2-word combinations by 2 years.
- Persistent hand asymmetry or a strict hand preference before 12 months (suggestive of a focal neurological issue, e.g. hemiplegic cerebral palsy).

The Roleplay

Station: Paediatric Consultation / Neurodevelopmental Screen

Candidate Role: FY2 Doctor in a Paediatric Outpatient Clinic

Patient/Roleplayer Role: Sarah Jenkins, mother of 18-month-old Tommy

Setting: Paediatric Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Tommy Jenkins, 18 months, brought in by his mother Sarah, worried because a friend's younger child seems more physically advanced.

Your Task:

- Address the mother's comparison anxiety supportively.
- Take a focused developmental history covering all four domains.
- Identify any red flags and formulate a safe management pathway.

(Sarah holds Tommy, who is contentedly chewing a toy. She looks anxious and self-conscious.)

Candidate: Hello Ms Jenkins, and hello Tommy! My name is Dr [Your Name]. I can see you've come in to chat about Tommy's development compared to other children his age. I know it can feel worrying as a parent noticing differences. How can I help most today?

Sarah: I've been sick with worry. My best friend's daughter is 15 months and already running around, saying words like 'cat' and 'more.' Tommy is 18 months — three months older — and still isn't walking on his own. He just crawls or shuffles on his bottom. Have I failed him?

Common Pitfall: *"Not walking at 18 months is a massive red flag for severe brain damage or muscular dystrophy. Your friend's child is normal, but Tommy is severely delayed. I'm ordering an emergency brain scan today." — Wildly alarmist and premature; a single isolated finding shouldn't be escalated to emergency imaging before completing the rest of the developmental screen.*

Model Response

Candidate: I can completely understand why you feel worried, Sarah — it's incredibly natural to compare our little ones to others, but you have absolutely not failed Tommy. Every child develops at their own pace, and coming in today is exactly the right step. While walking independently is something we look at closely around 18 months, we need to see the whole picture — his hands, his speech, and how he interacts. Do you mind if we go through what he's doing in those areas today?

Sarah: (relieved) Thank you, Doctor... I've been blaming myself for weeks. Yes, let's look at the whole picture.

Your Next Step: Screening the Remaining Domains

Candidate: Is he cruising — walking while holding onto furniture? How is he with his hands — can he pick up small objects or build things? Does he react to noises, and does he say any meaningful words? Does he smile, wave, or play interactively with you?

Sarah: He can pull himself up on the sofa and shuffle sideways along it, but won't let go to take a step alone. He loves his wooden blocks — he stacks three or four before they tumble, and gets so

proud. He tries hard with his spoon, gets some yoghurt in his mouth even if a lot ends up on the floor. And he copies me — if I wipe the coffee table, he grabs his blanket and 'helps.'

Your Next Step: Completing the Speech Screen and Background History

Candidate: That's really helpful. How's his hearing and talking — does he react to sounds, and does he say any words? And going back a little — was he born on time, any issues at birth, and has he ever lost a skill he used to have?

Sarah: His hearing is sharp — he turns round instantly when he hears the front door. He says 'Mama,' 'Dada,' 'bye-bye,' 'baba' for his bottle, and 'cat' — about four or five clear words. He understands a lot too — if I ask him to bring my shoes, he'll fetch them. Pregnancy and delivery were completely normal, born on his due date, just a bit of jaundice that cleared on its own. Fully up to date with his jabs. His health visitor noticed he wasn't walking at his 1-year check but said to give it time. His older brother was running at 11 months, which worries me even more. No family history of muscle or nerve problems, and he's never been seriously ill.

Your Next Step: Synthesis and Management

Tommy has normal speech, fine motor, and social development, but an isolated gross motor delay — cruising but not walking independently at 18 months. UK guidance is that any child not walking independently by 18 months should be referred to rule out an underlying neuromuscular condition or cerebral palsy, even when thriving in every other domain.

Candidate: Thank you for all of that detail, Sarah — it really helps build a complete picture. The good news is that Tommy's speech, his hand skills, and how he plays and interacts with you are all right on track for his age, which is reassuring. The one area we do want a specialist to look at is his walking — not walking independently by 18 months is something we always check further, even in children doing brilliantly everywhere else, just to make sure his muscles and nerves are developing exactly as they should. I'll refer him to our Community Paediatric Team, and I'll also loop in your Health Visitor, who can visit at home with some practical play and movement exercises while you wait for the appointment. Please bring him straight back if he ever loses a skill he already has — like stopping talking or stacking his blocks — but otherwise, this is simply a precautionary check.

Sarah: Thank you so much, Doctor. It's such a relief to hear his speech and hands are doing well. If a specialist needs to check his walking just to be safe, I'm completely on board. I feel much more at ease now.

Station Conclusion: An "Excellent Pass" Developmental Milestones Profile

Why your final choices scored maximum marks:

- **Systematic Four-Domain Screen:** You covered gross motor, fine motor, speech, and social development rather than fixating on the presenting concern alone.
- **Recovered to Check Background History:** You went back for birth history, immunisations, family history, and regression — completeness that's easy to skip under time pressure.
- **Correctly Applied the 18-Month Walking Red Flag:** You referred appropriately despite normal findings elsewhere, rather than reassuring without action.
- **Named the Correct UK Pathway:** You specified the Community Paediatric Team and Health Visitor involvement, rather than a vague "specialist referral."

- **Specific Safety-Netting:** You gave a concrete red flag (loss of an existing skill) rather than generic reassurance.

Station 62 — Paediatrics: Autism Spectrum Disorder Screening

An ASD screening station typically presents with a parent whose child has a speech delay, apparent "hearing concerns" (because the child seems to ignore them), or behavioural differences flagged by nursery staff. Your task is to screen for the core features of ASD using non-judgemental, everyday language, while managing a parent who may feel defensive or blindsided.

The Core Features of ASD

Current diagnostic classification (DSM-5) describes ASD across two core domains, rather than the older three-part "triad of impairments" model:

- **1. Social communication and interaction:** reduced eye contact, lack of joint attention (not following a point), reduced response to name, and atypical use of gesture or language to communicate (e.g. leading a parent's hand to an object instead of pointing or using words).
- **2. Restricted, repetitive patterns of behaviour, interests, or activities:** lining up toys rather than imaginative play, motor stereotypies (e.g. hand-flapping), insistence on sameness, and marked distress at small changes in routine.

Speech delay itself is not a diagnostic domain of ASD — many autistic children have typical or advanced language, and many children with isolated speech delay are not autistic. It remains a valuable and common associated feature to screen for, but isolated speech delay without features from the two domains above points toward a speech and language assessment rather than an autism-specific pathway.

The Roleplay

Station: Paediatric Consultation / Behavioural Concern

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: David Miller, father of 30-month-old Leo

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Leo Miller, 30 months, brought in by his father David. Leo's nursery suggested he be checked because he "doesn't mix with the other children" and "prefers to play alone."

Your Task:

- Address the father's defensiveness and manage the emotional dynamic.
- Take a focused history exploring social communication and repetitive behaviours.
- Formulate an appropriate NHS referral pathway without diagnosing prematurely.

(David sits upright, looking slightly defensive, holding a notebook tightly.)

Candidate: Hello Mr Miller, my name is Dr [Your Name]. I understand you've come in to talk about Leo and some feedback from his nursery. How are you feeling about everything?

David: Honestly, I'm a bit annoyed. The nursery implied there's something wrong with my boy just because he likes his own space — they used words like 'developmental assessment.' Leo is brilliant. He can count blocks and knows his shapes. He's just quiet, exactly like I was as a kid. Are they overreacting, or is there really something wrong with my son?

Common Pitfall: *"I understand you're annoyed, but the nursery staff are trained professionals. If they suspect autism, we must take it seriously. Let's do an autism checklist right now to see if your son has a mental disorder."* — Dismisses the father's feelings, uses stigmatising language ("mental disorder"), and jumps straight to a checklist before any rapport or history is established.

Model Response

Candidate: I completely understand why that feedback felt frustrating, David, especially knowing how bright Leo is with his shapes and numbers. It's incredibly common for children to have different personalities — some are naturally social, others more independent. The nursery just wants to make sure Leo gets the right support if he needs it. Would it be alright if we look at how he interacts and plays at home?

David: (relaxing) Yeah, that makes sense. I guess they're just trying to help — it just shocked me. Yes, let's talk about how he is at home.

Your Next Step: Core Screening

Candidate: Does Leo look at you when you speak to him, and how does he react to his name? If you point at something across the room, does he look to see what it is? How does he play with his cars or blocks — does he play imaginatively, or does he prefer lining them up or spinning the wheels? And if he wants something like juice, how does he let you know?

David: When I speak to him, he doesn't really look me in the eye — he looks past me. If I point at something and say 'look, Leo!' he just ignores me completely. With his toy cars, he spends hours turning them upside down and spinning the wheels, or lining them up perfectly straight — if anyone moves one, he has a massive meltdown. When he's excited or upset, he flaps his hands by his ears. As for asking for things — he doesn't use words. If he wants juice, he grabs my hand and drags it to the fridge handle, without saying anything or looking at me.

Your Next Step: Completing the Background History

Candidate: Since he doesn't always respond to his name, does he react to other loud noises — just to check his hearing is working well? Has he always been like this, or did he lose any words he used to have? And were there any complications in the pregnancy or at birth?

David: His hearing seems fine for loud bangs and things like that. He's always been like this, as far as I remember — no sudden loss of words. Pregnancy and birth were both completely normal, no complications at all.

Your Next Step: Explanation and Referral

David's history suggests reduced eye contact and joint attention, repetitive and restricted play, motor stereotypies, and atypical communication (using David's hand instrumentally) — features spanning both core ASD domains. This is not something to diagnose in a single GP appointment.

Candidate: Thank you so much for sharing all of that, David — it's clear how closely you pay attention to Leo, and you've given me a really helpful picture of how he experiences the world. Leo is physically well and clearly very bright with his shapes and numbers. But looking at the whole picture — how he plays, how he lets you know what he wants, and how he responds to changes in his routine — there are a few areas where he interacts and communicates a bit differently than we'd typically expect at two and a half.

David: Right... so what does that mean? Are you saying he has autism?

Candidate: It means Leo has a distinct pattern of communication and play. Some of these features are sometimes seen on the autism spectrum, but a formal diagnosis isn't something we can make in a single appointment — it needs a dedicated team who look closely at a child's strengths and differences over time. What I'd like to do, with your permission, is refer Leo to our Community Paediatric Team and a Speech and Language Therapist, who specialise in working with bright children like Leo to understand his communication style and put the right support in place. While we wait, I'll also loop in his Nursery and Health Visitor so some practical strategies can start straight away.

David: That actually sounds very sensible. If it helps with his talking and makes nursery easier for him, I'm completely happy with that.

Candidate: I'm glad we're on the same page. To keep him safe, if you notice him suddenly losing a skill he already has, or becoming so distressed you can't settle him, please bring him straight back. I'll get the referral letters sorted today, and we'll arrange a follow-up in a few weeks to see how you're both getting on.

Station Conclusion: An "Excellent Pass" ASD Screening Profile

Why your final choices scored maximum marks:

- **Validated Without Agreeing or Dismissing:** You acknowledged David's frustration while still taking the nursery's concern seriously, rather than picking a side.
- **Strengths-Based Opening:** You highlighted Leo's strengths early, which lowered David's guard before more sensitive questions.
- **Accurate Two-Domain Screening:** You screened both social communication/interaction and restricted/repetitive behaviour, rather than treating speech delay as a stand-alone diagnostic marker.
- **Avoided Premature Diagnosis:** You explicitly stated that diagnosis requires a specialist multidisciplinary team, not a single GP appointment — a critical exam safety point.
- **Complete, Multi-Agency Pathway:** You involved the Community Paediatric Team, Speech and Language Therapy, the Health Visitor, and the nursery together, rather than a single isolated referral.

The Genetic Counselling Framework

Genetic counselling stations test your ability to communicate complex, often incurable, hereditary information with sensitivity, while respecting patient autonomy and remaining strictly non-directive — you present information and options; you do not tell the patient what decision to make.

Core Principles

- **Non-directiveness:** your role is to inform, not to persuade — patients must reach their own decision about testing, disclosure, or reproductive choices.
- **The right not to know:** a competent adult can decline predictive genetic testing, even for a condition running clearly in their family, and this must be respected.
- **Children and adult-onset conditions:** predictive genetic testing for adult-onset conditions (such as Huntington's disease) is not performed on asymptomatic children — this is deferred until the child is a competent adult who can choose for themselves, preserving their own right not to know.
- **Family implications:** a genetic result can reveal information about other relatives' risk too (e.g. siblings, children); patients should be counselled on this before testing, though you cannot disclose a patient's genetic result to relatives without consent.
- **Multiple counselling sessions:** predictive testing for serious, incurable adult-onset conditions is not a single-appointment decision — it typically involves several counselling sessions over time, ideally with psychological support, before a test is actually performed.

Station 63 — Genetic Counselling: Huntington's Disease Predictive Testing

Huntington's disease is an autosomal dominant, progressive neurodegenerative condition caused by a CAG trinucleotide repeat expansion in the HTT gene, typically presenting in mid-adulthood with chorea, cognitive decline, and psychiatric symptoms. Each child of an affected parent has a 50% chance of inheriting the expanded gene. There is currently no cure or disease-modifying treatment, only symptomatic management.

The Roleplay

Station: Genetics Clinic / Predictive Testing Counselling

Candidate Role: FY2 Doctor in a Genetics Clinic

Patient/Roleplayer Role: Rachel Combe, 29 years old

Setting: Genetics Clinic Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Rachel, 29, whose father was diagnosed with Huntington's disease two years ago. She has booked an appointment asking to be tested to find out if she has inherited the gene.

Your Task:

- Explore her reasons for wanting testing now.
- Explain the condition, inheritance pattern, and implications of a positive result.
- Explain the UK predictive testing process, including its non-directive, staged nature.

(Rachel sits with her hands clasped tightly, looking determined but anxious.)

Candidate: Hello Rachel, I'm Dr [Your Name]. I understand you've come in today because you'd like to talk about genetic testing following your father's diagnosis. How are you doing with everything?

Rachel: Honestly, it's been really hard watching my dad decline over the last two years. I just need to know if I'm going to end up like him. I want the test done today if possible — I can't stand not knowing any longer.

Common Pitfall: *"Of course, we can arrange the blood test today, and I'll call you with the result once it's back." — Skips the mandatory, staged, non-directive counselling process entirely, treating a life-altering predictive test like a routine blood test.*

Model Response

Candidate: I can hear how difficult this has been, watching your dad go through this, and how much you want certainty. This is an incredibly important decision, and I want to make sure you have everything you need before any test happens — which is why this isn't something we'd do in a single visit. Can I ask what's made you want to find out now specifically, rather than at another point in your life?

Rachel: I'm actually thinking about starting a family soon, and I don't want to pass this on to a child if I can help it.

Your Next Step: Explaining the Condition and Implications

Candidate: That's a really important reason, and it's completely understandable. Because your father carries the gene, you have a 50% chance of having inherited it yourself — like a coin flip. If you were to test positive, it would mean you will almost certainly go on to develop the condition at some point, usually in mid-adulthood, though we can't predict exactly when or how severely. There's currently no cure or way to prevent it, only treatments to help manage symptoms as they arise. A positive result would also mean any children you have in the future would each have a 50% chance of inheriting it from you.

Rachel: I understand that... it's a lot to take in, but I'd still rather know.

Your Next Step: Explaining the UK Predictive Testing Pathway

Candidate: That's completely your choice to make, and I'll support you either way. What I do need to explain is that predictive testing for Huntington's isn't a single appointment — it involves several counselling sessions over time with our specialist genetics team and psychological support, precisely because of how much this result can affect someone's life, relationships, and mental health. It's also entirely voluntary at every stage — you're free to stop the process at any point, even after starting it, and you're also free to decide you'd rather not know after all.

Rachel: I didn't realise it was that involved. That actually makes me feel a bit more reassured, in a strange way — like I won't be rushed into it.

Your Next Step: Family and Insurance Implications

Candidate: A couple of other important things to be aware of. First, because this is a hereditary condition, a result also carries information about your wider family — for example, any siblings you have would also be at risk, and they may or may not want to know. We'd talk through how you'd want to handle that. Second, on the practical side — under the UK agreement between insurers and government, insurance companies generally cannot ask about predictive genetic test results, but Huntington's disease is actually a specific exception: insurers can ask about a confirmed result if you're applying for a large life insurance policy above a certain amount. That's worth knowing before you decide to test, since it could affect things like a mortgage-linked life insurance policy in future.

Rachel: Oh, I had no idea about the insurance side of things. I'm glad you told me before I went ahead.

Candidate: I'll refer you to our regional Clinical Genetics service, where you'll have your first proper counselling session to explore all of this fully before any decision is made about testing. In the meantime, please don't hesitate to contact us if you have any questions, or if you'd like some psychological support while you think things over.

Oral Examiner Follow-Up: What If a Parent Asks to Test Their Child?

Examiner: *"What would you say if Rachel later asked whether her own future child could be tested for the Huntington's gene at a young age, so the family would 'know for sure'?"*

Model Answer: Predictive testing for adult-onset conditions like Huntington's disease is not performed on asymptomatic children in the UK. This preserves the child's own future right to decide, as an adult, whether they want to know their genetic status — a choice that belongs to them, not their parents. The exception would be if the child developed symptoms in childhood

themselves, in which case diagnostic (not predictive) testing would be appropriate to guide their own care. Parents can, however, consider prenatal options such as preimplantation genetic diagnosis if they wish to avoid passing on the gene in the first place.

Station Conclusion: An "Excellent Pass" Genetic Counselling Profile

Why your final choices scored maximum marks:

- **Explored Her Motivation First:** You asked why she wanted testing now, rather than proceeding straight to logistics.
- **Accurate, Balanced Information:** You explained the 50% inheritance risk, the lack of a cure, and the implications for future children clearly and without minimising.
- **Correctly Described the Staged UK Pathway:** You explained that predictive testing requires multiple counselling sessions, not a single blood test, and remains voluntary throughout.
- **Covered Family and Insurance Implications:** You raised the impact on siblings' risk and correctly flagged Huntington's disease as the specific exception in UK insurance rules.
- **Protected the Rights of Future Children:** You knew that predictive testing isn't performed on asymptomatic minors, preserving their future autonomy.

Station 64 — Genetic Counselling: Down Syndrome Diagnosis in a Newborn

Down syndrome (trisomy 21) is the most common autosomal trisomy, occurring when a baby has three copies of chromosome 21 instead of two. This station tests breaking the news of a suspected diagnosis to new parents shortly after birth — a sensitive, non-directive communication task, alongside explaining the confirmatory testing and holistic care pathway.

Key Associated Conditions to Screen For

- **Congenital heart disease:** present in around 40–50% of babies with Down syndrome (e.g. atrioventricular septal defect) — requires an echocardiogram.
- **Duodenal atresia:** bowel obstruction that can present with vomiting and feeding difficulty.
- **Hypothyroidism:** more common; requires screening.
- **Hearing and vision problems:** more common; requires early assessment.
- **Hypotonia:** low muscle tone, common at birth.
- **Increased risk of leukaemia:** small but recognised increased risk in childhood.

The Roleplay

Station: Postnatal Ward / Breaking News

Candidate Role: FY2 Paediatric Doctor

Patient/Roleplayer Role: Mr and Mrs Taylor, new parents

Setting: Postnatal Ward, Private Room

The Prompt (read outside the cubicle):

Patient Details: Baby Taylor, born this morning. The paediatric team examining the baby noted physical features suggestive of Down syndrome (upward-slanting eyes, a single palmar crease, low muscle tone).

Your Task:

- Break the news of a suspected diagnosis sensitively and non-directively.
- Explain the confirmatory testing process.
- Outline the holistic care and support pathway.

(Mr and Mrs Taylor are sitting with their newborn, looking tired but happy, unaware of the paediatric team's concerns.)

Candidate: Hello, congratulations on the birth of your baby. My name is Dr [Your Name], one of the paediatric doctors. I've come to talk to you both about the examination we carried out this morning. Is now an alright time?

Mrs Taylor: Yes, of course — is everything okay with her?

Candidate: Physically, she's doing well and her observations are stable, which is reassuring. During our routine newborn check, though, we noticed a few physical features that can sometimes be associated with a genetic condition called Down syndrome. I want to be upfront with you about this so we can talk it through properly.

(Mrs Taylor's face falls; Mr Taylor puts a hand on her shoulder, looking stunned.)

Mr Taylor: Down syndrome? Are you sure? What does that even mean for her?

Common Pitfall: *"I'm afraid your daughter has Down syndrome. She'll likely have learning difficulties and health problems throughout her life." — States an unconfirmed finding as a certain diagnosis before genetic testing, and leads with a bleak, deficit-focused framing rather than balanced, honest information.*

Model Response

Candidate: I know this is a lot to take in, especially today of all days. To be clear, what we've noticed are physical features that can be associated with Down syndrome — things like the shape of her eyes, a particular crease on her palm, and slightly lower muscle tone. These are only ever a strong indication, though, not a confirmed diagnosis — to know for certain, we'd need to do a simple blood test to look at her chromosomes, which usually takes a few days to come back.

Mrs Taylor: (tearfully) A few days... okay. What does it actually mean if the test comes back positive?

Your Next Step: Explaining the Condition Sensitive

Candidate: Down syndrome happens when a baby has an extra copy of one chromosome — it's not caused by anything either of you did or didn't do during the pregnancy. Children with Down syndrome have a wide range of abilities, just like any other children — some go on to have significant learning needs, and others are much more mildly affected; every child is different. There can also be an increased chance of certain health conditions, particularly some heart conditions, so while we wait for the blood test result, we'll also arrange a heart scan and some other routine checks, just to make sure we're looking after her fully from day one.

Mr Taylor: This is... a lot. We weren't expecting anything like this today.

Candidate: That's completely understandable, and there's no right way to feel about this news. Take all the time you both need — I'm not going anywhere, and there's no rush to process everything at once.

Your Next Step: The Care Pathway and Support

Candidate: Whatever the blood test shows, we'll support you both every step of the way. If it does confirm Down syndrome, we'll arrange an echocardiogram to check her heart, screening for her thyroid function, and hearing and vision assessments, since these are all things that are a bit more common and easy to look after early. I'll also put you in touch with our paediatric team who specialise in this, and with a wonderful UK charity called the Down's Syndrome Association, who have brilliant new-parent support and can put you in touch with other families going through the same thing.

Mrs Taylor: I think I'd like to speak to other parents who've been through this... it might help to know we're not alone.

Candidate: Absolutely, and we'll make sure that connection happens. For now, let's focus on getting her settled and comfortable, and I'll come back to check on you both once we have the test results.

Station Conclusion: An "Excellent Pass" Down Syndrome Disclosure Profile

Why your final choices scored maximum marks:

- **Distinguished Suspicion From Confirmed Diagnosis:** You were careful to describe physical features as suggestive, not definitive, pending the chromosome test.
- **Balanced, Non-Deficit Framing:** You described a genuine range of ability and outcomes rather than a uniformly bleak picture.
- **Removed Parental Guilt:** You proactively clarified that nothing they did caused this.
- **Proactive Medical Screening:** You arranged the echocardiogram, thyroid screening, and hearing/vision checks without waiting to be asked.
- **Connected Them to Peer Support:** You signposted the Down's Syndrome Association, recognising that parent-to-parent support is often as valuable as clinical information at this stage.

Station 65 — Paediatrics: Prolonged Neonatal Jaundice — Biliary Atresia vs Unconjugated Hyperbilirubinaemia

Prolonged neonatal jaundice — persisting beyond 14 days in a term baby, or 21 days in a preterm baby — always requires investigation to distinguish a serious, time-critical obstructive cause (biliary atresia) from the much more common, generally benign unconjugated causes. This station tests the single most important discriminating step: checking the split bilirubin, and correlating it with stool and urine colour.

The Key Distinction

Feature	Unconjugated Hyperbilirubinaemia	Biliary Atresia (Conjugated)
Mechanism	Pre-hepatic: increased bilirubin production or reduced conjugation	Post-hepatic: obstruction to bile flow out of the liver
Common causes	Physiological jaundice, breastmilk jaundice, haemolysis (ABO/Rh incompatibility, G6PD deficiency, hereditary spherocytosis), hypothyroidism	Progressive fibro-obliteration of the extrahepatic bile ducts
Stool colour	Normal (brown/yellow)	Pale, chalky, or white (acholic) — bile cannot reach the gut to colour the stool
Urine colour	Normal (pale/clear)	Dark urine — conjugated bilirubin is water-soluble and excreted in urine
Urgency	Usually straightforward outpatient workup	Surgical emergency — Kasai portoenterostomy works best if performed before 6–8 weeks of age

The critical safety point: any jaundice persisting beyond 14 days (21 days if preterm) requires an urgent split (conjugated vs unconjugated) bilirubin measurement — this single test is what separates a routine, reassuring conversation from a surgical emergency.

The Roleplay

Station: Paediatric / Health Visitor Follow-Up Clinic

Candidate Role: FY2 Doctor in a Paediatric Outpatient Clinic

Patient/Roleplayer Role: Mrs Osei, mother of 3-week-old Kofi

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Kofi, 3 weeks old, born at term, referred by the health visitor because his jaundice, first noted at day 4, has not resolved.

Your Task:

- Take a focused history to differentiate benign from serious causes of prolonged jaundice.
- Specifically ask about stool and urine colour.
- Arrange the correct urgent investigation and explain the two possible pathways.

(Mrs Osei holds Kofi, who looks otherwise well but has a yellow tinge to his skin and eyes.)

Candidate: Hello Mrs Osei, I'm Dr [Your Name]. I understand Kofi's jaundice has been going on for a few weeks now — can you tell me more about it?

Mrs Osei: Yes, it started around day 4, which the midwife said was normal, but it's now three weeks later and he still looks quite yellow, especially in his eyes. He's feeding well and gaining weight, so I wasn't too worried, but the health visitor wanted him checked.

Candidate: Thank you, that's really helpful. Is he breastfed or formula fed? And can I ask you something that might seem like an unusual question — what colour have his nappies been, both the wee and the poo?

Mrs Osei: He's exclusively breastfed. That is an odd question, but now you mention it... his poo has looked quite pale, almost a chalky, whitish colour the last week or so, and his wee has looked darker than I'd expect, more of a strong yellow-brown.

Common Pitfall: *"Since he's feeding and gaining weight well, this is almost certainly just breastmilk jaundice, which is harmless and will resolve on its own — no further tests are needed." — Breastmilk jaundice is unconjugated and would not cause pale stools or dark urine; ignoring this specific detail risks missing a time-critical diagnosis of biliary atresia.*

Model Response

Candidate: Thank you for noticing that detail, Mrs Osei — that's actually really important information. While a lot of prolonged jaundice at this age is due to harmless causes like breastmilk jaundice, pale stools and dark urine can point to a different and more serious cause, where bile isn't draining properly from the liver into the gut. Because of this, I need to arrange an urgent blood test today called a split bilirubin, which tells us exactly what kind of jaundice this is, and I'd like to examine Kofi's tummy to check the size of his liver.

Mrs Osei: That sounds worrying... what would that more serious cause be?

Candidate: There's a rare condition called biliary atresia, where the bile ducts that normally drain from the liver don't form properly, causing bile to build up. I want to be upfront that this is a possibility we need to actively rule out, precisely because pale stools and dark urine are the specific pattern we look out for. If it turns out to be this, it's actually a situation where speed really matters, since the earlier it's treated with an operation, ideally before Kofi is 6 to 8 weeks old, the better the long-term outcome for his liver.

Mrs Osei: Oh goodness, I had no idea it could be anything like that. What happens now?

Candidate: We'll get that blood test done today, and depending on the result, we'll either be able to reassure you that this is one of the more common, harmless causes of prolonged jaundice, or, if it does point to bile duct obstruction, we'll refer Kofi urgently, likely the same day, to a specialist liver unit for further scans and assessment. I know waiting for results is hard, but we'll move quickly either way.

Station Conclusion: An "Excellent Pass" Neonatal Jaundice Profile

Why your final choices scored maximum marks:

- **Asked the Single Most Important Question:** You specifically asked about stool and urine colour, which is the key discriminator between benign and serious causes of prolonged jaundice.
- **Did Not Assume Benignity From Reassuring Feeding/Growth:** You recognised that good feeding and weight gain do not exclude biliary atresia, which can present in an otherwise thriving baby.
- **Ordered the Correct, Decisive Test:** You requested a split (conjugated/unconjugated) bilirubin as the specific test that resolves the diagnostic uncertainty.
- **Conveyed Genuine Time-Criticality:** You explained the importance of early diagnosis and the Kasai procedure window without being needlessly alarming before results were available.
- **Clear Two-Pathway Communication:** You explained what would happen under both a reassuring and a concerning result, rather than leaving the parent only with uncertainty.

SECTION:

PROFESSIONALISM & MEDICAL LEADERSHIP

Station 66 — Professionalism: Approaching a Late Junior Colleague

This station falls under Medical Leadership and Professionalism, testing your ability to act as a supportive senior colleague addressing a performance issue with a junior doctor, while avoiding a confrontational, manager-style interrogation. The correct NHS approach is supportive, non-judgemental exploration aimed at finding the root cause — burnout, health issues, or personal circumstances.

The Roleplay

Station: Professionalism / Colleague in Difficulty

Candidate Role: ST1 Paediatric Doctor / Medical Registrar

Patient/Roleplayer Role: Dr James Taylor, FY1 Doctor

Setting: Staff Room / Private Office

The Prompt (read outside the cubicle):

Scenario: The senior sister has pulled you aside to say James, the FY1, has arrived 30–45 minutes late for his shifts three times this week, leaving the ward uncovered during morning handover.

Your Task:

- Approach James privately and supportively.
- Explore the root cause without accusation.
- Formulate a collaborative support plan.

(James is sitting in the staff room, looking exhausted, staring blankly at a cup of coffee.)

Candidate: Hi James, do you mind if I sit with you for a quick moment? I just wanted to check in and see how you're getting on with the rotation lately.

James: Oh... hi, sure. To be honest, I'm just incredibly tired. This rotation is intense, but I'm managing... I think. Is everything okay? Did I miss something on the ward round?

Common Pitfall: *"James, the nurses have complained to me that you've been arriving 45 minutes late three times this week. This is unsafe for patients and unfair to the team. If this happens again, I'll have to report you to the Clinical Director."* — Names the complainants, opens with a threat, and skips any attempt to understand the cause — likely to make James defensive and shut down rather than disclose what's really going on.

Model Response

Candidate: You've been working incredibly hard, James, and it's a demanding ward. The reason I wanted to chat is that it's been brought to my attention, and I've noticed myself, that you've found it difficult to make it in on time for morning handover a few times this week. I'm not here to lecture you — I'm genuinely concerned. Is there anything going on at the moment that's making mornings difficult?

James: (dropping his gaze, rubbing his face) I'm so sorry... I didn't realise people were noticing that closely, but you're right. I've been sleeping through my alarms. My partner and I split up two weeks ago and I had to move out — I'm sleeping on a friend's sofa an hour's commute from the hospital. I haven't been sleeping at all from the stress, so I started taking some over-the-counter sleeping aids, but they leave me completely groggy in the morning. I feel like a total failure, and I'm terrified this is going to ruin my career before it's even started.

Your Next Step: Formulating the Support Plan

James has disclosed four distinct issues: relationship breakdown and insomnia, potentially unsafe self-medication with OTC sleeping aids, a logistical housing/commute crisis, and acute career-related fear. Address all four without judgement.

Candidate: Thank you so much for trusting me with all of that, James — that's a huge amount to be carrying alone, and none of it makes you a failure. Let's take this one step at a time. First, on the sleeping tablets and how groggy they're leaving you — I'd like you to speak to Occupational Health, since they can advise safely on sleep and help make sure nothing you're taking is putting you at risk at work. For the relationship breakdown and everything you're feeling, I'd strongly recommend the BMA counselling service, or our Practitioner Health Programme, which offers confidential support specifically for doctors. On the practical side, let's speak to the rota coordinator about a temporary shift adjustment, or even hospital accommodation, while you sort out permanent housing closer by — that commute alone would exhaust anyone.

James: (visibly relieved, eyes welling up) Wow... thank you. I was expecting a formal warning today. That's such a weight off my shoulders. I'd love to speak to BMA counselling, and if there's any chance of temporary accommodation or a shift adjustment, that would change everything.

Your Next Step: Safety and Documentation

Candidate: Because morning handover is so critical for patient safety, let's agree that if you think you might be running late on any morning before these adjustments kick in, you'll drop me a quick text so we can make sure the ward stays safely covered. This conversation stays between us, but I will write a brief, supportive note to your Educational Supervisor so they're aware you're accessing these resources — purely so they can support you too, not to flag a problem.

Station Conclusion: An "Excellent Pass" Late Colleague Profile

Why your final choices scored maximum marks:

- **Protected Confidentiality:** You said "it's been brought to my attention" rather than naming the nurses who complained.
- **Created Psychological Safety:** Your non-accusatory opening let James disclose the real root causes rather than becoming defensive.
- **Named Specific NHS Pathways:** Occupational Health, BMA counselling/Practitioner Health, and the rota coordinator were each matched to the specific problem they solve.

- **Balanced Support With Safety:** You still agreed a safety-net plan for the interim, rather than ignoring the clinical risk of continued lateness.

Station 67 — Professionalism: The Impaired Colleague (Suspected Alcohol)

This station tests your ability to prioritise immediate patient safety when a colleague may be unfit to work, while still handling the situation with compassion. Unlike the late-colleague scenario, safety concerns here are non-negotiable and must be acted on immediately, not explored gently first.

The Roleplay

Station: Professionalism / Colleague Safety

Candidate Role: Medical Registrar / Senior Colleague

Patient/Roleplayer Role: Dr Chloe Ahmed, FY1 Doctor

Setting: Quiet Office Off the Ward

The Prompt (read outside the cubicle):

Scenario: During the morning ward round, you notice Chloe, an FY1, smells strongly of alcohol, has slightly shaking hands, and seems unsteady. She is due to clerk patients this morning.

Your Task:

- Address the safety concern immediately and privately.
- Remove her from clinical duties without delay.
- Balance safety with compassion, and plan the escalation.

(You catch Chloe alone briefly by the notes trolley before the ward round begins.)

Candidate: Chloe, could I have a quick word with you somewhere private for a moment?

(You step into a side office together.)

Candidate: Chloe, I want to be direct with you because I'm concerned about your safety and the patients' safety this morning. I've noticed a smell of alcohol, and your hands seem to be shaking a little. Can you help me understand what's going on? For now, I need you to hand me your bleed and step back from clinical duties — this isn't a punishment, it's about keeping everyone safe while we sort this out.

Common Pitfall: *"Have you been drinking? You smell like a brewery — this is completely unacceptable and dangerous. I'm reporting you to the GMC right now." — Judgemental, insulting language escalates shame and panic rather than opening a path to disclosure, and threatens the GMC as a first response rather than following the proper internal escalation route.*

Chloe: (freezing, then bursting into tears) I'm so sorry... I'm so, so sorry. Please don't report me to the GMC. My life is falling apart. My dad was diagnosed with terminal cancer last month, and I haven't slept in weeks. I drank half a bottle of vodka last night just to stop my head from spinning so I could sleep, but I woke up late, rushed to work, and I guess it's still in my system. I would never hurt a patient, I swear. If I lose my licence, I have nothing left.

Your Next Step: Balancing Safety With Welfare

Chloe is now vulnerable, grieving, and terrified. You must maintain the non-negotiable safety boundary while showing genuine compassion, and be clear that this cannot remain a secret between the two of you.

Model Response

Candidate: Chloe, I am so sorry to hear about your dad — that's an enormous weight to be carrying, and it makes complete sense that you're struggling to cope and sleep. I want to be honest with you: stepping down from clinical duties today isn't optional, but please hear me — this is about helping you, not about ending your career. You are not going to be sent home alone or left to drive yourself — I'll arrange for someone to take you home safely, or call someone to collect you. I also need to let you know that I have to inform the consultant or clinical lead about this today; this isn't something that can stay between the two of us, both for your safety and for the ward's, but I will be with you every step of that conversation, and it will be handled supportively, not punitively.

Chloe: (quietly) Okay... I understand. I'm just so scared.

Candidate: That fear makes complete sense, but being honest about this today is what protects you, not what threatens you. I'd like to refer you to our Practitioner Health Programme, which offers completely confidential support for doctors dealing with exactly this kind of crisis — grief, stress, and difficulties with alcohol. Occupational Health will also be involved to support your safe return to work. Right now, let's just focus on getting you home safely.

Station Conclusion: An "Excellent Pass" Impaired Colleague Profile

Why your final choices scored maximum marks:

- **Immediate Safety Intercept:** You removed Chloe from clinical duties in the very first exchange — delaying this is a critical safety failure.
- **Objective, Non-Judgemental Language:** You described what you observed (smell, shaking hands) without insulting or shaming language.
- **Ensured Safe Transport:** You didn't allow her to drive or leave alone.
- **Mandatory, Transparent Escalation:** You were honest that the consultant must be informed, rather than promising to keep it secret — while framing this as protective, not punitive.
- **Connected Her to the Right Support:** The Practitioner Health Programme and Occupational Health are the correct, specific UK pathways for a doctor in crisis.

Station 68 — Professionalism: Social Media and Patient Confidentiality Breach

This station tests GMC Good Medical Practice standards on confidentiality and digital professionalism. A breach of patient confidentiality on social media is a serious fitness-to-practise issue, even if the patient's name is never mentioned — because unique clinical details can allow "jigsaw identification" by anyone who recognises the pieces.

The Roleplay

Station: Professionalism / Ethics & Social Media

Candidate Role: ST1 Medical Doctor

Patient/Roleplayer Role: Dr Ryan Finch, FY1 Doctor

Setting: Doctors' Mess (Empty)

The Prompt (read outside the cubicle):

Scenario: Last night, Ryan posted an X-ray of a rare abdominal foreign body on social media with the caption "Insane case on the surgical ward tonight! You won't believe what this guy swallowed. #LifeAsADoctor #NHS." The hospital logo is partially visible on the computer screen in the background.

Your Task:

- Approach Ryan privately about the post.
- Explain the confidentiality breach, including jigsaw identification.
- Guide him through the mandatory rectification steps.

(Ryan is eating lunch in the doctors' mess, looking at his phone.)

Candidate: Hi Ryan, do you mind if I grab a quick word in private? I wanted to chat about something I came across online last night.

Ryan: Oh, hey! Yeah, sure — is it about my tweet with the X-ray? It's gone totally viral, over 5,000 likes already. Pretty cool case, right?

Common Pitfall: *"That tweet is completely illegal — you've violated the Data Protection Act and you're going to get fired. Delete it this second or I'm reporting you to the medical director and the police right now."* — Overblown, threatening, and skips any explanation of why it's actually a breach, which teaches Ryan nothing and damages the working relationship unnecessarily.

Model Response

Candidate: It's definitely an unusual clinical case, Ryan, but I'm speaking to you because I'm concerned about patient confidentiality. Even though you didn't post the patient's name, the photo shows the hospital logo, and the specific details mean the patient or their family could easily recognise themselves. This is a direct breach of GMC guidance. For the patient's privacy and your own career, we need to take that post down right now — can you log on and delete it?

Ryan: (smile vanishing, going pale) Oh wow... I didn't even notice the hospital logo in the background. I was just fascinated by the X-ray. You're completely right — if the patient or their family saw this, they'd know instantly. I've just deleted it — it's gone. Doctor, I'm terrified now. If Trust management or the GMC saw it before I deleted it, is my career finished? What do I do?

Your Next Step: Rectifying and Reporting

Candidate: Try to take a breath — this is fixable, and how you handle it from here matters far more than the mistake itself. First, we need to log a formal incident report, called a Datix, to document that the breach happened and that it was corrected immediately. Second, I'd strongly recommend you speak to your Educational Supervisor today, before anyone else raises it — approaching them yourself to say "I made a mistake, I noticed it, deleted it, and logged it" shows real maturity and protects your integrity far more than staying quiet. Finally, it's worth writing a short reflective entry in your e-Portfolio about what you've learned about digital patient privacy — turning this into evidence of genuine professional growth.

Ryan: Okay... that makes sense. It's terrifying, but I agree being honest is much better than sweeping it under the carpet. I'll fill out that incident form right now while it's fresh in my mind. Thank you for catching this before it got worse.

Station Conclusion: An "Excellent Pass" Social Media Breach Profile

Why your final choices scored maximum marks:

- **Immediate Harm Mitigation:** You had the post deleted during the conversation, stopping ongoing exposure.
- **Explained the "Jigsaw Identification" Concept:** You clearly explained how the hospital logo plus rare case details could allow identification, rather than simply asserting it was "illegal."
- **Correct NHS Rectification Pathway:** You named the Datix incident report, proactive supervisor disclosure, and e-Portfolio reflection — the full accountability loop.
- **Reframed Panic Into Growth:** You turned a career-threatening moment into a constructive learning point, modelling exactly how a senior should coach a junior through a mistake.

Station 69 — Professionalism: Challenging Authority (Consultant Demanding a Relative's Records)

This advanced leadership station tests your ability to maintain professional composure and patient confidentiality when a senior colleague — outranking you significantly — is angry and demanding information. In the NHS, patient confidentiality and safety remain absolute regardless of a colleague's seniority.

The Roleplay

Station: Medical Professionalism / Challenging Senior Colleague

Candidate Role: FY2 Doctor in the Acute Medical Unit (AMU)

Patient/Roleplayer Role: Professor Robert Vance, Consultant Cardiologist at the same hospital

Setting: Ward Doctor's Office

The Prompt (read outside the cubicle):

Scenario: Margaret Vance, an elderly patient with confusion, was admitted to AMU two hours ago. Her son, Professor Robert Vance — a senior consultant cardiologist at this hospital — storms into your office demanding her blood results and treatment plan immediately.

Your Task:

- De-escalate his anger.
- Maintain professional boundaries around confidentiality.
- Formulate a safe, appropriate plan to update him.

(Professor Vance storms in, slamming a folder onto the desk, red-faced and aggressive.)

Professor Vance: Are you the doctor looking after Margaret Vance? This is an absolute joke! I'm a senior consultant in this hospital, and my mother has been sitting in a cubicle for two hours with a suspected stroke or severe infection, and nobody has done anything! Hand me her file and tell me her white cell count right now. I don't have time for your junior doctor incompetence!

Common Pitfall: *"I'm so sorry, Professor Vance! I didn't realise you were a Consultant here. Let me print out all her blood results for you right now."* — Caves to seniority and hands over confidential medical information without any check on consent or capacity, a major pass/fail safety breach in this station.

Model Response

Candidate: Professor Vance, I can completely understand why you're so upset and anxious right now, and I would feel exactly the same if it were my mother. I want to assure you we're actively looking after her. However, to make sure I'm keeping her safe and following hospital policy, I need to check her chart first to see if she's given consent for her medical details to be shared, and verify her current treatment plan. Could you give me just a moment to look into this safely for you?

Professor Vance: (arms crossed, scowling, voice sharper but quieter) Look, she has severe dementia and confusion — she can't physically sign a piece of paper or give you 'verbal consent'

right now! I am her son, her next of kin, and a senior clinician in this building. I know exactly what her rights are. Stop hiding behind red tape — she doesn't have capacity to consent, so as her next of kin, I'm demanding to see her blood results and head scan right now!

Your Next Step: Navigating Capacity, Next of Kin, and Escalation

Professor Vance has raised two classic legal misconceptions: that "next of kin" grants an automatic legal right to medical records, and that this overrides standard process. Under UK law, "next of kin" has no formal legal status to consent to treatment or access records. If a patient lacks capacity, the correct step is to check for a registered Lasting Power of Attorney for Health and Welfare; if none exists, decisions are made in the patient's best interests, which involves consulting family, but does not mean handing over full records to an angry relative to read freely.

Model Response

Candidate: I hear you, and I know this is incredibly hard to watch as her son. If your mother currently lacks capacity, what I'd need to check is whether there's a registered Lasting Power of Attorney for Health and Welfare in place — that's the specific legal authority that would let us share full details with you. If there isn't one, we're still very happy to update you on her general condition and comfort, and to make decisions in her best interests together with you, but I wouldn't be able to hand over her full file to read independently. What I can do right now is have my registrar or the AMU consultant come and speak with you directly, so you can have that conversation peer-to-peer while I confirm her management plan.

Professor Vance: (shoulders dropping, posture relaxing) Look, I apologise for storming in and shouting. I know you're just doing your job — it's just, she's my mum, and seeing her that confused panicked me. I am actually her legal Power of Attorney for Health and Welfare, and I have the paperwork in her handbag. Thank you for being patient. If you can double-check her management plan, catch up with your senior colleague, and meet me by her cubicle, I'd really appreciate that. I'll go sit with her now.

Station Conclusion: An "Excellent Pass" Challenging Authority Profile

Why your final choices scored maximum marks:

- **De-escalated Without Absorbing the Insult:** You validated his distress as a worried son without reacting defensively to "junior doctor incompetence."
- **Held the Confidentiality Boundary Under Pressure:** You didn't hand over records simply because of his seniority — a major pass/fail line in this station.
- **Correctly Identified the Legal Framework:** You recognised that "next of kin" alone doesn't grant record access, and correctly introduced the Lasting Power of Attorney for Health and Welfare as the relevant legal authority.
- **Offered Peer-to-Peer Escalation:** You offered to involve your registrar or consultant so a senior-to-senior conversation could happen, the standard NHS approach when a senior colleague from another department is involved.
- **Refocused on the Patient:** You redirected the conversation toward Margaret's actual care and comfort, rather than letting it remain about her son's authority.

SECTION: COMMUNICATION SKILLS & CONFLICT RESOLUTION

Station 70 — Communication Skills: De-Escalating an Angry, Resistant Patient (Medication Overuse Headache)

Handling a resistant or angry patient is a common test of Interpersonal Skills in PLAB 2. Examiners want to see whether you can stay calm, validate frustration, avoid becoming defensive, and safely negotiate a plan without breaking the doctor-patient relationship. This station uses a classic clinical scenario — medication overuse headache — as a grounded example of the general conflict-resolution sequence, which is transferable to many other difficult-conversation stations.

The Clinical Background: Medication Overuse Headache

Medication overuse headache (MOH) occurs when frequent use of acute headache medication — simple analgesics on 15 or more days a month, or triptans/opioids/combination analgesics on 10 or more days a month, for over 3 months — paradoxically causes more frequent headaches. The core management is stopping the overused medication. Simple analgesics, NSAIDs, and triptans are usually stopped abruptly; opioid- or barbiturate-containing medications are more often tapered gradually, given the risk of a more significant withdrawal syndrome. Headaches typically worsen for one to two weeks after stopping before gradually improving over one to two months.

The NURSE Protocol for Empathic Statements

- **Naming:** naming the emotion you observe ("I can see you're really frustrated by this.")
- **Understanding:** conveying that their reaction makes sense ("That's a completely understandable reaction.")
- **Respecting:** acknowledging their effort or strength ("You've been doing everything you can to manage this yourself.")
- **Supporting:** committing to stay involved ("I'm not going anywhere — we'll work through this together.")
- **Exploring:** inviting them to say more ("Can you tell me more about what's worrying you most?")

The Roleplay

Station: General Practice / Conflict Resolution

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Sarah Kowalski, 38 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Sarah, 38, has chronic daily headaches and has been taking paracetamol and codeine most days for several months. Your assessment suggests medication overuse headache, and the correct management is to withdraw the overused analgesics.

Your Task:

- De-escalate her anger and validate her frustration without being defensive.
- Reframe the plan collaboratively rather than as a punishment.
- Negotiate a shared, safety-netted agreement.

(Sarah is sitting with her arms crossed, visibly frustrated.)

Sarah: Are you joking? You want me to just stop my pills and suffer? I came here for help, not to be tortured!

Common Pitfall: *"I understand you're upset, but medically this is the right thing to do, and I need you to trust me on this." — Technically reasonable but rushes past validating her anger, and asserting authority ("trust me") rather than genuinely exploring her concerns is likely to entrench her resistance rather than dissolve it.*

Model Response: Immediate De-escalation

Let her fully vent without interrupting. Maintain soft eye contact, uncross your arms, lean slightly forward, and lower the pitch and pace of your voice.

Candidate: I completely understand why you're angry, Sarah. Hearing that the advice today is to stop the very things you rely on to get through the day must sound incredibly frustrating. I can see how much pain you're in, and the last thing I want is for you to feel like I'm ignoring your suffering.

Sarah: (arms still crossed, but listening) Well... at least you're not just lecturing me.

Your Next Step: From Confrontation to Collaboration

Candidate: You've been doing everything right by trying to manage this pain yourself so you can keep working and looking after your family — the tablets are what anyone in your position would reach for. The reason I'm suggesting we change course isn't to punish you. It's because the evidence shows that when we keep adding stronger painkillers for headaches like this, the brain actually gets trapped in a cycle where the pain never fully goes away. I want to help you get rid of these headaches for good, not just patch them up for an hour at a time.

Sarah: I hear what you're saying, but I can't just stop — I'll lose my job if I miss work!

Your Next Step: Handling Specific Resistance

Candidate: That's a very valid concern. Let's look at the calendar together — we don't have to start this on a day that clashes with a big deadline. Could we plan to start over a weekend, or during a week you have some time off, so the worst of the adjustment happens while you're at home?

Sarah: I suppose that could work... but what if it's not headaches at all? What if there's something seriously wrong, like a tumour? Can't you just give me a brain scan to prove it?

Candidate: I completely hear how frightening the thought of a tumour is. Based on the clear pattern here — your headaches happening right as your medication wears off — and the fact that you have none of the warning signs we look for, like visual changes or being sick first thing in the morning, a scan isn't medically necessary right now. It would also expose you to unnecessary radiation and waiting time, without addressing what's actually driving your daily pain.

Sarah: (voice softening, but still anxious) So you're just going to leave me with absolutely nothing?

Candidate: Absolutely not — I'm not abandoning you. While we can't continue the regular painkillers, I can prescribe a short-term anti-sickness medication if the withdrawal makes you feel nauseous, provide a sick note if you need a few days off work, and I want to book a phone call with you in exactly one week to see how you're holding up.

Your Next Step: The Shared Agreement

Candidate: I can't force you to stop the tablets — that choice is entirely yours. But as your doctor, I'd strongly recommend we give this a try. What if we agree to trial this for two weeks, and if it becomes completely unbearable, you come straight back in and we reassess together? How does that feel to you?

Sarah: (uncrossing her arms) Okay... I think I can try that, especially knowing I'm not just being left to cope alone.

Your Next Step: Finalising and Safety-Netting

Candidate: So, our plan is to stop the paracetamol and codeine completely from this weekend. We expect the headaches to feel worse for a few days to a couple of weeks, but then gradually improve over the following month. If at any point you develop a sudden, blinding headache unlike anything you've felt before, a high fever, a stiff neck, or sudden weakness anywhere, don't wait for our follow-up call — go straight to the nearest A&E.

Station Conclusion: An "Excellent Pass" Conflict Resolution Profile

Why your final choices scored maximum marks:

- **Let Her Vent Fully Before Responding:** You didn't interrupt or rush to correct her, which is often the first mark lost in these stations.
- **Validated Without Capitulating:** You acknowledged her anger and effort genuinely, while still holding the clinically correct line on stopping the medication.
- **Addressed Each Resistance Point Individually:** You handled the job/scheduling concern, the fear of a serious diagnosis, and the fear of abandonment as three distinct, specific concerns rather than one generic reassurance.
- **Respected Autonomy While Still Recommending:** You were clear you couldn't force her, framed it as a trial with a built-in review point, and let her choose to opt in.
- **Concrete Safety-Netting:** You gave specific red flags for secondary headache causes, not just "come back if it's bad."

SECTION: UROLOGY

Station 71 — Urology: Mumps Orchitis

Mumps orchitis is inflammation of one or both testicles caused by the mumps virus, typically developing in post-pubertal males 4 to 8 days after parotid gland swelling. This station tests managing a painful, embarrassing testicular presentation with empathy, ruling out the surgical emergency of testicular torsion, and addressing fertility anxiety.

Ruling Out Testicular Torsion (The Critical Red Flag)

- **Onset:** torsion causes sudden, agonising, excruciating pain; mumps orchitis develops more gradually, often alongside or shortly after parotid swelling.
- **Nausea/vomiting:** common in torsion; less typical in uncomplicated orchitis.
- **Radiation:** torsion pain often radiates to the lower abdomen.
- **Cremasteric reflex:** classically absent on the affected side in torsion.
- **Prehn's sign:** elevating the testicle may relieve pain in epididymo-orchitis but not in torsion — a classic teaching point, though it is not considered reliable enough on its own to rule torsion in or out clinically.

Any suspicion of torsion mandates immediate urological review and surgical exploration — this cannot be safely excluded by history and examination alone if genuinely in doubt.

The Roleplay

Station: General Practice / Urology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Tom Fletcher, 19-year-old university student

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Tom, 19, developed painful swelling of the right testicle two days ago, six days after his cheeks and jaw became swollen and tender.

Your Task:

- Take a sensitive history, ruling out testicular torsion.
- Address his fertility concerns honestly.
- Explain management, infection control, and public health duties.

(Tom sits awkwardly, clearly uncomfortable discussing the problem.)

Candidate: Hello Tom, I'm Dr [Your Name]. I understand you've been having some pain and swelling — this can be an uncomfortable thing to talk about, but you're in a completely private, confidential space, and I'm here to help. Can you tell me what's been happening?

Tom: It's really embarrassing, but my right testicle has become swollen and really painful over the last two days. It actually started about six days after my face and jaw got all swollen and sore — I thought that was just a bad cold, but now this.

Candidate: Thank you for trusting me with that — there's nothing to be embarrassed about, this is a very common thing for us to see. Can I ask, did the testicle pain come on very suddenly and severely, like a switch flipping, or has it built up more gradually? And have you felt sick or vomited alongside it?

Tom: It built up gradually over about a day, and no vomiting, just feeling generally rough with a bit of a fever.

Common Pitfall: *"Sudden testicular pain always means torsion, so since yours came on gradually, we can completely rule that out and I won't need to examine you."* — Oversimplifies risk assessment; any residual diagnostic uncertainty about torsion still warrants examination and a low threshold for urgent urological referral, since torsion is a time-critical surgical emergency.

Model Response

Candidate: That's really helpful, thank you. The gradual onset, combined with the timing after your jaw swelling, and the absence of vomiting, all point away from the more urgent problem I need to rule out, called testicular torsion — but I'd still like to gently examine you to be completely sure, since that's something we can never afford to miss.

Tom: Okay, that makes sense. Can I ask... is this going to affect me having kids one day? That's honestly what's scaring me most.

Your Next Step: Addressing Fertility Concerns

Candidate: That's a completely understandable thing to worry about, and I'm glad you asked directly. What you're describing sounds like mumps orchitis — inflammation of the testicle caused by the same virus that gave you the facial swelling. It is genuinely uncomfortable, but it's a self-limiting viral illness that gets better on its own. While the affected testicle can sometimes shrink slightly afterwards, complete infertility is very uncommon, especially since only one side is affected in your case — the risk is higher if both testicles become involved, which isn't the case for you. If you're ever concerned in future, we can easily arrange a simple sperm count test for peace of mind.

Tom: That's a huge relief to hear. What can I actually do about the pain in the meantime?

Your Next Step: Symptomatic Management

Candidate: Rest is important during the painful phase — try to avoid strenuous activity. Simple painkillers like paracetamol or ibuprofen will help. Wearing supportive underwear, like briefs rather than boxers, or placing a rolled towel under the area while resting, can really ease the dragging discomfort. An ice pack wrapped in a cloth, applied for 15 to 20 minutes at a time, can also help bring the swelling down.

Your Next Step: Infection Control and Public Health Duty

Candidate: One more important thing — mumps is highly infectious, so you'll need to stay away from university, work, and public places for 5 days from when your face first started swelling. I also need to let you know that mumps is a notifiable disease in the UK, which means I have a legal duty to inform our local public health team, purely to help protect others in the community — this isn't something to worry about personally, it's just part of how we manage infectious diseases here.

Tom: Oh, I didn't realise it was that contagious. I'll definitely stay away from everyone for now. Thanks for being so straightforward about everything.

Your Next Step: Safety-Netting

Candidate: If the pain suddenly becomes much worse or unbearable, if you develop persistent vomiting or a high fever that won't come down, or if you notice spreading redness or warmth across the skin there, please go straight to A&E rather than waiting to see us again.

Station Conclusion: An "Excellent Pass" Mumps Orchitis Profile

Why your final choices scored maximum marks:

- **Ruled Out Torsion Properly:** You screened onset, associated vomiting, and still examined the patient, rather than excluding a surgical emergency on history alone.
- **Addressed Fertility Fears Honestly and Accurately:** You gave a realistic, reassuring answer that distinguished unilateral from bilateral involvement, rather than an blanket dismissive reassurance.
- **Complete Symptomatic Management:** You covered rest, analgesia, scrotal support, and cold packs, not just "take some painkillers."
- **Correct Public Health Duty:** You explained the 5-day isolation period and the legal duty to notify public health, framing it as protective rather than punitive.
- **Clear, Specific Safety-Netting:** You gave concrete red flags for returning urgently, rather than a vague "come back if it's not improving."

SECTION: IMMUNISATION & PUBLIC HEALTH

Station 72 — Immunisation: Tetanus Prophylaxis and Wound Management

Tetanus prophylaxis tests your ability to classify a wound, assess vaccination history, and apply the UK Health Security Agency (Green Book) prophylaxis matrix — deciding between no treatment, a vaccine booster alone, or a booster combined with Human Tetanus Immunoglobulin (TIG).

Step 1: Classify the Wound

- **Clean wounds:** minor, clean wounds or burns with no significant contamination.
- **Tetanus-prone wounds:** contaminated with soil, manure, faeces, or saliva (e.g. animal or human bites); deep puncture wounds or those containing a foreign body (e.g. rusty nails, splinters); wounds with significant devitalised tissue, avulsions, or crush injury; or any wound where surgical treatment has been delayed more than 6 hours.
- **High-risk tetanus-prone wounds:** a tetanus-prone wound with heavy contamination (e.g. soil or manure with a high likelihood of tetanus spores), extensive devitalised tissue, or significant delay to treatment — this category can warrant TIG even in a fully immunised patient, which the standard table below does not capture on its own.

Step 2: Check Vaccination History

A full UK primary course is 5 doses: three primary doses in infancy (as part of the 6-in-1 vaccine, typically at 8, 12, and 16 weeks), a pre-school booster (around 3 years 4 months), and a teenage school booster (around 14 years). Ask when the last dose was given, and whether it was more or less than 10 years ago.

Step 3: Apply the UK Prophylaxis Matrix

Vaccination History	Clean Wound	Tetanus-Prone Wound	High-Risk Tetanus-Prone Wound
Fully immunised (5 doses), last dose < 10 years ago	No vaccine needed	No vaccine needed	Consider TIG regardless of history, given the heavy contamination/delay
Fully immunised (5 doses), last dose > 10 years ago	No vaccine needed	Give 1 reinforcing dose of vaccine	Give 1 reinforcing dose of vaccine plus TIG

Vaccination History	Clean Wound	Tetanus-Prone Wound	High-Risk Tetanus-Prone Wound
Incomplete or unknown history	Give 1 dose of vaccine now, then arrange to complete the 5-dose course	Give 1 dose of vaccine plus TIG	Give 1 dose of vaccine plus TIG

In the UK, single-antigen tetanus vaccine is no longer used — boosters are given as the combined Revaxis vaccine (tetanus, diphtheria, and inactivated polio). If TIG is required, it is given as a separate injection into a different limb from the vaccine, using separate syringes.

The Roleplay

Station: Emergency Department / Wound Management

Candidate Role: FY2 Doctor in the Emergency Department

Patient/Roleplayer Role: Mark Fletcher, 45 years old

Setting: Emergency Treatment Cubicle

The Prompt (read outside the cubicle):

Patient Details: Mark, 45, stepped on a rusty nail in his garden shed an hour ago. He isn't sure when he last had a tetanus jab.

Your Task:

- Classify the wound and assess vaccination history.
- Apply the correct prophylaxis pathway.
- Explain TIG (if needed) and safety-net appropriately.

Candidate: Hello Mark, I'm Dr [Your Name]. I understand you've stepped on a rusty nail — let's take a look and make sure we get this sorted properly. Can you tell me exactly what happened?

Mark: Yeah, I was clearing out my shed and stood right on an old rusty nail sticking up from a plank. It went in fairly deep into the sole of my foot. It's pretty painful, but I've cleaned it up a bit myself already.

Candidate: Thank you for that detail — a deep puncture wound from a rusty nail in a shed is exactly the kind of wound we call 'tetanus-prone,' since it can carry tetanus spores from the soil and rust. Do you know roughly how many tetanus vaccinations you've had in your life, and when your last one was?

Mark: Honestly, I couldn't tell you. I think I had my childhood jabs, but I've no idea about anything since school, and that was about 30 years ago.

Common Pitfall: *"Since you can't confirm your history, I'll just give you the vaccine booster today and that should cover you." — Fails to give Human Tetanus Immunoglobulin, which is required for a tetanus-prone wound in a patient with an incomplete or unknown vaccination history, not just the vaccine alone.*

Model Response

Candidate: Thank you for being upfront about that — it's a really common situation, and it's exactly the information I need. Because your vaccination history is unclear and this is a tetanus-prone wound, I'm going to give you two things today: a booster vaccine called Revaxis, which also covers diphtheria and polio, and a separate injection called Human Tetanus Immunoglobulin, or TIG. The vaccine helps your body build its own long-term protection, but that takes time, so the TIG gives you ready-made antibodies that act immediately to neutralise any tetanus toxin right now, while your own immunity develops. We'll give these as two separate injections into different arms.

Mark: Two separate injections, got it. What about the wound itself?

Candidate: I'm going to thoroughly wash and flush the wound with sterile saline, and remove any dirt, debris, or dead tissue, since tetanus bacteria thrive in exactly that kind of environment. Once we've done that and given the injections, you'll be in a good position. You might notice some redness, mild swelling, or soreness at the injection sites for a day or two, and possibly feel slightly unwell or run a mild temperature — that's all normal. But please come back straight away if you notice spreading redness, worsening pain, heat, or pus from the wound itself, or if you develop jaw stiffness, difficulty swallowing, or stiffness in your neck or tummy muscles — those would need urgent attention.

Mark: That all makes sense, thank you for explaining exactly why I need both things rather than just the one jab.

Alternate Presentation 1: The Pregnant Patient

A variant of this station presents a pregnant patient, either needing emergency wound prophylaxis or asking about her routine antenatal vaccination.

Patient: I'm 24 weeks pregnant, and I've just been told I need a tetanus jab because of a cut. Will this harm my baby?

Model Answer: I understand you want to protect your baby, and that's a very natural thing to ask about. I want to reassure you that this vaccine is inactivated, meaning it contains no live virus at all — it's considered completely safe at any stage of pregnancy and will not harm your baby. The same wound-assessment principles apply exactly as they would for anyone else: if your wound is tetanus-prone and your vaccination history isn't fully up to date, you'd be offered a booster, and TIG if needed, exactly as we'd offer a non-pregnant adult. Separately, since you're now over 16 weeks, this is also around the time you'd be offered your routine antenatal whooping cough vaccine, which is usually given between 20 and 32 weeks — this routine vaccine also contains a tetanus and diphtheria component, so if you have it, it will update your tetanus protection as well. Some services may even be able to combine this with today's wound-related dose, so you're not needing separate injections unnecessarily — I'd check with the team giving your vaccine today about the best way to sequence this for you.

Note: UK antenatal vaccine brands are periodically updated by the Green Book, so a specific product name should always be confirmed against the current edition or local formulary rather than assumed. The clinically important, well-established teaching points are that tetanus vaccines are inactivated and safe throughout pregnancy, and that the routine antenatal pertussis-containing vaccine also provides tetanus/diphtheria cover.

Alternate Presentation 2: The Fully Vaccinated Child

A variant presents an anxious parent whose child, fully vaccinated for their age, has a tetanus-prone wound.

Parent: My son cut his foot on some rusty metal at the park. He's had all his usual childhood jabs — does he need a tetanus booster or that immunoglobulin thing?

Model Answer: That's a great question, and the reassuring answer is no — because your son is fully up to date with his routine childhood immunisations for his age, his body already has strong, active protection against tetanus, regardless of how dirty or deep this wound looks. Giving an additional booster or immunoglobulin wouldn't add any real clinical benefit in that situation. What we will do is thoroughly clean the wound, removing any dirt or dead tissue, since good wound care is the single most important step in preventing infection. I'd just ask you to keep an eye out for normal signs of a wound infection — spreading redness, increasing warmth, worsening pain, or pus — and bring him back if you notice any of those.

Examiner Follow-Up: *"Is there ever an exception to this, even in a fully vaccinated child?"*

Model Answer: Yes — if the child is significantly immunocompromised, for example undergoing active chemotherapy or living with a severe immune deficiency, their body may not be able to mount or maintain a full protective antibody response despite having received all their scheduled vaccines. In that specific situation, they should be treated as if unvaccinated for the purposes of a tetanus-prone wound, and given Human Tetanus Immunoglobulin regardless of their vaccination record.

Station Conclusion: An "Excellent Pass" Tetanus Prophylaxis Profile

Why your final choices scored maximum marks:

- **Accurate Wound Classification:** You correctly identified a deep puncture wound from a rusty nail as tetanus-prone, not simply "dirty."
- **Correctly Applied the Matrix:** You recognised that an unknown vaccination history plus a tetanus-prone wound requires both a vaccine dose and TIG, not either alone.
- **Explained the Rationale for Two Separate Products:** You explained why both the vaccine and TIG are needed (immediate passive protection versus longer-term active immunity) rather than just stating the plan.
- **Adapted the Same Framework Across Populations:** You applied the identical wound-classification logic correctly to a pregnant patient and a fully vaccinated child, adjusting only the population-specific details (fetal safety reassurance; recognising that a fully vaccinated child needs no further treatment except in immunocompromise).
- **Concrete Safety-Netting:** You gave specific signs of wound infection and early tetanus symptoms (jaw stiffness, swallowing difficulty) rather than a vague "come back if it's not healing."

Station 73 — Immunisation: The UK Routine Vaccination Schedule

Knowing the UK routine immunisation schedule lets you counsel worried parents, recognise when a child is overdue, and correctly answer common questions about missed or delayed vaccines. The schedule changed significantly from January 2026, with a new appointment added at 18 months and a combined measles/mumps/rubella/varicella (MMRV) vaccine introduced — since a child's exact entitlement can depend on their date of birth during this transition, always check the current schedule for an individual patient's specific cohort if in doubt.

The Routine Childhood Schedule (From January 2026)

Age	Vaccines Given
8 weeks	6-in-1 (diphtheria, tetanus, pertussis, polio, Hib, hepatitis B); MenB; Rotavirus (oral)
12 weeks	6-in-1 (2nd dose); Rotavirus (2nd dose)
16 weeks	6-in-1 (3rd dose); MenB (2nd dose); Pneumococcal (PCV)
1 year	MMR (1st dose); Pneumococcal booster; MenB booster
18 months (new appointment from Jan 2026)	6-in-1 (4th dose, providing a Hib booster); MMRV (combined 2nd MMR dose plus varicella/chickenpox) for eligible birth cohorts
3 years 4 months	4-in-1 preschool booster (diphtheria, tetanus, pertussis, polio); 2nd MMR dose (for children not already covered via MMRV at 18 months)
12–13 years	HPV vaccine (single dose, offered to all genders)
14 years	3-in-1 teenage booster (tetanus, diphtheria, polio); MenACWY

Key recent changes worth knowing: the combined Hib/MenC vaccine (Menitorix) has been discontinued and replaced by an extra 6-in-1 dose at 18 months, since the adolescent MenACWY programme has made a separate meningitis C dose in infancy unnecessary. The MMRV vaccine (combining MMR with varicella) is now offered as part of the routine schedule for children born on or after 1 September 2022, rather than chickenpox vaccination being absent from the routine schedule as it was previously.

Vaccination in Pregnancy and Adulthood

- **Pertussis-containing vaccine:** offered to all pregnant women, ideally around 20 weeks and no later than 32 weeks, protecting the newborn against whooping cough before their own vaccinations begin.
- **RSV vaccine in pregnancy:** more recently introduced to reduce severe RSV disease in infants.
- **Flu vaccine:** annual vaccination for eligible groups, including young children (usually as a nasal spray from age 2), pregnant women, those 65 and over, and people with certain long-term health conditions.
- **Shingles vaccine:** offered to older adults, with the exact eligible age band having been adjusted over recent years — always check current eligibility criteria rather than assuming a fixed age.
- **Pneumococcal vaccine:** offered to adults 65 and over, and to those in clinical risk groups.
- **RSV vaccine for older adults:** introduced for eligible older age groups given their higher risk of severe RSV disease.

The Roleplay

Station: General Practice / Public Health Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Hannah Reid, mother of 18-month-old Oscar and 4-year-old Lily

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Hannah has booked an appointment confused because her younger son Oscar has just been invited for an 18-month vaccination appointment, but her older daughter Lily never had one at that age.

Your Task:

- Explain why the schedule differs between her two children.
- Clearly describe what Oscar's appointment involves.
- Reassure her that neither child is being under- or over-vaccinated.

Candidate: Hello Hannah, I understand you had a query about Oscar's vaccination invitation — how can I help?

Hannah: Yes, Oscar's just been invited for a jab at 18 months, but Lily never had an appointment at that age when she was little. Has something been missed for one of them?

Common Pitfall: *"That does seem odd, let me look into whether Lily missed a vaccination." — Assumes an error has occurred rather than recognising this as an expected difference due to a nationwide schedule change, which risks causing unnecessary alarm and an unnecessary catch-up referral.*

Model Response

Candidate: That's a really sensible thing to ask about, and I can reassure you straight away — nothing has been missed for either of your children. The national vaccination schedule was updated from January 2026, and a brand new appointment at 18 months was introduced as part of that change. Because Lily is older, she was vaccinated under the previous schedule, which didn't include this appointment, whereas Oscar falls into the age group who now benefits from it.

Hannah: Oh, that makes much more sense. What will Oscar actually have done at this appointment?

Candidate: He'll have a 4th dose of his combination vaccine, which boosts his protection against six different infections including whooping cough and tetanus, and he'll also be offered a newer combined vaccine that protects against measles, mumps, rubella, and chickenpox all in one, since he was born after the date that makes him eligible for that combined version.

Hannah: That's really reassuring, thank you. I was worried something had gone wrong for one of them.

Candidate: Not at all — these updates happen periodically as new evidence comes in, and it's completely normal for siblings born a couple of years apart to end up on slightly different schedules. If you're ever unsure what either of them is due, you're always welcome to check with us directly.

Station Conclusion: An "Excellent Pass" Vaccination Schedule Profile

Why your final choices scored maximum marks:

- **Recognised a Schedule Change Rather Than an Error:** You correctly identified the discrepancy as an expected consequence of a national schedule update, not a missed vaccination for either child.
- **Accurate, Current Schedule Knowledge:** You correctly described the 18-month appointment and the MMRV vaccine, reflecting the current UK schedule.
- **Clear, Non-Technical Explanation:** You explained what the appointment involved in plain language without overwhelming her with vaccine brand names or codes.
- **Proactive Reassurance:** You explicitly addressed her underlying worry that something had gone wrong, rather than only answering the literal question asked.

SECTION: GASTROENTEROLOGY & HEPATOLOGY

Station 74 — Hepatology: Suspected Liver Cancer and the Abdominal Examination

A suspected liver cancer station tests focused history-taking around chronic liver disease risk factors, a structured abdominal examination looking for signs of both the tumour and underlying cirrhosis, and correctly triggering an urgent cancer pathway. Hepatocellular carcinoma (HCC) usually arises on a background of chronic liver disease, most often cirrhosis from hepatitis B or C, or alcohol-related or metabolic (fatty liver) disease.

Focused History

- **Risk factors:** chronic hepatitis B or C infection, long-term heavy alcohol use, known cirrhosis or fatty liver disease, family history of liver disease.
- **Symptoms:** right upper quadrant or epigastric pain, early satiety, unintentional weight loss, abdominal swelling, jaundice, itching.
- **Constitutional red flags:** fevers, night sweats, profound fatigue.

The Abdominal Examination: What to Look For

- **General inspection:** jaundice (skin and sclera), muscle wasting, spider naevi on the chest/arms, palmar erythema, gynaecomastia, bruising.
- **Abdominal inspection:** distension (ascites), caput medusae (dilated periumbilical veins from portal hypertension), visible masses.
- **Palpation:** hepatomegaly — in HCC, the liver edge may feel hard, irregular, or nodular, rather than the smooth edge typical of simple fatty infiltration; also check for splenomegaly, a sign of portal hypertension.
- **Percussion:** shifting dullness, suggesting ascites.
- **Fluid thrill:** a further sign of large-volume ascites.

The Roleplay

Station: General Practice / Gastroenterology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Derek Hughes, 58 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Derek, 58, with a long history of heavy alcohol use and known liver cirrhosis, has booked an appointment for worsening abdominal discomfort and unintentional weight loss.

Your Task:

- Take a focused history exploring risk factors and red-flag symptoms.
- Propose and interpret a structured abdominal examination.
- Explain the urgent referral pathway.

(Derek looks thin and tired, with visibly yellowed sclera.)

Candidate: Hello Mr Hughes, I'm Dr [Your Name]. I understand you've been having some tummy discomfort and losing weight — can you tell me more?

Derek: Yeah, it's this dull ache under my right ribs that's been getting worse over the last couple of months. I've also gone off my food completely, and I've lost about a stone without trying. I just feel exhausted all the time.

Candidate: Thank you for describing that clearly. I know you have a history of liver problems — can you tell me a bit more about that, and how much alcohol you've been drinking recently?

Derek: I was told I had cirrhosis about three years ago from years of heavy drinking. I've cut back a lot since then, but I still have a few drinks most evenings. My skin's looked a bit yellow the last few weeks too, and my belly's looking more swollen, though I put that down to the beer.

Common Pitfall: *"Since you already have cirrhosis, this new discomfort and weight loss is probably just your liver disease getting a bit worse — let's just continue your usual monitoring." — Fails to recognise that new, progressive symptoms on a background of cirrhosis, especially with weight loss, are a red flag for a superimposed liver cancer, not simply the expected course of stable cirrhosis.*

Model Response

Candidate: Thank you for that background, Derek. Because you have known cirrhosis, and you're now describing worsening pain, weight loss, jaundice, and increasing abdominal swelling, I want to examine your tummy properly today to look for anything that's changed, and organise some urgent tests.

Your Next Step: Examination and Findings

Candidate: I'm going to look at your skin and eyes for jaundice, check for any spider-like blood vessels on your chest, feel your abdomen for the size and texture of your liver and spleen, and tap and check for any fluid in your tummy.

Examiner's Findings Card

- Jaundiced sclera; several spider naevi across the upper chest; mild gynaecomastia.
- Abdomen distended, with visible dilated veins around the umbilicus.
- Liver edge palpable 4cm below the costal margin, firm and irregular in texture.
- Spleen tip palpable.
- Shifting dullness present, consistent with ascites.

Your Next Step: Explaining the Findings and Referral

Candidate: Derek, based on your history of cirrhosis, together with the jaundice, the swelling in your abdomen, and finding your liver feels larger than before with an irregular edge, I'm concerned this could represent a growth in the liver, on top of your existing cirrhosis. I want to be honest with you about that possibility now, rather than waiting for test results. I'm arranging an urgent scan of your liver, and a blood test called alpha-fetoprotein, which can be raised with this type of liver cancer, alongside your usual liver blood tests. I'll refer you urgently to the liver specialist team so this can be looked into without delay.

Derek: (quietly) I was worried it might be something like that. What happens if it is cancer?

Candidate: If a growth is confirmed, the specialist team will look at exactly how big it is, whether it's affected one area or spread more widely, and how well the rest of your liver is working, since all of that determines the treatment options — these can range from targeted procedures directly on the liver, to medication, depending on what's found. I know this is frightening, but getting you seen urgently now gives you the best chance of the full range of treatment options being available.

Derek: Thank you for being straight with me, Doctor. I'd rather know and get moving on this quickly than not know.

Station Conclusion: An "Excellent Pass" Liver Cancer Profile

Why your final choices scored maximum marks:

- **Recognised New Symptoms on Stable Cirrhosis as a Red Flag:** You didn't attribute worsening symptoms to "expected" cirrhosis progression without further assessment.
- **Systematic, Verbalised Examination:** You explicitly described looking for stigmata of chronic liver disease, hepatomegaly with an irregular edge, splenomegaly, and ascites.
- **Correct Investigation Choice:** You requested liver imaging and alpha-fetoprotein alongside routine liver blood tests, rather than blood tests alone.
- **Honest, Direct Communication:** You named the possibility of cancer directly rather than using euphemisms, while still explaining next steps clearly.
- **Urgent, Specialist-Led Pathway:** You arranged urgent specialist referral rather than routine monitoring, recognising the time-sensitive nature of the presentation.

SECTION: SLEEP MEDICINE

Station 75 — Sleep Medicine: Obstructive Sleep Apnoea

Obstructive sleep apnoea (OSA) occurs when the throat muscles intermittently relax and narrow or block the airway during sleep, causing repeated pauses in breathing, fragmented sleep, and daytime symptoms. This station tests recognition of the classic symptom pattern, use of a validated screening tool, and the correct UK management and safety pathway.

Key Screening Tools

- **STOP-BANG:** Snoring, Tiredness during the day, Observed apnoeas, high blood Pressure, BMI over 35, Age over 50, Neck circumference over 40cm, and male Gender — a higher score indicates greater risk of significant OSA.
- **Epworth Sleepiness Scale:** a questionnaire scoring the likelihood of dozing off in various everyday situations, used to quantify daytime sleepiness and monitor response to treatment.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Paul Henderson, 52 years old, lorry driver

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Paul, 52, a lorry driver, brought in by his wife due to loud snoring and excessive daytime sleepiness.

Your Task:

- Take a focused history including STOP-BANG risk factors.
- Explain the diagnostic pathway.
- Address the driving safety implications directly.

(Paul looks exhausted, yawning repeatedly; his wife sits beside him looking concerned.)

Candidate: Hello Mr Henderson, I'm Dr [Your Name]. I understand you've been having some trouble with sleep — can you tell me more?

Paul: My wife says I snore like a freight train, and she's noticed me actually stop breathing for a bit before gasping myself awake. I feel exhausted all day, no matter how much sleep I get, and I've caught myself nearly nodding off at the wheel a couple of times recently, which has really scared me.

Candidate: Thank you for telling me that so honestly, especially the part about nearly falling asleep while driving — that's really important, and we'll come back to it. Can I ask a few more questions first? How's your blood pressure normally, and could you tell me your rough neck size, or do you know your BMI?

Paul: My blood pressure's been creeping up, actually, my GP mentioned it last year. I'm not sure on my neck size, but my shirts have gone up a couple of collar sizes. And my BMI's around 33 I think, based on my last weigh-in.

Common Pitfall: *"Since you're not driving right now, there's no urgency — let's just get you booked in for a routine sleep study in a few months and see how you get on."* — Fails to address the immediate road safety risk of a professional driver who has already nearly fallen asleep at the wheel, which needs direct action today, not routine follow-up.

Model Response

Candidate: Thank you, that's really helpful. Everything you've described — the loud snoring, witnessed pauses in breathing, high blood pressure, and increasing neck size and weight — strongly suggests a condition called obstructive sleep apnoea, where your airway repeatedly narrows during sleep, interrupting your breathing and fragmenting your sleep without you fully waking up each time. I'd like to arrange a sleep study, which monitors your breathing, oxygen levels, and sleep patterns overnight, either at home or in a sleep clinic, to confirm this and see how severe it is.

Paul: That makes a lot of sense. What about my driving, though? I do it for a living.

Candidate: I need to be very direct with you about that, because it's a genuine safety issue. Given that you've already had near-miss episodes of falling asleep at the wheel, you have a legal duty to inform the DVLA about your symptoms, and you shouldn't drive, especially professionally, until your sleepiness is properly assessed and, if needed, treated. I know that feels like a big step, especially with your job, but this protects you and everyone else on the road.

Paul: (sighing) I was worried you'd say that... but you're right, I can't risk falling asleep with a lorry full of cargo. I'll stop driving until this is sorted and let the DVLA know.

Your Next Step: Explaining Management

Candidate: Thank you for taking that seriously, Paul — that's exactly the right call. If the sleep study confirms moderate or severe sleep apnoea, the main treatment is something called CPAP — a small machine that gently blows air through a mask you wear overnight, keeping your airway open so you don't have those pauses in breathing. It's very effective, and most people notice a big difference in their energy levels within days to weeks of starting it. Alongside this, losing some weight, cutting back on alcohol in the evenings, and avoiding sedative medications before bed can all help significantly. Once you're established on treatment and your daytime sleepiness has resolved, you'll usually be able to safely return to driving, including professional driving, following DVLA guidance.

Paul: That's reassuring — I was worried this might be the end of my career, but it sounds like there's a proper way through this.

Station Conclusion: An "Excellent Pass" OSA Profile

Why your final choices scored maximum marks:

- **Complete STOP-BANG Risk Assessment:** You systematically covered snoring, observed apnoeas, blood pressure, BMI, and neck size, rather than relying on the presenting complaint alone.
- **Immediate Action on the Driving Risk:** You addressed the DVLA duty and driving cessation directly and urgently, given the disclosed near-miss episodes, rather than treating it as a routine follow-up matter.
- **Correct Diagnostic Pathway:** You arranged a sleep study to confirm the diagnosis and severity before committing to a specific treatment.
- **Accurate CPAP-Centred Management:** You explained CPAP as the primary treatment for confirmed moderate-severe OSA, alongside lifestyle measures.
- **Reassured Regarding His Livelihood:** You gave him a realistic path back to driving once treated, rather than leaving him with only the immediate bad news.

Station 76 — Sleep Medicine: Insomnia in a Heavy Drinker of Normal Weight

This station deliberately avoids the common assumption that sleep problems only occur in overweight patients with suspected OSA. A patient of normal weight with poor sleep tests your ability to consider a broader differential, and specifically to recognise alcohol as a common, under-reported cause of fragmented, non-restorative sleep.

Why Alcohol Disrupts Sleep

Alcohol is initially sedating and can help someone fall asleep faster, but as it's metabolised over the course of the night, it causes rebound arousal, fragmented sleep, and disrupted sleep architecture (including REM rebound), leading to frequent waking and unrefreshing sleep — even though the total number of hours in bed may seem adequate.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Rebecca Hall, 41 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Rebecca, 41, of normal build and weight, has booked an appointment for chronic poor sleep and daytime exhaustion.

Your Task:

- Take a broad sleep history without assuming OSA based on appearance.
- Screen sensitively for alcohol use as a potential cause.
- Explain the mechanism and management plan.

(Rebecca is of average build, looking tired but otherwise unremarkable in appearance.)

Candidate: Hello Ms Hall, I'm Dr [Your Name]. I understand you've been struggling with your sleep — can you tell me more about what's been happening?

Rebecca: I just can't seem to get a proper night's sleep. I fall asleep quickly enough, but I keep waking up through the night, and I wake up in the morning feeling like I haven't slept at all. It's been going on for months now, and I'm exhausted all day.

Common Pitfall: *"Since your weight looks normal, this is unlikely to be sleep apnoea, so it's probably just stress — try some relaxation techniques before bed." — Jumps to a conclusion based on appearance alone and skips a proper sleep and lifestyle history, missing an important and common contributor to fragmented sleep.*

Model Response

Candidate: Thank you for describing that so clearly. To help me understand what might be behind this, can you walk me through your evening routine — what time you go to bed, and anything you have to eat or drink in the hours beforehand?

Rebecca: I usually have dinner around 7, and then I'll have a glass or two of wine while winding down in front of the TV — sometimes more if I've had a stressful day, which is often. I go to bed around 11, fall asleep pretty quickly, but then I'm awake again by 2 or 3am, and it's a real struggle to get back off.

Candidate: Thank you for being open about that. Can I ask a bit more about your drinking generally — roughly how many days a week would you say you're having wine in the evening, and has anyone ever commented on it, or have you ever felt you should cut down yourself?

Rebecca: Most evenings, if I'm honest — it's become part of my routine to relax. My partner has mentioned it a couple of times, actually, and I suppose if I really think about it, I probably should cut down, but it doesn't feel like a big problem.

Your Next Step: Explaining the Mechanism

Candidate: This is really helpful, and I think it explains a lot about your sleep. Alcohol can actually make it easier to fall asleep initially, but as your body processes it through the night, it disrupts the normal structure of your sleep and causes exactly the kind of early, fragmented waking you're describing — which is why you can sleep a full night in bed and still wake up feeling exhausted. This is an extremely common but often overlooked cause of poor sleep, and it's not related to weight or classic snoring-type sleep apnoea at all.

Rebecca: I honestly never connected the two — I thought the wine was helping me relax and sleep better, if anything.

Candidate: It's a really common misconception, so please don't feel bad about that. Rather than reaching for a sleeping tablet, which wouldn't address the underlying cause and can carry its own risks of dependence, I'd really recommend we focus on gradually reducing your evening alcohol intake as the main step — even cutting back a couple of nights a week could make a real difference to your sleep quality. Alongside that, keeping a regular bedtime and wake time, avoiding screens right before bed, and steering clear of caffeine in the afternoon can help reinforce better sleep.

Rebecca: That actually makes so much more sense than I expected. I'll try cutting back on the wine and see how that goes before jumping to anything else.

Station Conclusion: An "Excellent Pass" Alcohol-Related Insomnia Profile

Why your final choices scored maximum marks:

- **Avoided Appearance-Based Assumptions:** You didn't rule out or rule in a cause based on her weight, and instead took a genuine, broad sleep and lifestyle history.
- **Uncovered the Hidden Contributor:** You asked specifically and non-judgementally about evening alcohol use, revealing a pattern she hadn't connected to her sleep problem.
- **Explained the Physiological Mechanism Clearly:** You explained why alcohol disrupts rather than improves sleep quality, correcting a common misconception.
- **Avoided Reaching for a Prescription:** You recommended addressing the underlying cause (alcohol reduction) rather than defaulting to a sleeping tablet.

- **Non-Judgemental Tone Throughout:** You normalised her situation rather than making her feel criticised for her drinking habits.

SECTION: RHEUMATOLOGY & CHRONIC PAIN

Station 77 — Chronic Pain: Postherpetic Neuralgia

Postherpetic neuralgia (PHN) is persistent neuropathic pain in the same dermatomal distribution as a prior episode of shingles (herpes zoster), typically defined as pain continuing more than 3 months after the rash has healed. It is more common in older patients, those who had a more severe acute rash or pain, and those with ophthalmic (facial) involvement.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Margaret Ellis, 74 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Margaret, 74, had shingles on her right chest wall 4 months ago. The rash has fully healed, but she continues to have severe pain in the same area.

Your Task:

- Confirm the diagnosis of postherpetic neuralgia.
- Explain the condition and its management.
- Provide safety-netting and discuss prevention for the future.

Candidate: Hello Mrs Ellis, I'm Dr [Your Name]. I understand you're still having pain from the shingles you had a few months ago — can you describe it for me?

Margaret: Yes, the rash itself cleared up ages ago, but this pain just won't go away. It's a burning, stabbing sensation on the right side of my chest, exactly where the rash was. Even the light touch of my clothing against the skin sets it off, and it's really affecting my sleep and my mood.

Common Pitfall: *"Since the rash has completely healed, this pain isn't related to the shingles anymore, so let's look for another cause." — Fails to recognise that pain persisting well beyond rash resolution, in the same dermatomal distribution, is the defining feature of postherpetic neuralgia rather than evidence against it.*

Model Response

Candidate: Thank you for describing that so clearly, Mrs Ellis. What you're experiencing is actually a recognised condition called postherpetic neuralgia — even though the rash has healed, the shingles virus can leave the nerves in that area damaged and oversensitive, causing ongoing burning or stabbing pain, and even making normal touch, like your clothing, feel painful. This is a genuine nerve pain condition, not a sign that something new or separate is going on.

Margaret: That's a relief to have a name for it. Is there anything that can actually help?

Your Next Step: Management

Candidate: Yes, there are several good options. The main treatments are medications that specifically calm overactive nerve signals, rather than standard painkillers like paracetamol or ibuprofen, which don't tend to work well for this type of pain. We'd typically start with either a tablet called amitriptyline, taken at night, or one of two other options called gabapentin or pregabalin. For pain that's more localised, like yours, we can also add a topical treatment, such as a lidocaine patch placed directly over the painful area, or a capsaicin cream, which can help desensitise the nerves over time. If the pain doesn't settle well with these, I can refer you to a specialist pain clinic for further options.

Margaret: I'll try the tablets first, then. Is there anything I could have done to prevent this in the first place?

Candidate: That's a really good question. Starting antiviral treatment early, within 72 hours of the shingles rash first appearing, can reduce the risk and severity of postherpetic neuralgia — something worth remembering if you're ever unlucky enough to get shingles again in a different area. There's also a shingles vaccine available on the NHS for older adults, which reduces both the risk of getting shingles at all and the risk of this kind of long-term nerve pain if it does occur — definitely worth asking about if you haven't already had it.

Your Next Step: Safety-Netting

Candidate: Please come back if the pain isn't improving despite these treatments, if you develop any new rash or blistering in the area, which could suggest a further outbreak, or if the pain is significantly affecting your mood or you're finding it hard to cope — we can offer more support with that too.

Station Conclusion: An "Excellent Pass" Postherpetic Neuralgia Profile

Why your final choices scored maximum marks:

- **Correctly Linked Persistent Pain to the Prior Shingles Episode:** You recognised that pain outlasting rash resolution in the same dermatome is expected in PHN, not a sign against the diagnosis.
- **Neuropathic-Specific Management:** You reached for amitriptyline/gabapentin/pregabalin rather than standard analgesics, which are not effective for this type of pain.
- **Offered Topical Adjuncts:** You mentioned lidocaine patches and capsaicin cream for localised pain, not just oral medication.
- **Addressed Future Prevention:** You covered both early antiviral treatment for future shingles and the shingles vaccine as prevention strategies.
- **Holistic Safety-Netting:** You checked in on her mood and coping, recognising the significant psychological burden of chronic pain, not just the physical symptom.

Station 78 — Rheumatology: Polymyalgia Rheumatica

Polymyalgia rheumatica (PMR) is an inflammatory condition causing bilateral shoulder and/or pelvic girdle pain and stiffness, typically in patients over 50. It has a crucial and dangerous overlap with giant cell arteritis (GCA), which must always be actively screened for given the risk of irreversible vision loss if missed.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Barbara Coleman, 68 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Barbara, 68, has had worsening pain and stiffness in both shoulders and hips over the last 6 weeks.

Your Task:

- Take a focused history supporting a diagnosis of PMR.
- Actively screen for symptoms of giant cell arteritis.
- Explain investigations and the diagnostic response to steroids.

Candidate: Hello Mrs Coleman, I'm Dr [Your Name]. I understand you've been having pain and stiffness — can you tell me where, and how it's been affecting you?

Barbara: It's both my shoulders and my hips, Doctor. It's been building up over about six weeks now. The stiffness in the mornings is the worst part — it takes me well over an hour to loosen up enough to even get dressed properly. I've also just felt generally worn out and a bit low, and I've lost a bit of weight without trying.

Candidate: Thank you for describing that so clearly — that sounds like it's really affecting your mornings and your energy. I do need to ask a few specific questions to make sure we're not missing anything important alongside this: have you had any new headaches, particularly around your temples, any pain in your jaw when chewing, tenderness on your scalp, like when brushing your hair, or any changes to your vision?

Common Pitfall: *"This sounds like classic polymyalgia rheumatica — I'll start you on steroids and we'll review in a few weeks." — Fails to actively screen for giant cell arteritis before starting treatment, missing a potentially blindness-causing emergency that requires a different, more urgent steroid dose and same-day ophthalmology involvement if present.*

Barbara: No headaches, no jaw pain, and my vision's been fine, thankfully. My scalp isn't tender either.

Model Response

Candidate: That's really reassuring to hear, thank you. Based on your age, the pattern of pain and stiffness in both shoulders and hips, the prolonged morning stiffness, and the fatigue and weight

loss, this fits well with a condition called polymyalgia rheumatica, which is caused by inflammation affecting the muscles around your shoulder and hip joints. I want to check some blood tests today, particularly your inflammatory markers, which are almost always raised in this condition, and some general blood tests to rule out other causes.

Barbara: That makes sense. What's the treatment if it is that?

Candidate: The main treatment is a steroid tablet called prednisolone, and one of the interesting things about this condition is that it responds dramatically well — most people feel significantly better within just a few days of starting it, which actually helps confirm the diagnosis itself. You'll likely need to stay on a reducing dose of steroids for around one to two years, and because of that, we'll also start you on calcium and vitamin D to protect your bones, and consider a bone-strengthening medication too.

Barbara: A year or two sounds like a long time on steroids — is that normal?

Candidate: It is, for this particular condition — the dose reduces gradually over that time as your symptoms allow. It's also really important that if you ever develop any of those symptoms I asked about earlier — new headaches, jaw pain, scalp tenderness, or any change in your vision — even after starting treatment, you contact us immediately, as this can indicate a related but more serious condition affecting the blood vessels, which would need urgent same-day assessment.

Barbara: I'll definitely keep an eye out for that and let you know straight away if anything like that happens.

Station Conclusion: An "Excellent Pass" Polymyalgia Rheumatica Profile

Why your final choices scored maximum marks:

- **Actively Screened for Giant Cell Arteritis:** You explicitly asked about headache, jaw claudication, scalp tenderness, and visual disturbance before finalising the plan, rather than treating PMR as an isolated diagnosis.
- **Correct Diagnostic Reasoning:** You recognised the classic pattern (bilateral girdle pain, prolonged morning stiffness, systemic symptoms) and the role of inflammatory markers.
- **Explained the Dramatic Steroid Response:** You correctly described the rapid, near-diagnostic improvement with steroids as a defining feature of PMR.
- **Addressed Long-Term Steroid Safety:** You proactively arranged bone protection given the prolonged steroid course required.
- **Ongoing GCA Safety-Netting:** You reinforced that new GCA symptoms at any point, even after treatment starts, need urgent same-day review.

Station 79 — Chronic Disease: Chronic Fatigue Syndrome (ME/CFS)

Chronic fatigue syndrome (also known as myalgic encephalomyelitis, or ME/CFS) is a complex, long-term condition causing persistent, disabling fatigue not resolved by rest. Current UK guidance (NICE NG206, 2021) places post-exertional malaise — a worsening of symptoms following activity, often delayed by a day or more — at the centre of diagnosis, and has moved decisively away from previously recommended generic graded exercise therapy as a treatment.

Diagnostic Features

- **Persistent fatigue:** present for at least 3 months, not caused by ongoing excessive exertion and not substantially relieved by rest.
- **Post-exertional malaise (PEM):** a disproportionate worsening of symptoms after physical, cognitive, or emotional exertion, often delayed by hours to days — this is considered a hallmark, near-essential feature.
- **Unrefreshing sleep or sleep disturbance.**
- **Cognitive difficulties:** often described by patients as "brain fog" — problems with concentration, memory, or word-finding.

Other causes of chronic fatigue (anaemia, hypothyroidism, coeliac disease, depression, sleep disorders) should be excluded with baseline investigations before confirming the diagnosis.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Emma Talbot, 34 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Emma, 34, has had persistent, disabling fatigue for the last 8 months, previously investigated with normal blood tests.

Your Task:

- Take a focused history confirming the diagnostic pattern, including post-exertional malaise.
- Explain the diagnosis validatingly.
- Outline current, correct UK management.

(Emma looks pale and drained, moving slowly as she sits down.)

Candidate: Hello Ms Talbot, I'm Dr [Your Name]. I understand you've been struggling with fatigue for quite a while now — can you tell me more about it?

Emma: It's been going on for about eight months now. It's not just being tired — it's this crushing exhaustion that doesn't go away no matter how much I sleep. The really strange thing is, if I push

myself and do too much on a good day, like a short walk or cleaning the house, I end up completely wiped out for the next two or three days afterwards, far worse than the activity itself should explain.

Candidate: Thank you for explaining that so precisely — that pattern you've just described, where doing more than usual causes a delayed crash lasting days, is actually a really important and specific clue. Do you also find your sleep unrefreshing, even when you get a full night, and any difficulties with concentration or memory, sometimes described as 'brain fog'?

Emma: Yes to both, actually. I can sleep for nine hours and still wake up feeling like I haven't slept at all. And my memory and concentration have gotten so much worse — I've had to write everything down at work because I just can't hold information in my head like I used to.

Common Pitfall: *"The best thing for this is to push through gradually and build back up your exercise tolerance with a structured graded exercise programme, increasing your activity a little bit every week regardless of how you feel."* — This reflects outdated guidance; current NICE guidance no longer recommends generic graded exercise therapy, given the risk that increasing activity without regard for post-exertional malaise can worsen the condition.

Model Response

Candidate: Thank you for being so specific, Emma — everything you've described, especially that delayed crash after exertion, together with the unrefreshing sleep and cognitive difficulties, fits with a condition called Chronic Fatigue Syndrome, sometimes also called myalgic encephalomyelitis, or ME/CFS. I want to be clear from the start: this is a real, recognised medical condition, not something that's "all in your head" or something you could simply push through with willpower.

Emma: Thank you for saying that... I've had people imply I'm just being lazy, which has been awful to hear.

Candidate: That must have been incredibly hurtful, and I want to reassure you that's absolutely not what's happening here. Now that your earlier blood tests have already ruled out other causes like anaemia or an underactive thyroid, I'd like to refer you to a specialist ME/CFS team, who can help build a personalised management plan with you.

Emma: What does that actually involve? I've heard some people say you just need to exercise more to get better.

Candidate: I'm really glad you raised that, because the guidance on this has actually changed. We no longer recommend a generic, steadily increasing exercise programme, because pushing through activity regardless of how you're feeling can trigger exactly the kind of crash you've described and make things worse, not better. Instead, the specialist team will help you with something called energy management, or pacing — learning to plan and balance activity and rest specifically around your own individual limits, so you can gradually do more over time without triggering that post-exertional crash. Some people are also offered a talking therapy like CBT, but it's used to help you cope with living with the condition and manage its impact, not as a cure or a way of proving it's psychological.

Emma: That's such a relief to hear the exercise advice has changed — I tried pushing myself before and it made me so much worse, and I thought I was just doing it wrong.

Station Conclusion: An "Excellent Pass" Chronic Fatigue Syndrome Profile

Why your final choices scored maximum marks:

- **Recognised Post-Exertional Malaise as the Key Feature:** You specifically identified the delayed, disproportionate crash after activity as central to the diagnosis, not just generic tiredness.
- **Validated the Condition as Genuine:** You proactively addressed the stigma of being seen as "lazy" or the illness being dismissed as psychological.
- **Confirmed Prior Exclusion of Other Causes:** You referenced that reversible causes had already been ruled out before settling on this diagnosis.
- **Up-to-Date, Correct Management:** You explicitly corrected the outdated "push through with graded exercise" advice, reflecting current NICE guidance favouring individualised energy management/pacing.
- **Clarified the Role of CBT:** You explained CBT as a coping and support tool, not a cure or an implicit claim that the condition is psychological.

Station 80 — Rheumatology: Acute Gouty Arthritis

Gout is caused by deposition of monosodium urate crystals in a joint, classically presenting as a sudden, excruciatingly painful, hot, red, swollen monoarthritis, most often affecting the first metatarsophalangeal joint (podagra). This station tests recognition of the classic presentation, the critical differential of septic arthritis, and correct acute and long-term management.

Risk Factors and Key Differential

- **Risk factors:** diet high in purines (red meat, seafood, beer/alcohol), obesity, diuretic use (particularly thiazides), chronic kidney disease, and family history.
- **Critical differential — septic arthritis:** must always be considered and actively excluded, especially with fever or systemic upset, since missing a septic joint can rapidly destroy the joint and be life-threatening.

The Roleplay

Station: Emergency Department / GP Consultation

Candidate Role: FY2 Doctor in the Emergency Department

Patient/Roleplayer Role: Gary Palmer, 54 years old

Setting: Emergency Treatment Cubicle

The Prompt (read outside the cubicle):

Patient Details: Gary, 54, woke at 3am with sudden, severe pain in his right big toe, which is now red, hot, and swollen.

Your Task:

- Take a focused history, actively excluding septic arthritis.
- Explain the diagnosis and confirm it appropriately.
- Outline acute and long-term management.

Candidate: Hello Gary, I'm Dr [Your Name]. I understand your toe's been very painful — can you tell me what happened?

Gary: I woke up at 3 this morning with the worst pain I've ever felt in my right big toe. Even the bedsheet touching it was agony. It's gone completely red and swollen, and it's hot to touch. I had a big steak dinner and quite a few beers at a work event last night, if that's relevant.

Candidate: Thank you, that's really helpful detail. Have you had any fevers, chills, or felt generally unwell? And has anything like this happened to you before?

Gary: No fevers, and I feel fine in myself otherwise, just this awful toe. This is actually the second time this has happened — I had something similar in the same toe about a year ago, which settled on its own after about a week.

Common Pitfall: *"This is clearly gout given the classic story — no need for any further tests, I'll just start you on allopurinol today to bring your urate down."* — Starting or adjusting urate-

lowering therapy like allopurinol during an acute attack can actually worsen and prolong the flare, and skipping joint aspiration in a first or diagnostically uncertain presentation risks missing septic arthritis.

Model Response

Candidate: Thank you, Gary, that's a really classic story for something called gout — a build-up of uric acid crystals in the joint causing sudden, severe inflammation, often triggered by heavy alcohol and rich food. Because you have no fever and feel otherwise well, an infected joint is less likely, but to be completely safe, I'd like to take a small sample of fluid from the joint with a needle, which lets us look directly for the crystals under a microscope and rules out infection at the same time.

Gary: Okay, that makes sense. What's the treatment for the pain right now?

Candidate: For the acute attack, we'll start you on a strong anti-inflammatory, like naproxen, alongside something to protect your stomach, assuming there's no reason that's unsafe for you. If anti-inflammatories aren't suitable, we have an alternative tablet called colchicine. It's important to know that a blood test for uric acid can actually look falsely normal during an acute attack, so we don't rely on that alone to confirm or rule out gout.

Gary: Good to know. Since this has happened before, is there anything I can do to stop it coming back?

Candidate: Yes — reducing alcohol, particularly beer, cutting back on red meat and seafood, staying well hydrated, and losing some weight if that applies can all help reduce future attacks. Because this is your second attack, once this current flare has fully settled, usually a few weeks from now, we'd consider starting a long-term urate-lowering tablet called allopurinol to prevent further episodes — but we deliberately wait until the acute attack has resolved before starting that, since starting it during a flare can actually make the current attack worse and last longer.

Gary: That's really helpful, thank you — I'll definitely cut back on the beer and steak nights for a while.

Station Conclusion: An "Excellent Pass" Gouty Arthritis Profile

Why your final choices scored maximum marks:

- **Actively Excluded Septic Arthritis:** You screened for fever and systemic upset and still proposed joint aspiration despite a classic-sounding history, rather than assuming gout on story alone.
- **Correct Diagnostic Test:** You identified joint aspiration and crystal microscopy as the gold-standard test, which also excludes infection simultaneously.
- **Avoided the Serum Urate Trap:** You knew that serum urate can be misleadingly normal during an acute attack and shouldn't be used alone to exclude gout.
- **Correct Sequencing of Urate-Lowering Therapy:** You explicitly avoided starting allopurinol during the acute attack, knowing it should be deferred until the flare has settled.
- **Comprehensive Lifestyle Advice:** You covered alcohol, diet, hydration, and weight as modifiable risk factors, not just the drug treatment.

SECTION: MENOPAUSE & VULVAL HEALTH

Station 81 — Menopause: Lichen Sclerosus vs Vaginal Atrophy

Vulval itching and soreness in a postmenopausal woman has two common, sometimes coexisting, causes with very different implications: vaginal atrophy (part of the genitourinary syndrome of menopause, due to declining oestrogen), and lichen sclerosus (a distinct chronic inflammatory skin condition carrying a small but real long-term risk of vulval cancer). This station tests differentiating the two and applying the correct management and monitoring for each.

Key Distinguishing Features

Feature	Vaginal Atrophy (Genitourinary Syndrome of Menopause)	Lichen Sclerosus
Cause	Declining oestrogen thinning the vaginal walls	Chronic inflammatory skin condition of unclear cause
Appearance	Thin, pale, dry vaginal walls; may bleed easily on contact	White, thickened or crinkly ("cigarette-paper") patches, sometimes in a figure-of-eight pattern around the vulva and anus
Key symptoms	Dryness, burning, dyspareunia, urinary frequency/urgency, recurrent UTIs	Intense itching (often worse at night), soreness, splitting/fissuring, pain
Long-term risk	No malignant potential	Small but real risk (around 4–5%) of vulval squamous cell carcinoma over a lifetime — requires monitoring
Management	Vaginal moisturisers/lubricants; topical vaginal oestrogen for more significant symptoms	Potent topical corticosteroid (e.g. clobetasol propionate); long-term follow-up given cancer risk

The Roleplay

Station: General Practice / Gynaecology Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Janet Cole, 67 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Janet, 67, postmenopausal, has booked an appointment about persistent vulval itching over the last 4 months.

Your Task:

- Take a sensitive history distinguishing the two most likely causes.
- Explain the differing implications, including cancer risk where relevant.
- Arrange appropriate examination, treatment, and monitoring.

(Janet looks uncomfortable discussing the topic.)

Candidate: Hello Janet, I'm Dr [Your Name]. I understand you've been having some discomfort — can you tell me more about it, in whatever way feels comfortable?

Janet: It's embarrassing, but I've had this awful itching down below for about four months now. It's actually worse at night — sometimes it wakes me up, and the skin feels quite tight and sore, especially around the opening. I've also noticed the skin looks different, sort of pale and crinkly.

Candidate: Thank you for trusting me with that, there's nothing to be embarrassed about — this is a very common thing we see. Have you noticed any dryness or discomfort during sex, or any urinary symptoms, like needing to go more often or more urgently? And has the skin ever split or cracked, or become painful rather than just itchy?

Janet: Sex has become uncomfortable, yes, quite dry. No particular urinary problems though. And yes, actually, I have noticed some small splits in the skin, especially where it's most tight-feeling.

Common Pitfall: *"This all sounds like normal vaginal dryness from the menopause — I'll prescribe some vaginal oestrogen cream and that should settle everything."* — Overlooks the description of pale, crinkly, splitting skin, which points to lichen sclerosus rather than simple atrophy, and fails to examine or mention the associated cancer risk that requires monitoring.

Model Response

Candidate: Thank you for describing all of that so clearly, Janet. Some of what you're telling me — the vaginal dryness and discomfort during sex — does fit with a very common change after menopause called vaginal atrophy, caused by lower oestrogen levels thinning the tissues. However, the pale, crinkly skin you've described, along with the splitting and the itching being worse at night, makes me want to examine the area properly, because that pattern can also point to a separate skin condition called lichen sclerosus, which sometimes occurs alongside vaginal atrophy.

Janet: Oh — is that something serious?

Candidate: It's a manageable, chronic skin condition, and I want to be straightforward with you: it does carry a small increased lifetime risk of a rarer type of skin cancer in that area, which is exactly why, if it turns out to be this, we'd want to treat it properly and keep an eye on it with

regular follow-up, rather than just leaving it. Let's examine the area together now, with your consent, so I can see exactly what we're dealing with.

(On examination: pale, thickened, crinkled skin in a figure-of-eight pattern around the vulva and perianal area, with a small fissure; no ulceration or thickened, non-healing area seen.)

Candidate: Based on what I can see, this does look like lichen sclerosus, alongside some vaginal atrophy as well. For the lichen sclerosus, I'll prescribe a strong steroid ointment, which is very effective at controlling the itching and skin changes, and I'd like to see you back in a few months to check how it's responding and keep a regular eye on the skin long-term. For the vaginal dryness, a topical vaginal oestrogen cream, alongside a plain moisturiser for everyday comfort, should help significantly.

Janet: Thank you for explaining both parts separately — I had no idea there might be two different things going on.

Candidate: It's genuinely common for both to occur together at your age. Please come back sooner than your review if you notice any area that becomes thickened, doesn't heal, ulcerates, or looks different from the rest, since that's what we'd want to check promptly.

Station Conclusion: An "Excellent Pass" Lichen Sclerosus/Vaginal Atrophy Profile

Why your final choices scored maximum marks:

- **Recognised Two Coexisting Conditions:** You didn't force the entire picture into a single diagnosis, correctly identifying overlapping vaginal atrophy and lichen sclerosus.
- **Examined Rather Than Assumed:** You proposed examination given specific features (pale, crinkly, splitting skin) rather than treating empirically based on history alone.
- **Honest About Cancer Risk:** You disclosed the small vulval cancer risk associated with lichen sclerosus honestly, framing ongoing monitoring as protective rather than alarming.
- **Separate, Correct Treatments for Each Condition:** You prescribed potent topical steroid for lichen sclerosus and vaginal oestrogen/moisturiser for atrophy, rather than a single treatment for both.
- **Specific Safety-Netting:** You gave concrete, specific red flags (thickening, non-healing area, ulceration) rather than a vague "come back if it doesn't improve."

Station 82 — Menopause: Perimenopausal Symptoms and Hormone Replacement Therapy

This station tests counselling a perimenopausal woman about her symptoms, correctly diagnosing menopause clinically where appropriate, and providing balanced, individualised information about hormone replacement therapy (HRT), following NICE guideline NG23.

Key Clinical Points

- **Diagnosis:** in women over 45 with typical symptoms, menopause can usually be diagnosed clinically (based on symptoms and 12 months of amenorrhoea) without blood tests; FSH testing is reserved for atypical presentations or women under 45.
- **HRT composition:** women with a uterus need combined oestrogen and progestogen (the progestogen protects the womb lining from unopposed oestrogen); women who have had a hysterectomy can usually take oestrogen-only HRT.
- **Route matters:** transdermal (patch/gel/spray) oestrogen avoids first-pass liver metabolism and does not increase the risk of blood clots the way oral oestrogen can — an important choice for women with risk factors such as obesity, smoking, or migraine with aura.
- **Risks:** combined HRT carries a small increased risk of breast cancer that increases with duration of use; oral (not transdermal) oestrogen carries a small increased risk of venous thromboembolism. For most women starting HRT around the time of menopause, these risks are generally low and should be weighed individually against the benefits.
- **Contraindications:** current or past hormone-sensitive cancer (e.g. breast cancer), active or past venous thromboembolism, undiagnosed vaginal bleeding, and active liver disease.

The Roleplay

Station: General Practice / Menopause Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Deborah Marsh, 49 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Deborah, 49, has booked an appointment for hot flushes, night sweats, and irregular periods over the last 8 months.

Your Task:

- Take a focused history to confirm perimenopause.
- Explore her ideas and concerns about HRT.
- Give balanced, individualised information about HRT options and risks.

(Deborah looks tired, fanning herself occasionally.)

Candidate: Hello Deborah, I'm Dr [Your Name]. I understand you've been having some hot flushes and changes to your periods — can you tell me more?

Deborah: It's been going on for about eight months now. My periods have become really irregular — sometimes six weeks apart, sometimes closer together. And these hot flushes are relentless, especially at night — I'm drenched in sweat and can't sleep properly. I feel like a different person, honestly, my mood's all over the place too.

Candidate: That sounds exhausting, and it's completely understandable that it's affecting your mood when your sleep is so disrupted. Based on your age and everything you've described, this fits with the perimenopause — the transition period leading up to your final period. Do you still have your womb, or have you ever had a hysterectomy?

Deborah: No hysterectomy, I've still got everything. I've heard a bit about HRT from friends — some swear by it, but I've also heard scary things about it causing breast cancer. What's your honest opinion?

Common Pitfall: *"HRT is completely safe with no real risks, you should definitely go on it" or conversely "HRT causes cancer, so it's best avoided altogether" — Both extremes misrepresent the evidence; the correct approach is a balanced, individualised discussion of the actual risks and benefits, not a blanket recommendation for or against.*

Model Response

Candidate: That's a really important question, and I want to give you an honest, balanced answer rather than just telling you what to do. For most women starting HRT around your age, the benefits for symptoms like yours are significant, and the risks are generally low. Because you still have your womb, you'd need a combined preparation with both oestrogen and a progestogen, which protects the lining of your womb. There is a small increased risk of breast cancer with combined HRT, and that risk does increase gradually the longer you take it, but for most women it remains a small risk in absolute terms. There's also a slightly increased risk of blood clots, but that's specifically linked to oestrogen taken as a tablet — if we use a patch, gel, or spray instead, applied through the skin, that particular risk isn't increased in the same way, which might be worth considering depending on your preferences.

Deborah: That's really helpful to actually understand the difference. I think I'd like to try it — what would you recommend?

Candidate: Given what you've told me, I'd suggest starting with a transdermal, combined preparation — for example, an oestrogen patch or gel alongside a progestogen — since that avoids the small clot risk associated with tablets. We'll start at a standard dose and review how you're getting on in about three months, since it can take a little while to find the right balance for you. In the meantime, simple things like keeping your bedroom cool, wearing layers, and reducing caffeine and alcohol in the evenings can also help with the flushes.

Deborah: That all makes sense, thank you for being so straightforward about the risks rather than just brushing them aside.

Station Conclusion: An "Excellent Pass" Menopause/HRT Profile

Why your final choices scored maximum marks:

- **Clinical Diagnosis Without Unnecessary Testing:** You diagnosed perimenopause based on age and symptom pattern, without defaulting to unnecessary hormone blood tests.

- **Correctly Identified the Need for Combined HRT:** You checked for an intact uterus before confirming she needs a progestogen component alongside oestrogen.
- **Balanced, Non-Directive Risk Discussion:** You avoided both dismissing her fears and overstating the risks, giving an honest, proportionate answer.
- **Route-Specific Safety Advice:** You explained that transdermal oestrogen avoids the clot risk associated with oral oestrogen, tailoring your recommendation accordingly.
- **Holistic Management:** You included simple lifestyle measures alongside HRT, rather than presenting medication as the only option.

SECTION: NEUROLOGY

Station 83 — Neurology: Headache Differential Diagnosis

A headache history-taking station tests your ability to rapidly differentiate common primary headaches (tension-type, migraine, cluster) from dangerous secondary causes, using a structured framework, while addressing the patient's very common underlying fear of a brain tumour.

The SOCRATES Framework Applied to Headache

- **Site:** all over (tension), one-sided (migraine or cluster), or strictly behind one eye (cluster).
- **Onset:** gradual build-up, or an instant "thunderclap" onset (a red flag for subarachnoid haemorrhage)?
- **Character:** throbbing/pulsating (migraine), a tight band (tension), or sharp/stabbing (cluster or trigeminal neuralgia)?
- **Associations:** nausea/vomiting, photophobia/phonophobia, visual aura (migraine); watery eye, blocked nose, or drooping eyelid on the affected side (cluster).
- **Exacerbating/relieving factors:** worse with coughing, straining, or bending forward (raised intracranial pressure); better lying in a dark, quiet room (migraine).

Absolute Red Flags to Screen For

- **Raised intracranial pressure:** worse in the morning, on coughing/straining/bending forward, or with early morning vomiting.
- **Meningitis:** fever, neck stiffness, or a non-blanching rash.
- **Giant cell arteritis (if over 50):** scalp tenderness, jaw claudication, or visual changes.
- **Malignancy/systemic:** unexplained weight loss, night sweats, or a history of cancer.
- **Neurological deficit:** new weakness, numbness, speech difficulty, or balance problems.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Laura Bennett, 29 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Laura, 29, has had recurrent headaches over the last 6 months, occurring roughly twice a month.

Your Task:

- Take a structured SOCRATES history.
- Screen systematically for red flags.
- Address her underlying fear and deliver a diagnosis and management plan.

Candidate: Hello Laura, I understand you've been getting headaches — can you tell me a bit more about what's been happening?

Laura: They've been going on for about six months, roughly twice a month. It's usually one-sided, throbbing pain, and I feel really sick with it, sometimes I'm actually sick. Bright lights make it so much worse, I have to lie down in a dark room. Sometimes, just before it starts, I see these odd zig-zag lines in my vision.

Candidate: Thank you, that's a really clear description. How long does a typical episode last, and does anything specific seem to bring it on?

Laura: It usually lasts anywhere from a few hours to most of a day. I've noticed it's often worse around my period, and also after a stressful week at work with not much sleep.

Your Next Step: Red Flag Screening

Candidate: I want to ask a few important safety questions. Is the headache ever worse first thing in the morning, or when you cough, strain, or bend forward? Have you had any fevers, a stiff neck, or a rash that doesn't fade under pressure? Any sudden weakness, numbness, difficulty speaking, or problems with your balance?

Laura: No to all of that, thankfully. It's not worse in the mornings or with straining, no fevers or stiff neck, and definitely no weakness or speech problems. I have to admit, though, I've been secretly terrified this might be a brain tumour.

Common Pitfall: *"Don't worry, it's definitely not a tumour."* — Dismisses her fear too briefly and without explaining the reasoning, which can feel unconvincing rather than genuinely reassuring.

Model Response

Candidate: I really appreciate you telling me that, Laura — it's an extremely common fear, and I want to explain exactly why I'm not worried about that, rather than just telling you not to be. The pattern you've described — one-sided, throbbing pain with sensitivity to light, feeling sick, and those zig-zag visual changes beforehand — is actually very classic for a condition called migraine, not a tumour. Importantly, none of the specific warning signs I just asked you about are present, which is reassuring. Tumour-related headaches tend to be different: often constant, gradually worsening over weeks, worse in the morning, and accompanied by other neurological changes, none of which fit your story.

Laura: That actually makes so much sense when you explain it that way. I feel a lot better hearing the reasoning, not just being told not to worry.

Your Next Step: Management

Candidate: For managing the migraines, I'd suggest keeping a headache diary to help spot your triggers, since stress, poor sleep, and your monthly cycle all seem to play a role for you. For an acute attack, a combination of a specific migraine medication called a triptan alongside an anti-inflammatory like ibuprofen, taken as early as possible once a migraine starts, tends to work well. If you find yourself needing painkillers more than a couple of days a week, it's worth mentioning to us, as overusing them can actually cause a separate type of headache. If your migraines become frequent or severe despite this, there are also preventative daily medications we can discuss.

Laura: That's really helpful, thank you — I'll start with the diary and try the triptan next time one starts.

Station Conclusion: An "Excellent Pass" Headache Differential Profile

Why your final choices scored maximum marks:

- **Complete SOCRATES History:** You systematically covered site, character, associations, and timing rather than a vague general description.
- **Thorough Red-Flag Screen:** You explicitly asked about raised ICP symptoms, meningitis symptoms, and neurological deficits before reaching a diagnosis.
- **Addressed Her Fear With Genuine Reasoning:** You explained why the pattern didn't fit a tumour rather than offering a bare reassurance.
- **Correct Migraine Management:** You covered trigger identification, acute treatment (triptan plus NSAID), and screened for medication overuse risk.
- **Mentioned the Preventative Option:** You flagged escalation to preventative treatment for frequent or severe migraines, showing awareness of the full management ladder.

SECTION: ONCOLOGY & CANCER SCREENING

Station 84 — Oncology: Suspected Ovarian Cancer

Ovarian cancer is sometimes called a "silent killer" because its symptoms are vague and easily mistaken for benign conditions like irritable bowel syndrome (IBS), often leading to delayed diagnosis. A key UK safety principle is that new, IBS-like symptoms in a woman aged 50 or over should prompt investigation for ovarian cancer, since IBS rarely presents for the first time at this age.

Key Symptoms — Often Mistaken for IBS

- Persistent bloating (not coming and going).
- Early satiety or loss of appetite.
- Pelvic or abdominal pain.
- Increased urinary urgency or frequency.
- Unexplained weight loss, fatigue, or a change in bowel habit.

Risk Factors

- **Increased risk:** increasing age, BRCA1/BRCA2 mutation, family history, endometriosis, never having been pregnant, early menarche/late menopause.
- **Reduced risk:** combined oral contraceptive pill use, pregnancy, and breastfeeding.

Investigation Pathway

- **CA125 blood test:** first-line investigation in primary care for a woman with persistent or frequent relevant symptoms.
- **If CA125 is 35 IU/mL or greater:** arrange an urgent pelvic/abdominal ultrasound scan.
- **Risk of Malignancy Index (RMI):** combines menopausal status, ultrasound findings, and the CA125 level to stratify risk and guide referral to specialist gynaecological oncology services.
- **Immediate 2-Week Wait referral:** if examination or imaging reveals ascites and/or a pelvic or abdominal mass.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Patricia Doyle, 58 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Patricia, 58, has booked an appointment for bloating and reduced appetite over the last 2 months, which she has been self-managing as "IBS."

Your Task:

- Take a focused history covering the key symptoms and risk factors.
- Explain why this needs investigation rather than a presumed IBS diagnosis.
- Arrange the correct investigation pathway.

Candidate: Hello Patricia, I'm Dr [Your Name]. I understand you've been having some bloating and appetite changes — can you tell me more?

Patricia: Yes, for about two months now. My stomach feels constantly bloated, almost like I'm pregnant by the end of the day, and I just can't manage full meals anymore — I get full so quickly. I assumed it was IBS, so I've been trying to manage it myself with some over-the-counter remedies, but nothing's really helped.

Candidate: Thank you for describing that so clearly. Have you noticed any pain in your tummy or pelvis, any changes in how often you need to pass urine, any changes to your bowel habit, or any weight loss you didn't intend?

Patricia: Now that you mention it, I have needed the toilet more urgently than usual the last few weeks, and I think I've lost a bit of weight too, though I put that down to eating less because of the fullness.

Common Pitfall: *"This does sound like classic IBS — let's try a course of antispasmodics and see how you get on." — New-onset, IBS-like symptoms in a woman of 58 should never be presumed to be IBS without first excluding ovarian cancer, since IBS very rarely starts for the first time at this age.*

Model Response

Candidate: Thank you for all of that detail, Patricia. I want to be upfront with you: because these symptoms have started newly at your age, rather than automatically treating this as IBS, our guidelines specifically recommend we rule out a condition affecting the ovaries first, since the symptoms can look very similar. I'd like to arrange a blood test called CA125 today, which can be a helpful marker, alongside an ultrasound scan of your pelvis and tummy.

Patricia: Oh — you think this could be ovarian cancer? That's frightening to hear.

Candidate: I understand that's a scary thing to consider, and I want to be honest with you rather than dismiss your symptoms as something simple without checking properly. Many women with these symptoms turn out to have a benign, treatable cause, but because the symptoms of ovarian cancer can mimic IBS so closely, especially at your age, it's important we investigate properly rather than assume. Depending on what the blood test and scan show, we'll use a specific scoring system that combines those results with your menopausal status to work out exactly how urgently you need to be seen by a specialist.

Patricia: I appreciate you being straightforward with me rather than brushing this off. Let's get the tests done.

Station Conclusion: An "Excellent Pass" Ovarian Cancer Profile

Why your final choices scored maximum marks:

- **Avoided the IBS Trap:** You recognised that new IBS-like symptoms at age 58 should prompt investigation for ovarian cancer rather than a presumed benign diagnosis.
- **Correct First-Line Investigation:** You arranged CA125 and pelvic ultrasound, the correct primary-care investigation pathway.
- **Explained the RMI Concept:** You described how the results would be combined to stratify urgency, rather than presenting the tests as an endpoint in themselves.
- **Honest but Balanced Communication:** You named the possibility directly without either alarming her unnecessarily or minimising the reason for investigation.

Station 85 — Oncology: Breast Cancer Family History and Genetic Risk Assessment

This station complements Station 2 (symptomatic breast lump) by testing a different scenario: a well patient concerned about her family history of breast cancer, asking whether she should be tested for a genetic mutation. This tests your ability to apply UK familial risk referral criteria correctly, rather than referring everyone with any family history, or conversely reassuring everyone without proper assessment.

NICE-Aligned Referral Criteria (Any One of the Following Warrants Referral for Specialist Assessment)

- One first-degree relative diagnosed with breast cancer under the age of 40.
- Two or more relatives on the same side of the family diagnosed with breast and/or ovarian cancer.
- Any relative with bilateral breast cancer, male breast cancer, or triple-negative breast cancer diagnosed under 60.
- A relative with both breast and ovarian cancer.
- Ashkenazi Jewish ancestry combined with a family history of breast or ovarian cancer.

A single distant relative, or an isolated case of breast cancer at a typical older age, generally does not meet the threshold for genetic referral on its own.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Amanda Foster, 32 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Amanda, 32, is well with no breast symptoms, but is anxious after her mother was recently diagnosed with breast cancer at age 38, and her maternal aunt had ovarian cancer.

Your Task:

- Take a structured family history.
- Determine whether she meets referral criteria.
- Explain the next steps and what genetic testing would involve.

Candidate: Hello Amanda, I'm Dr [Your Name]. I understand you're worried about your family history of cancer — can you tell me more about that?

Amanda: Yes, my mum was diagnosed with breast cancer just a few months ago — she's only 38. And my aunt, my mum's sister, had ovarian cancer a couple of years back. I'm terrified I'm going to get this too. Should I just ask for a mammogram now, or maybe get tested for the cancer gene?

Common Pitfall: *"Since it's just your mum and aunt, that's probably not significant enough to worry about — let's leave it for now." — This family history pattern — a first-degree relative diagnosed under 40, plus a same-side relative with ovarian cancer — clearly meets referral criteria, and dismissing it risks missing a genuine hereditary cancer syndrome.*

Model Response

Candidate: Thank you for telling me that, and it's completely understandable to feel anxious given what your family's been through. Let me ask a few more details to build the full picture — do you know if your mum or aunt have had any genetic testing themselves, and are there any other relatives on your mum's side with breast or ovarian cancer?

Amanda: My mum hasn't had genetic testing yet, as far as I know. No other relatives that I'm aware of, just those two.

Candidate: Thank you. Based on what you've told me — your mother being diagnosed under 40, combined with your aunt having ovarian cancer on the same side of the family — this does meet our criteria for referral to a specialist genetics service, since this pattern raises the possibility of an inherited faulty gene, such as BRCA1 or BRCA2, running in your family.

Amanda: Does that mean I definitely have it too?

Candidate: Not at all — it means it's worth properly assessing. Ideally, the specialist team would first try to test your mum or aunt directly, since finding a specific gene fault in a relative who's already had cancer gives the clearest answer. If a mutation is found in the family, you could then be offered a specific test for that exact change. If no relative can be tested, they can still assess your own risk based on the family pattern and offer you testing directly if appropriate.

Amanda: That makes sense. What would happen if I did test positive?

Candidate: If you were found to carry a mutation, you'd be offered enhanced surveillance, such as earlier and more frequent breast screening with MRI scans starting well before the usual national screening age, and a specialist team would discuss further options with you, including risk-reducing surgery or medication, entirely based on your own preferences and circumstances. None of that is decided today — the first step is simply the referral for proper assessment.

Amanda: Thank you, that's much clearer. I'd like to go ahead with that referral.

Station Conclusion: An "Excellent Pass" Genetic Risk Assessment Profile

Why your final choices scored maximum marks:

- **Correctly Applied Referral Criteria:** You recognised that a first-degree relative diagnosed under 40 plus a same-side relative with ovarian cancer meets the threshold for genetic referral.
- **Explained Cascade Testing Logic:** You explained that testing an affected relative first is the most informative approach, rather than testing Amanda in isolation as a first step.
- **Avoided Overstating Certainty:** You clarified that meeting referral criteria doesn't mean she definitely carries a mutation.
- **Outlined Options Without Pressuring a Decision:** You described surveillance and risk-reducing options factually, without implying she needed to decide anything at this appointment.

Station 86 — Public Health: UK National Cancer Screening Programmes

This station tests your knowledge of the three main UK population-based cancer screening programmes — breast, cervical, and bowel — testing whether you can explain the correct age ranges, intervals, and tests clearly to a patient asking about screening in general.

The Three UK Screening Programmes

Programme	Age Range	Interval	Test
Breast	50 up to the 71st birthday (first invitation between 50–53)	Every 3 years	Mammogram (breast X-ray)
Cervical	25–49; then 50–64	Every 3 years (25–49); every 5 years (50–64)	HPV test first, then cytology if HPV positive
Bowel	50–74 (England, Scotland, Wales); 60–74 in Northern Ireland	Every 2 years	FIT (faecal immunochemical test) home test kit

People above the upper age limit for breast or bowel screening are not automatically invited but can request ongoing screening directly. All three programmes are opt-out, not opt-in — eligible people are automatically invited if registered with a GP.

The Roleplay

Station: General Practice Consultation

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Diane Osborne, 51 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Diane, 51, has come in with general questions about which cancer screening tests she should be having now that she's over 50.

Your Task:

- Explain all three relevant national screening programmes clearly.
- Correct any misconceptions she has.
- Reinforce that screening does not replace vigilance for new symptoms.

Candidate: Hello Diane, I'm Dr [Your Name]. I understand you wanted to check which cancer screening tests apply to you now — how can I help?

Diane: I turned 51 last month, and a friend mentioned there are a few different screening tests I should be getting, but I'm honestly not sure which ones, or whether I need to book them myself.

Candidate: That's a great question, and it's really common to be unsure, since there are three separate national programmes. The good news is that all three are automatic — you don't need to book them yourself, as long as you're registered with a GP, which you are.

Common Pitfall: *"You just need to make sure you book your own mammogram and smear test — the NHS won't do that for you automatically."* — Incorrectly implies these are opt-in programmes requiring the patient to proactively book, when in fact eligible people are automatically invited by letter without needing to request anything.

Model Response

Candidate: For breast cancer, you'll be automatically invited for a mammogram, which is a breast X-ray, every 3 years from age 50 up until your 71st birthday — your first invitation would have come between your 50th and 53rd birthday, so you might already be due one. For cervical screening, since you're 51, you're now in the group invited every 5 years rather than every 3, which is the interval for those between 25 and 49 — that test checks for a virus called HPV first, and only looks at the cells themselves if that comes back positive. And for bowel cancer, you'll automatically be sent a simple home test kit called a FIT test every 2 years from age 50 to 74 — you just provide a small stool sample at home and post it off, no appointment needed.

Diane: That's really helpful, thank you — I had no idea the bowel one was just done at home. What if I haven't received an invitation for any of these yet?

Candidate: If you think you're overdue for any of them, it's always worth contacting your GP practice or the relevant screening service directly, since letters can occasionally go astray, especially if you've moved house recently. One really important point, though: screening is for people without symptoms. If you ever notice something new or concerning in between your screening appointments — a breast lump, unexpected bleeding, or a change in your bowel habit, for example — please come and see us straight away rather than waiting for your next scheduled screening, since screening tests are done at set intervals and aren't a substitute for getting new symptoms checked.

Diane: That makes complete sense, thank you for laying it all out so clearly. I feel like I actually understand what to expect now.

Station Conclusion: An "Excellent Pass" Cancer Screening Overview Profile

Why your final choices scored maximum marks:

- **Accurate Ages and Intervals for All Three Programmes:** You gave the correct age ranges and intervals for breast, cervical, and bowel screening without conflating them.
- **Corrected the "Opt-In" Misconception:** You clarified that all three programmes are automatic, opt-out invitations, not something the patient needs to arrange themselves.
- **Explained the HPV-First Cervical Pathway:** You accurately described that HPV testing comes first, with cytology only performed if HPV is positive.

- **Reinforced Symptom Vigilance:** You explicitly stated that screening doesn't replace seeking urgent review for new symptoms between appointments — a frequently missed safety point.

SECTION: CLINICAL PROCEDURAL SKILLS

Introduction to Procedural Skills Stations

Procedural Skills stations test manual dexterity, adherence to Aseptic Non-Touch Technique (ANTT), and clear communication with both the patient (or mannequin) and the examiner. Unlike consultation stations, you are expected to narrate your actions out loud throughout, since the examiner is scoring what you say as much as what you do. Common threads across every procedure in this section: obtain informed consent before starting, maintain a clearly defined sterile field, narrate each safety check as you perform it, and know the exact stopping rule for when something goes wrong.

Station 87 — Procedural Skills: Male Urinary Catheterisation

Male urinary catheterisation is a high-frequency, safety-critical procedural station. Your objectives are to maintain an absolute sterile field, minimise urethral trauma, and safely manage a common clinical emergency: acute urinary retention.

High-Yield Procedural Step Matrix

Step	Core Action	Critical UK Safety Rule
Preparation	Consent, assemble kit, apply an apron, wash hands, open the outer tray using ANTT.	Use an appropriate catheter size — typically 12–14 Ch/Fr for a standard adult male.
Cleaning	Don sterile gloves, place a fenestrated drape, clean the glans penis with sterile saline wipes.	Clean in a single downward stroke from the urethral meatus outward. Use each wipe only once.
Anaesthesia	Instil roughly 11ml of 2% lidocaine gel (Instillagel) into the urethra using the sterile syringe nozzle.	Wait a full 3–5 minutes for anaesthesia and lubrication to take effect before inserting the catheter.
Insertion	Hold the penis at a 90-degree angle to straighten the urethra. Advance gently until urine flows, then advance to the hilt.	Never force the catheter against resistance (e.g. at the prostate) — stop immediately to avoid a false passage.
Anchoring	Inflate the retention balloon slowly with 10ml of sterile water, once urine flow confirms correct placement.	Reposition the foreskin immediately afterwards to prevent paraphimosis.

The Roleplay

Station: Clinical Procedural Skills

Candidate Role: FY2 Doctor in the Acute Medical Unit (AMU)

Patient/Model Role: Male catheterisation pelvic mannequin (Arthur Pendleton, 68)

Setting: Treatment Room, AMU

The Prompt (read outside the cubicle):

Patient Details: Arthur Pendleton, 68, admitted with severe lower abdominal pain and complete inability to pass urine for 9 hours. A bladder scan shows 850ml of retained urine.

Your Task:

- Counsel the patient and obtain informed consent.
- Perform male urinary catheterisation on the mannequin safely.
- Narrate safety measures to the examiner throughout.

(The mannequin is draped on the couch. The examiner sits in the corner with a marking sheet.)

Candidate: Hello Mr Pendleton, my name is Dr [Your Name], one of the medical doctors on duty today. I can see you're in a huge amount of pain from that full bladder — we're going to relieve that pressure right away. I'd like to insert a small, flexible tube called a catheter into your bladder to drain the trapped urine safely. It involves numbing the area first with a special gel. Do I have your permission to go ahead?

Arthur: Yes, please do it, Doctor. The pressure is agonising. Is it going to hurt?

Common Pitfall: *"It might hurt a bit, but we don't have time to wait — I'll push the tube straight in and inflate the balloon quickly." — Skips the mandatory local anaesthetic window entirely, causing unnecessary pain and urethral trauma; there is always time for the 3–5 minute Instillagel window, even in urgent retention.*

Model Response

Candidate: I understand you're in a lot of discomfort, Arthur, and we'll move as gently as possible. To make sure you don't feel pain, I'm going to instil a specialised numbing and lubricating gel directly into your waterworks first, and leave it for a full 3 to 5 minutes to make the passage completely numb and smooth before the tube goes in. I'll now wash my hands, set up my sterile field, and prepare the equipment.

Your Next Step: Cleaning and Anaesthesia

Candidate: I'm holding the penis with a sterile gauze pad using my non-dominant hand — this hand is now non-sterile and will stay in place. With my dominant, sterile hand, I'm cleaning the glans in a single downward stroke away from the tip, using each wipe only once. I'm now introducing the nozzle of the lidocaine syringe into the meatus, instilling the gel smoothly, and gently pinching the tip closed to keep the gel inside the urethra for the full 3 to 5 minutes.

Your Next Step: Insertion and Anchoring

Common Pitfall: *"I'll hold the penis flat against the legs and push firmly. If I feel a hard bump near the prostate, I'll twist sharply and push past it quickly." — Forcing a catheter against resistance risks creating a false passage or urethral tear; any resistance must prompt an immediate stop, never increased force.*

Model Response

Candidate: I'm holding the penis at a 90-degree angle to straighten the urethra, and feeding the catheter in gently. Once I see urine flowing into the tube, I'll advance it fully to the hilt, then slowly inflate the retention balloon with 10ml of sterile water, and pull back gently until it anchors. I'll connect the drainage bag, and immediately reposition the foreskin back over the glans to prevent paraphimosis. Finally, I'll measure the initial urine output.

Station Conclusion: An "Excellent Pass" Male Catheterisation Profile

Why your final choices scored maximum marks:

- **Flawless ANTT:** You maintained a clear sterile field, split your hands into a non-sterile "holding" hand and sterile "inserting" hand, and cleaned in a single outward motion.
- **Respected the Anaesthetic Window:** You waited the full 3–5 minutes for the lidocaine gel to take effect, rather than rushing.
- **Prevented Paraphimosis:** You repositioned the foreskin immediately after anchoring the balloon — leaving it retracted is an automatic safety fail.
- **Mastery of the Resistance Protocol:** You stopped immediately at any resistance rather than forcing the catheter, preventing a false passage or urethral tear.

Station 88 — Procedural Skills: Female Urinary Catheterisation

Female catheterisation follows the same core principles as the male procedure, adapted for different anatomy. The single highest-yield teaching point in this station is how to safely handle accidental vaginal misplacement of the catheter — a very common practical error that examiners specifically test for correct containment, not for avoiding the error altogether.

The Roleplay

Station: Clinical Procedural Skills

Candidate Role: FY2 Doctor on the Ward

Patient/Model Role: Female catheterisation pelvic mannequin

Setting: Treatment Room

Your Task:

- Obtain consent and prepare a sterile field.
- Identify the anatomy correctly and administer local anaesthetic gel.
- Safely manage accidental vaginal misplacement if it occurs.

Your Next Step: Exposure, Identification, and Anaesthesia

Common Pitfall: *"I'll part the labia with my dominant sterile hand and wipe the area back and forth with gauze to clear discharge quickly, then inject gel and insert the catheter immediately."* — Contaminates the sterile hand by using it for a non-sterile exposure task, wipes in a manner that risks faecal contamination of the urethra, and skips the mandatory anaesthetic window.

Model Response

Candidate: I'm using my non-dominant hand to part the labia majora and minora firmly, and stating clearly that this hand is now non-sterile and will stay in place to maintain exposure. I can identify the urethral meatus directly above the vaginal opening and below the clitoris. With my dominant, sterile hand, I'm cleaning the meatus in a single downward motion from front to back using saline wipes. I'm now introducing the nozzle of the lidocaine syringe, instilling the gel smoothly, and waiting a full 3 to 5 minutes for complete anaesthesia.

Your Next Step: Handling Vaginal Misplacement

The examiner tells you: as you advance the catheter, it accidentally slides into the vaginal opening instead of the urethra. This is the single most important safety test in this station.

Common Pitfall: *"I'll pull the catheter out of the vagina, wipe it with a sterile saline cloth, and re-insert the exact same tube into the urethra to save equipment."* — A catheter that has entered the vagina is contaminated and must never be reinserted into the urethra, even after wiping, as this risks introducing infection directly into the bladder.

Model Response

Candidate: I'll leave the misplaced catheter exactly where it is in the vagina — I won't touch or remove it, since it now acts as a helpful landmark showing me where not to go again. I'll state clearly to the examiner that this catheter is contaminated. I'll request a brand-new sterile catheter kit, don fresh sterile gloves, and feed the second catheter into the correct urethral meatus, which I can now see directly above the first, blocked, vaginal opening. Once urine flows through the new catheter, I'll inflate the balloon, and only then will I remove and discard the first, contaminated catheter.

Station Conclusion: An "Excellent Pass" Female Catheterisation Profile

Why your final choices scored maximum marks:

- **Correct Anatomical Identification:** You correctly located the urethral meatus and maintained clear exposure with a designated non-sterile hand.
- **Front-to-Back Cleaning:** You cleaned in a single front-to-back motion, minimising the risk of introducing faecal bacteria into the urethra.
- **Safely Contained the Vaginal Misplacement Error:** You left the misplaced catheter in place as a landmark rather than removing it, and used a fresh sterile kit for the second attempt — the correct, examinable response to this very common error.
- **Never Reused a Contaminated Catheter:** You recognised that a catheter which entered the vagina can never be reinserted into the urethra, regardless of cleaning.

Station 89 — Procedural Skills: Arterial Blood Gas (ABG) Sampling

An ABG station tests extracting blood directly from the radial artery to assess respiratory, metabolic, and acid-base status. Because you are puncturing a high-pressure artery rather than a vein, strict adherence to pre-procedural safety checks and post-procedural haemostasis is essential.

High-Yield Procedural Step Matrix

Step	Core Action	Critical UK Safety Rule
Preparation	Gain consent, assemble the heparinised blood gas syringe, position the wrist extended over a small pillow.	Perform the modified Allen's test before skin puncture to verify adequate collateral (ulnar) blood supply.
Palpation & cleaning	Palpate the radial pulse just proximal to the wrist crease; clean with 2% chlorhexidine in 70% isopropyl alcohol for 30 seconds and let it air-dry.	Do not re-palpate the exact entry point after cleaning unless wearing sterile gloves.
Puncture	Hold the syringe like a dart; insert at a 45-degree angle, bevel up, directly into the pulsating artery.	Allow the syringe to self-fill from arterial pressure — do not pull back on the plunger manually.
Haemostasis	Withdraw the needle, activate the safety shield immediately, and apply firm continuous digital pressure for a minimum of 5 full minutes.	Increase to 10 minutes if the patient is anticoagulated. Never ask the patient to hold the pressure themselves.
Processing	Expel air bubbles, cap the syringe, gently roll it between your palms to mix the heparin, and analyse immediately.	The sample must be analysed within 15 minutes of collection to ensure accurate values.

The Roleplay

Station: Clinical Procedural Skills

Candidate Role: FY2 Doctor in the AMU

Patient/Model Role: ABG forearm mannequin (Arthur Pendleton, 54)

Setting: Treatment Room, AMU

The Prompt (read outside the cubicle):

Patient Details: Arthur, 54, admitted with a severe acute exacerbation of COPD, breathless, requiring an urgent ABG.

Your Task:

- Counsel the patient and obtain consent.
- Perform the modified Allen's test and radial artery puncture safely.
- Demonstrate correct haemostasis and sample processing.

Candidate: Hello Arthur, my name is Dr [Your Name]. I can see you're working quite hard with your breathing. To find out exactly how much oxygen and carbon dioxide are in your blood so we can treat you correctly, I need to take a small sample from an artery in your wrist. It can feel a bit sharper than a usual blood test, but I'll move quickly and gently. Do I have your permission to go ahead?

Arthur: Yes, go ahead, Doctor. My chest feels so tight, just do whatever you need to.

Common Pitfall: *"I'll tie a tourniquet around your upper arm and push the needle straight into the pulse point to get the sample quickly." — A tourniquet is not used for arterial sampling, and skipping the Allen's test before puncturing a high-pressure artery risks proceeding without confirming safe collateral circulation to the hand.*

Model Response

Candidate: Before I prepare the needle, I need to perform a quick, painless check on your wrist called an Allen's test, which confirms the alternative blood vessel in your hand is working well to keep your hand safe during the test. I'll now wash my hands, apply my apron, and set up my equipment.

Your Next Step: The Modified Allen's Test and Puncture

Candidate: I'm firmly compressing both the radial and ulnar arteries at the wrist simultaneously, and asking you to open and close your fist tightly three times until your palm turns pale. I'm now releasing pressure only from the ulnar artery, while keeping the radial artery compressed, and watching for the palm to flush red within 5 to 10 seconds, which would confirm good collateral flow.

(The palm flushes back to normal colour within the expected time — a positive Allen's test.)

Candidate: The Allen's test is positive, confirming safe collateral circulation. I'm cleaning the site with chlorhexidine in alcohol, letting it air-dry, then inserting the needle at a 45-degree angle with the bevel facing upward, directly into the pulsating artery, and allowing the syringe to self-fill from arterial pressure rather than pulling back on the plunger.

Your Next Step: Haemostasis and Processing

Common Pitfall: *"I'll pull the needle out, slide the cap back on by hand, and ask the patient to press on their own wrist while I walk away. I'll shake the syringe to mix the heparin." — Manually re-sheathing a needle risks a needlestick injury, asking the patient to self-apply pressure risks inadequate haemostasis on a high-pressure artery, and vigorous shaking causes haemolysis, distorting the results.*

Model Response

Candidate: I'm withdrawing the needle smoothly, immediately activating the integrated safety shield, and placing it straight into the sharps bin. I'm applying firm, continuous digital pressure with a sterile gauze pad to the wrist myself for a full 5 minutes without stopping, to prevent a haematoma — I'd extend this to 10 minutes if Arthur were on an anticoagulant. For the sample, I'm expelling any air bubbles, capping it, gently rolling it between my palms to mix the heparin without causing haemolysis, and taking it to the blood gas analyser immediately, since it needs to be processed within 15 minutes for accurate results.

Station Conclusion: An "Excellent Pass" ABG Sampling Profile

Why your final choices scored maximum marks:

- **Mandatory Allen's Test:** You verified collateral ulnar circulation before puncturing the radial artery.
- **Correct Puncture Dynamics:** You used the 45-degree angle, bevel up, and let arterial pressure self-fill the syringe rather than aspirating manually.
- **Flawless Sharps Safety and Haemostasis:** You activated the safety shield immediately, disposed of the sharp correctly, and personally maintained 5 minutes of continuous pressure rather than delegating it to the patient.
- **Preserved Sample Integrity:** You rolled rather than shook the syringe, and processed it within the 15-minute window.

Station 90 — Procedural Skills: IV Cannulation and Venepuncture

Cannulation and venepuncture are among the most frequently performed practical skills in UK hospitals, and are correspondingly high-yield PLAB 2 procedural stations. The core skill is the same for both — locating and accessing a vein safely — with venepuncture drawing a sample and cannulation leaving a device in place for ongoing access.

The Roleplay

Station: Clinical Procedural Skills

Candidate Role: FY2 Doctor on the Ward

Patient/Model Role: IV cannulation arm mannequin

Setting: Treatment Room

Your Task:

- Obtain consent and prepare equipment using ANTT.
- Perform the cannulation safely.
- Secure the cannula and dispose of sharps correctly.

Candidate: Hello, my name is Dr [Your Name]. I need to place a small plastic tube into a vein in your arm so we can give you fluids and medications directly into your bloodstream. It involves a quick needle scratch to get it in, but the needle itself doesn't stay in — only a soft, flexible tube does. Are you happy for me to go ahead?

Common Pitfall: *"I'll apply the tourniquet, quickly wipe the arm, and insert the cannula straight in at a steep angle." — Rushing skips proper vein selection, uses too steep an insertion angle risking through-puncture of the vein, and doesn't allow the antiseptic wipe to dry before puncture, all of which increase the risk of a failed attempt or infection.*

Model Response

Candidate: I'm applying the tourniquet approximately 10 to 15 centimetres above my chosen site, and asking you to open and close your fist to help the veins fill. I'm inspecting and palpating for a suitable vein — ideally straight, bouncy, and well-supported — and I've chosen a vein on the back of the hand or forearm, avoiding areas of flexion. I'm cleaning the site with a chlorhexidine in alcohol wipe for 30 seconds and letting it air-dry fully before touching the skin again.

Candidate: I'm inserting the cannula bevel-up at approximately 15 to 30 degrees to the skin. Once I see a flashback of blood in the chamber, I'll lower the angle slightly, advance a couple of millimetres to ensure the tip is fully within the vein, then withdraw the needle slightly while advancing the plastic cannula fully off the needle and into the vein.

Your Next Step: Securing and Sharps Safety

Candidate: I'm releasing the tourniquet now, and applying pressure just proximal to the cannula tip with one finger while I fully withdraw the needle, immediately activating the safety shield and disposing of it directly into the sharps bin. I'll attach the cap or extension set, flush the cannula

with saline to confirm it flushes easily without swelling or pain, and secure it with a clear adhesive dressing so the insertion site remains visible for monitoring. I'll dispose of all my equipment appropriately and document the date, size, and site of insertion.

Candidate: For venepuncture specifically, rather than a cannula, I'd attach a vacutainer needle and holder instead, and draw blood directly into the required sample bottles, filling them in the correct order if multiple tests are needed, before applying pressure and a dressing in the same way.

Station Conclusion: An "Excellent Pass" Cannulation Profile

Why your final choices scored maximum marks:

- **Correct Vein Selection:** You chose a straight, well-supported vein away from areas of flexion, rather than the first vein seen.
- **Proper Antiseptic Technique:** You allowed the antiseptic to air-dry fully before puncture, rather than wiping and immediately inserting.
- **Correct Puncture Angle and Flashback Recognition:** You used a shallow 15–30 degree angle and correctly advanced the cannula fully off the needle after confirming flashback.
- **Immediate Sharps Safety:** You activated the safety shield and disposed of the needle immediately, minimising needlestick risk.
- **Complete Documentation:** You documented the date, size, and site of the cannula, which is essential for infection monitoring and timely removal.

Station 91 — Procedural Skills: Nasogastric (NG) Tube Insertion

Nasogastric tube insertion is a common but genuinely high-risk procedural station, since a misplaced tube used for feeding can cause fatal aspiration into the lungs. UK patient safety guidance is extremely strict about how placement is confirmed, and this is the single most heavily marked element of the station.

The Critical Safety Point: Confirming Placement

Do not commence feeding or giving medication through the tube until placement is confirmed by one of the following accepted methods:

- **First-line:** aspirate a small amount of fluid from the tube and test it with pH paper — a pH of 5.5 or below confirms gastric placement.
- **If aspirate cannot be obtained, or pH is above 5.5:** arrange a chest X-ray, checking that the tube follows the midline down past the carina and the tip is clearly visible below the left hemidiaphragm.

The old "whoosh test" — injecting air down the tube while listening with a stethoscope over the stomach — is no longer considered a safe or acceptable method of confirming placement in current UK practice, following patient safety alerts linked to fatal misplaced feeding. Do not use it, and do not mention it as an acceptable confirmatory step in this station.

The Roleplay

Station: Clinical Procedural Skills

Candidate Role: FY2 Doctor on the Ward

Patient/Model Role: NG tube insertion mannequin

Setting: Treatment Room

Your Task:

- Obtain consent and measure the correct insertion length.
- Insert the tube safely, guiding the patient through it.
- Confirm placement correctly before declaring the tube ready for use.

Candidate: Hello, my name is Dr [Your Name]. I need to pass a thin, soft tube through your nose and down into your stomach, which we can use to give you feeds or medication directly. It might feel a bit uncomfortable and make you want to gag briefly, but I'll go slowly and it will be over quickly. Sipping some water when I ask will really help it pass smoothly. Are you happy for me to go ahead?

Common Pitfall: *"I'll insert the tube to a rough estimated length, and once I have it in, I'll inject some air down the tube and listen over the stomach with my stethoscope to confirm it's in the right place." — The whoosh test is not a safe or accepted method of confirming NG tube placement in current UK practice; only pH testing of aspirate or chest X-ray should be used, since air auscultation has caused missed lung placements leading to patient deaths.*

Model Response

Candidate: Before inserting, I'm measuring the correct length using the NEX method — from the tip of the nose, to the earlobe, and down to the xiphisternum — and noting this measurement so I know roughly how far to pass the tube. I'm lubricating the tip, and gently inserting it through your nostril, along the floor of the nose, backward and slightly downward. As it reaches the back of your throat, I'll ask you to sip some water and swallow, which helps guide the tube past your throat and down into the stomach rather than into your airway.

Candidate: Once the tube is at the measured length, I'll secure it temporarily. To confirm correct placement, I'm aspirating a small amount of fluid from the tube using a syringe, and testing it on pH paper. If the pH reads 5.5 or below, that confirms the tube is safely in the stomach, and I can secure it properly and it's then safe to use. If I can't get any aspirate, or the pH reads higher than 5.5, I will not use the tube for anything until we've arranged a chest X-ray to confirm the position directly.

Station Conclusion: An "Excellent Pass" NG Tube Insertion Profile

Why your final choices scored maximum marks:

- **Correct Length Measurement:** You used the NEX method (nose–earlobe–xiphisternum) to estimate insertion depth rather than guessing.
- **Guided the Patient Through Swallowing:** You used sips of water and swallowing to help the tube pass safely into the oesophagus rather than the airway.
- **Used Only Accepted Confirmation Methods:** You confirmed placement with pH testing of aspirate, and knew to escalate to chest X-ray if aspirate was unobtainable or inconclusive — critically, you did not use the outdated and unsafe whoosh test.
- **Withheld Use Until Confirmed Safe:** You explicitly stated the tube would not be used for feeding or medication until placement was properly confirmed.

Station 92 — Procedural Skills: Adult Basic Life Support (BLS)

Basic Life Support tests your ability to recognise cardiac arrest and deliver high-quality CPR according to the UK Resuscitation Council algorithm, on a mannequin, while clearly narrating each step and directing any bystanders or team members present.

The UK Resuscitation Council Adult BLS Algorithm

- **1. Danger:** check the scene is safe for you, the patient, and any bystanders.
- **2. Response:** gently shake the shoulders and shout loudly, "Can you hear me? Open your eyes!"
- **3. Shout for help:** call out for assistance immediately if unresponsive.
- **4. Airway:** open the airway with a head-tilt, chin-lift.
- **5. Breathing:** look, listen, and feel for normal breathing for no more than 10 seconds — agonal gasping does not count as normal breathing.
- **6. Call 999/emergency team and get a defibrillator:** if not breathing normally, call for emergency help and request a defibrillator immediately, ideally delegating this to a bystander while you begin compressions.
- **7. Start CPR:** 30 chest compressions to 2 rescue breaths, continuing this ratio.
- **8. Attach the defibrillator as soon as it arrives:** switch it on and follow its voice prompts without delay.

The Roleplay

Station: Clinical Procedural Skills / Emergency Simulation

Candidate Role: FY2 Doctor, first on the scene

Patient/Model Role: Adult CPR training mannequin

Setting: Hospital Corridor / Ward Bay

The Prompt (read outside the cubicle):

Scenario: You walk past a bay and find a patient collapsed on the floor, unresponsive.

Your Task:

- Assess safety and responsiveness.
- Recognise cardiac arrest and initiate the correct sequence.
- Deliver high-quality CPR, narrating each step, until the defibrillator arrives.

(A mannequin lies motionless on the floor. A nurse is nearby.)

Candidate: I'm checking the area is safe for myself and the patient before approaching.

Candidate: (kneeling beside the patient, gently shaking the shoulders) Can you hear me? Open your eyes!

(No response.)

Common Pitfall: *"The patient isn't responding, so I'll start chest compressions straight away." — Skips confirming the airway is open and checking for breathing first; a patient who is unresponsive but still breathing normally does not need chest compressions and instead needs a different emergency response.*

Model Response

Candidate: (shouting) I need help here now! (opening the airway with a head-tilt, chin-lift) I'm now looking, listening, and feeling for normal breathing for no more than 10 seconds.

(No normal breathing is detected — only occasional, irregular gasps.)

Candidate: This patient is in cardiac arrest. Please call the crash team on 2222 now and bring the defibrillator and the crash trolley immediately.

Your Next Step: Starting CPR

Candidate: I'm placing the heel of my hand on the centre of the chest, on the lower half of the sternum, with my other hand interlocked on top. I'm delivering chest compressions at a depth of 5 to 6 centimetres, at a rate of 100 to 120 per minute, allowing full chest recoil between each compression, and minimising any interruptions. I'll deliver 30 compressions, followed by 2 rescue breaths, and continue this cycle.

(The defibrillator arrives.)

Candidate: The defibrillator is here — I'm switching it on immediately and following its voice prompts, attaching the pads to the chest as instructed, while continuing compressions until the machine tells me to stand clear for rhythm analysis.

Station Conclusion: An "Excellent Pass" Basic Life Support Profile

Why your final choices scored maximum marks:

- **Followed the Full Sequence in Order:** You checked danger, response, airway, and breathing before concluding cardiac arrest, rather than jumping straight to compressions.
- **Correctly Identified Agonal Gasping:** You recognised that irregular gasping is not normal breathing and still indicates cardiac arrest requiring CPR.
- **Delegated the Emergency Call Immediately:** You directed a bystander to call for help and bring the defibrillator rather than delaying compressions to do this yourself.
- **Correct Compression Technique:** You specified correct hand position, depth (5–6cm), rate (100–120/min), and full recoil, rather than a vague description of "doing CPR."
- **Immediate Defibrillator Use:** You attached and activated the defibrillator the moment it arrived, following its prompts without delay.

Station 93 — Procedural Skills: Adrenaline Auto-Injector (EpiPen) Technique

This station tests your ability to both manage an anaphylactic emergency and teach correct adrenaline auto-injector technique — a skill patients and carers rely on being taught clearly and confidently.

Key Facts

- **Dosing by weight:** 300 micrograms (0.3mg) IM for adults and children over 30kg (roughly over 6 years); 150 micrograms (0.15mg) IM for children 15–30kg.
- **Injection site:** the anterolateral (outer) thigh, given firmly at a 90-degree angle — this can be given through light clothing in a genuine emergency if needed.
- **Hold time:** varies slightly by device brand (commonly 3–10 seconds) — always check the specific device's instructions, since this detail differs between manufacturers.
- **Always call 999 immediately, even if symptoms improve:** a biphasic reaction (symptoms recurring hours later) can occur, so emergency assessment is required regardless of apparent improvement.
- **A second dose can be given after 5–15 minutes:** if there's no improvement or symptoms return, while waiting for the ambulance.
- **Positioning:** lie the patient flat with legs raised if they feel faint or their blood pressure is low; sit them up if breathing is the main difficulty; use the recovery position if unconscious but breathing.

The Roleplay

Station: Clinical Procedural Skills / Patient Education

Candidate Role: FY2 Doctor in an Allergy Follow-Up Clinic

Patient/Roleplayer Role: Sophie Turner, mother of 9-year-old Max, recently diagnosed with a peanut allergy

Setting: Consultation Room

Your Task:

- Teach correct adrenaline auto-injector technique using a trainer device.
- Explain when and how to use it.
- Safety-net regarding calling emergency services.

Candidate: Hello Mrs Turner, I'm Dr [Your Name]. I understand Max has recently been diagnosed with a peanut allergy, and we've prescribed him an adrenaline auto-injector. I'd like to show you exactly how to use it today using this trainer device, so you feel completely confident.

Sophie: Yes, please — I'm honestly terrified I'll do it wrong if we ever really needed it.

Common Pitfall: *"Just pull off the cap and jab it anywhere on his body quickly, then call an ambulance if he still looks unwell."* — Gives no guidance on the correct injection site (outer

thigh), the correct angle, or the fact that an ambulance should always be called immediately regardless of how the patient looks afterwards.

Model Response

Candidate: That's a completely understandable fear, and that's exactly why we're practising together now. First and most importantly: if Max ever has a severe allergic reaction — difficulty breathing, swelling of the throat or lips, or feeling faint — you use the pen straight away, and call 999 immediately, even before or right after using it. You remove the safety cap, then hold the pen firmly against the outer part of his thigh, through clothing if you need to, at a 90-degree angle, and press firmly until you hear or feel a click. You then hold it in place for the count specified on this particular device — let's check the leaflet together, since this varies by brand.

Sophie: Okay, and what if he doesn't get better after that?

Candidate: Even if he seems to improve, you still need to call 999 and go to hospital, because these reactions can come back a few hours later even after the first dose helps. If there's no improvement, or symptoms come back, after 5 to 15 minutes, and the ambulance hasn't arrived yet, you can give a second dose from a spare pen. While waiting, lie him flat with his legs raised if he seems faint, or let him sit up if he's struggling to breathe — never let him stand up suddenly, as this can make a reaction more dangerous.

Sophie: That's really clear, thank you. I feel much more confident now.

Station Conclusion: An "Excellent Pass" EpiPen Technique Profile

Why your final choices scored maximum marks:

- **Correct Site and Technique:** You specified the outer thigh, 90-degree angle, and firm pressure until a click, rather than a vague "anywhere on the body."
- **Emphasised the Universal 999 Rule:** You stressed calling emergency services regardless of apparent improvement, given the risk of a biphasic reaction.
- **Explained the Second-Dose Rule:** You gave the correct 5–15 minute window for a repeat dose if needed.
- **Positioning Advice:** You explained why the patient shouldn't stand suddenly, and how to position them based on their main symptom.

Station 94 — Procedural Skills: Inhaler Technique

Poor inhaler technique is one of the most common, correctable reasons for apparently "poorly controlled" asthma or COPD. This station tests your ability to teach correct technique for a standard pressurised metered-dose inhaler (pMDI), and to know when a spacer or a dry powder inhaler needs a different approach.

Key Technique Differences

- **Standard pMDI ("puffer"):** shake well, breathe out fully away from the device, seal lips around the mouthpiece, and breathe in slowly and deeply while pressing the canister once, then hold the breath for around 10 seconds.
- **Spacer device:** recommended for anyone who struggles with coordination (young children, older adults, or during an acute attack) — press the inhaler into the spacer, then take slow, normal tidal breaths in and out through the spacer, or a single slow deep breath with a 10-second hold.
- **Dry powder inhalers (e.g. Turbuhaler, Accuhaler):** require the opposite technique — a fast, forceful, deep inhalation, since the force of breathing in is what disperses the powder; a slow breath (as used for a pMDI) will not work.
- **After a steroid-containing inhaler:** rinse the mouth with water and spit it out, to reduce the risk of oral thrush.

The Roleplay

Station: Clinical Procedural Skills / Patient Education

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: David Ellis, 45, recently diagnosed with asthma

Setting: Consultation Room

Your Task:

- Teach correct pMDI inhaler technique using a placebo device.
- Correct any technique errors observed.
- Explain steroid inhaler mouth care.

Candidate: Hello David, I understand you've been started on some new inhalers. Before you go, I'd like to check your technique with them, since this makes a huge difference to how well they work. Could you show me how you'd normally take a puff?

(David shakes the inhaler, puts it to his lips, and presses the canister while taking one quick, shallow sniff, then immediately breathes out.)

Common Pitfall: "That looks fine, keep doing it that way." — Fails to notice and correct a rushed, shallow breath with no breath-hold, which means most of the medication will deposit in the mouth and throat rather than reaching the lungs.

Model Response

Candidate: Thank you for showing me — that's really helpful, because I can see a couple of small adjustments that will make a big difference. Let's go through it together. First, breathe all the way out, away from the inhaler, to empty your lungs. Then seal your lips around the mouthpiece, and as you start to breathe in slowly and deeply, press the canister once, right at the start of your breath. Keep breathing in slowly for as long as you can, then hold your breath for about 10 seconds before breathing out.

David: (trying again, more slowly) Like that?

Candidate: That's much better — a nice slow, deep breath with a proper hold at the end. If you ever find it hard to coordinate pressing and breathing in together, a spacer device can make this much easier and gets more medication into your lungs rather than your mouth. One more important thing: because one of your inhalers contains a steroid, please rinse your mouth with water and spit it out after using it, to reduce the risk of a sore mouth or oral thrush.

David: I didn't realise the technique mattered this much, or about the mouth rinsing — that's really useful to know.

Station Conclusion: An "Excellent Pass" Inhaler Technique Profile

Why your final choices scored maximum marks:

- **Observed Before Correcting:** You asked David to demonstrate his technique first, rather than simply lecturing him.
- **Identified and Corrected the Specific Error:** You pinpointed the rushed, shallow breath and missing breath-hold, rather than giving generic advice.
- **Offered the Spacer as a Solution:** You mentioned a spacer for anyone struggling with coordination, rather than assuming technique alone would fix everything.
- **Steroid Mouth Care:** You proactively advised rinsing and spitting after steroid inhaler use to prevent oral thrush.

Station 95 — Procedural Skills: Peak Flow Meter Technique

Peak expiratory flow rate (PEFR) measurement is a simple bedside test used both to monitor chronic asthma control and to assess the severity of an acute asthma attack. This station tests correct technique and your ability to interpret the reading against a patient's predicted or personal best value.

Correct Technique

- Stand or sit up straight, to allow full lung expansion.
- Slide the marker/indicator to the bottom of the numbered scale.
- Take a full deep breath in.
- Seal the lips tightly around the mouthpiece.
- **Blow out as hard and as fast as possible — a short, sharp blast, not a slow, sustained exhale.** This is the opposite technique to spirometry, which requires a full, sustained exhale.
- Note the reading, then repeat the whole process 3 times, recording the best (highest) of the 3 attempts.

Using Peak Flow to Grade Acute Asthma Severity (BTS/SIGN)

- **Moderate:** PEFR 50–75% of best or predicted.
- **Severe:** PEFR 33–50% of best or predicted.
- **Life-threatening:** PEFR below 33% of best or predicted.

The Roleplay

Station: Clinical Procedural Skills / Patient Education

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Roleplayer Role: Rachel Cole, 29, with newly diagnosed asthma

Setting: Consultation Room

Your Task:

- Teach correct peak flow meter technique.
- Explain how to use the readings for self-monitoring.

Candidate: Hello Rachel, since you've been started on asthma treatment, I'd like to show you how to use a peak flow meter — this is a simple device you can use at home to keep an eye on how your asthma's doing.

(Rachel takes a small breath in, purses her lips loosely around the mouthpiece, and blows a slow, steady breath out.)

Common Pitfall: *"That's a good effort, we'll go with that reading." — Fails to correct a slow, sustained exhale with a loose lip seal, both of which will produce a falsely low reading that doesn't reflect her true peak flow.*

Model Response

Candidate: Thank you for trying that — let's adjust a couple of things, since I want to make sure we get an accurate reading. First, take as full and deep a breath in as you can. Then seal your lips tightly all the way around the mouthpiece, so no air can escape around the sides. Then, rather than blowing steadily, I want you to blow out as hard and as fast as you possibly can — think of it like blowing out a candle from across the room in one short, sharp blast, rather than a slow, sustained breath.

Rachel: (tries again, with a sharp, forceful blast) Like that?

Candidate: Perfect, that's exactly right. We'll do that three times in total, and take the best of the three readings as your result. At home, I'd like you to do this once or twice a day and keep a simple diary. Once we know your personal best reading, we can set up green, amber, and red zones for you — for example, if your reading drops into the amber zone, that tells you to increase your regular inhaler use, and if it drops into the red zone, that's a sign to seek urgent medical help.

Rachel: That's a really clear way to know when I need to worry. Thank you.

Station Conclusion: An "Excellent Pass" Peak Flow Technique Profile

Why your final choices scored maximum marks:

- **Corrected the Exhale Technique:** You identified and fixed the slow, sustained exhale, replacing it with the correct short, forceful blast.
- **Ensured an Airtight Seal:** You specifically checked and corrected her lip seal around the mouthpiece.
- **Best-of-Three Recording:** You explained the standard technique of repeating the measurement three times and recording the best result.
- **Practical Self-Management Application:** You connected the technique to a real-world personalised asthma action plan with green/amber/red zones, rather than teaching the skill in isolation.

Station 96 — Procedural Skills: 12-Lead ECG Recording

This station tests correct electrode placement for a standard 12-lead ECG, patient preparation, and awareness of common technical errors that can distort the recording. The 12 leads are generated from just 10 physical electrodes: 4 limb electrodes and 6 chest (precordial) electrodes.

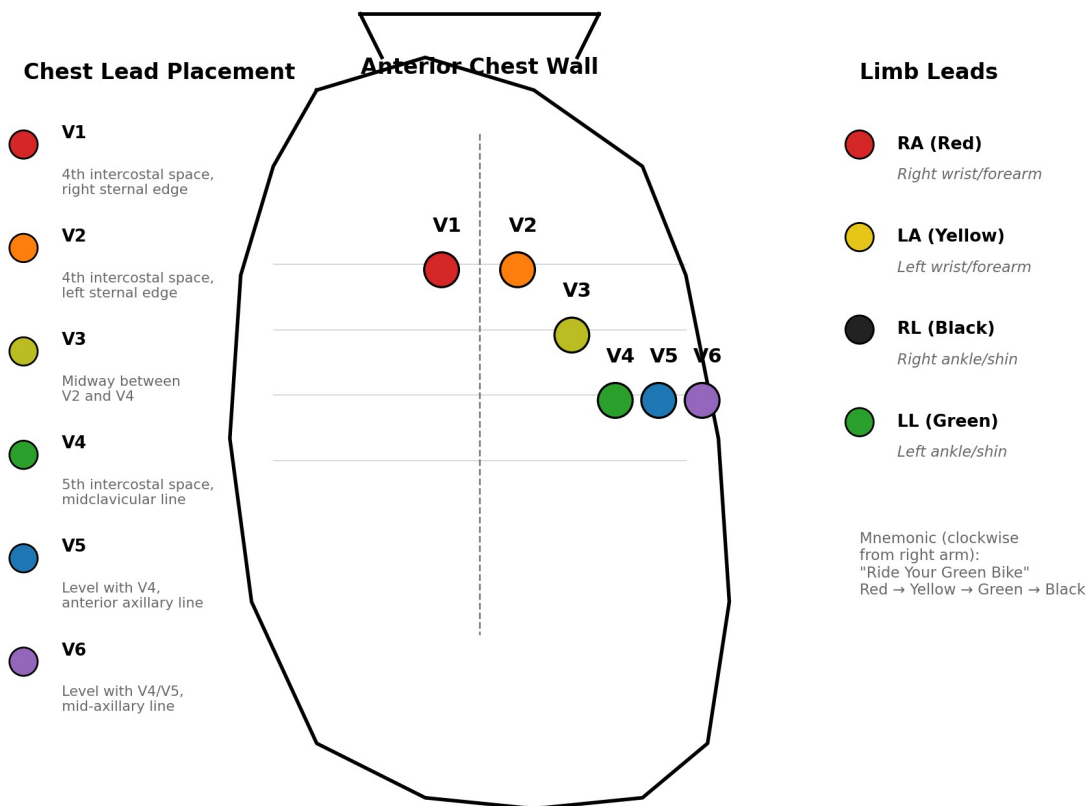


Figure: Precordial (V1–V6) chest lead placement and limb lead colour coding for a standard 12-lead ECG.

Electrode Placement

- **V1:** 4th intercostal space, right sternal edge.
- **V2:** 4th intercostal space, left sternal edge.
- **V3:** midway between V2 and V4.
- **V4:** 5th intercostal space, midclavicular line.
- **V5:** same horizontal level as V4, anterior axillary line.

- **V6:** same horizontal level as V4/V5, mid-axillary line.
- **Limb leads:** placed on the limbs themselves (wrists and ankles), or proximally on the limbs if a patient is missing a limb or moving excessively — right arm (red), left arm (yellow), left leg (green), right leg (black, the neutral/earth lead).

Mnemonic for the clockwise colour order starting from the right arm: "Ride Your Green Bike" — Red, Yellow, Green, Black.

Lead Groupings for Anatomical Correlation (Useful Context)

- **Inferior leads:** II, III, aVF.
- **Anterior/septal leads:** V1–V4.
- **Lateral leads:** I, aVL, V5, V6.

The Roleplay

Station: Clinical Procedural Skills

Candidate Role: FY2 Doctor on the Ward

Patient/Model Role: ECG electrode placement mannequin

Setting: Treatment Room

Your Task:

- Prepare the patient and explain the procedure.
- Place all 10 electrodes correctly.
- Recognise and avoid common technical errors.

Candidate: Hello, my name is Dr [Your Name]. I need to record a tracing of your heart's electrical activity, called an ECG. It's completely painless — I'll place ten small sticky electrodes on your chest, arms, and legs, and it only takes a couple of minutes. Would that be alright?

Common Pitfall: *"I'll just place all six chest electrodes roughly in a line across the middle of the chest, and put the limb electrodes anywhere convenient on the shoulders and hips." — Incorrect precordial spacing distorts the ECG's anterior/lateral picture, and placing limb electrodes on the torso instead of the limbs can introduce artefact and alter the recorded axis.*

Model Response

Candidate: I'm asking you to lie back comfortably and expose your chest, and I'll keep you covered elsewhere for your privacy. I'm placing V1 in the 4th intercostal space at the right edge of your breastbone, and V2 in the same intercostal space on the left edge. V4 goes in the 5th intercostal space in line with the middle of your collarbone, and V3 sits midway between V2 and V4. V5 and V6 continue along that same horizontal level, out towards the side of your chest. For the limb electrodes, I'm placing red on your right wrist, yellow on your left wrist, green on your left ankle, and black, the neutral electrode, on your right ankle.

Candidate: Before recording, I'll ask you to lie as still and relaxed as possible and breathe normally, since movement or muscle tension can distort the tracing. I'll also make sure all electrodes have good skin contact, cleaning the skin first if needed for someone who is very hairy or sweaty, since poor contact is a common cause of a poor-quality recording.

Station Conclusion: An "Excellent Pass" ECG Recording Profile

Why your final choices scored maximum marks:

- **Precise Precordial Placement:** You correctly identified the intercostal spaces and anatomical landmarks for V1–V6, rather than estimating positions roughly.
- **Correct Limb Lead Placement and Colour Coding:** You placed limb leads on the limbs themselves with the correct colour-to-position mapping.
- **Anticipated Common Artefact Sources:** You proactively addressed patient movement, muscle tension, and poor skin contact as causes of a poor-quality trace.
- **Maintained Dignity:** You kept the patient covered elsewhere while exposing only the chest as needed for electrode placement.

Station 97 — Procedural Skills: Rinné's and Weber's Tuning Fork Tests

Rinné's and Weber's tests use a vibrating tuning fork (classically 512Hz) to distinguish conductive from sensorineural hearing loss at the bedside, and to work out which ear is affected. This station tests correct technique and, critically, correct interpretation of the combined result.

Rinné's Test

- **Technique:** strike the tuning fork and place the base firmly on the mastoid process (testing bone conduction). Ask the patient to say when they can no longer hear it. Then immediately move the still-vibrating fork so the prongs are held about 2cm in front of the ear canal (testing air conduction), and ask if they can hear it again.
- **Rinné positive (normal):** air conduction is heard louder/longer than bone conduction ($AC > BC$) — this is the normal finding, and is also the finding in sensorineural hearing loss (both are reduced, but air still beats bone).
- **Rinné negative (abnormal):** bone conduction is heard louder/longer than air conduction ($BC > AC$) — this indicates a conductive hearing loss in that ear.

Weber's Test

- **Technique:** strike the tuning fork and place the base firmly in the midline, either on the forehead or the top of the head (vertex). Ask the patient where they hear the sound — in the middle, or louder in one ear.
- **Normal:** the sound is heard equally, in the midline.
- **Lateralises to the affected ear:** suggests a conductive hearing loss in that ear (there is less competing ambient air-conducted sound to mask the bone-conducted signal in the affected ear, so it sounds louder there).
- **Lateralises to the unaffected/better ear:** suggests a sensorineural hearing loss in the other (quieter-sounding) ear — the diseased cochlea or nerve on that side cannot process the sound as well even via bone conduction.

Combined Interpretation

Finding	Rinné (Affected Ear)	Weber	Interpretation
Normal hearing	Positive ($AC > BC$)	Midline	No hearing loss
Conductive hearing loss (e.g. earwax, otitis media)	Negative ($BC > AC$)	Lateralises to the affected ear	Conductive loss in that ear
Sensorineural hearing loss (e.g. presbycusis, acoustic neuroma)	Positive ($AC > BC$), though both reduced	Lateralises to the better (unaffected) ear	Sensorineural loss in the quieter ear

Examiner pearl: a "false Rinne negative" can occur in a severe unilateral sensorineural hearing loss — the bone-conducted vibration transmits across the skull and is picked up by the good cochlea on the opposite side, mimicking a conductive pattern in the deaf ear. If the Weber result doesn't match what the Rinne result would predict, consider this possibility rather than assuming the Rinne result is simply wrong.

The Roleplay

Station: Clinical Procedural Skills

Candidate Role: FY2 Doctor in a GP Clinic

Patient/Model Role: Harold Jennings, 70 years old

Setting: Consultation Room

The Prompt (read outside the cubicle):

Patient Details: Harold, 70, reports reduced hearing in his right ear for the last 2 weeks, following a bad cold.

Your Task:

- Explain and perform Rinne's and Weber's tests correctly.
- Interpret the combined findings accurately.
- Explain the likely cause and next steps.

Candidate: Hello Mr Jennings, I'm Dr [Your Name]. I understand your hearing in your right ear hasn't been quite right since your cold — I'd like to do two quick, painless tests with a tuning fork to help understand why. Is that alright?

Harold: Yes, of course, Doctor.

Common Pitfall: *"I'll just tap the fork once and hold it near your ear to check your hearing." — Skips the actual bone-versus-air conduction comparison entirely, and omits the Weber test altogether, meaning no meaningful conductive-versus-sensorineural distinction can be made.*

Model Response

Candidate: I'm going to strike this tuning fork and place the base firmly on the bony part behind your ear. Please tell me as soon as you can no longer hear it.

Harold: Okay... it's gone now.

Candidate: Thank you — now I'm moving it to just in front of your ear canal, still vibrating. Can you hear it again now?

Harold: Yes, actually, I can hear it again, a bit.

Candidate: That's helpful. Now for the second test, I'm placing the vibrating fork right in the middle of your forehead. Can you tell me if you hear it equally in both ears, or louder on one side?

Harold: That one's strange — it's definitely louder in my right ear, the one that's been blocked.

Your Next Step: Interpreting the Findings

Rinné's test showed air conduction still beating bone conduction, but Weber lateralised to the affected right ear — this combination points to a conductive cause in that ear, consistent with his recent cold.

Candidate: Thank you, Harold, that's really useful. These results suggest the reduced hearing is likely due to a blockage somewhere in the outer or middle part of your ear, rather than a problem with the nerve itself — very consistent with fluid or congestion left over from your cold. I'd like to take a look inside your ear with an otoscope to check for wax or fluid behind the eardrum, and if it doesn't settle over the next couple of weeks, we can arrange a formal hearing test to confirm.

Harold: Oh, that's a relief — I was worried it might be something permanent.

Station Conclusion: An "Excellent Pass" Tuning Fork Test Profile

Why your final choices scored maximum marks:

- **Correct Technique for Both Tests:** you performed a genuine bone-to-air conduction comparison for Rinné's, and a proper midline placement for Weber's, rather than a superficial single-tap check.
- **Accurate Combined Interpretation:** you correctly synthesised a positive Rinné with lateralisation to the affected ear as indicating a conductive cause, rather than interpreting either test in isolation.
- **Linked the Finding to the Clinical Context:** you connected the conductive pattern to his recent cold, rather than presenting the result as an isolated, unexplained fact.
- **Appropriate Follow-Up:** you planned otoscopic examination and a safety-netted formal hearing test if symptoms persisted, rather than treating the tuning fork result as the final answer.

SECTION: SPECIALIST REFERRAL PATHWAYS

Station 98 — Knowing Where and How Fast: The UK Referral Pathway Blueprint

Knowing exactly where to send a patient, how urgently, and why, is a core part of the Clinical Management marking domain across almost every PLAB 2/UKMLA station. The NHS uses a tiered referral system so that the most time-critical cases are seen fastest. Below is a structured reference for the referral pathways most commonly tested.

1. The Red Flag Pathway: Two-Week Wait (2WW) Referral

Timing rule: the patient must be seen by a specialist within 14 days of the referral being made.

Clinical indication: used when cancer is a genuine possibility that needs to be actively excluded, not confirmed cancer itself.

Pathway	Key NICE-Aligned Criteria
Breast Clinic	Aged 30 or over with an unexplained breast lump, with or without pain; or aged 50 or over with unilateral nipple discharge, retraction, or other concerning nipple change.
Lower GI / Colorectal	Aged 40+ with unexplained weight loss AND abdominal pain; aged 50+ with unexplained rectal bleeding; aged 60+ with iron-deficiency anaemia OR a change in bowel habit (either alone is sufficient — these are separate criteria, not a combined requirement); or a positive faecal occult blood (FIT) test, or a palpable rectal/abdominal mass.
Lung / Respiratory	A chest X-ray suggestive of lung cancer; or aged 40+ with unexplained haemoptysis.
Gynaecology (Endometrial)	Postmenopausal bleeding — current NICE guidance specifically centres this pathway around age 55 and over; women under 55 with postmenopausal bleeding and additional risk factors should still be discussed with a specialist, but the automatic threshold sits at 55.

The single most commonly confused point here: the colorectal criteria are four separate, independent triggers, not one combined rule — a patient needs to meet only one of them, and the specific age/symptom pairing matters (e.g. weight loss pairs with abdominal pain at 40+, not with anaemia at 60+).

2. Specialised Outpatient Clinics (Routine vs Urgent Referral)

These are standard hospital clinics, not cancer pathways. In the exam, specify whether the referral is Routine (weeks to months) or Urgent (typically within 2–4 weeks, but not on the cancer pathway) based on symptom severity.

A. GUM (Genitourinary Medicine) / Sexual Health Clinic

- **Timing:** urgent (within days) for a new acute diagnosis; routine for stable ongoing monitoring.
- **Indications:** newly diagnosed HIV; active, symptomatic STIs requiring partner notification; PrEP (pre-exposure prophylaxis) counselling.

B. Community Eating Disorders Service (CEDS)

- **Timing:** depends on physical risk. A patient meeting high-risk criteria on a formal risk assessment (such as MARSIPAN) — for example, marked bradycardia, hypotension, hypothermia, or a very low BMI — bypasses outpatient referral entirely and goes straight to same-day medical admission. Otherwise, an urgent outpatient referral is made.
- **Indications:** suspected or confirmed anorexia nervosa or bulimia nervosa in young people or adults.

C. CMHT (Community Mental Health Team)

- **Timing:** routine to urgent (roughly 1–4 weeks).
- **Indications:** severe, persistent mental illness (e.g. schizophrenia, bipolar disorder, treatment-resistant depression) where the patient's immediate physical risk is low.

Critical distinction: if the patient has an active plan to end their life, this is not a CMHT referral — escalate immediately to the Crisis Resolution and Home Treatment Team (CRHTT) for same-day assessment.

D. TIA (Transient Ischaemic Attack) Clinic

- **If symptoms occurred within the last 7 days:** must be assessed by a specialist within 24 hours.
- **If symptoms occurred more than 7 days ago:** still refer urgently, but the specialist assessment window is within 7 days rather than 24 hours — this second half of the rule is frequently forgotten and worth remembering explicitly.
- **Indications:** a temporary facial droop, arm weakness, or slurred speech that has fully resolved — the urgency exists precisely because a TIA is a warning sign that a major stroke may follow imminently.

3. Immediate Same-Day Escalation (A&E / Medical Admission)

In these situations, any outpatient clinic pathway is inappropriate — the patient needs same-day hospital assessment or admission.

- **Medical/SimMan emergencies:** anaphylaxis, acute severe asthma (unable to complete a sentence in one breath), major postpartum haemorrhage.
- **Psychiatric emergencies:** active, intent-driven suicidal ideation with a plan, command hallucinations instructing self-harm, or extreme self-neglect.
- **Surgical emergencies:** suspected mechanical bowel obstruction, a leaking abdominal aortic aneurysm, or acute appendicitis.

The "Name, Time, Purpose" Phrasing Technique

When delivering a referral plan to a patient, naming these three elements explicitly scores consistently well across stations:

Candidate: Maya, based on what you've described, I'm making an urgent 2-Week Wait referral to our specialist breast clinic. This means a specialist will see you face-to-face within 14 days to examine you and arrange an ultrasound scan. We do this as a precautionary safeguard, to rule out anything serious, including cancer, as quickly as possible.

Notice the three components: the specific pathway named ("2-Week Wait referral to the breast clinic"), the concrete timeframe ("within 14 days"), and the plain-language purpose ("to rule out anything serious, including cancer") — all three should appear together, not just the referral name alone.

Worked Example: Applying the Technique to the TIA Pathway

Candidate: Mr Patel, because your face drooped and your speech was slurred for about twenty minutes yesterday before it completely recovered, this sounds like a TIA — sometimes called a mini-stroke. Because this happened within the last week, I'm making an urgent referral to our TIA clinic, and you'll be seen by a specialist within 24 hours. This is because a TIA can be an early warning sign, and getting you assessed and started on the right treatment quickly significantly lowers the risk of a full stroke happening in the days afterwards.

Section Conclusion: Common Exam Pitfalls in Referral Pathways

- **Conflating separate 2WW criteria:** treating the colorectal criteria as one combined rule rather than four independent triggers with different age/symptom pairings.
- **Sending a crisis patient to CMHT instead of CRHTT:** active suicidal intent always needs same-day crisis team assessment, never a routine or urgent-but-delayed CMHT referral.
- **Forgetting the second half of the TIA rule:** symptoms more than 7 days old still need urgent referral, just with a 7-day rather than 24-hour specialist review window.
- **Naming a referral without the timeframe or purpose:** saying "I'll refer you to the breast clinic" alone scores lower than explicitly stating the urgency category, the timeframe, and why.

SECTION: PRESCRIBING & DRUG CHART SKILLS

Station 99 — Prescribing: UK Drug Chart Fundamentals and High-Yield Interactions

Prescribing stations test a distinct skill set from consultation stations: writing safely and legibly on a UK drug chart, correctly using the BNF, and recognising the small set of drug interactions that come up disproportionately often in the exam. The task usually varies: explain a new medication and prescribe it, counsel on a side effect, address a patient's concern, or adjust an existing prescription.

General Drug Chart Rules

- Write in block capital letters, using a black ball-point pen.
- Essential details (patient name, NHS/hospital number) must be written on every sheet of the chart, not just the front page.
- Always check and record drug sensitivities/allergies before writing anything else.
- Use the generic drug name (e.g. salbutamol), never a brand name (e.g. Ventolin).
- Write the dose clearly and unambiguously.
- **Only "g" and "mg" are acceptable abbreviations for units; "micrograms" and "nanograms" must always be written in full, never abbreviated to "mcg" or "ng",** since these are common sources of tenfold dosing errors.
- Use numbers/figures to denote the quantity of a sachet or unit dose (e.g. "2 sachets"), not words.
- Always specify the dose of inhaled drugs by both strength and number of puffs (e.g. "Salbutamol 100 micrograms/puff, 2 puffs"), since inhaler strengths vary between products.
- Always write a stop date/duration for a course of treatment (e.g. "5/7" for 5 days), rather than leaving it open-ended.
- If no clear alternative is available for a stated task, say you would liaise with the relevant team (e.g. pharmacy, microbiology) rather than guessing.

A single wrong patient identifier (name, date of birth, or number) on an otherwise perfect prescription is treated as an automatic fail for that station — always double-check these first.

Chart Abbreviations

Abbreviation	Meaning
OD	Once daily
BD	Twice daily

Abbreviation	Meaning
TDS	Three times daily
QDS	Four times daily
Mane	In the morning
Nocte	At night
PRN	As required

High-Yield Drug Interactions

1. Warfarin Interactions

- **Macrolide/quinolone antibiotics (clarithromycin, erythromycin, ciprofloxacin) and metronidazole:** inhibit warfarin metabolism, raising INR and bleeding risk.
- **Azole antifungals (e.g. fluconazole):** significantly increase warfarin levels.
- **Enzyme inducers (rifampicin, carbamazepine):** reduce warfarin's effect, risking clot or stroke.

2. Combined Oral Contraceptive Pill (COCP) and Enzyme Inducers

A classic scenario: a patient on the COCP is prescribed a new medication that reduces contraceptive efficacy (e.g. carbamazepine, phenytoin, rifampicin, or St John's Wort). You must counsel on additional contraceptive precautions (e.g. condoms) or switching to a method not affected by enzyme induction, such as the intrauterine system or contraceptive injection.

3. SSRIs and NSAIDs

Commonly tested in an older patient, or one with a history of depression, presenting with new pain. Taking an SSRI (e.g. citalopram) alongside an NSAID (e.g. ibuprofen or naproxen) significantly increases the risk of gastrointestinal bleeding. Management: suggest a proton pump inhibitor for gastric protection, or an alternative analgesic such as paracetamol.

4. Statins with Macrolides or Grapefruit Juice

A patient on simvastatin or atorvastatin prescribed clarithromycin or erythromycin for a chest infection — or who drinks grapefruit juice regularly — is at risk of a marked rise in statin levels, which can cause rhabdomyolysis (muscle breakdown that can also damage the kidneys). Tip: stop the statin for the duration of the antibiotic course, then restart it the day after finishing.

5. Methotrexate and Trimethoprim

A significant patient safety scenario. Both drugs interfere with folate metabolism, and taking them together can cause severe, potentially fatal bone marrow suppression (pancytopenia). A classic setup: a GP reviewing a patient on methotrexate for rheumatoid arthritis who develops symptoms of a UTI — trimethoprim must not be prescribed; choose an alternative antibiotic instead.

6. ACE Inhibitors and Potassium-Sparing Diuretics

Combining an ACE inhibitor with spironolactone, or with potassium supplements, can cause life-threatening hyperkalaemia. Monitor renal function and potassium closely if this combination is genuinely necessary.

Worked Mock Dialogue 1: Warfarin and Clarithromycin

Candidate: Hello Mr Ainsworth, I understand we're starting you on a course of clarithromycin for your chest infection. Before you leave, I'd like to explain how this new antibiotic might affect your warfarin — is that okay?

Patient: Sure, Doctor. Will it be a problem?

Candidate: It's not necessarily a problem, but they do interact. Clarithromycin can make your warfarin considerably stronger in your body, which means your blood could become too thin, increasing your risk of bleeding.

Patient: Oh, that sounds serious — should I stop the warfarin?

Candidate: No, please don't stop it. Instead, we need to monitor you more closely — I'd like you to come in for an extra INR blood test in 2 to 3 days, so we can adjust your dose if needed.

Patient: I see. What should I look out for in the meantime?

Candidate: Please keep an eye out for any unusual bleeding — bleeding gums when you brush your teeth, unexplained bruising, or dark or red-coloured urine or stools. If you notice any of that, please contact us immediately.

Worked Mock Dialogue 2: Statin and Clarithromycin

Candidate: Hello Mr Whitfield, I've reviewed your symptoms and I'm going to prescribe a course of clarithromycin for your chest infection. I noticed from your records that you take simvastatin every evening for your cholesterol — is that correct?

Patient: Yes, Doctor, I've been on that for years.

Candidate: Right — it's important that while you're taking this antibiotic, you stop your simvastatin temporarily. Is that alright?

Patient: Stop it? Is that safe for my heart?

Candidate: It's safe to stop it for a short time. The issue is that clarithromycin interacts with simvastatin — it stops your body breaking the statin down properly, so the level builds up dangerously high in your system. This can lead to a serious condition where your muscles start to break down, which can even damage your kidneys.

Patient: I see — when can I start it again?

Candidate: You should wait until you've finished the entire course of antibiotics. Once the course is complete, you can restart the simvastatin as usual the following day.

Patient: What if I accidentally took both together?

Candidate: If you notice any unexplained muscle pain, tenderness, or weakness — especially if your urine looks unusually dark, like strong tea or cola — you must contact us immediately or go to A&E, as those are signs of muscle damage.

High-Yield Prescribing Scenarios

- **Acute pain (post-operative or trauma):** simple analgesia stepped up to co-codamol or morphine as needed. Trap: never prescribe co-codamol alongside separate paracetamol, since co-codamol already contains paracetamol — the combination risks accidental paracetamol overdose. Paracetamol dosing: 1g four times a day, maximum 4g in 24 hours.
- **VTE prophylaxis (DVT/PE prevention):** low molecular weight heparin (e.g. dalteparin), once daily, subcutaneously. Trap: the dose is weight-dependent — always check the patient's weight on the chart before prescribing, rather than using a standard dose for everyone.
- **Asthma exacerbation:** prednisolone 40mg once daily in the morning for 5 days is a standard example dose. Counselling point: advise taking it with food, since it can upset the stomach.
- **Hypoglycaemia (drug emergency):** glucagon 1mg intramuscularly if there is no IV access.

Station Conclusion: An "Excellent Pass" Prescribing Profile

Why your final choices scored maximum marks:

- **Flawless Chart Technique:** block capitals, generic names, units written correctly (micrograms in full), and a clear stop date.
- **Recognised the Interaction Before Prescribing:** you checked the existing medication list and identified the interaction proactively, rather than being prompted.
- **Did Not Simply Stop a Vital Medication:** for warfarin, you arranged closer monitoring rather than stopping it outright, since stopping warfarin can itself be dangerous.
- **Clear, Jargon-Free Explanation and Safety-Netting:** you explained the mechanism in plain language and gave specific, concrete red flags to watch for.

Station 100 — Prescribing: Fluid Charts, Emergency Drug Doses, and Specialist Prescribing

This station covers the reference material most commonly needed for fluid prescribing, emergency drug dosing, and specialist restricted-antibiotic charts (such as vancomycin) — the kind of detail-heavy content that's easy to blank on under exam pressure if not deliberately reviewed beforehand.

Drug Chart Sections Explained

- **Once-only/stat section:** used for a single emergency dose (e.g. "IV acetazolamide 500mg" for acute glaucoma, or emergency drugs given once during an acute deterioration).
- **Regular medications section:** used for chronic treatments or a defined course of antibiotics (e.g. "Amoxicillin 500mg TDS").
- **PRN (as-required) section:** used for symptom-based medication such as analgesia (e.g. "Paracetamol 1g every 4–6 hours as required for pain").

Emergency/Stat Drug Doses — Quick Reference

Drug	Dose / Route	Indication
Adrenaline	500 micrograms (0.5mg) IM	Anaphylaxis — given into the mid-anterolateral thigh
Aspirin	300mg oral, chewed	Suspected acute coronary syndrome/MI (stat dose)
Naloxone	400 micrograms IV, repeat every 2–3 minutes	Opioid overdose/toxicity
Glucagon	1mg IM	Hypoglycaemia with no IV access
Acetazolamide	500mg IV	Acute angle-closure glaucoma

Paediatric Fluid Prescribing

- **Emergency resuscitation bolus ("the B bolus"):** for a child with signs of severe dehydration (around 8%) or circulatory collapse (e.g. capillary refill over 2 seconds, cool peripheries) — give 20ml/kg of 0.9% sodium chloride, given in under 15 minutes. Exception: if the child has trauma or diabetic ketoacidosis, use a more cautious 10ml/kg bolus instead.
- **Routine maintenance ("the M fluid", the 4:2:1 rule):** for a stable child unable to take oral fluids — 4ml/kg/hour for the first 10kg of body weight, plus 2ml/kg/hour for the second 10kg, plus 1ml/kg/hour for each kilogram above 20kg. As a rough adult-equivalent ceiling, this is usually capped around 2000–2500ml/day for a larger child/adolescent.

- **Fluid deficit replacement ("the D fluid"):** replaces fluid already lost to dehydration — typically calculated as 5% (moderate dehydration) or up to 8% (severe dehydration) of body weight; do not use a higher percentage than this in the calculation.

Adult Fluid Prescribing

- **Daily maintenance requirement:** approximately 25–30ml/kg/day of water, roughly 1mmol/kg/day of sodium (around 50–100mmol/day) and roughly 1mmol/kg/day of potassium (around 40–80mmol/day) for an average adult.
- **Resuscitation/initial volume replacement:** a balanced crystalloid such as Hartmann's solution is generally the safer first choice, particularly in the first 24 hours, unless the patient has significant liver failure (in which case seek senior/specialist input).
- **Maintenance fluid choice:** current UK guidance favours isotonic fluids (e.g. 0.9% sodium chloride, or a balanced crystalloid, with appropriate potassium added) for routine maintenance, rather than older hypotonic dextrose-saline mixes, given the risk of dangerous hyponatraemia with hypotonic solutions — this is a patient-safety point worth remembering explicitly.
- **Potassium infusion safety:** do not exceed 10mmol of potassium per hour via a peripheral line, to avoid cardiac arrhythmia.

Vancomycin Prescribing (A Restricted-Use Antibiotic Chart)

Vancomycin charts are used in the UK for serious infections requiring a "restricted use" antibiotic, such as MRSA or endocarditis. Dosing is weight-based, and the following is illustrative of a typical simplified example — always check the current local protocol and patient-specific factors:

Weight	Example Dose	Infusion Time
Under 40kg	750mg in 250ml	Over 90 minutes
40–59kg	1g in 250ml	Over 120 minutes
60–90kg	1.5g in 500ml	Over 180 minutes (3 hours)
Over 90kg	2g in 500ml	Over approximately 240 minutes (4 hours)

- **Renal adjustment:** dosing interval is adjusted based on eGFR — the lower the renal function, the longer the interval between doses; discuss with a clinical microbiologist if eGFR is very low.
- **Safe administration:** infuse slowly — a rate exceeding roughly 10mg/minute risks "red man syndrome" (a histamine-related flushing reaction, not a true allergy). Reconstituted vancomycin must be further diluted (typically in 0.9% sodium chloride or 5% dextrose) to a concentration that does not exceed 5mg/ml.
- **Monitoring (trough levels):** for twice-daily dosing, sample before the 3rd or 4th dose; for once-daily dosing, before the 2nd or 3rd dose. Always give the scheduled dose on time, without waiting for the trough sample result. Target trough levels are typically around 10–20mg/L for standard infections, or 15–20mg/L for deep-seated infections.

- **Safety-netting:** advise the patient to report any change in hearing (a sign of ototoxicity) or a marked reduction in urine output (a sign of nephrotoxicity), both recognised side effects.

VTE Risk Assessment

Every inpatient should have a formal VTE (venous thromboembolism) risk assessment on admission, weighing clotting risk against bleeding risk, and categorising the patient as high, intermediate, or low risk.

- **High risk (and no significant bleeding risk):** prophylactic low molecular weight heparin, often continued for an extended period after high-risk surgery (e.g. major hip or knee replacement, or cancer surgery).
- **Lower risk:** mechanical prophylaxis (e.g. compression stockings or intermittent pneumatic compression), early mobilisation, and maintaining good hydration.

For a pregnant patient admitted with even a minor injury or reduced mobility, always mention performing a VTE risk assessment using the obstetric-specific chart, since pregnancy itself is a VTE risk factor requiring its own assessment tool.

Example weight-banded VTE prophylaxis dosing (dalteparin, illustrative only — confirm against current local protocol):

Weight	Dalteparin Dose
Under 50kg	2500 units once daily
50–90kg	5000 units once daily
91–130kg	7500 units once daily
131–170kg	10,000 units once daily

A Note on Nitrofurantoin

Nitrofurantoin should be avoided in patients with significant renal impairment — generally avoided if eGFR is below 30, and used with caution between 30–44 — since it relies on being concentrated in the urine to work, and impaired renal function both reduces its effectiveness and increases the risk of toxicity. It has no specific interaction with penicillin allergy.

Station Conclusion: An "Excellent Pass" Fluid and Specialist Prescribing Profile

Why your final choices scored maximum marks:

- **Correct Fluid Choice by Context:** you chose a balanced crystalloid for resuscitation and an isotonic (not hypotonic) solution for maintenance, avoiding the hyponatraemia risk of older dextrose-saline regimens.
- **Weight-Based Dosing Throughout:** you checked the patient's weight before prescribing VTE prophylaxis or vancomycin, rather than using a single default dose.

- **Correct Monitoring and Safety-Netting for Vancomycin:** you knew when to sample trough levels, the maximum safe infusion rate, and the two key toxicities (ototoxicity, nephrotoxicity) to warn the patient about.
- **Recognised the Special Case of Pregnancy:** you flagged the need for an obstetric-specific VTE risk assessment tool for a pregnant inpatient.

A Note to the Reader

Disclaimer

This book is a preparatory revision resource compiled by Dr. Sampson Owhin for candidates studying for the PLAB 2/UKMLA examination. It is intended as a study aid to support, not replace, structured teaching, clinical experience, and the current editions of standard medical textbooks and official guidelines (including NICE, the UK Resuscitation Council, the BNF, and relevant Royal College guidance).

Medicine is a constantly evolving field, and clinical guidelines are revised over time. Every effort has been made to ensure the clinical content in this book is accurate and reflects current UK practice at the time of writing, but readers are strongly encouraged to independently verify all clinical facts, drug doses, referral pathways, and thresholds against current official guidelines and standard reference texts before relying on them in practice or in an examination setting. This book should not be used as a sole or definitive clinical reference.

Fictitious Scenarios

All patient names, ages, personal details, and clinical scenarios presented throughout this book are entirely fictitious and have been created for educational and illustrative purposes only. Any resemblance to real individuals, living or deceased, or to real patient cases, is entirely coincidental and unintended.

Sources and Acknowledgement

The material in this book was freely compiled from a wide range of offline and online educational resources, official guidelines, and general medical knowledge, and has been systematically reorganised, rewritten, and personalised into the consistent station format used throughout this book. It is offered in good faith as a study companion, in the hope that it proves genuinely useful to readers preparing for their exam.

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Good luck with your exam.

— Dr. Sampson Owhin

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